AED Operation (Automated External Defibrillator) — Adult

Israel — ADULT

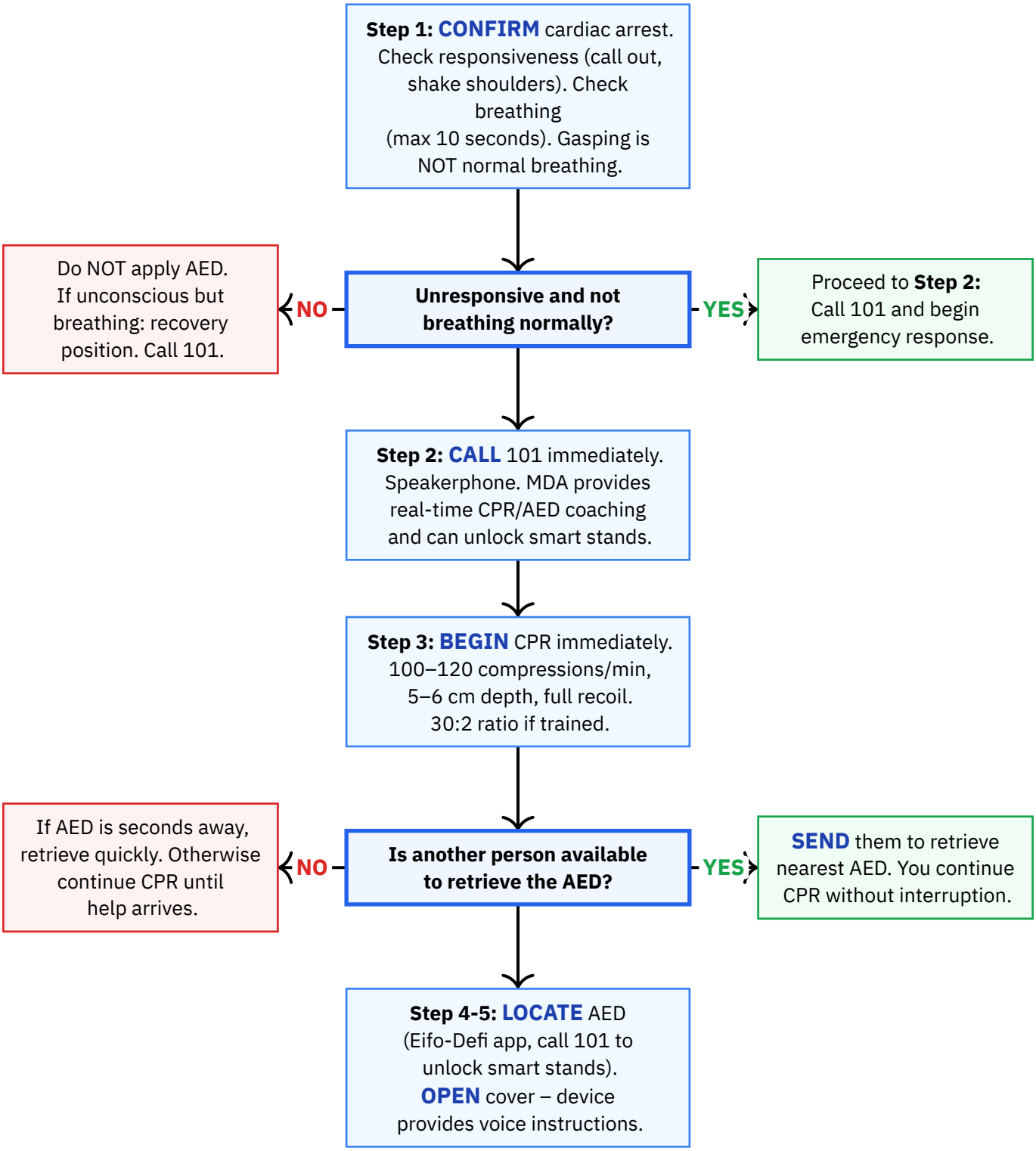
CALL 101 (MDA) — EVERY MINUTE OF DELAY REDUCES SURVIVAL BY 7–10%

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

An adult (age 8+) is found unresponsive, not breathing normally (absent breathing or only gasping/agonal breathing), and suspected to be in cardiac arrest.

Confirm Cardiac Arrest and Begin CPR



AED Application and Shock Delivery

Step 6: PREPARE chest:
expose bare chest, ensure dry.
Remove medication patches.
Shave excessive hair.

↓

Step 7: APPLY electrode pads:
Pad 1: below right collarbone.
Pad 2: left side below armpit.
Follow diagrams on pads.

↓

Step 8: AED analyses rhythm.
STOP CPR. Ensure nobody
is touching the patient.

WARNING
Do NOT touch patient
during analysis. Any
contact may produce
inaccurate results.

↓

Does the AED advise a shock?

NO →
Rhythm not shockable.
RESUME CPR
immediately.
AED re-analyses after
2 minutes.

YES →
Proceed to **Step 9:**
deliver shock.

↓

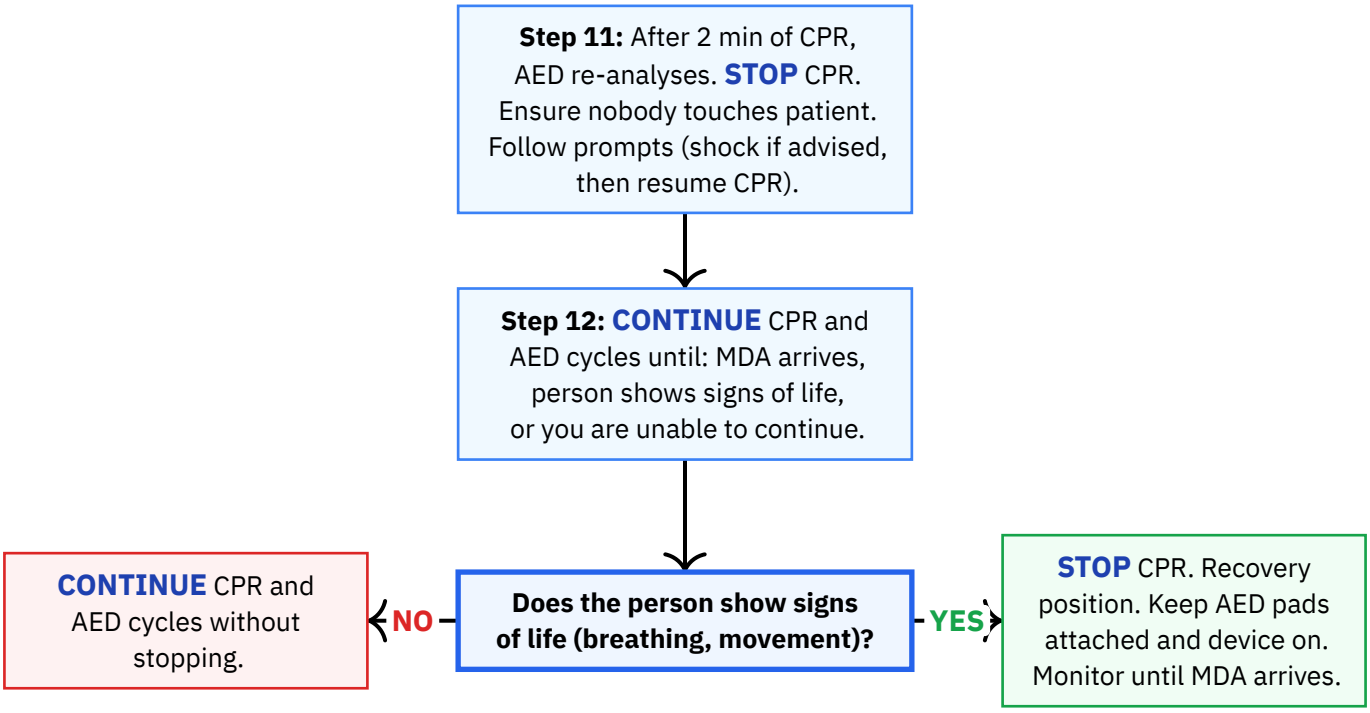
Step 9: Ensure all clear.
Call out "Hitrachku!"
(Stand clear!)
PRESS shock button.

WARNING
Visually confirm nobody
is in contact with
the patient before
pressing shock button.

↓

Step 10: RESUME CPR
immediately after shock.
Continue for 2 minutes
(5 cycles of 30:2).

CPR/AED Cycles and Continuation



Key AED Parameters

Compression rate: 100–120 / min
CPR cycle: 2 min (5 cycles of 30:2)
Pad 1: Below right collarbone
Emergency number: 101 (MDA)
Compression depth: 5–6 cm
AED re-analysis: Every 2 minutes
Pad 2: Left side below armpit
AED locator: Eifo-Defi app

DO NOT:

X Do NOT use the AED on a person who is conscious and breathing normally.
X Do NOT apply pads while the person is in water -- move to dry ground first.
X Do NOT use near flammable gases or oxygen-enriched atmospheres.
X Do NOT place pads directly over a pacemaker -- place at least 2.5 cm away.
X Do NOT place pads over medication patches -- remove and wipe skin first.
X Do NOT touch the patient during rhythm analysis or shock delivery.
X Do NOT turn off the AED or disconnect pads, even if the person regains consciousness.
X Do NOT delay defibrillation -- survival decreases by 7--10% per minute of delay.
X Do NOT interrupt compressions for more than 10 seconds for any reason.
X Do NOT allow pads to touch or overlap each other.

Equipment needed:

— AED (from MDA smart stands, public locations, or Eifo-Defi app locator)
— Razor (for shaving chest hair -- often included in AED kits)
— Towel or cloth (for drying the chest)
— Scissors (for cutting away clothing -- often included in AED kits)
— Phone (to call 101 and use speakerphone for dispatcher-guided CPR/AED use)

Source: Magen David Adom (MDA) — MDA AED Operation Protocol, AHA 2025 Guidelines
URL: https://www.mdais.org/kursim/defibrillator
Publication date: 2025-01-01 · **Edition:** AHA 2025 Guidelines
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Israeli AED law (2008): public spaces with 500+ daily visitors must have an AED. Eifo-Defi app locates nearest AED. MDA smart stands: call 101 to remotely unlock.

Anaphylaxis / Allergic Reaction — Adult

Israel — ADULT

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

Severe allergic reaction with involvement of breathing, circulation, or consciousness. High probability when: acute onset with skin/mucosal involvement plus respiratory compromise or hypotension; or two or more of skin symptoms, respiratory compromise, hypotension, persistent GI symptoms after allergen exposure.

Summary: Recognition and first aid treatment of anaphylaxis in adults, including EpiPen administration and monitoring until EMS arrival.

Recognition and Initial Response

Step 1: CHECK for signs of severe allergic reaction.
Skin: rash, hives, swelling
Breathing: wheeze, stridor
Circulation: weak pulse, pallor
GI: nausea, vomiting

↓

Signs of anaphylaxis? (breathing difficulty, circulatory compromise, altered consciousness)

→ **NO** → **MONITOR** for worsening. Mild skin reaction only: observe closely. **CALL** 101 if worsens.

↓ **YES**

Step 2: CALL 101 (MDA) immediately. Speaker mode. **FOLLOW** dispatcher instructions.

↓

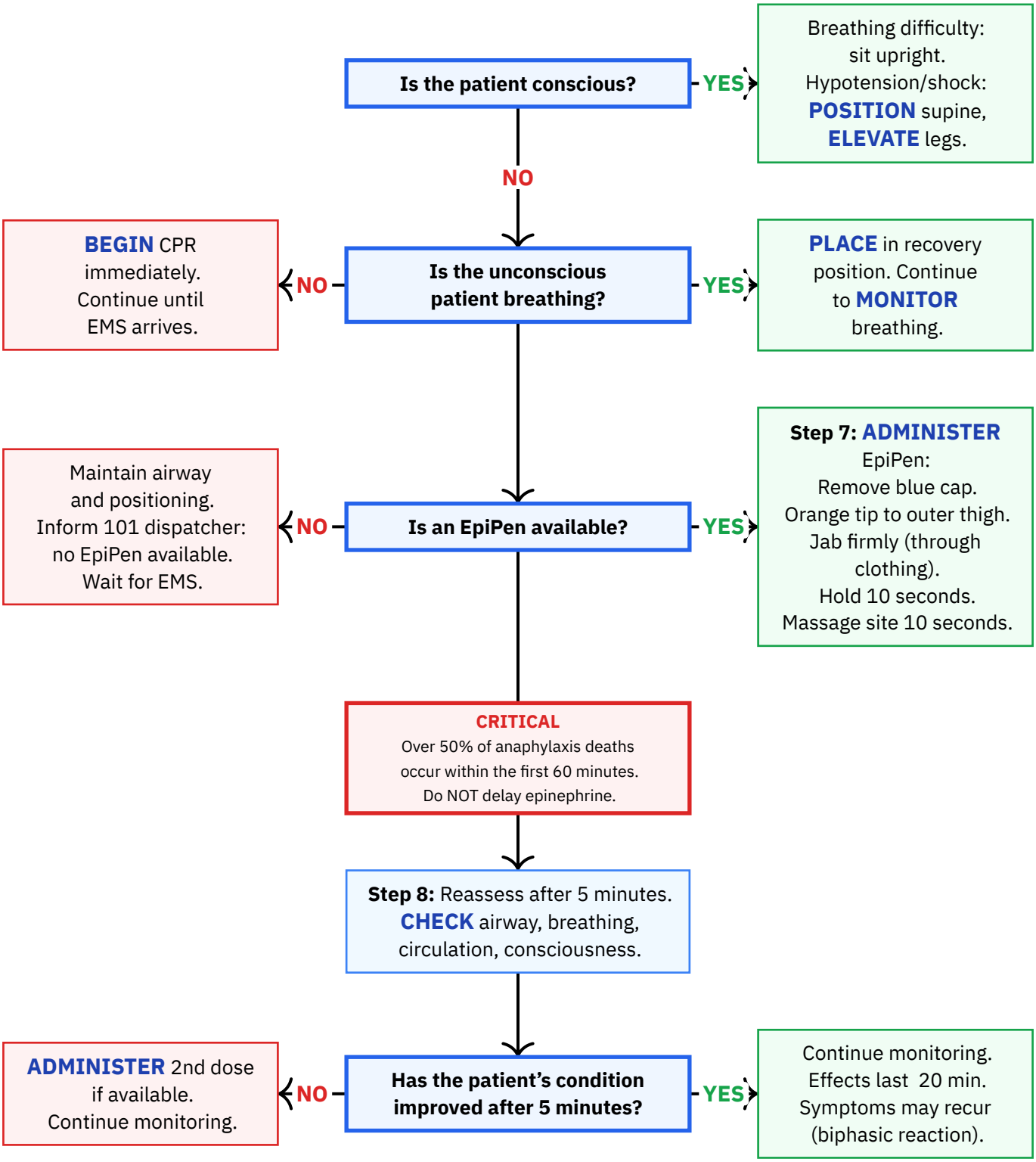
Step 3: REMOVE the offending agent if possible. Scrape bee stinger sideways. Stop infusion/medication.

↓

Step 4: Manage the airway. **OPEN** airway, clear debris. **LOOSEN** tight clothing around neck and chest.

WARNING
Airway swelling can progress rapidly. Continuous monitoring is essential.

Positioning and Epinephrine



Step 9: MONITOR continuously until EMS arrives. Be prepared to **BEGIN** CPR if patient loses consciousness and stops breathing. All patients with suspected anaphylaxis require hospital evaluation, even if symptoms fully resolve.

WARNING: Biphasic reactions can occur hours after the initial event. Hospital observation is mandatory.

Reference

DO NOT:

X Do not delay calling 101 -- anaphylaxis is a life-threatening emergency.
X Do not delay epinephrine administration if anaphylaxis is suspected. Epinephrine is the only effective first-line treatment.
X Do not substitute antihistamines (e.g., diphenhydramine) for epinephrine. Antihistamines are not effective against anaphylaxis.
X Do not make the patient stand or sit up suddenly if they are hypotensive -- this can cause cardiac arrest.
X Do not leave the patient unattended after epinephrine administration -- symptoms may recur.
X Do not assume the patient is safe after symptoms resolve -- all suspected anaphylaxis requires hospital evaluation.
X Do not waste time removing clothing before injecting the EpiPen -- it can be administered through trousers.
X Do not administer the EpiPen into a vein -- it is an intramuscular injection into the outer thigh only.
X Do not give food or drink to a patient experiencing anaphylaxis.

Equipment needed:

— Epinephrine auto-injector (EpiPen) -- adult 0.3 mg
— Phone to call MDA at 101

Source: Magen David Adom (MDA) — "MDA First Aid Guidance on Allergies and ALS Protocol (April 2024); Israeli Ministry of Health Anaphylaxis Guidance"
URL: https://www.mdais.org/101/allergy
Publication date: 2024-04-01 · **Edition:** ALS Manual April 2024
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Both protocols are substantially aligned on Sampson diagnostic criteria and EpiPen technique. MDA is used as the authoritative default per project rules.

Asthma Attack — Adult

Israel — ADULT

CALL 101 (MDA) — IF INHALER FAILS OR NO INHALER AVAILABLE

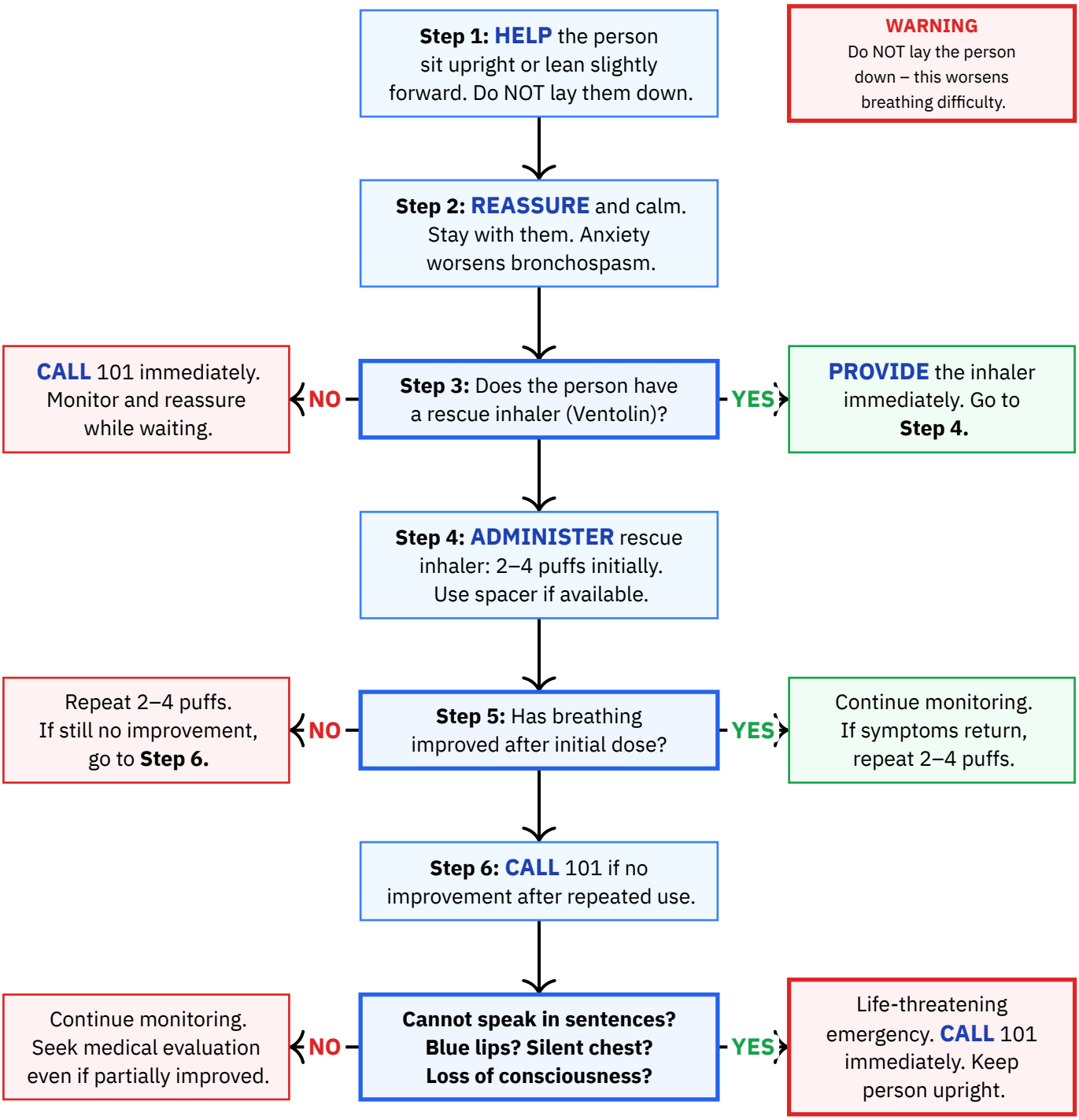
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

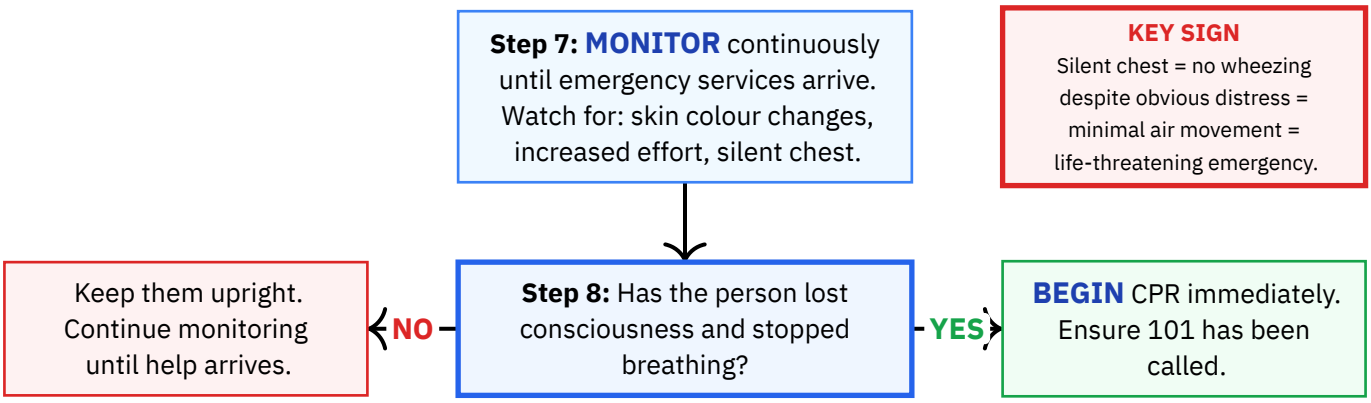
Person is experiencing an asthma attack: shortness of breath, wheezing, persistent coughing, chest tightness, difficulty speaking, or visible breathing effort. Severe signs: inability to complete a sentence, cyanosis (blue lips/fingertips), silent chest, or no response to inhaler.

Summary: Position upright, administer rescue inhaler, and call 101 if symptoms do not improve.

Asthma Response Protocol



Continued Monitoring



DO NOT:

X Do NOT lay the person down -- the upright position is essential for breathing.
X Do NOT leave the person alone during an attack.
X Do NOT administer controller medications (inhaled corticosteroids) for acute relief -- these do not provide immediate effect.
X Do NOT delay calling 101 if no inhaler is available.
X Do NOT ignore a 'silent chest' -- absence of wheezing during obvious distress indicates a life-threatening emergency.

Equipment needed:

— Rescue inhaler (Ventolin / Salbutamol)
— Spacer device (merkhav) if available

Source: Magen David Adom (MDA) — MDA ALS Protocol Manual 2022
URL: https://www.mdais.org/101/first-aid
Publication date: 2022-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16

Blast / Shrapnel Injuries — Adult

Israel — ADULT

CALL 101 (MDA) — HEMORRHAGE CONTROL IS THE LIFE-SAVING PRIORITY

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Person has been injured by an explosion, blast, or shrapnel. Signs include: visible wounds, active bleeding, burns, signs of shock, difficulty breathing, hearing loss, embedded objects. Blast victims frequently have injuries from multiple mechanisms simultaneously.

Scene Safety and Hemorrhage Control

Step 1: CONFIRM scene safety. Wait for security forces. Beware secondary devices.

WARNING
Secondary devices (second bombs) are a known tactic. Maintain distance until area is declared safe.

Step 2: CALL 101. Report location, number of casualties, type of incident (explosion).

Step 3: ASSESS for life-threatening hemorrhage. Hemorrhage control is priority.

Life-threatening bleeding from a limb?

- **NO** → Check for embedded objects, burns, breathing difficulty. Go to **Step 7.**
- **YES** → Proceed to **Step 4:** hemorrhage control. This takes priority over all other care.

Step 4: APPLY direct pressure. Use hand or gauze. Transition to Israeli bandage when possible.

Has bleeding stopped or slowed significantly?

- **NO** → Escalate: wound packing (**Step 5**) or tourniquet (**Step 6**).
- **YES** → Maintain pressure or secure with Israeli bandage. Check for other injuries.

Step 5: PACK large wounds with gauze if direct pressure is insufficient.
Step 6: APPLY tourniquet if packing fails. Tighten until bleeding stops. Document time.

WARNING
Do NOT remove tourniquet once applied. If bleeding continues, apply a second tourniquet above the first.

Secondary Assessment and Monitoring

Step 7: STABILISE embedded objects. Do NOT remove or move them. Apply pressure on either side.

WARNING
Never remove embedded objects – removal may cause uncontrollable bleeding.

↓

Step 8: LOOK for additional wounds. Blast victims often have multiple injury sites.

↓

Step 9: ASSESS and treat burns. Cool with clean water. Do not remove clothing stuck to burns. Cover with clean dressing.

↓

Step 10: MONITOR for shock and reassure the victim. Keep still and warm.

↓

Is the victim conscious and breathing?

- **NO** → Unconscious + breathing: recovery position. Not breathing: **BEGIN** CPR (30:2).
- **YES** → Continue monitoring and reassurance. Keep still and warm until MDA arrives.

DO NOT:

X Do NOT approach the scene until security forces confirm it is safe -- secondary devices are a known tactic.
X Do NOT remove embedded shrapnel or debris from the body -- this may cause uncontrollable bleeding.
X Do NOT remove a tourniquet once applied -- only medical professionals should make this decision.
X Do NOT remove clothing stuck to burns.
X Do NOT let the victim move or walk until MDA arrives.
X Do NOT use head-tilt for airway management in blast/trauma victims -- use jaw-thrust to protect cervical spine.
X Do NOT administer epinephrine in hypovolemic trauma.
X Do NOT assume only one injury site -- blast victims typically have injuries from multiple mechanisms.

Equipment needed:

— Phone to call 101 (MDA)
— Sterile gauze or clean cloth for direct pressure
— Israeli bandage (emergency bandage)
— Gauze for wound packing
— Tourniquet (CAT tourniquet preferred; improvised as last resort)
— Non-adherent dressings for burns
— Clean water for cooling burns
— Blanket for warmth

Source: Magen David Adom (MDA) — MDA First Aid Guidelines - Blast and Shrapnel Injuries
URL: https://www.mdais.org/media/4954/mda-first-aid-guidelines-en.pdf
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah sources. Blast injuries classified into four mechanisms: primary (barotrauma), secondary (penetrating/shrapnel), tertiary (blunt/displacement), quaternary (thermal/burns).

Burns First Aid Protocol — Adult

Israel — ADULT

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

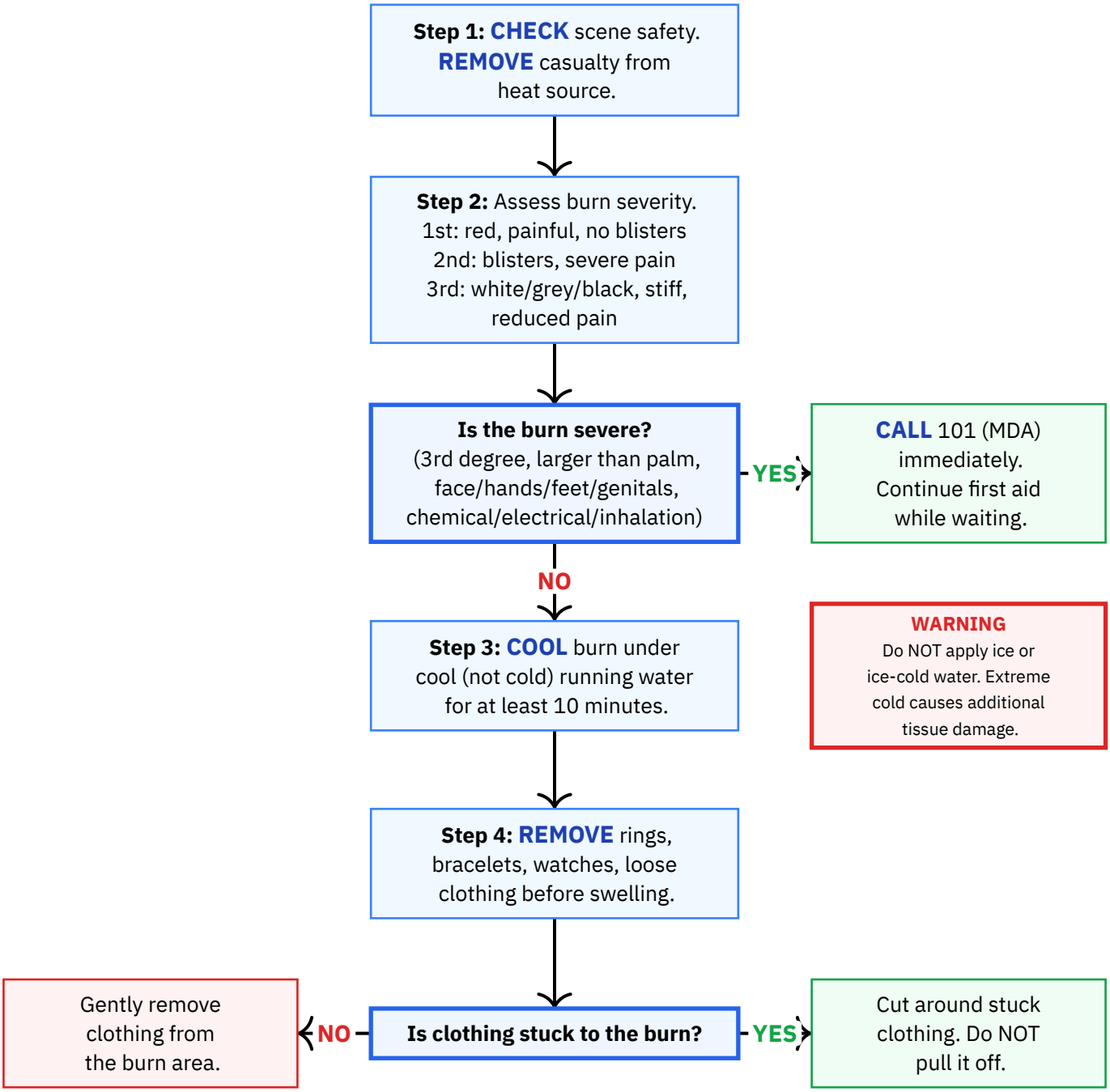
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

When an adult has sustained a burn from heat, chemicals, electricity, or radiation. Signs: redness, swelling, blisters, white/grey/black discolouration of skin, or pain at injury site.

Summary: First aid for thermal, chemical, and electrical burns: cooling, protecting the wound, and recognising when to call emergency services.

Initial Assessment and Treatment



Wound Protection and Special Burns

Step 5: COVER burn loosely with cling film, clean plastic bag, or non-fluffy dressing. Do not wrap tightly.

WARNING
Do NOT apply toothpaste, coffee, raw egg, yogurt, butter, or home remedies. They trap heat.

Step 6: POSITION patient lying down. Keep warm to prevent hypothermia.

Was the burn caused by a chemical substance?

YES → **REMOVE** contaminated clothing (protect yourself). **FLUSH** with massive cool water 15–20 min. Eyes: flush 15 min. Do NOT neutralise. **CALL** 101.

NO

Was the burn caused by electricity?

YES → Ensure power source disconnected first. **CALL** 101. Be prepared for CPR. Internal injuries may not be visible.

NO

Step 9: MONITOR for complications: infection, respiratory issues, shock. For mild 1st-degree burns: ibuprofen or paracetamol for pain relief.

DO NOT:

X Do NOT apply ice or ice-cold water to the burn -- extreme cold causes additional tissue and blood vessel damage.
X Do NOT apply toothpaste, ground coffee, raw egg, yogurt, butter, or other home remedies -- these trap heat and increase infection risk.
X Do NOT rupture or pop blisters -- the fluid inside protects the underlying tissue during healing.
X Do NOT use fat-based creams or ointments on fresh burns.
X Do NOT remove clothing that is stuck to the burn -- cut around it instead.
X Do NOT attempt chemical neutralisation on chemical burns.
X Do NOT approach an electrical burn casualty until the power source is confirmed disconnected.

Reference

Equipment needed:

— Cool running water source
— Cling film (food wrap) or clean plastic bag
— Clean non-fluffy dressings or bandages
— Scissors (to cut clothing around stuck areas)
— Blanket (to keep patient warm and prevent hypothermia)

Source: Magen David Adom (MDA) — “MDA Burns First Aid Guidelines”
URL: https://www.mdais.org/101/burns
Publication date: 2026-01-01
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. MDA specifies “cool (not cold) running water” for at least 10 minutes. MDA is used as the authoritative default per project rules.

Choking (Foreign Body Airway Obstruction) — Adult

Israel — ADULT

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

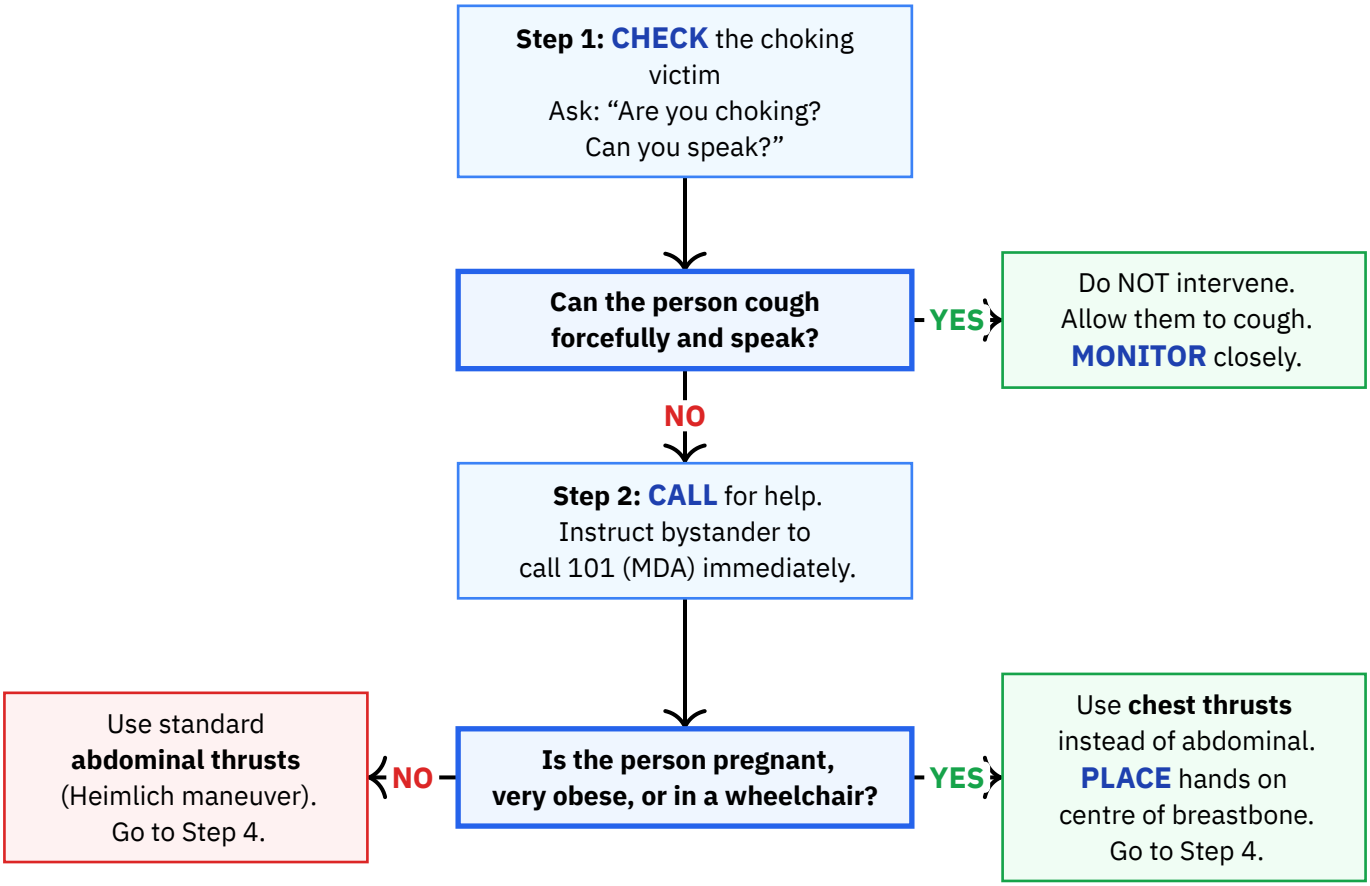
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Person is choking: grasping neck, unable to speak/cough/breathe, face turning red or blue, high-pitched wheezing sounds, sudden distress while eating.

Summary: Abdominal thrusts (Heimlich maneuver) for conscious adults with complete airway obstruction; transition to CPR if unconscious.

Assessment Phase



Heimlich Maneuver and Reassessment

Step 4: APPLY abdominal thrusts
Stand behind the person.
Wrap arms around waist.
Fist above navel, below breastbone.
Thrust inward and upward.
Repeat until object expelled.

↓

Has the object been expelled? Can the person breathe?

- YES → **STOP** thrusts. **MONITOR** the person. Seek medical evaluation (internal injuries possible).

NO ↓

Is the person still conscious?

- YES → Repeat Step 4 (abdominal thrusts). Continue until object expelled or person loses consciousness.

NO ↓

Step 6: Person unconscious.
STOP abdominal thrusts.
Carefully lower to ground onto firm, flat surface.

WARNING
Do NOT continue abdominal thrusts on an unconscious person.

↓

Step 7: CALL 101 (MDA) if not already done.

↓

Step 8: BEGIN CPR:
30 compressions, then 2 breaths.
Before each breath, look in mouth for visible object.
If visible: **REMOVE** carefully.
If not visible: do NOT finger sweep.
Continue 30:2 until EMS arrives.

Self-Treatment (If Choking While Alone)

Step 9: Lean over a firm object (back of a chair or countertop edge). **PRESS** your upper abdomen forcefully against the object to generate an upward thrust. Repeat until the object is expelled. **CALL** 101 as soon as you can speak.

Reference

DO NOT:

X Do NOT perform abdominal thrusts on a person who can cough forcefully and speak -- you may worsen the obstruction or cause injury.
X Do NOT perform back blows (slaps on the back) on an adult. The Israeli/MDA protocol explicitly describes this as incorrect and potentially harmful for adults.
X Do NOT perform blind finger sweeps -- only remove an object from the mouth if you can clearly see it.
X Do NOT attempt to give water or any liquid to a choking person.
X Do NOT continue abdominal thrusts on an unconscious person -- transition to CPR.
X Do NOT use abdominal thrusts on pregnant women or very obese individuals -- use chest thrusts instead.

Equipment needed:

No special equipment required. Use a firm object (chair back, countertop) for self-treatment if alone.

Source: Magen David Adom (MDA) — “How to Treat Choking from a Foreign Body”; corroborated by MDA BLS Guide 2016
URL: https://www.mdais.org/101/asphyxia
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: MDA/Israeli protocol explicitly discourages back blows for adults, diverging from AHA and ERC guidelines. MDA is used as the authoritative default per project rules.

CPR (Cardiopulmonary Resuscitation) — Adult

Israel — ADULT

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

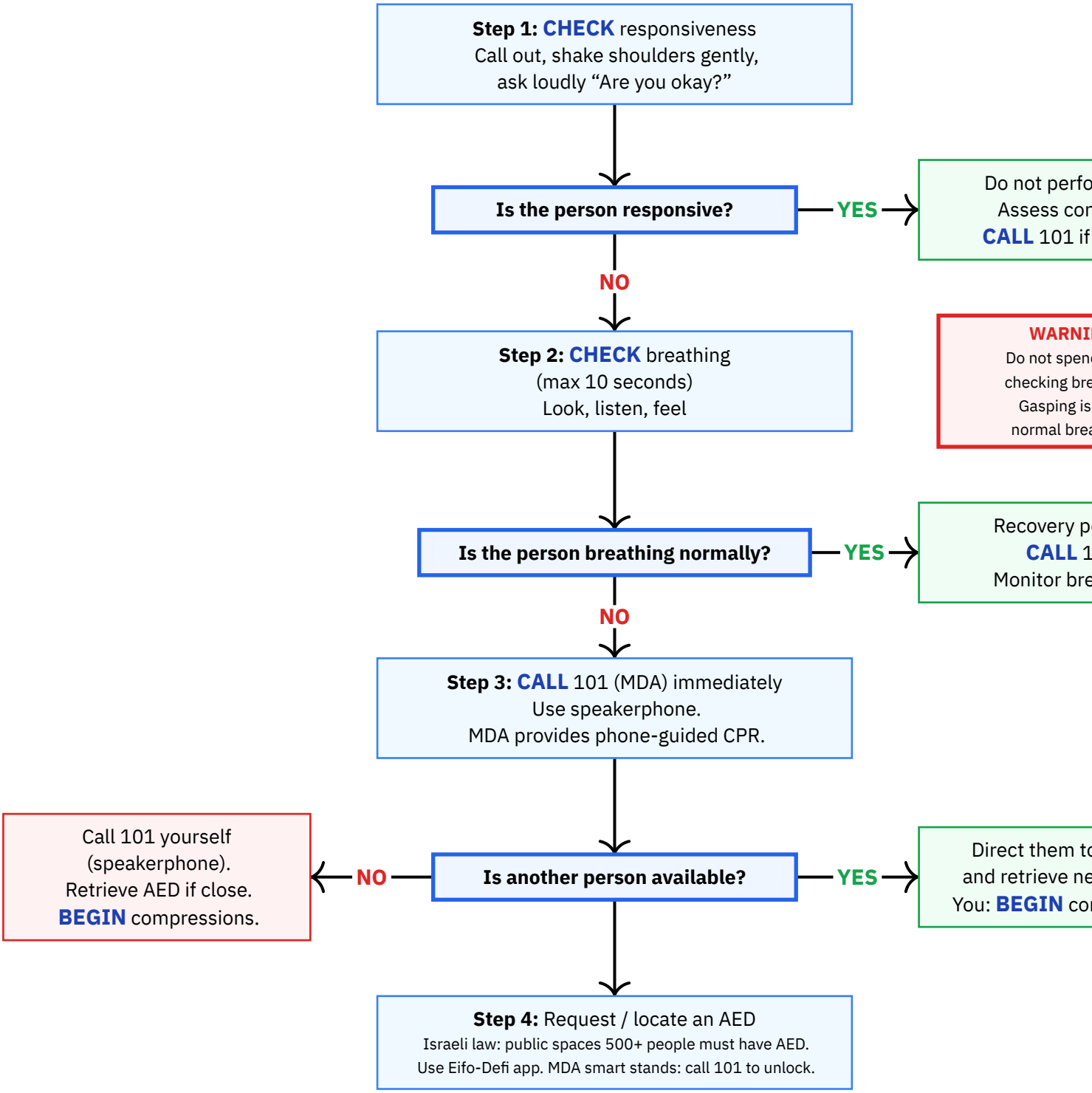
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

An adult (age 12+) is found unresponsive and is not breathing normally (absent breathing or only gasping/agonal breathing).

Summary: Bystander and first-responder CPR protocol for unresponsive adults not breathing normally, following MDA/AHA guidelines with Israel-specific considerations.

Assessment Phase



CPR Technique

Step 5: POSITION person supine (face up) on firm, flat surface

In extreme heat: move to
Hot surfaces: place ba
underneath.

↓

Step 6: PLACE hands on chest
Heel of one hand centre of breastbone (between nipples). Other hand on top, fingers interlocked. Arms straight.

↓

Step 7: BEGIN chest compressions
Rate: 100–120 per minute
Depth: 5–6 cm (2–2.4 in)
Allow full chest recoil.
Minimise interruptions (under 10s pauses).

WARNING
Inadequate depth/ra
incomplete recoil, a
excessive interruptio
reduce effectivenes

↓

Trained in CPR and willing to give rescue breaths?

YES → 30 compressions then 2 rescue breaths Go to Step 8

NO → Hands-only CPR continuous compres 100–120/min. No br Skip to Step 11

Rescue Breaths (if trained)

Step 8: After 30 compressions, **OPEN** airway: head-tilt / chin-lift
Tilt head back, lift chin.

↓

Step 9: DELIVER 2 rescue breaths
Pinch nose, seal mouth, 1 second per breath, watch for chest rise.

WARNING
Do not over-ventilat
Excessive ventilatio
increases gastric infla

↓

Does the chest rise visibly with each breath?

NO → Reposition head (re-tilt). Reattempt. If still no rise: **RESUME** compressions. Do not delay.

YES → **RESUME** compres immediately. Go to Step 10

CPR Cycles and AED

Step 10: RESUME compressions immediately after 2 breaths.
Continue 30:2 cycles.
(2 min for 5 cycles)

↓

Step 11: When AED arrives,
TURN ON and apply pads immediately.
Right pad: under right collarbone.
Left pad: under/left of left breast.

↓

Step 12: FOLLOW AED voice prompts.
Device analyses heart rhythm.

↓

Does the AED advise a shock?

NO → **RESUME** CPR immediately.
AED re-analyses after 2 minutes.

YES → Ensure all
PRESS shoc
RESUME
immediately a

WARNI
Ensure NO
touching the
during analysi
Announce "Sta

↓

Step 13: RESUME CPR immediately after shock (or no-shock).
Do not wait for AED to re-analyse.

↓

Step 14: REASSESS every 2 minutes
(5 cycles of 30:2).
Rotate compressors if 2+ rescuers
(switch in under 10 seconds).

↓

Step 15: CONTINUE CPR until:
EMS arrives and takes over,
person shows signs of life,
AED prompts to stop, or
you are physically unable.

↓

Has the person begun breathing normally?

NO → **CONTINUE** CPR cycles and AED use.
Do not stop until EMS takes over or unable.

YES → **STOP**
Recovery p
(on side, leg be
Monitor until E

Reference

Key CPR Parameters

Compression rate: 100–120 / min
Compression:breath ratio: 30 : 2
Max pause: < 10 seconds
Emergency number: 101 (MDA)
Compression depth: 5–6 cm (2–2.4 in)
Reassess interval: Every 2 min (5 cycles)
Breath duration: 1 second each
AED re-analysis: Every 2 minutes

DO NOT:

X Do not delay calling 101 -- early activation of EMS is critical for survival.
X Do not spend more than 10 seconds checking for breathing.
X Do not interrupt chest compressions for more than 10 seconds for any reason.
X Do not lean on the chest between compressions -- allow full recoil.
X Do not compress deeper than 6 cm (2.4 inches) to avoid internal injury.
X Do not over-ventilate -- deliver each breath over 1 second only, just enough for visible chest rise.
X Do not touch the person while the AED is analysing the rhythm or delivering a shock.
X Do not delay AED use -- apply it as soon as it arrives, even if CPR is in progress.
X Do not wait for the AED to re-analyse before resuming compressions after a shock.
X Do not move the person unnecessarily -- perform CPR where they are unless the scene is unsafe.
X Do not assume gasping (agonal breathing) is normal breathing -- it is a sign of cardiac arrest.

Equipment needed:

— AED (Automated External Defibrillator)
— Barrier device or pocket mask (for rescue breaths, if available)
— Phone (to call 101 and use speakerphone for dispatcher-guided CPR)

Source: Magen David Adom (MDA) — "MDA CPR Protocol – synthesised from MDA public CPR page, Maccabi Health adult resuscitation guide, and AHA 2025 Guidelines for CPR and ECC (adopted by MDA)"
URL: https://www.mdais.org/101/cpr-hands-only
Publication date: 2025-01-01 · **Edition:** AHA 2025 Guidelines
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah reference material. No protocol-level differences were identified between the two organisations; both follow AHA guidelines. MDA is used as the authoritative default per project rules.

Dehydration and Heat-Related Illness — Adult

Israel — ADULT

CALL 101 (MDA) — IMMEDIATELY FOR HEAT STROKE

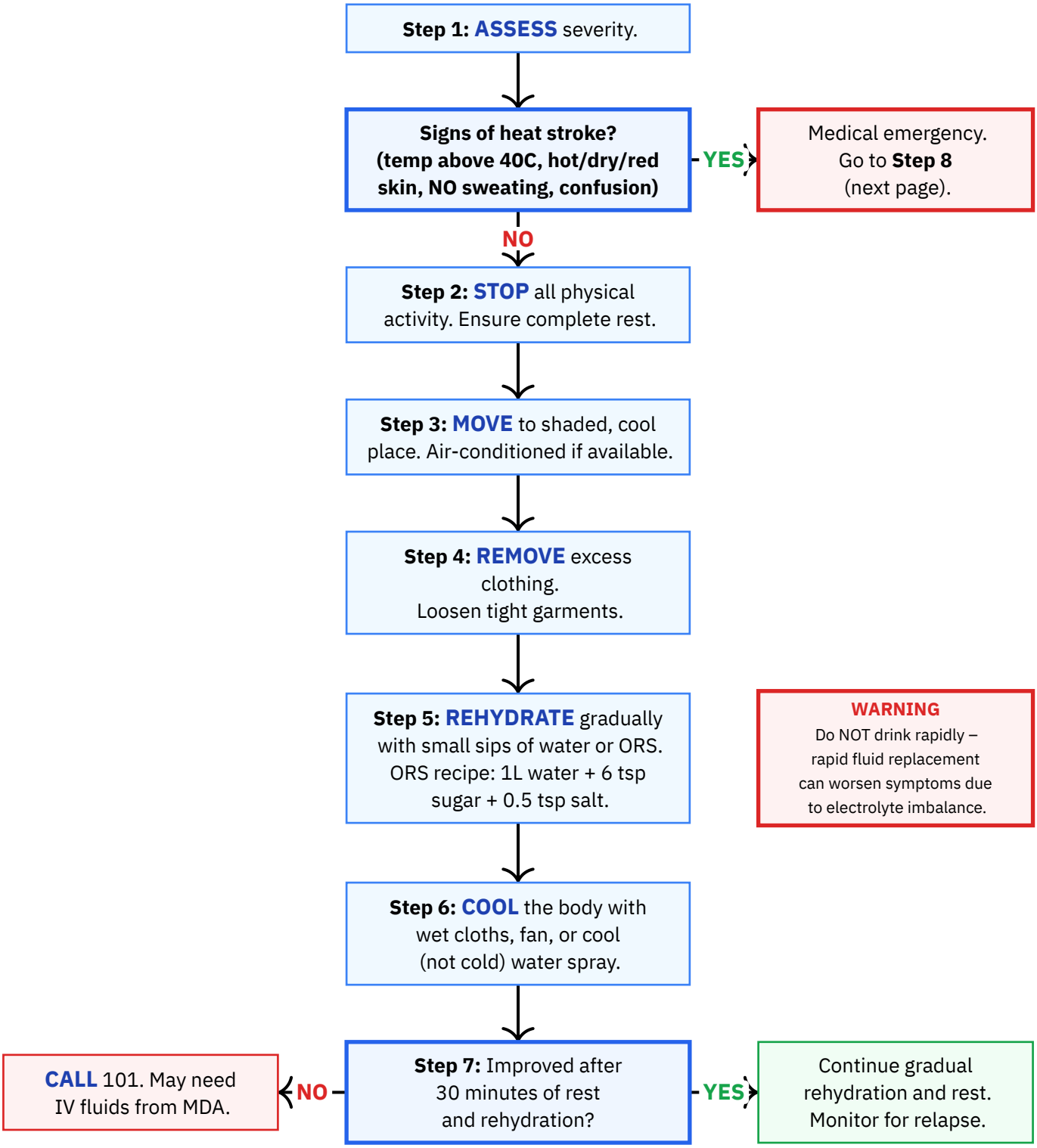
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

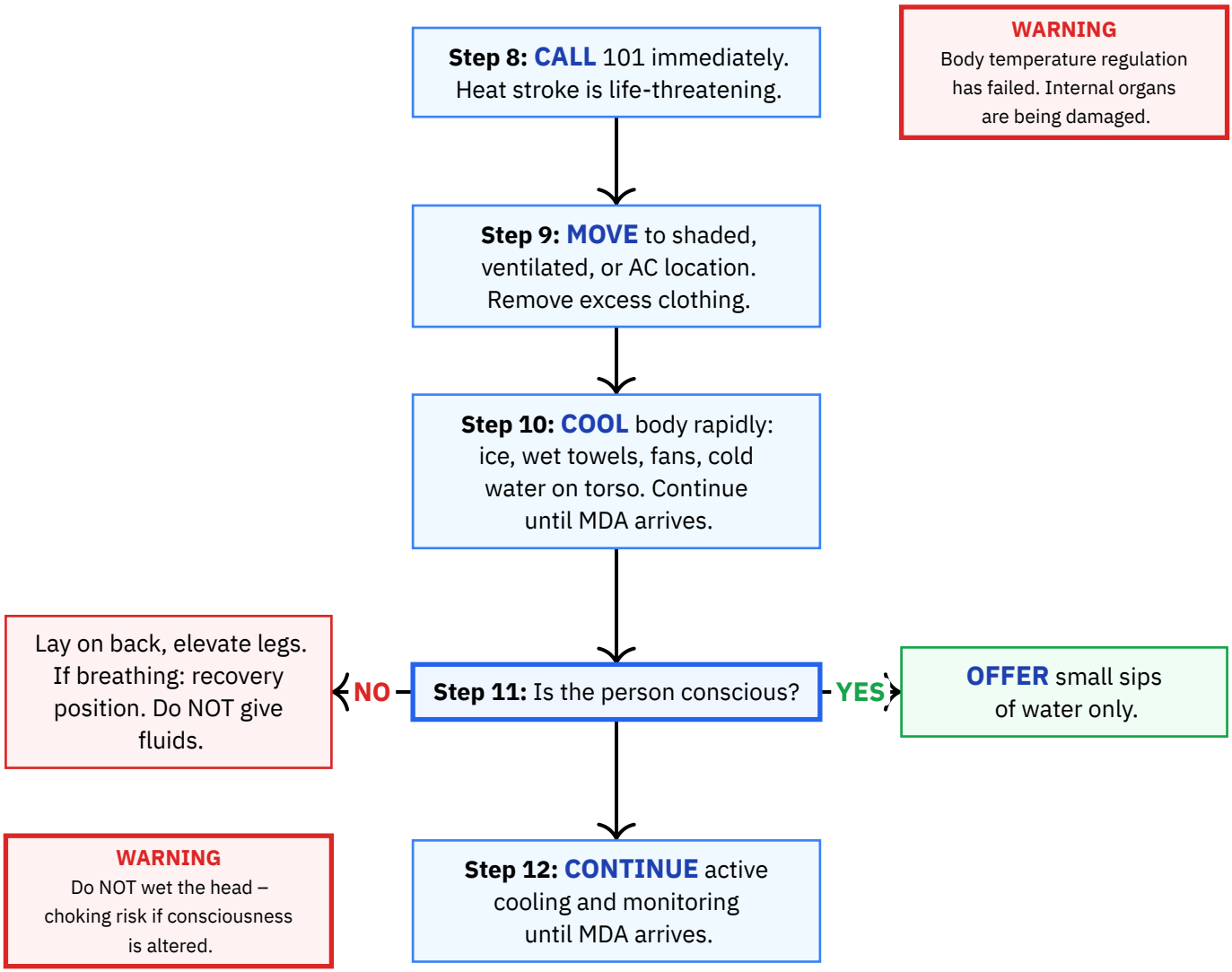
Person shows signs of dehydration: increased thirst, dry mouth, reduced/dark urine, fatigue, dizziness, headache, rapid pulse. Or signs of heat stroke: body temperature above 40 degrees C, hot dry skin (sweating has stopped), confusion, loss of consciousness.

Key Distinction: Dehydration involves continued sweating. Heat stroke involves ABSENT sweating with high body temperature – this is a medical emergency.

Triage and Dehydration Management



Heat Stroke Emergency Protocol



DO NOT:

X Do NOT give large quantities of water quickly -- rapid fluid replacement disrupts electrolyte balance.
X Do NOT give sugary drinks, coffee, or alcohol.
X Do NOT wet the head of a heat stroke patient -- choking risk if consciousness is altered.
X Do NOT ignore the key distinction: dehydration involves continued sweating; heat stroke involves ABSENT sweating with high body temperature.
X Do NOT delay calling 101 for heat stroke -- it is a medical emergency requiring hospital treatment.
X Do NOT give fluids to an unconscious person.

Equipment needed:

— Water or ORS (Oral Rehydration Solution)
— Fan or method of creating air flow
— Wet cloths or towels for cooling
— Ice packs (for heat stroke)
— Straw (for controlled sipping)

Source: Magen David Adom (MDA) — Dehydration and Heat-Related Illness First Aid
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and Hatzalah Without Borders sources. Both emphasise gradual rehydration with small sips.

Diabetic Emergencies — Adult

Israel — ADULT

CALL 101 (MDA) — WHEN IN DOUBT, GIVE SUGAR

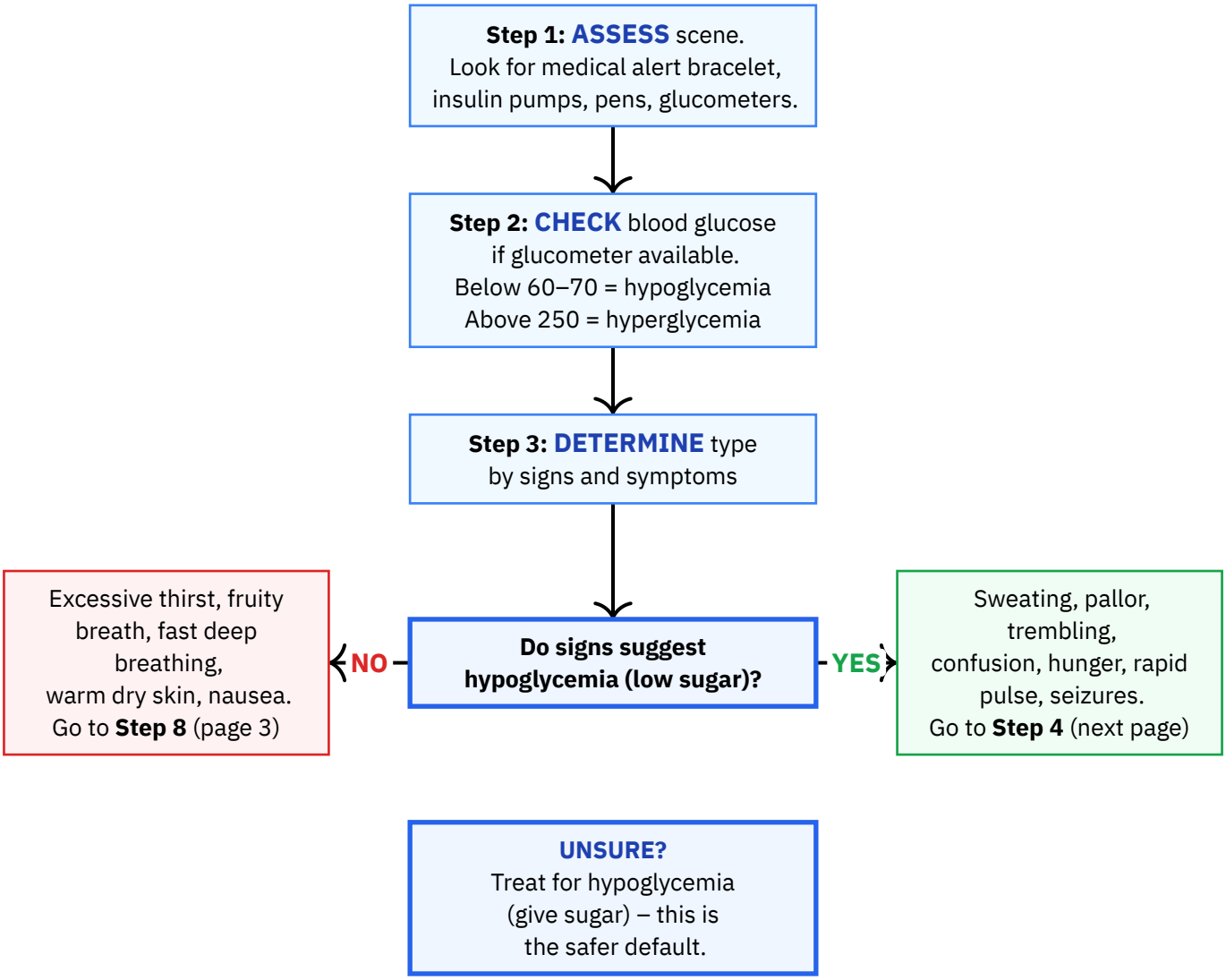
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

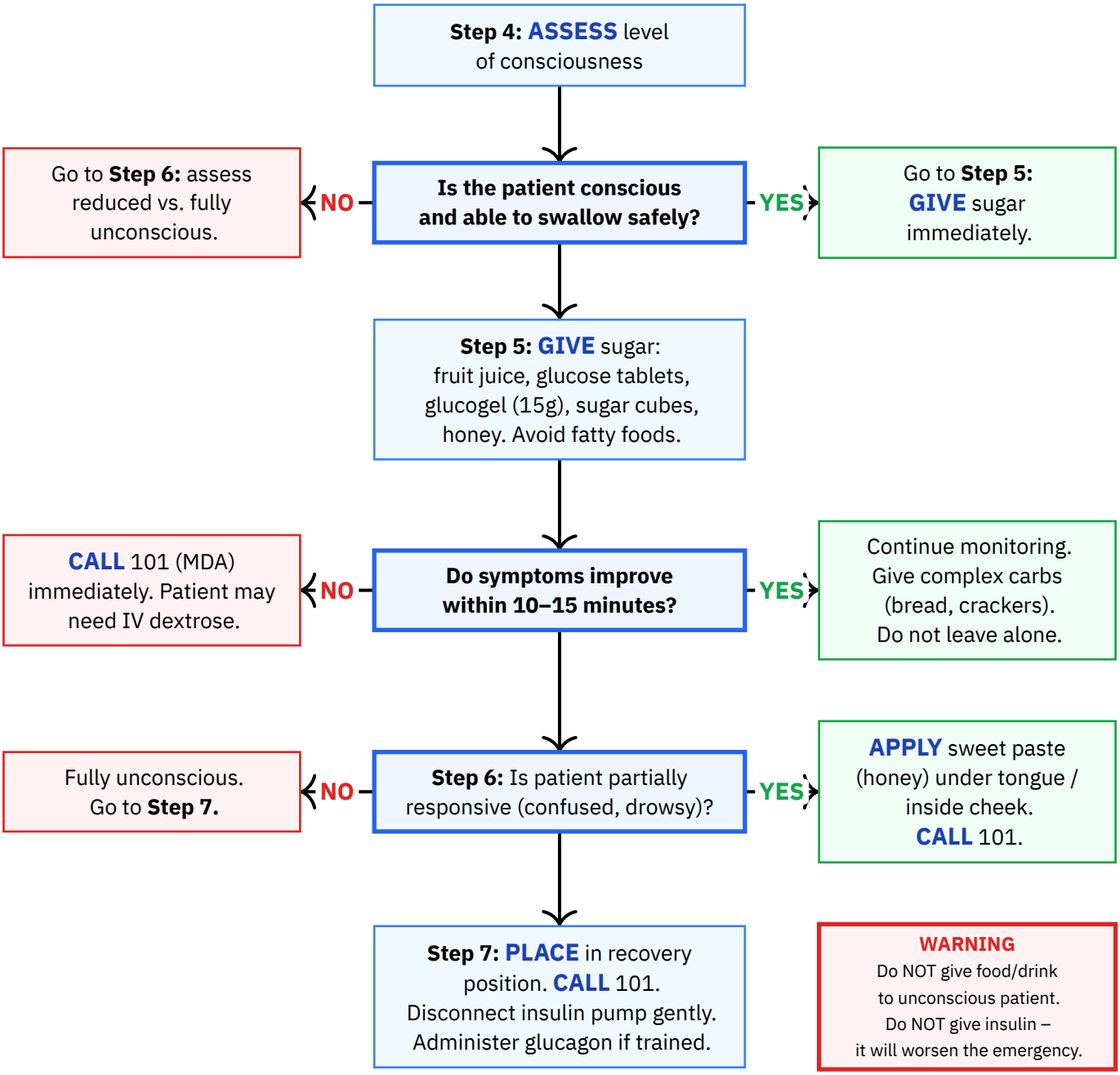
Person is a known or suspected diabetic showing signs of altered consciousness, confusion, weakness, trembling, sweating, behavioural changes, excessive thirst, fruity breath odour, or rapid deep breathing. Also apply when any person has unexplained altered consciousness.

Key Rule: When in doubt whether blood sugar is high or low, **GIVE** sugar – this is the safer default. Giving sugar to a hypoglycaemic patient is life-saving.

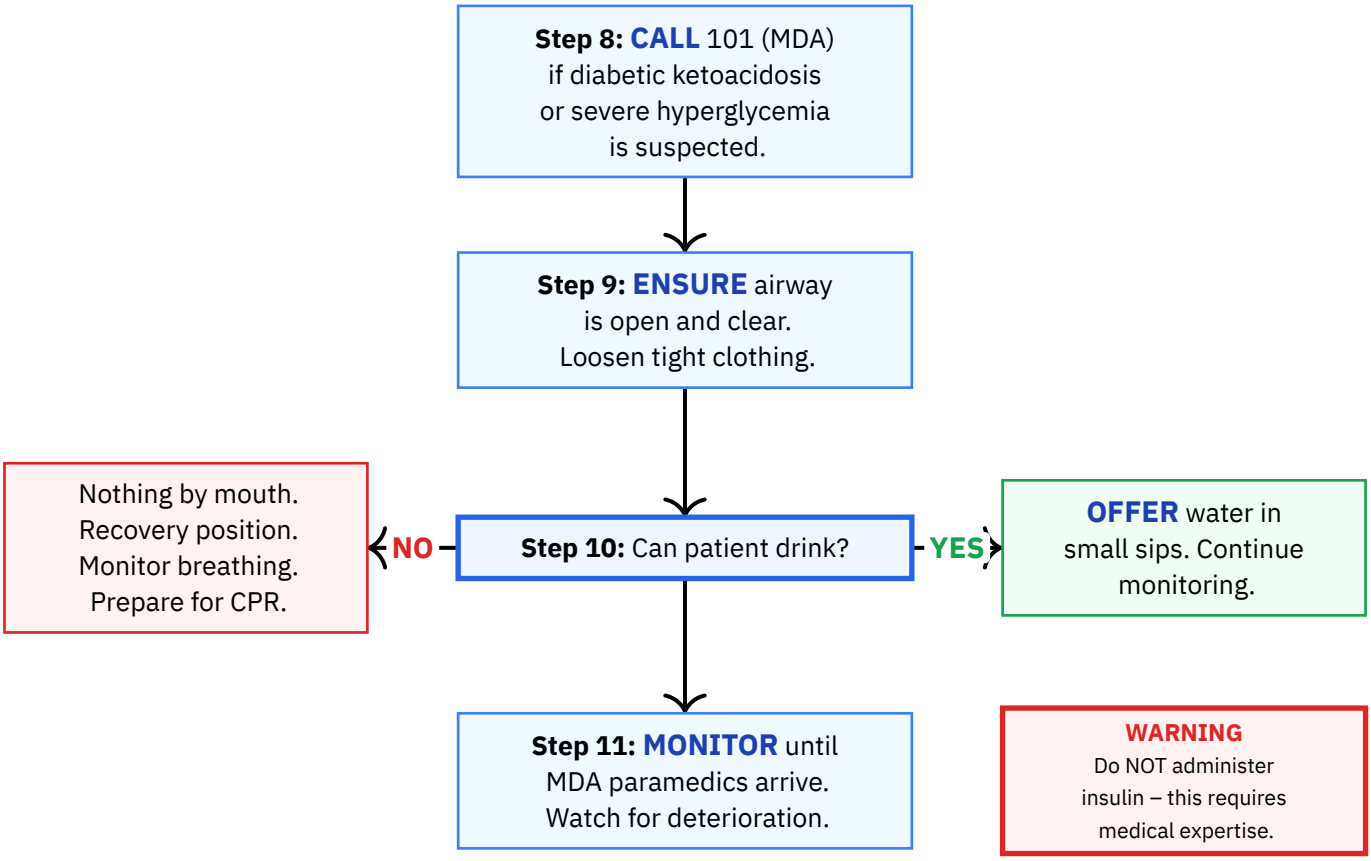
Initial Assessment



Hypoglycemia Pathway (Low Blood Sugar)



Hyperglycemia Pathway (High Blood Sugar)



DO NOT:

X Do NOT administer insulin as a first aider -- insulin dosing requires medical expertise.
X Do NOT give food or drink to an unconscious patient -- risk of choking and aspiration.
X Do NOT give diet drinks or sugar-free foods when treating hypoglycemia.
X Do NOT give sugary foods high in fat (chocolate bars, ice cream) as first-line treatment -- fat slows sugar absorption.
X Do NOT withhold sugar when in doubt -- giving sugar to a hypoglycaemic patient is life-saving.
X Do NOT leave a diabetic emergency patient unattended.
X Do NOT delay calling 101 if patient is unconscious, seizing, or not improving.
X Do NOT disconnect an insulin pump roughly.

Equipment needed:

— Phone to call 101 (MDA) or 1221 (United Hatzalah)
— Glucometer and test strips (if available)
— Rapidly absorbed sugar source (glucose tablets, glucogel, fruit juice, sugar cubes, honey)
— Complex carbohydrate for follow-up (bread, crackers)
— Glucagon injection kit (if available and trained to use)

Source: Magen David Adom (MDA) — How to Treat a Diabetes Patient
URL: https://www.mdais.org/101/diabetes
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Both sources are broadly consistent on all key points.

Drowning (Near-Drowning) — Adult

Israel — ADULT

CALL 101 (MDA) — CALL IMMEDIATELY

MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

Person found submerged or immersed in water, rescued from water who is unresponsive or not breathing, or who has experienced a near-drowning event in any body of water including pools, sea, lakes, or the Dead Sea.

Summary: Rescue from water, assess breathing, perform CPR (ABC sequence preferred for trained rescuers), and transport to hospital – even if the person recovers.

Rescue and Initial Assessment

WARNING
Untrained rescuers who enter the water frequently become drowning victims.

Step 1: ENSURE your own safety before attempting rescue. Use reaching aids if possible.

Are you a trained swimmer or lifeguard?

- **NO** → Do NOT enter water. Use pole, rope, or throw buoyant object. Wait for trained rescuer.
- **YES** → You may enter water to perform rescue. Use flotation aids.

Step 2: CALL 101 (MDA) immediately. Speakerphone. Provide location. Send bystander for AED.

Step 3: REMOVE person from water as quickly as possible. Move to dry area.

Step 4: CHECK responsiveness and breathing. Call out, tap shoulder, observe chest.

Is the person responsive and breathing normally?

- **NO** → Immediate CPR needed. Proceed to Step 5.
- **YES** → Recovery position. Monitor breathing. Keep warm – remove wet clothing, cover with dry blankets. Go to Step 8.

CPR for Drowning

Step 5: POSITION person on back on firm, flat, dry surface away from water.

↓

Step 6: BEGIN CPR. Drowning is a hypoxic emergency – rescue breaths are especially important.

↓

Are you trained in full CPR with rescue breaths?

YES →

Use ABC approach: **OPEN** airway, give 2 rescue breaths, then 30 compressions. Continue 30:2 ratio. Rate: 100/min.

NO →

Hands-only CPR: continuous compressions at 100/min. Do not stop until MDA arrives.

↓

Step 7: USE AED if available. Dry the chest thoroughly. Ensure dry surface. Follow AED voice prompts.

WARNING
Using AED on wet patient or wet surface risks electrical injury. Always dry chest and move to dry area first.

↓

Step 8: CONTINUE CPR or monitoring until MDA arrives. If person regains consciousness, recovery position and monitor.

↓

Step 9: ENSURE hospital transport regardless of apparent recovery.

WARNING
Do NOT allow person to refuse treatment. Secondary drowning can cause death 1–24 hours after the incident. Electrolyte imbalances can be fatal.

Reference

DO NOT:

X Do NOT enter the water to rescue someone unless you are a trained swimmer or lifeguard -- untrained rescuers frequently become drowning victims.
X Do NOT attempt to remove water from the victim's lungs. MDA states this is impossible, dangerous, and a waste of critical time.
X Do NOT perform the Heimlich maneuver, turn the person upside down, or use any method to 'drain' water from the lungs.
X Do NOT use an AED on a wet patient or on a wet surface -- dry the chest and move to a dry area first.
X Do NOT allow a near-drowning victim to refuse hospital transport, even if they appear fully recovered. Secondary drowning and electrolyte imbalances can be fatal hours later.
X Do NOT stop CPR until the person is breathing on their own or professional medical help takes over.
X Do NOT delay calling 101 -- call immediately, even before attempting rescue if you are alone.

Equipment needed:

— AED (defibrillator) -- commonly available at lifeguard stations and public facilities in Israel
— Reaching aids (pole, rope, flotation device) for water rescue
— Dry blankets or clothing to keep the victim warm after extraction

Source: Magen David Adom (MDA) — “Drowning – MDA 101 Public First Aid Guidance Series; corroborated by MDA ALS Manual April 2024”
URL: https://www.mdais.org/101/drowning
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Includes Israel-specific context for Dead Sea, Mediterranean, and Kinneret hazards.

Electrocution / Electrical Injury — Adult

Israel — ADULT

CALL 101 (MDA) + 102 (Fire) + 103 (IEC)

MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **IEC:** 103

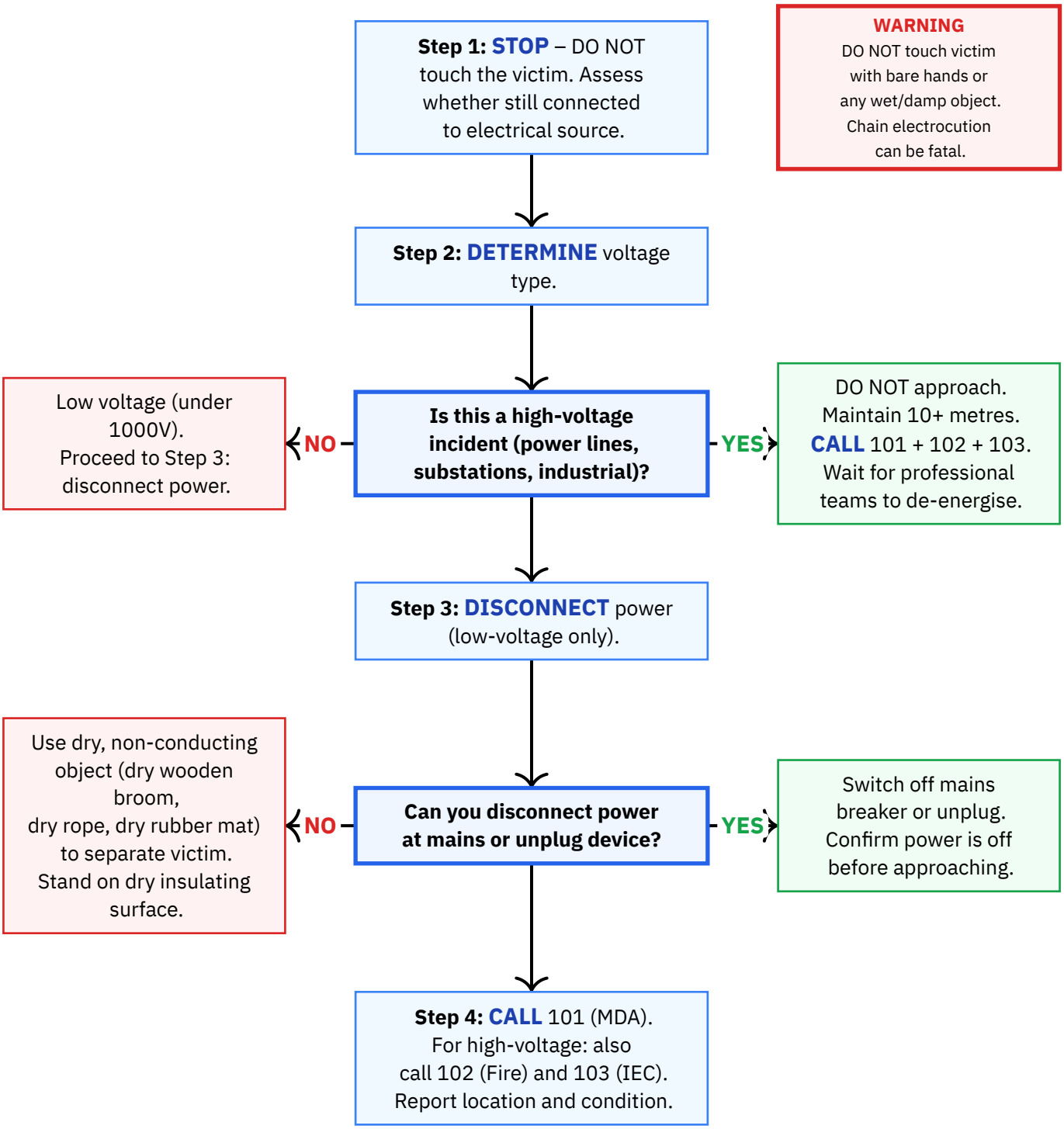
When to apply:

An adult has received an electrical shock from any source: household appliances, power lines, industrial equipment, lightning. Signs include burns (entry/exit wounds), loss of consciousness, muscle spasms, breathing difficulty, cardiac arrest, confusion, or visible contact with an electrical source. Israel uses 220V AC / 50Hz household supply.

Summary: Scene safety, power disconnection before any contact, CPR readiness, burns assessment, and mandatory hospital evaluation.

Personal reference only — not medical advice

Scene Safety and Power Disconnection



Assessment and Resuscitation

Step 5: ASSESS consciousness and breathing (once scene safe). Call out, tap shoulders. Look, listen, feel (max 10s).

↓

Is the victim responsive?

- **YES** → Keep victim still. Proceed to Step 8 (burns assessment). Monitor for deterioration.
- **NO** ↓

Step 6: CHECK breathing of unresponsive victim.

↓

Is the victim breathing normally (not just gasping)?

- **YES** → Recovery position (on side). Monitor breathing continuously. Be prepared to begin CPR.
- **NO** ↓

Step 7: BEGIN CPR.
30 compressions : 2 breaths.
Rate: 100–120/min.
Depth: 5–6 cm.
Full chest recoil.
If AED available: dry chest, apply pads, follow prompts.

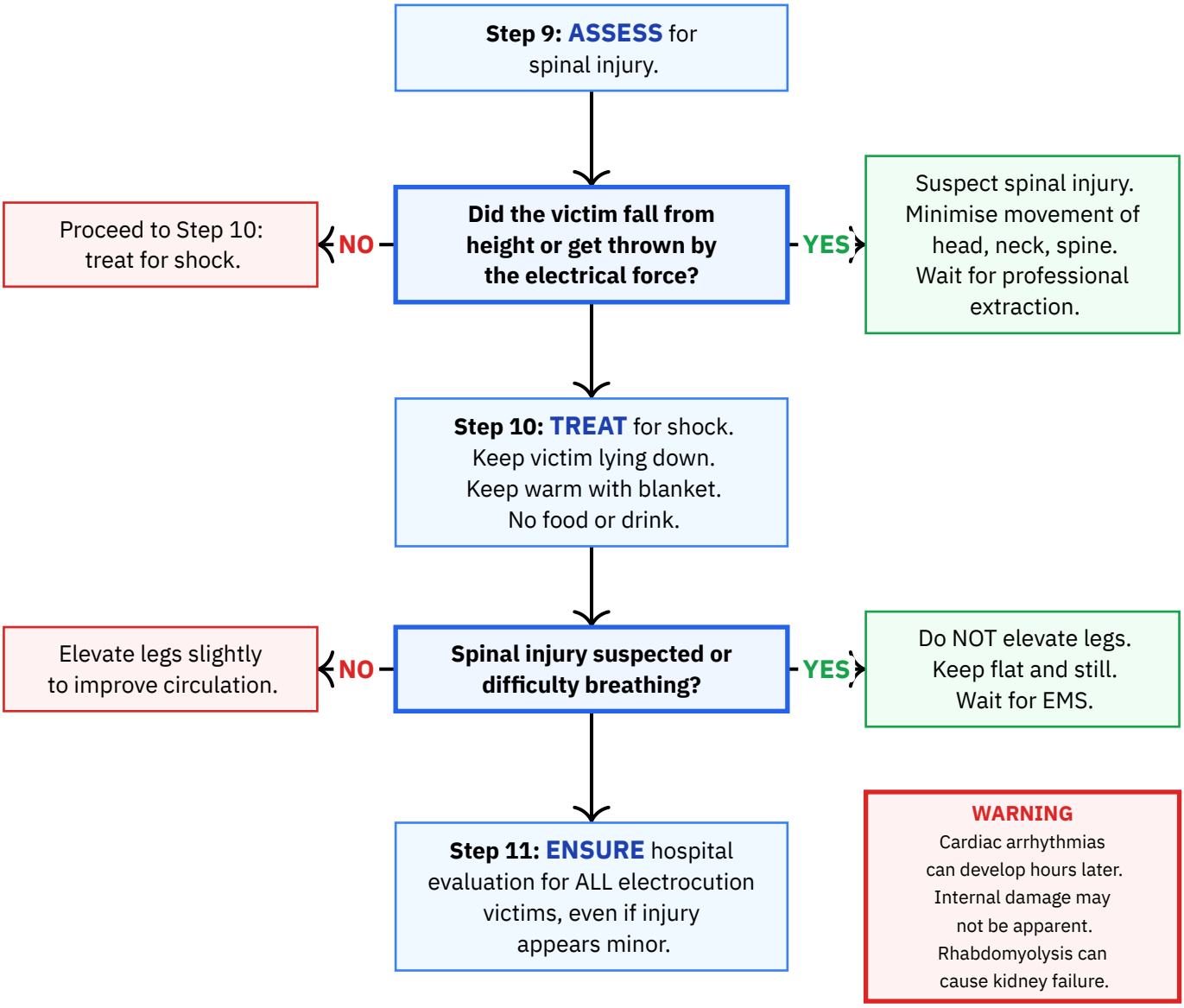
WARNING
Electrocution victims have higher-than-average survival rate if CPR started promptly. Do not give up.

↓

Step 8: ASSESS and treat burns.
Look for entry/exit wounds.
Cool visible burns with cool running water (10+ min).
Cover with cling film or clean non-fluffy dressing.

WARNING
External burns may appear small but conceal severe internal tissue damage along the current pathway.

Spinal Injury, Shock, and Hospital Transport



DO NOT:

X Do NOT touch the victim while they may still be in contact with the electrical source -- chain electrocution can kill the rescuer.
X Do NOT use wet or damp objects to separate the victim from the electrical source.
X Do NOT approach high-voltage incidents (power lines, substations) -- maintain at least 10 metres and wait for IEC (103).
X Do NOT perform CPR on a conscious victim.
X Do NOT apply water to burns while the victim is still connected to the electrical source.
X Do NOT apply creams, ointments, or home remedies to electrical burns.
X Do NOT assume the injury is minor based on small visible burns -- internal damage may be severe.
X Do NOT give food or drink to an electrocution victim.

Equipment needed:

— Dry non-conducting object (dry wooden broom handle, dry rope, dry rubber mat)
— Dry insulating surface to stand on (dry wood, dry rubber mat, newspapers)
— AED (automated external defibrillator)
— Cool running water source (for burn cooling after scene is safe)
— Cling film or clean non-fluffy dressings (for burn covering)
— Blanket (for shock prevention)

Source: Magen David Adom (MDA) — "MDA Electrocution First Aid Guidelines"
URL: https://www.mdais.org/101/electrocution
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Israel uses 220V AC / 50Hz household supply. High-voltage: also call Fire (102) and IEC (103).

Eye Injuries — Adult

Israel — ADULT

CALL 101 (MDA) — FOR PENETRATING, CHEMICAL, OR SEVERE INJURIES

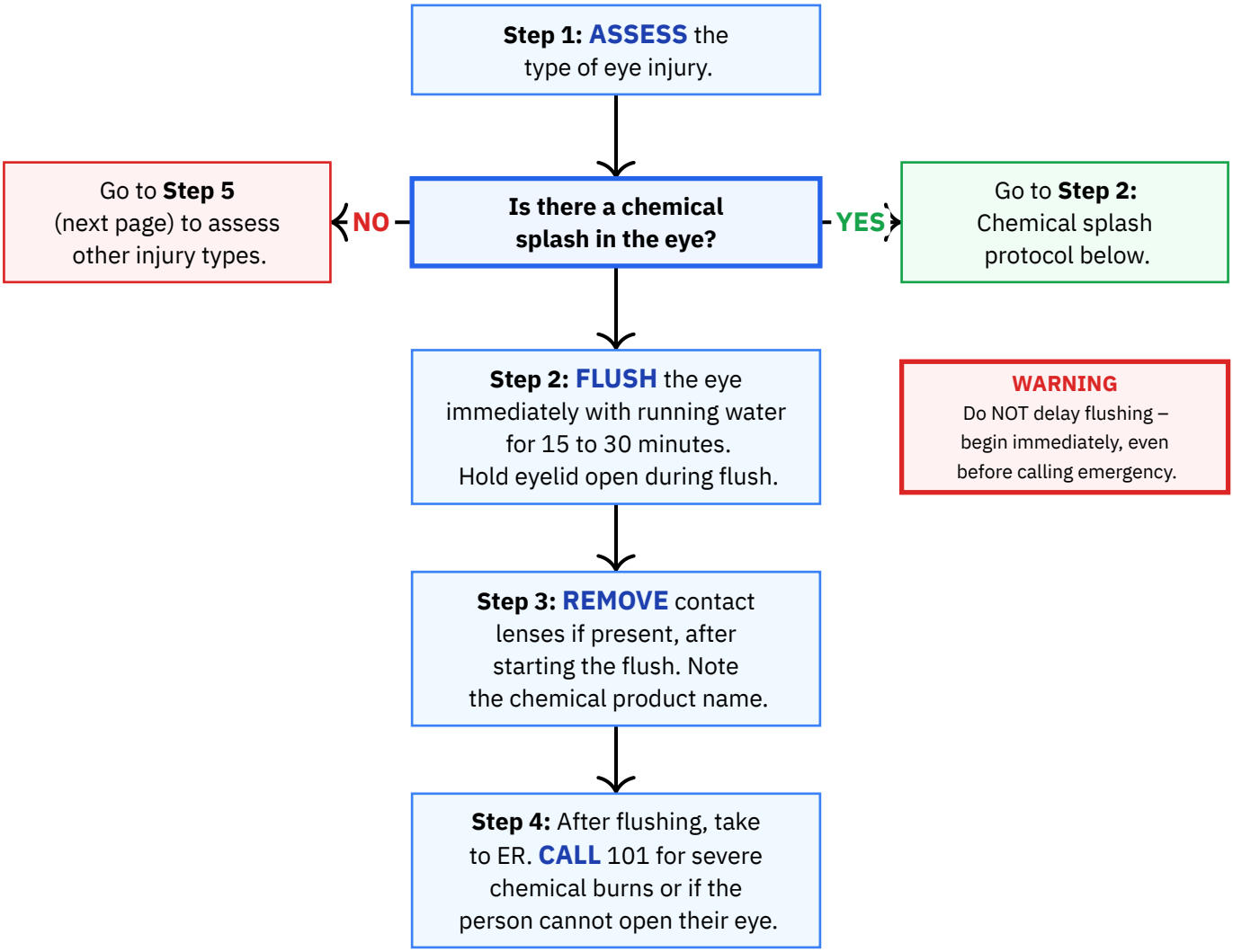
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

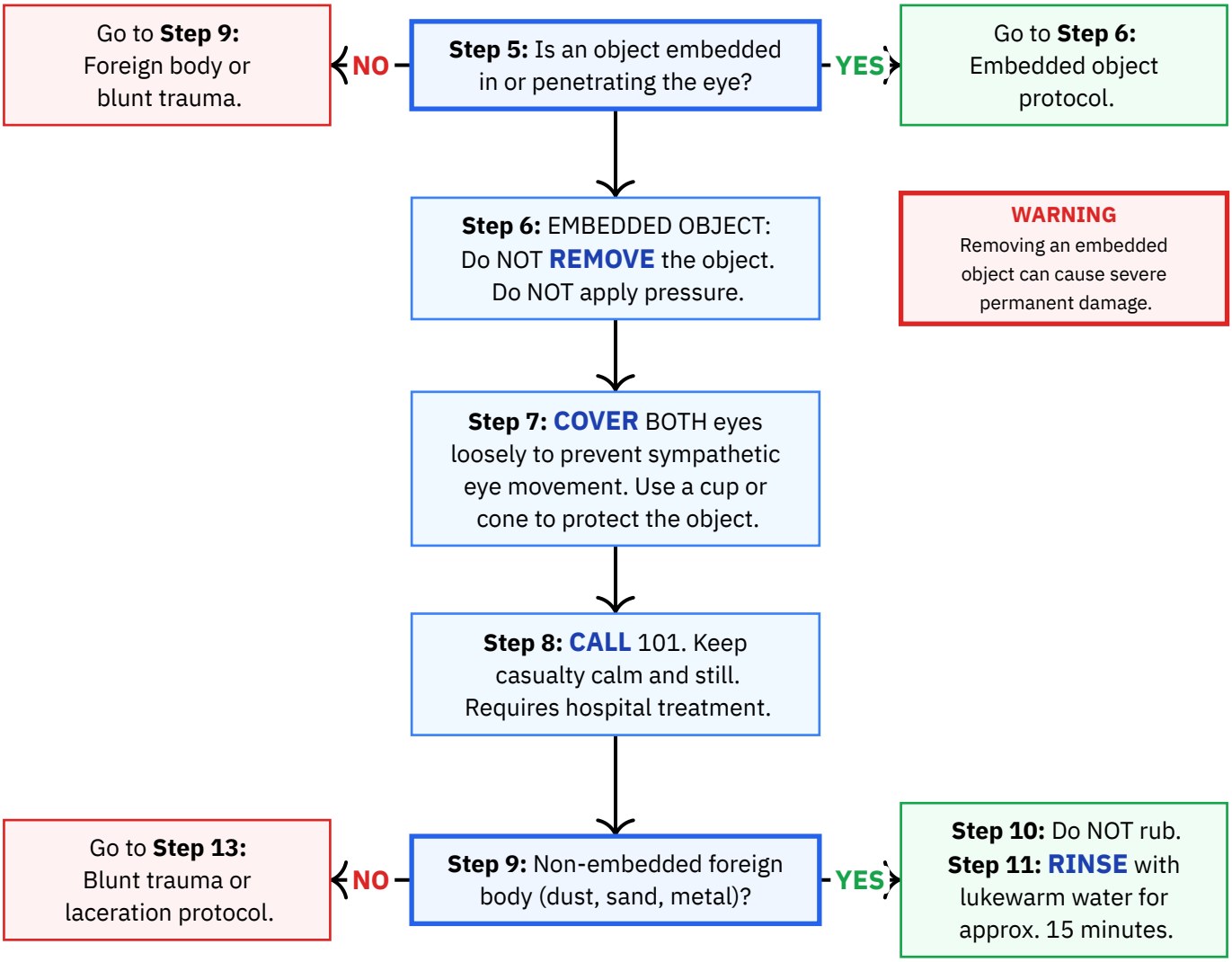
Person has sustained an eye injury from a foreign body, chemical splash, penetrating object, blunt force, or laceration.

Summary: First aid for eye injuries including foreign bodies, chemical splashes, embedded objects, blunt trauma, and lacerations. Covers multiple injury types with specific protocols for each.

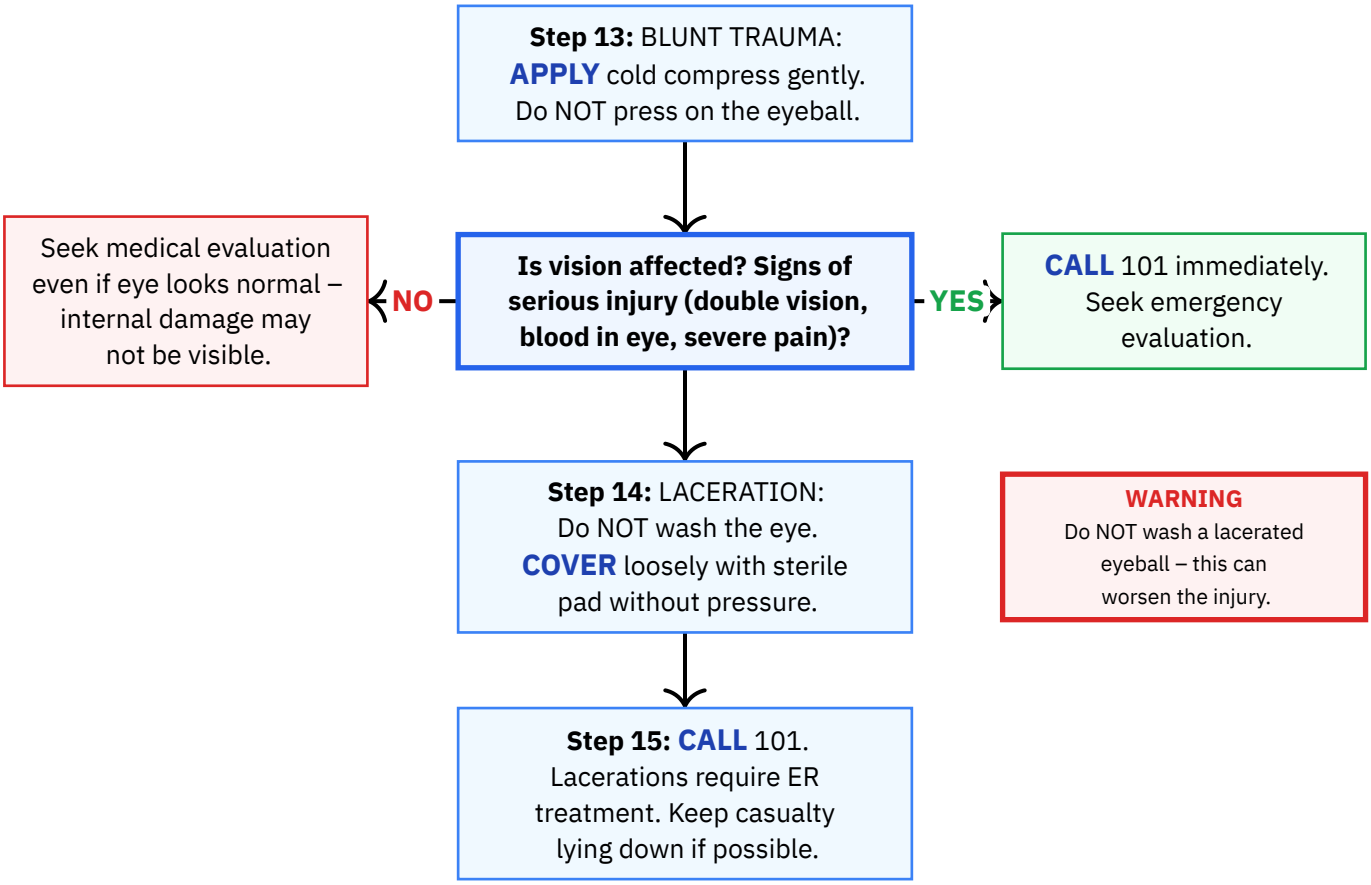
Triage and Chemical Splash Protocol



Embedded Object and Foreign Body Protocols



Blunt Trauma and Laceration Protocols



DO NOT:

X Do NOT rub the eye when a foreign body is present.
X Do NOT attempt to remove an embedded or penetrating object.
X Do NOT apply pressure to an injured eye or embedded object.
X Do NOT wash the eye if there is a laceration to the eyeball.
X Do NOT delay flushing for chemical splashes -- start immediately.
X Do NOT press a cold compress onto the eyeball for blunt trauma.

Equipment needed:

— Clean running water source for flushing
— Sterile eye pads or clean cloth
— Cold compress (for blunt trauma)
— Cup or cone (to protect embedded objects)
— Bandages to loosely cover both eyes if needed

Source: Magen David Adom (MDA) — Eye Injury First Aid
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and Hatzalah Without Borders sources. Covers chemical splash, embedded objects, foreign body, blunt trauma, and lacerations.

Fractures and Sprains — Adult

Israel — ADULT

CALL 101 (MDA) FOR ALL SUSPECTED FRACTURES

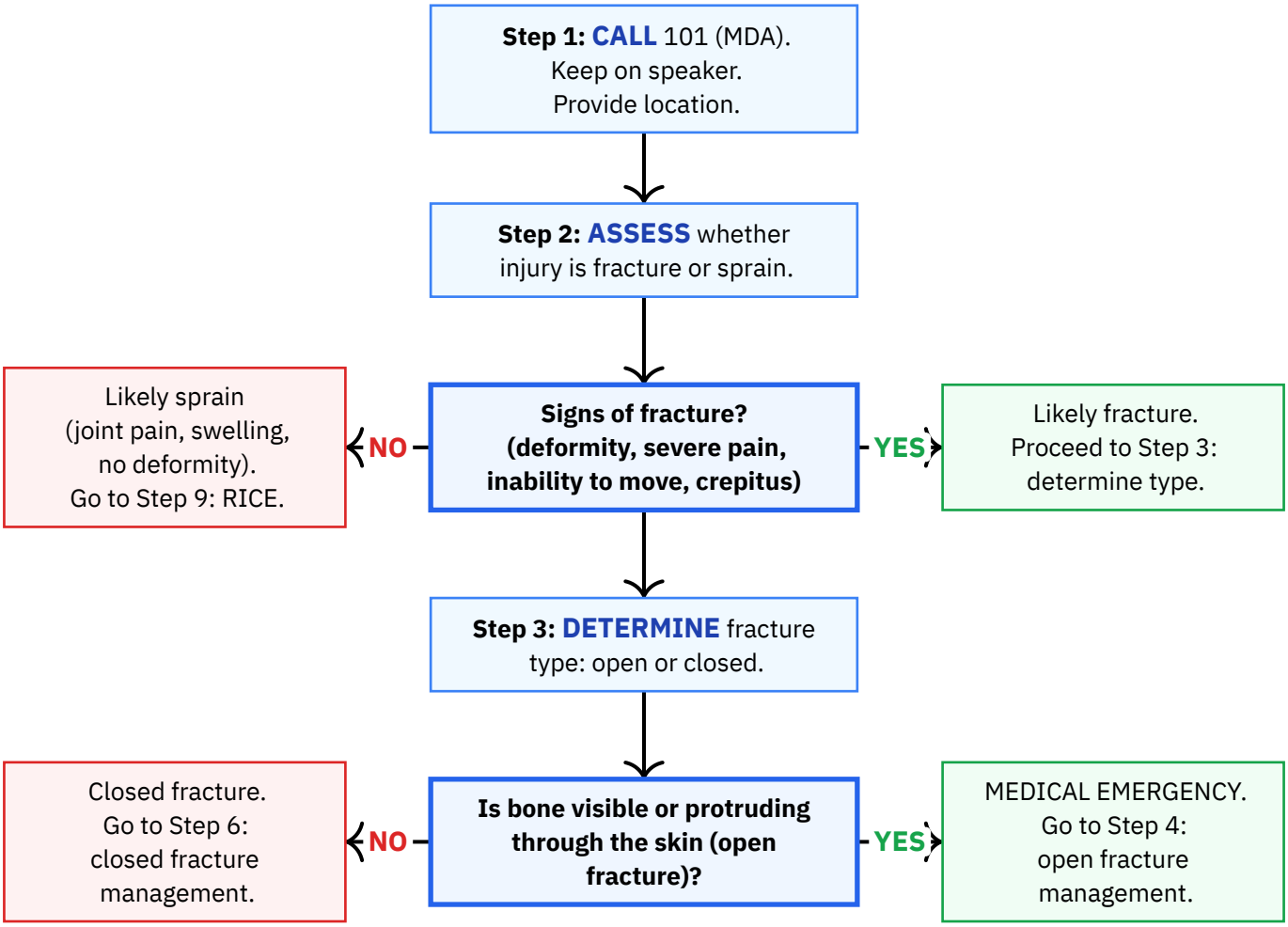
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

An adult has sustained an injury to a bone or joint. Signs include severe pain worsening with movement, swelling and bruising, visible deformity, inability to move the affected limb, possible crepitus, or a popping sensation at the time of injury.

Summary: Immobilisation, RICE for sprains, and recognising when to call emergency services. Core principle: minimum movement, maximum stability.

Assessment and Classification



Open Fracture Management

Step 4: CONTROL bleeding. Apply direct pressure around (not on) protruding bone. Use tourniquet if severe.

WARNING
Do NOT push the bone back in. Leave it as found.

Step 5: COVER wound with sterile dressing. Support limb in position found. Do NOT attempt to realign. Monitor for shock. Go to Step 8.

Closed Fracture Management

Step 6: Do NOT move the limb unnecessarily. Allow comfortable position.

WARNING
Do NOT straighten or realign the limb. This may tear arteries and nerves.

Step 7: IMMOBILISE the injured area.

Is the fracture in the arm or upper limb?

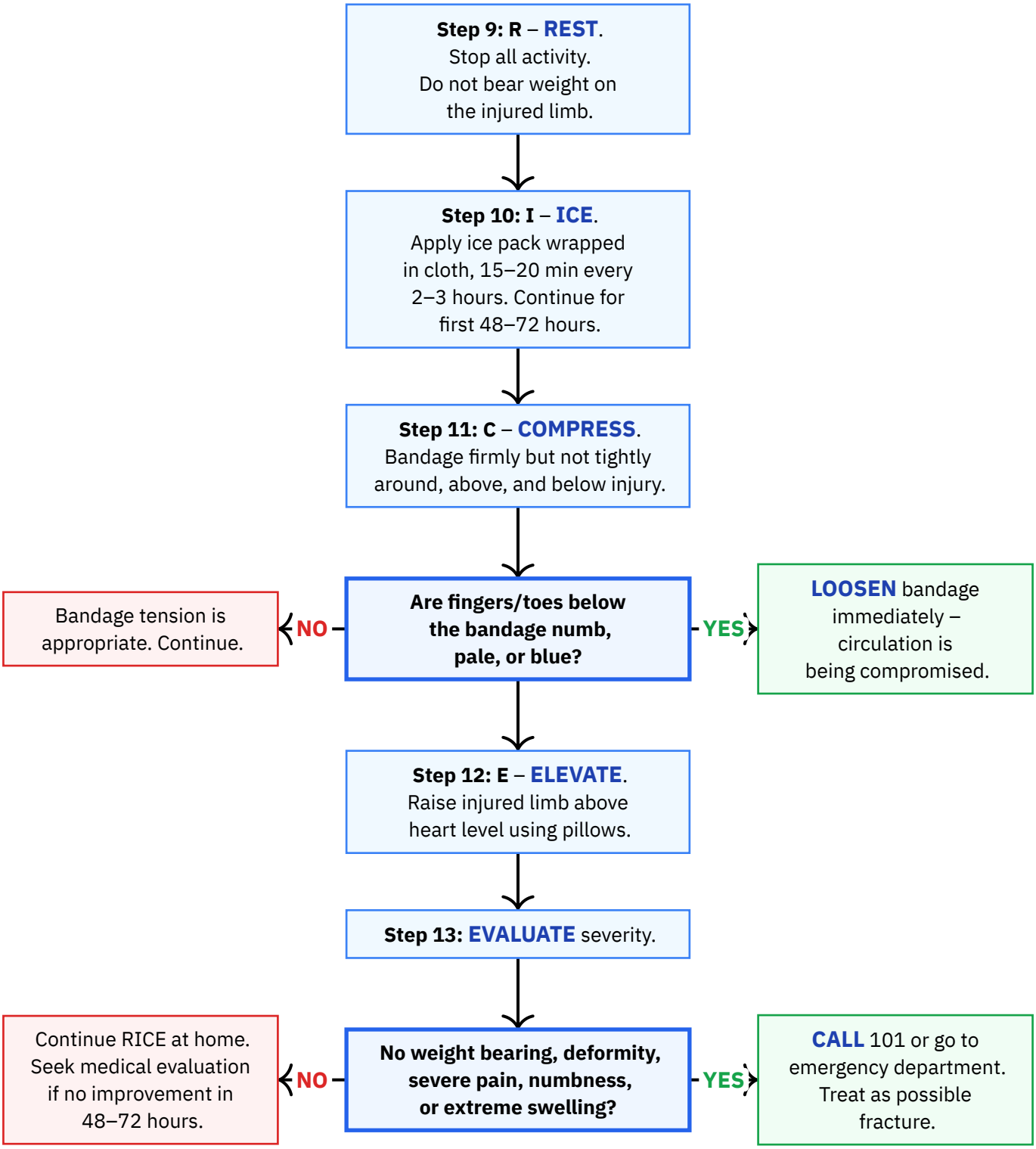
NO → Leg/lower limb: do not move. Place pillows or rolled blankets on both sides. Secure with triangular cloth.

YES → Support with healthy hand. Use improvised sling (shirt, scarf). Attach to chest. Tie at neck.

Step 8: APPLY cold therapy (ice wrapped in cloth, 15 min). **REMOVE** rings, watches, jewellery from affected limb. Monitor for shock until MDA arrives.

WARNING
Do NOT apply ice directly to skin. Always wrap in cloth.

Sprain: RICE Protocol



Reference

DO NOT:

X Do NOT attempt to straighten or realign a broken limb -- this may tear arteries and nerves.
X Do NOT push a protruding bone back in during an open fracture.
X Do NOT apply ice directly to skin -- always wrap in cloth first.
X Do NOT apply heat to a fresh fracture or sprain -- heat increases swelling.
X Do NOT allow the patient to walk on a suspected leg fracture.
X Do NOT remove embedded objects from a wound associated with a fracture.

Equipment needed:

— Triangular bandage or improvised sling (shirt, scarf)
— Ice pack or cooling spray (wrapped in cloth)
— Elastic bandage (for compression of sprains)
— Pillows, blankets, or rolled towels (for leg immobilisation and elevation)
— Sterile dressings or clean cloth (for open fracture wounds)
— Tourniquet (for severe bleeding in open fractures)

Source: Magen David Adom (MDA) — “MDA Fractures and Sprains First Aid Guidelines”
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Both organisations are fully consistent on fracture and sprain treatment.

Head and Spinal Injury — Adult

Israel — ADULT

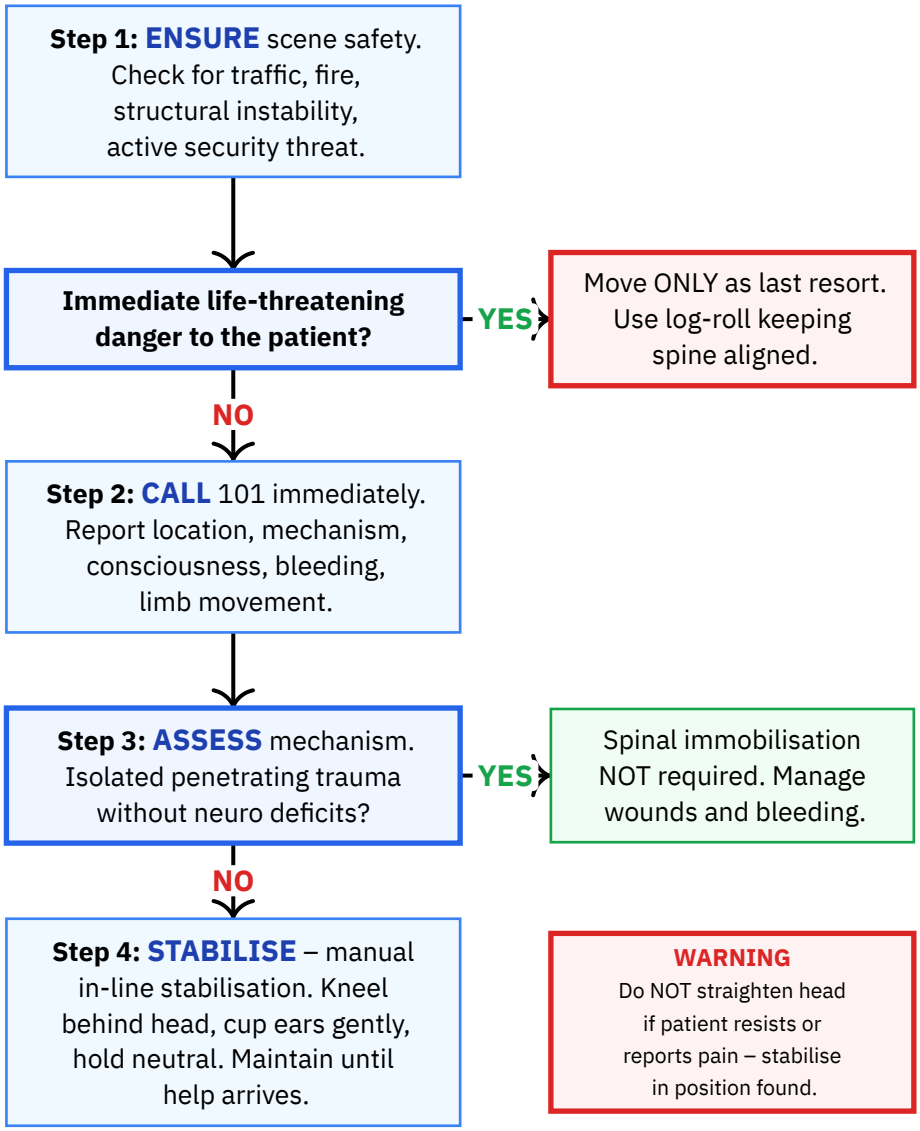
CALL 101 (MDA) — DO NOT MOVE THE PATIENT

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

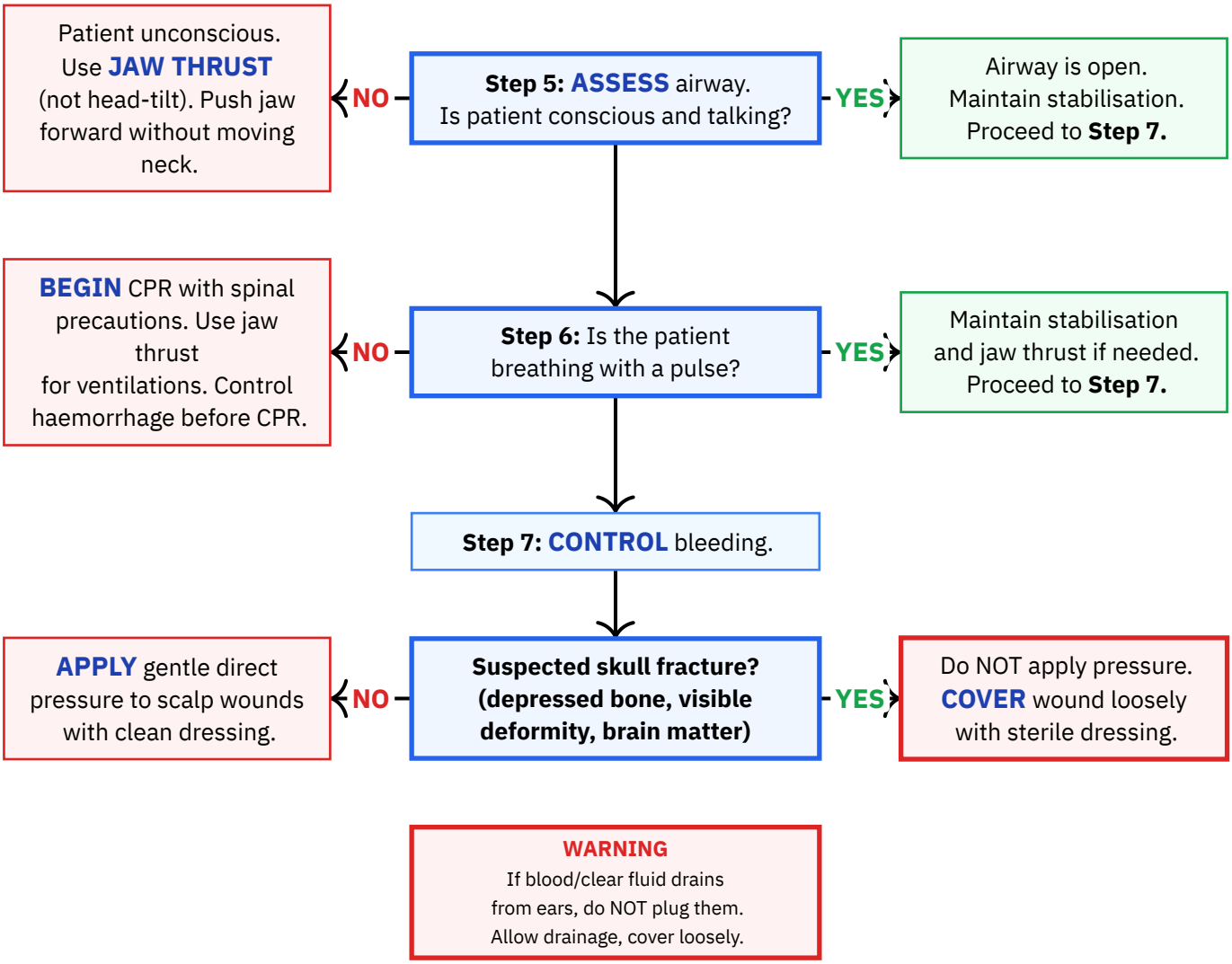
When to apply:

Adult has sustained trauma to the head, neck, or back. Common scenarios in Israel: road traffic accidents, falls from height (hiking in wadis), security incidents (blast, shrapnel), diving into shallow water, sports injuries, falls from ladders or rooftops.

Scene Safety and Initial Response



Airway, Breathing, and Bleeding Control



Neurological Assessment and Monitoring

Step 8: CHECK sensation and movement in all limbs. Compare left vs right. Check pupils.

↓

Step 9: APPLY cervical collar if available. Second person maintains stabilisation during application.

NOTE
A collar alone is NOT sufficient. Continue to support the head even with a collar applied.

↓

Step 10: MONITOR continuously.
Note changes in consciousness, vomiting, seizures, unequal pupils. Keep warm and shaded.

↓

Step 11: ASSESS concussion signs: headache, confusion, dizziness, nausea, light sensitivity. Loss of consciousness NOT required for concussion.

↓

Step 14: Helmet present and patient cannot breathe?

NO → Leave helmet in place. Open face shield for communication and airway monitoring.

YES → **REMOVE** helmet with two trained responders: one maintains C-spine, other removes helmet.

Steps 12–13: Log-Roll Technique

If unconscious patient is vomiting or must be moved due to life-threatening situation, use log-roll with minimum 3 people. Person at head commands the roll. Maintain spinal alignment throughout. Move the patient as a single unit.

Reference

DO NOT:

X Do NOT move the patient unless there is immediate danger to life.
X Do NOT twist, flex, or extend the neck.
X Do NOT attempt to sit the patient up.
X Do NOT use head-tilt chin-lift to open the airway -- use jaw thrust only.
X Do NOT remove a helmet without proper training and a second person.
X Do NOT give the patient food or drink.
X Do NOT leave the patient alone.
X Do NOT apply direct pressure to a suspected skull fracture.
X Do NOT plug the ears or nose if blood or clear fluid is draining.
X Do NOT pull the patient by the arms or legs.
X Do NOT attempt to extract a patient from a vehicle unless immediate life threat.
X Do NOT give oral fluids even if dehydrated (risk of vomiting and aspiration).
X Do NOT attempt to straighten the head if the patient resists or reports pain.

Equipment needed:

— Disposable gloves
— Clean dressings or bandages for wound care
— Sterile dressing for skull fracture coverage
— Cervical collar (if available and properly sized)
— Blanket or thermal protection
— Tourniquet (CAT) for associated limb haemorrhage
— High-visibility vest (if responding at a road accident)

Source: Magen David Adom (MDA) — MDA Head and Spinal Injury First Aid Guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2024-01-01 · **Edition:** MDA BLS Guide 2016, ALS Professional Manual 2024
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Both agree on jaw thrust, manual in-line stabilisation, and penetrating trauma exception.

Heart Attack / Chest Pain — Adult

Israel — ADULT

CALL 101 (MDA) — CALL IMMEDIATELY

MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

An adult presents with chest pain or pressure (tightness, squeezing, heaviness, or burning) lasting more than 15 minutes, possibly with radiating pain to arms, neck, jaw, back, or upper abdomen; shortness of breath; cold sweating; nausea; dizziness; or feeling of impending doom. Atypical presentations (especially in women) may include extreme fatigue, upper back pain, or symptoms resembling heartburn.

Summary: First aid protocol for suspected heart attack in adults, following MDA guidelines with Israel-specific considerations.

Personal reference only — not medical advice

Initial Response

Step 1: CALL 101 (MDA) immediately. State you suspect a heart attack. Stay on line. Also call 1221 (Hatzalah).

↓

Step 2: STOP all physical activity. Patient must rest completely. No walking.

WARNING
Any physical activity, even minor, can worsen heart muscle damage.

↓

Step 3: POSITION patient in half-sitting position (semi-reclined), back supported, knees slightly bent. Loosen tight clothing.

↓

Step 4: ASK patient about aspirin allergy.

↓

Is the patient allergic to aspirin?

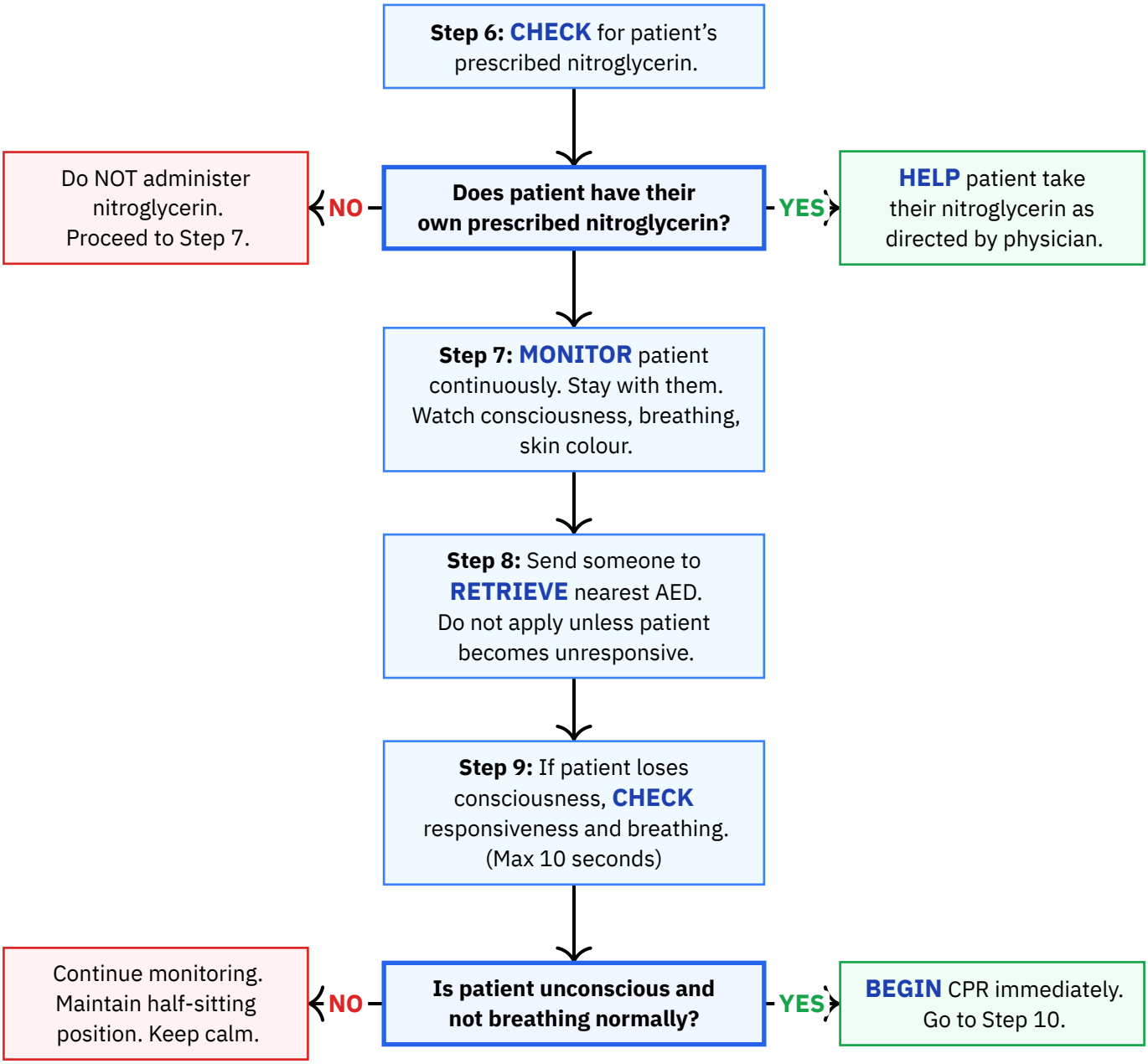
- **NO** → Proceed to Step 5: **GIVE** aspirin.
- **YES** → Do NOT give aspirin. Inform MDA team about the allergy. Skip to Step 6.

↓

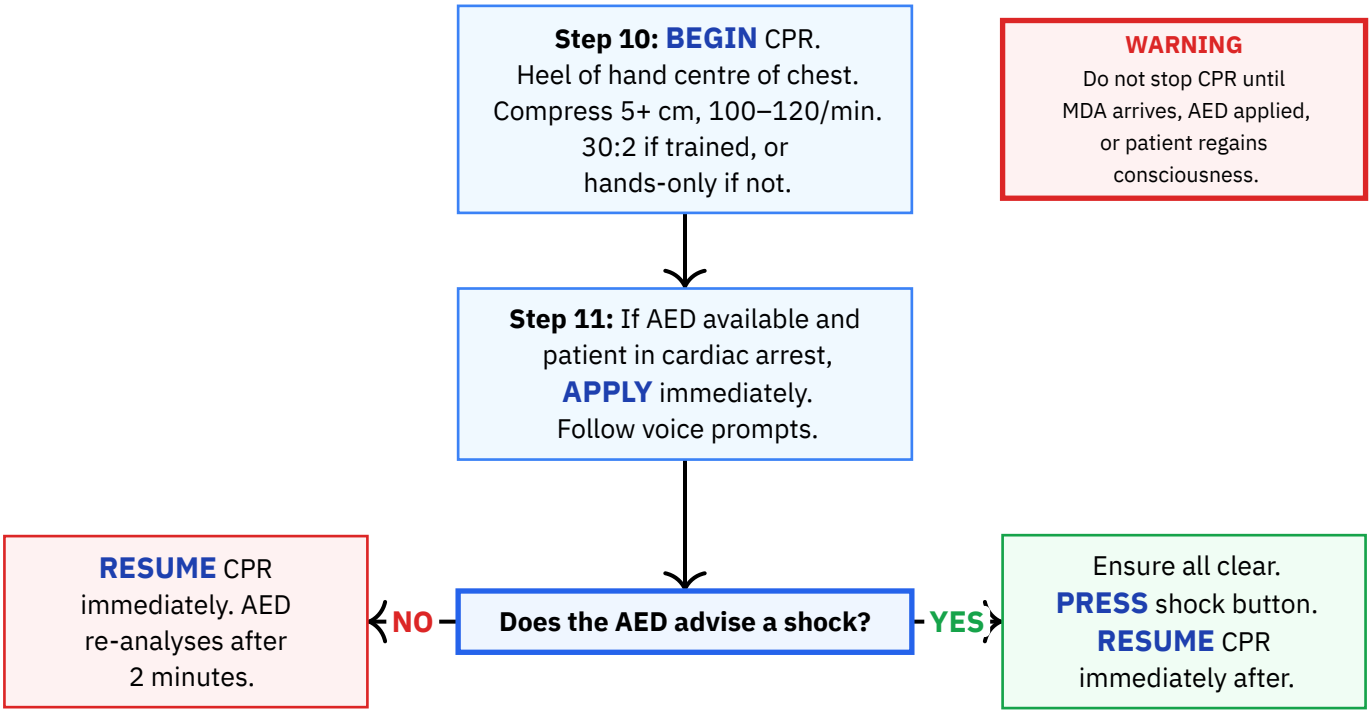
Step 5: GIVE 100–300 mg aspirin. Patient must CHEW the tablet, not swallow whole.
Israel: 100 mg Micro-Aspirin, give 1–3 tablets.

WARNING
Paracetamol (Acamol) is NOT a substitute. Only aspirin provides the antiplatelet effect.

Monitoring and Escalation



CPR and AED (if cardiac arrest)



Reference

DO NOT:

X Do not allow the patient to walk, exert themselves, or dismiss their symptoms.
X Do not give the patient food or beverages.
X Do not leave the patient alone (except briefly to call for help).
X Do not give aspirin if the patient is allergic to it.
X Do not give nitroglycerin that was not prescribed to this specific patient.
X Do not wait to see if symptoms resolve on their own -- every minute of delay increases irreversible heart muscle damage.
X Do not drive the patient to hospital yourself -- call 101 and wait for the ambulance. MDA has cardiac monitoring equipment and can begin treatment en route.
X Do not substitute paracetamol (Acamol) for aspirin -- only aspirin provides the needed antiplatelet effect.
X Do not give aspirin as a whole tablet -- it must be chewed for rapid absorption.

Equipment needed:

— Aspirin tablets (100 mg, 1-3 tablets for 100-300 mg total)
— AED (Automated External Defibrillator) -- if available
— Phone (to call 101 MDA and optionally 1221 United Hatzalah)
— Nitroglycerin (only if prescribed to the patient)

Source: Magen David Adom (MDA) — "MDA Heart Attack Protocol – synthesised from MDA public heart attack page, MDA BLS guide (2016), MDA ALS protocol (2024), and corroborating Israeli medical sources"
URL: https://www.mdais.org/101/mi-signs
Publication date: 2024-04-01 · **Edition:** MDA ALS Protocol 2024 / BLS Guide 2016
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah reference material. Aspirin dosage of 100-300 mg is consistent across all Israeli sources and aligns with international guidelines.

Heatstroke and Heat Exhaustion — Adult

Israel — ADULT

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

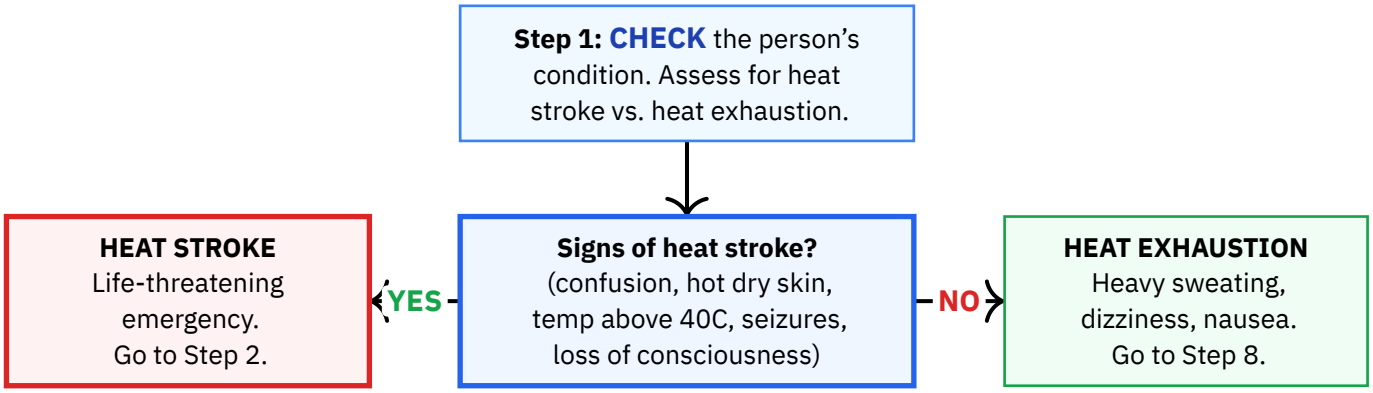
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Person showing signs of heat-related illness after prolonged heat exposure or physical exertion in hot conditions. Signs: heavy sweating, dizziness, nausea, confusion, hot dry skin, or loss of consciousness.

Summary: Assessment and first aid for heat exhaustion and heat stroke in adults, including recognition, cooling, and escalation to emergency services.

Assessment: Heat Stroke vs. Heat Exhaustion



Heat Stroke Pathway (EMERGENCY)

Step 2: CALL 101 (MDA) immediately. Heat stroke is a life-threatening emergency.

WARNING
Do not delay calling 101 for suspected heat stroke.

↓

Step 3: Move person to shade or air-conditioned area.

↓

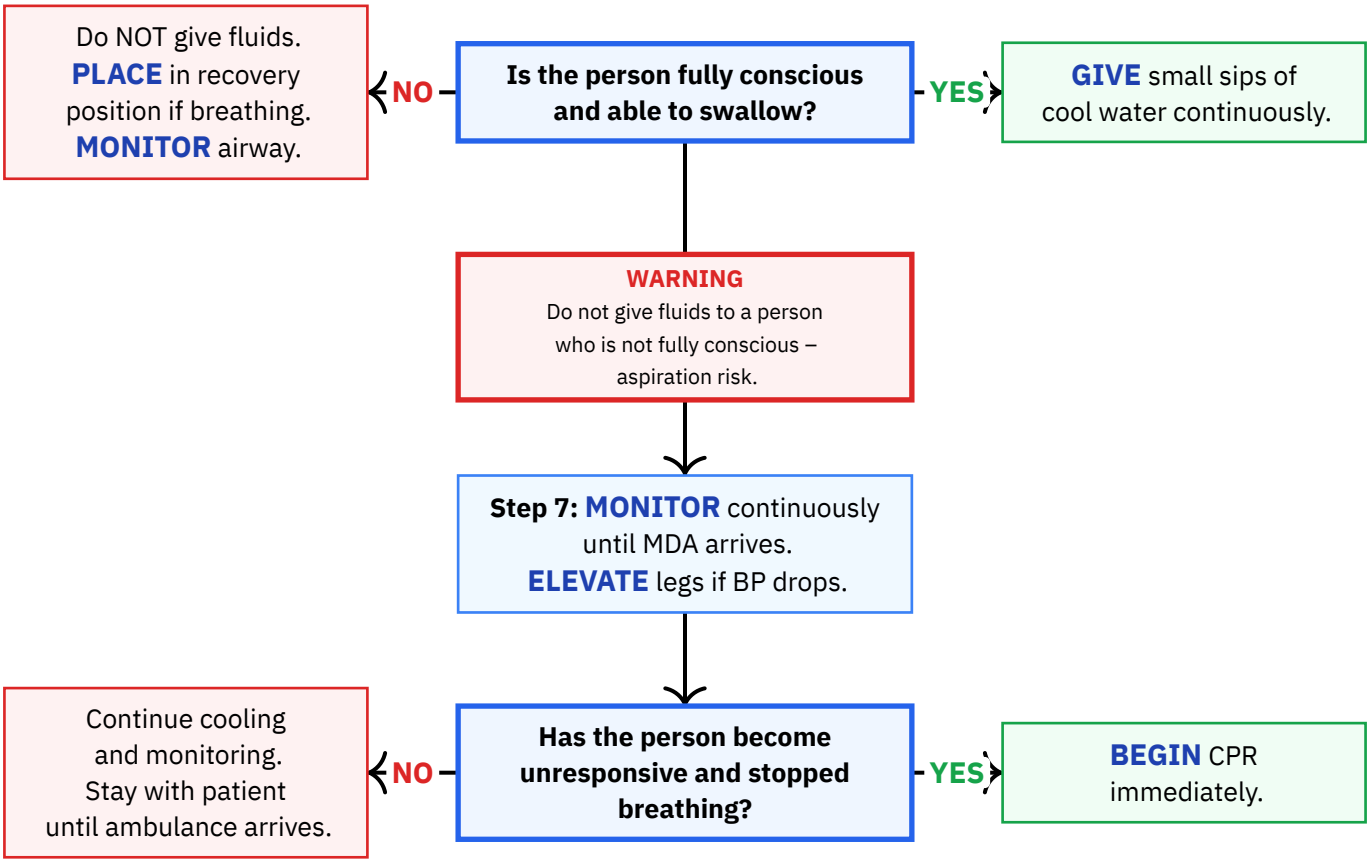
Step 4: REMOVE unnecessary clothing.

↓

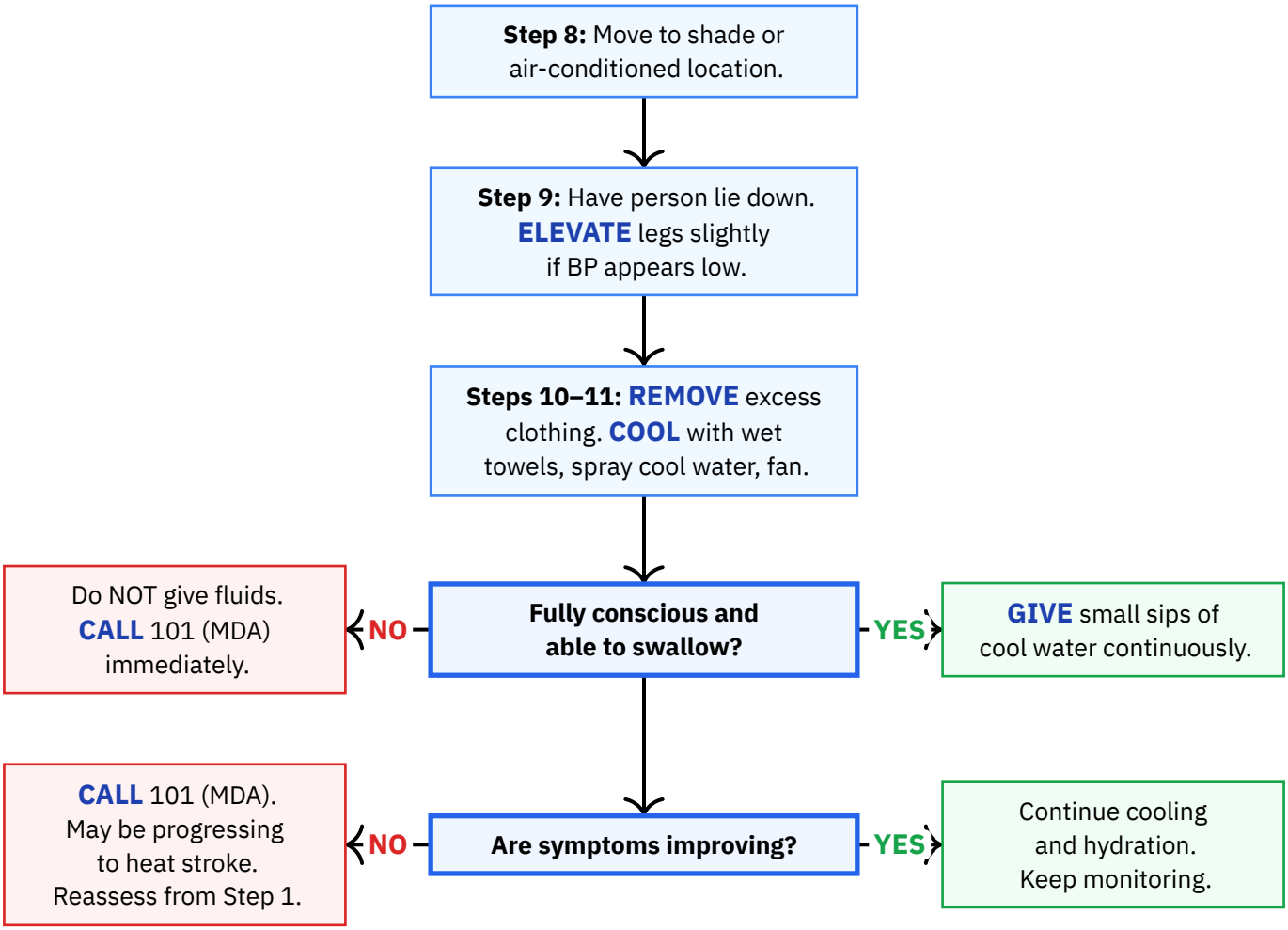
Step 5: COOL aggressively: Wet towels on neck, armpits, groin. Spray/pour cool water. Fan vigorously. Cold packs to neck, armpits, groin.

Target areas where major blood vessels are near the skin for maximum cooling.

Heat Stroke: Fluids and Monitoring



Heat Exhaustion Pathway



Reference

DO NOT:

X Do not administer fever-reducing medications (paracetamol, ibuprofen) -- antipyretics do not work for environmental hyperthermia and may cause harm.
X Do not give fluids to a person who is not fully conscious or who cannot swallow safely.
X Do not delay calling 101 for suspected heat stroke.
X Do not ignore early signs of heat exhaustion -- it can progress to life-threatening heat stroke.
X Do not leave the person unattended while waiting for emergency services.

Equipment needed:

— Wet towels or cloths
— Cool/cold water (for spraying, pouring, and drinking)
— Cold packs or ice packs (if available)
— Fan or other means of air circulation (if available)

Source: Magen David Adom (MDA) — "MDA First Aid Guidelines – Heat-Related Illness"
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. MDA allows small sips of cool water if patient is fully conscious (Hatzalah recommends withholding all fluids in confirmed heat stroke). MDA guidance followed here.

Hypothermia / Cold Exposure — Adult

Israel — ADULT

CALL 101 (MDA) — IMMEDIATELY UPON RECOGNISING HYPOTHERMIA

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Person shows signs of hypothermia: shivering, confusion, pale/cold skin, fatigue, slurred speech, muscle rigidity, or loss of consciousness after cold exposure. Body temperature below 35 degrees C (95 degrees F).

Summary: First aid for hypothermia through gradual rewarming, starting with the torso. Rewarming must be gradual – rapid rewarming can cause dangerous cardiac arrhythmias.

Immediate Response and Rewarming

Step 1: CALL MDA at 101 immediately. Request ambulance.

↓

Step 2: MOVE person to warm, dry shelter. Protect from wind, rain, cold.

↓

Step 3: REMOVE wet clothing promptly. Replace with dry layers.

↓

Step 4: COVER with blankets and layers, including the head. Significant heat lost through head.

↓

Step 5: WARM the room to approx. 22 degrees C (72 degrees F) if indoors.

↓

Step 6: Is the person conscious and able to swallow?

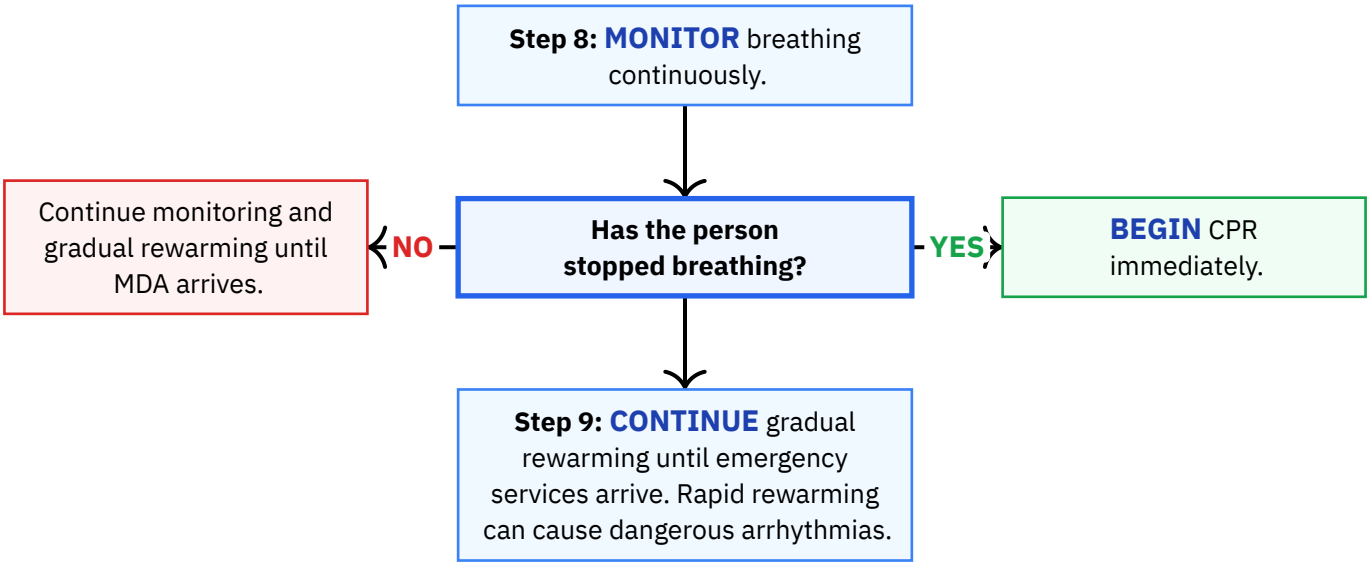
- **NO** → Do NOT give any fluids. Monitor breathing closely. Prepare for CPR.
- **YES** → **PROVIDE** warm (NOT hot) beverages. Avoid alcohol and caffeine.

↓

Step 7: APPLY heat packs to chest, abdomen, back ONLY. Use cloth barrier. Warm torso FIRST, not extremities.

WARNING
Warming extremities first can cause cold blood to rush to the heart, triggering cardiac arrest (afterdrop phenomenon).

Continued Monitoring



DO NOT:

X Do NOT rub extremities -- friction can cause tissue damage and cardiac complications.
X Do NOT use direct intense heat (hot water baths, heating pads directly on skin) -- rapid rewarming can cause dangerous cardiac arrhythmias.
X Do NOT give alcohol -- it dilates blood vessels and accelerates heat loss.
X Do NOT give caffeine -- it worsens hypothermia.
X Do NOT warm extremities first -- cold blood rushing to the heart can trigger cardiac arrest (afterdrop).
X Do NOT apply vigorous external rewarming.

Equipment needed:

— Blankets or thermal blankets (space blankets)
— Dry clothing
— Heat packs with cloth barrier
— Warm (not hot) beverages (for conscious patients only)

Source: Magen David Adom (MDA) — MDA Emergency Medicine Manual 2024
URL: https://www.mdais.org/101/first-aid
Publication date: 2024-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16

Poisoning — Adult

Israel — ADULT

CALL 101 (MDA) + Poison Center 04-7771900

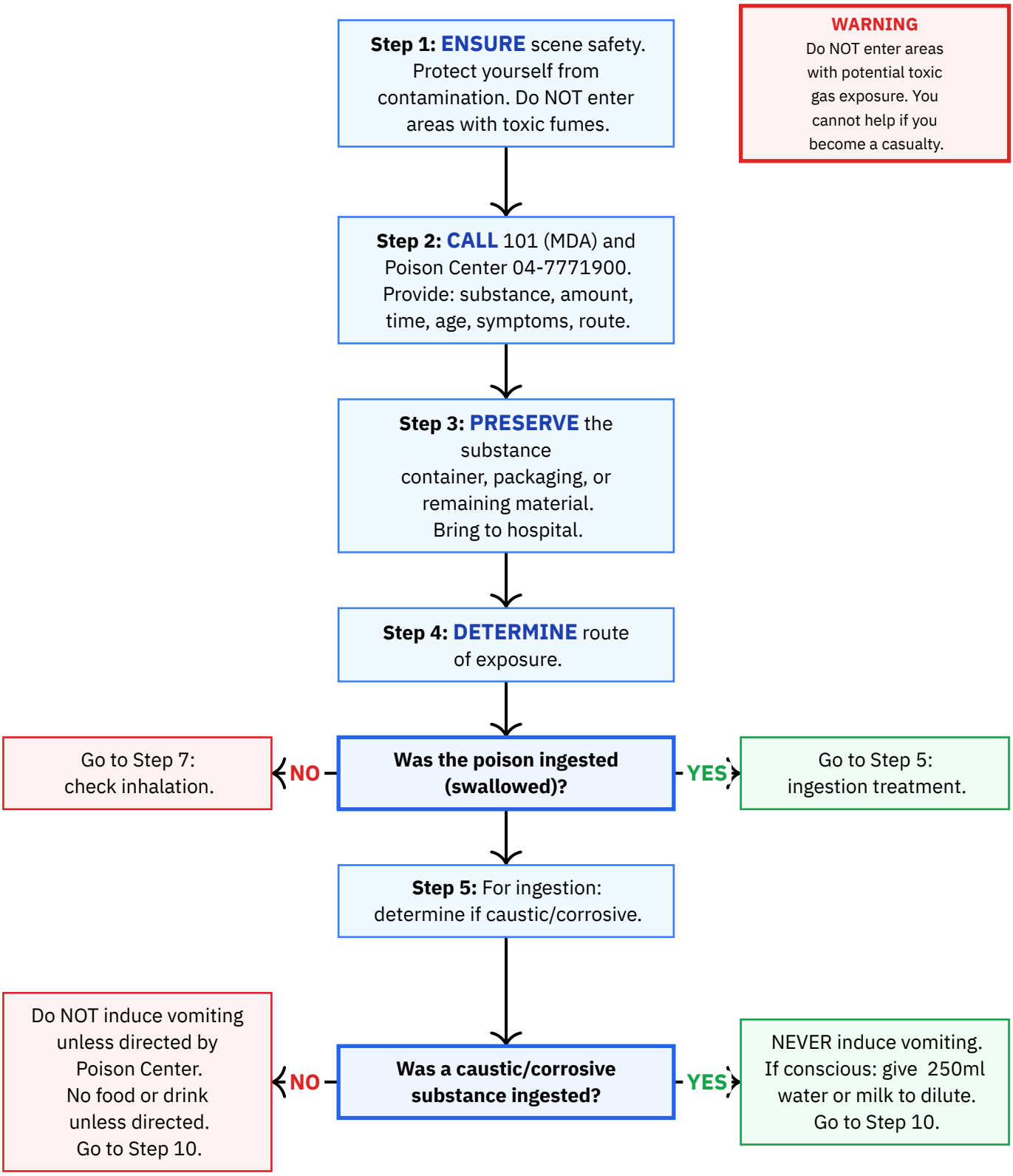
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

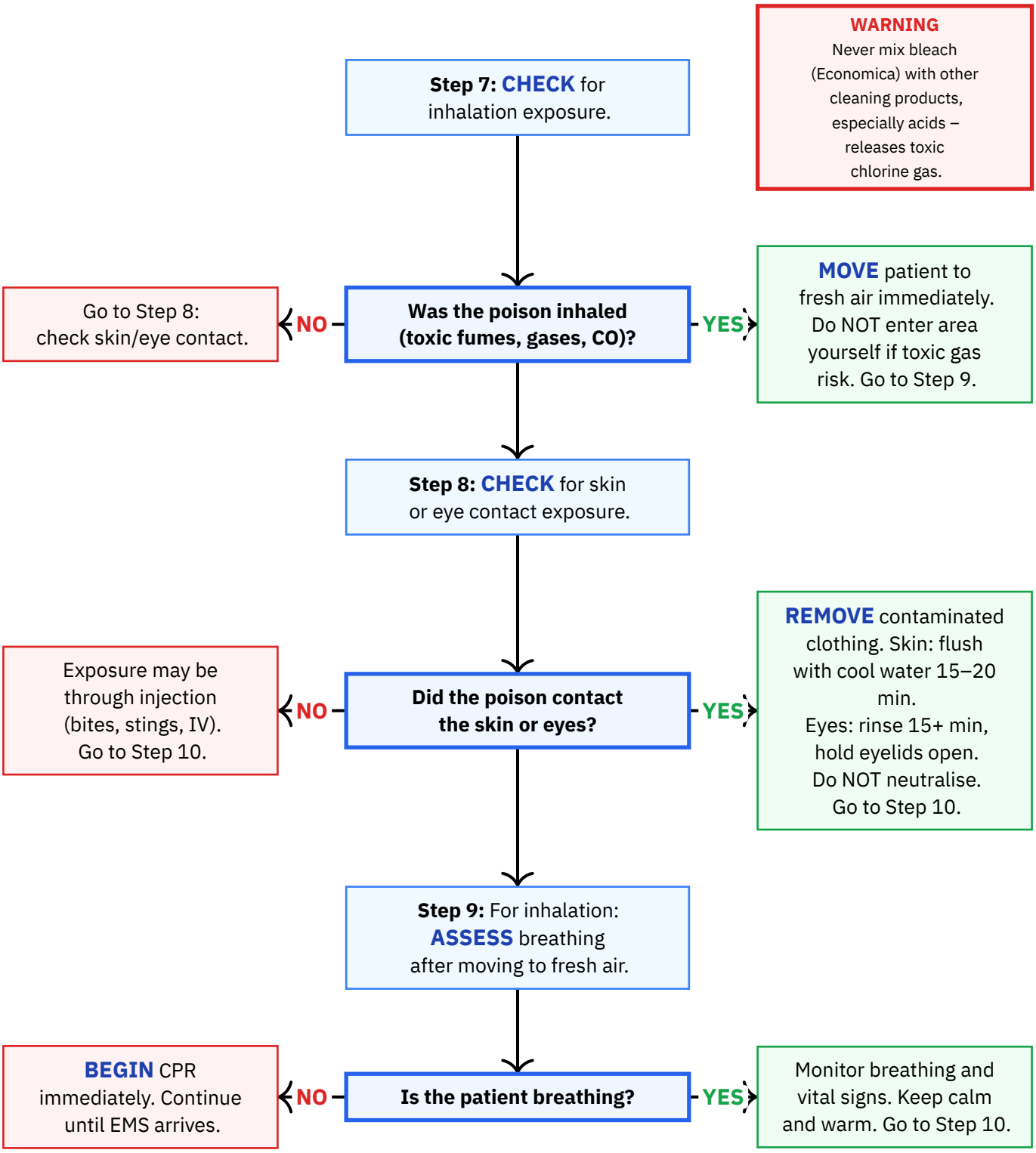
Suspected poisoning through any route: ingestion (medications, cleaning products, chemicals, plants), inhalation (carbon monoxide, chlorine gas, chemical fumes), skin or eye contact with hazardous chemicals, or injection (venomous bites/stings, IV drug use). Signs include nausea, vomiting, confusion, drowsiness, breathing difficulty, seizures, chemical burns, or loss of consciousness.

Summary: Scene safety, call for help, identify route of exposure, provide route-specific first aid, preserve the substance, and monitor until emergency services arrive.

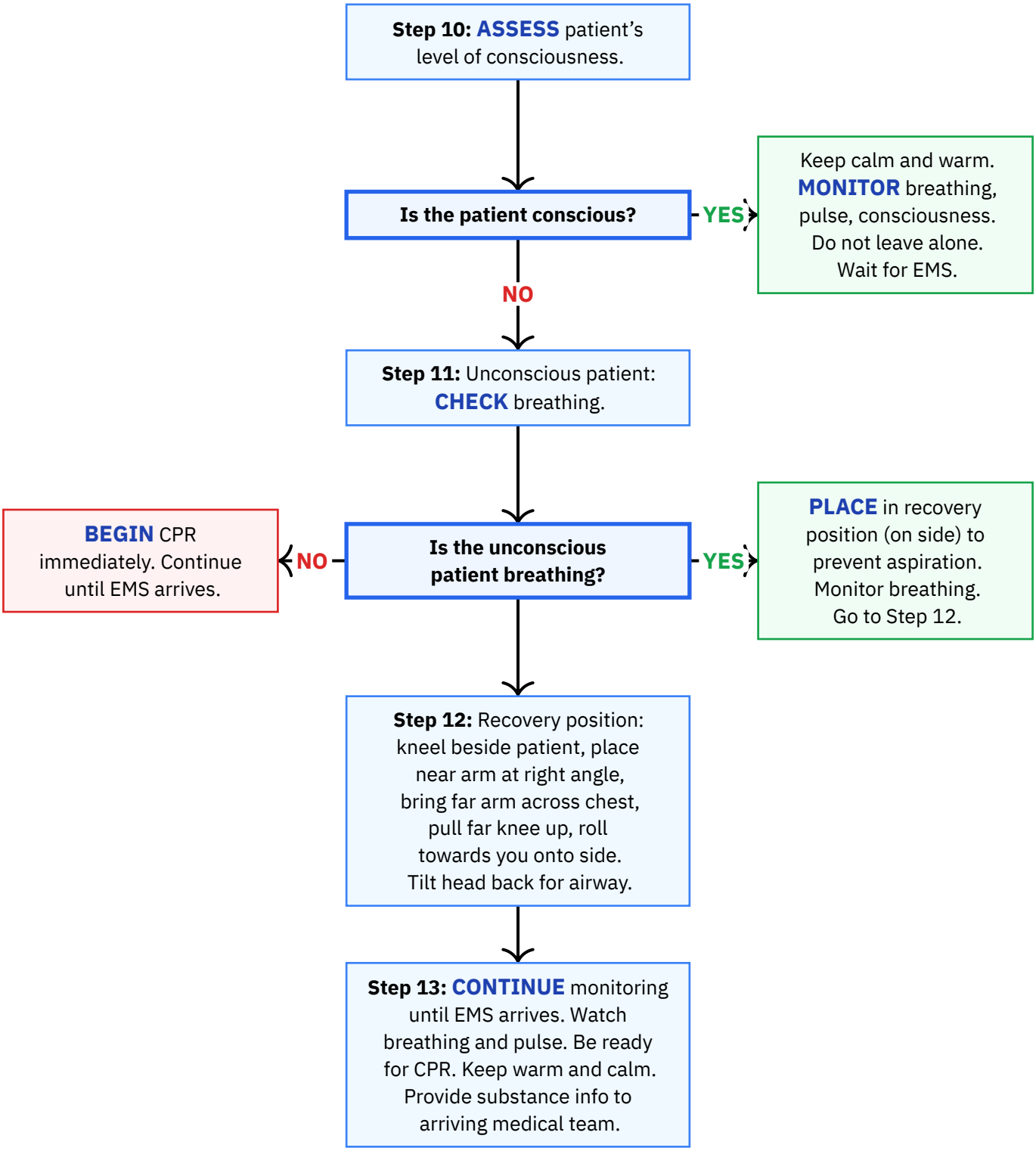
Scene Safety and Ingestion Path



Inhalation and Contact Exposure



Consciousness Assessment and Monitoring



Step 6 Note — Activated Charcoal: Do NOT give activated charcoal unless directed by medical professionals. It is effective only within 1–2 hours and only for certain substances. This is a medical-level intervention, not a first aid action.

Reference

DO NOT:

X Do NOT induce vomiting unless specifically directed by the Poison Information Center (04-7771900) or a physician.
X Do NOT give food or drink unless directed by medical professionals (exception: water or milk for conscious patients who ingested caustic substances).
X Do NOT attempt to neutralise the poison (e.g., giving acid for base ingestion or vice versa).
X Do NOT use home remedies (salt water, mustard, raw eggs, etc.).
X Do NOT give activated charcoal without medical direction.
X Do NOT enter a confined space with potential toxic fumes without proper protection.
X Do NOT delay calling for help -- even if symptoms appear mild, they can deteriorate rapidly.
X Do NOT administer antidotes -- this is a hospital-level intervention only.
X Do NOT leave the patient alone.

Equipment needed:

— Gloves (to protect against contamination)
— Running water source (for skin or eye decontamination)

Source: Magen David Adom (MDA) — "Poisoning First Aid Guidance – MDA 101 Public First Aid Guidance Series; corroborated by Rambam National Poison Information Center guidance"
URL: https://www.mdais.org/101/poisoningn
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Israel Poison Information Center: 04-7771900 (Rambam Health Care Campus, Haifa).

Recovery Position — Adult

Israel — ADULT

CALL 101 (MDA) — UNCONSCIOUS PERSON ALWAYS NEEDS MEDICAL EVALUATION

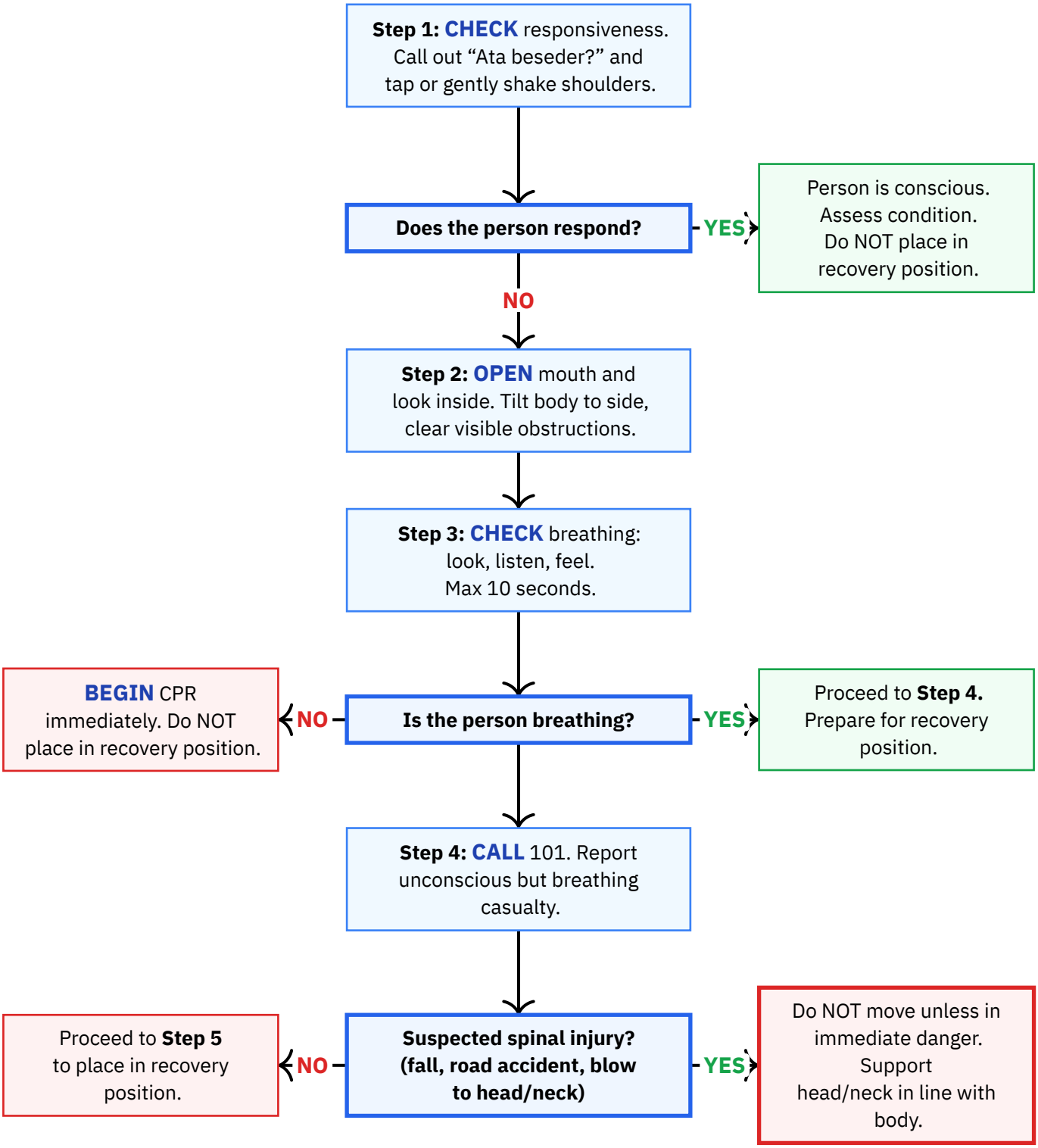
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Person is unconscious (unresponsive) but breathing, with no suspected spinal injury and no need for CPR.

Summary: Place an unconscious but breathing person in the recovery position to maintain an open airway and prevent aspiration.

Assessment Phase



Recovery Position Technique

Step 5: KNEEL beside the person lying on their back.

↓

Step 6: EXTEND the nearest arm at right angle to body, palm facing up.

↓

Step 7: FOLD far arm across chest. Back of hand rests against nearest cheek. Hold.

↓

Step 8: BEND the far knee up to a right angle.

↓

Step 9: ROLL toward you by pulling on the bent knee. They roll smoothly onto side.

↓

Step 10: ADJUST top leg so both hip and knee are bent at right angles for stability.

↓

Step 11: TILT head back slightly to keep airway open. Adjust hand under cheek.

↓

Step 12: MONITOR continuously until MDA arrives. Check breathing.
Keep warm with blanket.

Stopped breathing?
YES: Roll onto back, begin CPR immediately.
NO: Continue monitoring. Turn to opposite side after 30 minutes.

DO NOT:

X Do NOT place in recovery position if the person is not breathing -- begin CPR instead.
X Do NOT move a person with suspected spinal injury unless they are in immediate danger.
X Do NOT leave the person unattended -- monitor breathing continuously.
X Do NOT place the person on their back if they are unconscious and breathing -- risk of airway obstruction and aspiration.
X Do NOT forget to call 101 -- an unconscious person always requires professional medical evaluation.

Equipment needed:

— Blanket (to keep the person warm)
— Phone to call MDA at 101

Source: Magen David Adom (MDA) — MDA Emergency Medicine Manual 2024; MDA BLS Guide 2016
URL: https://www.mdais.org/101/first-aid
Publication date: 2024-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Hebrew terminology: Tanukhat Hit'oshshut (recovery position) or Tanukhat Tzad (lateral position).

Scorpion Stings — Adult

Israel — ADULT

CALL 101 (MDA) — ALL SCORPION STINGS REQUIRE HOSPITAL EVALUATION

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Person has been stung or suspected stung by a scorpion. Signs include: immediate intense pain at sting site, swelling and redness, numbness or tingling. May progress to systemic symptoms: restlessness, altered consciousness, seizures, difficulty breathing, excessive sweating, tachycardia, nausea, or vomiting. Local symptom severity does NOT reliably predict systemic danger.

Summary: Call 101 immediately, keep the victim calm and still, immobilise the affected limb, remove constrictive items, monitor for systemic reaction, and transport to hospital for mandatory observation (minimum 6 hours).

Immediate Response

Step 1: CALL MDA at 101
Request ambulance. All scorpion stings require hospital evaluation. Follow dispatch instructions.

↓

Step 2: KEEP victim calm and still. Have them lie down. Minimise all movement – movement accelerates venom spread.

↓

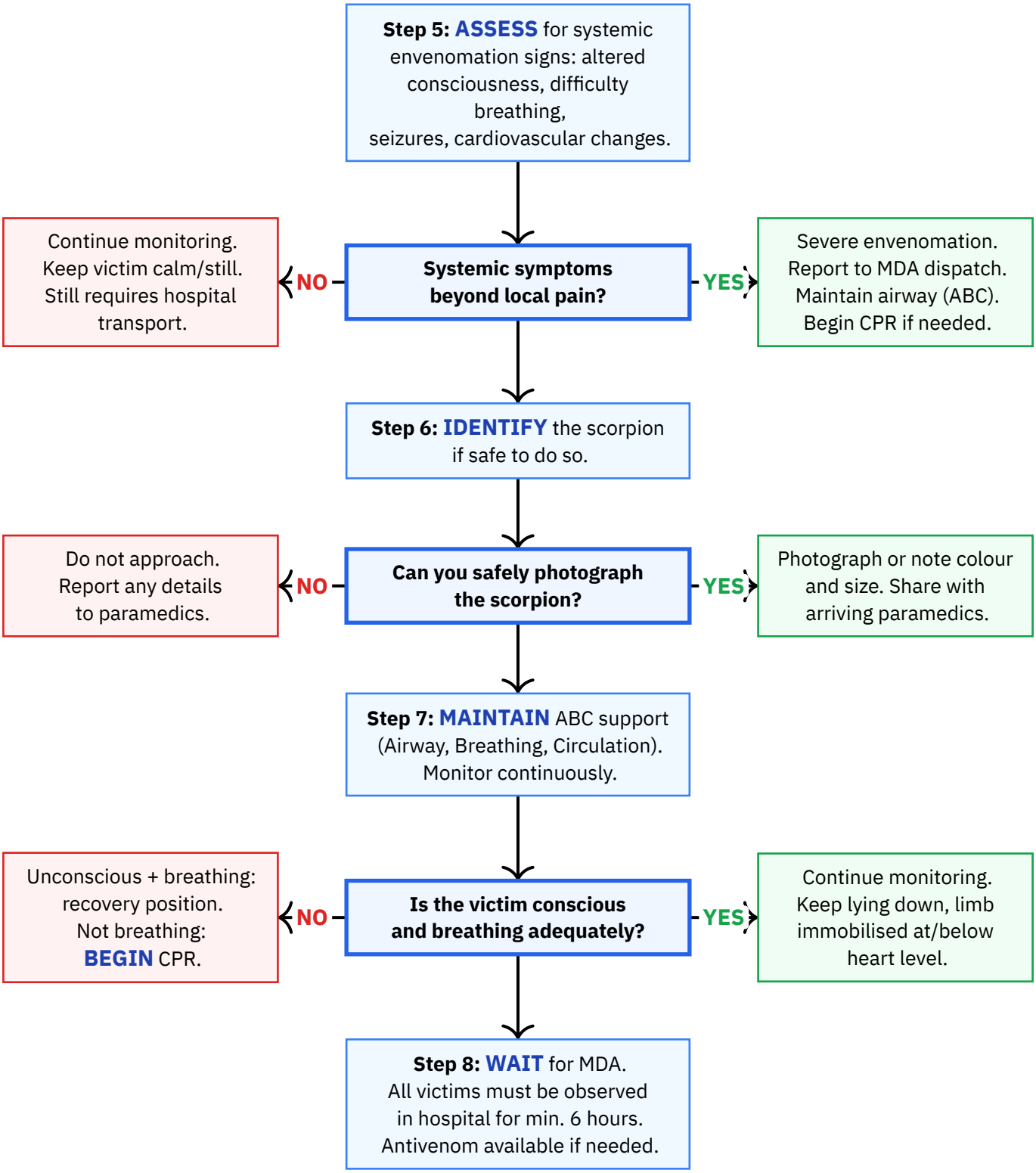
Step 3: IMMOBILISE the affected limb. Keep at or below heart level to slow venom distribution.

↓

Step 4: REMOVE jewellery, watches, rings, tight clothing near the sting site.

WARNING
Swelling can progress rapidly. Remove all constrictive items as early as possible.

Assessment and Monitoring



Reference

DO NOT:

X Do NOT apply a tourniquet -- it traps venom locally and causes tissue damage; it does not prevent systemic spread.
X Do NOT suck the venom out of the wound -- this is ineffective and potentially harmful.
X Do NOT cut the sting area -- this increases infection risk and does not remove venom.
X Do NOT apply ice or cool the sting area -- the Israeli Ministry of Health (2026) explicitly advises against cooling.
X Do NOT give the victim food or drink -- this risks venom dispersal and may complicate hospital treatment.
X Do NOT allow the victim to walk or move the stung limb -- movement accelerates venom circulation.

Equipment needed:

— Phone to call 101 (MDA) and optionally 04-7771900 (Poison Center)
— Phone or camera to photograph the scorpion if safe to do so

Source: Magen David Adom (MDA) — First Aid for Scorpion Stings
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Israel Poison Information Center 24/7 hotline: 04-7771900. Primary dangerous species: Leiurus quinquestriatus (deathstalker/yellow scorpion). The common myth that black scorpions are not dangerous is FALSE.

Seizures (Epileptic Seizure / Convulsions) — Adult

Israel — ADULT

CALL 101 (MDA) IF SEIZURE LASTS OVER 5 MINUTES

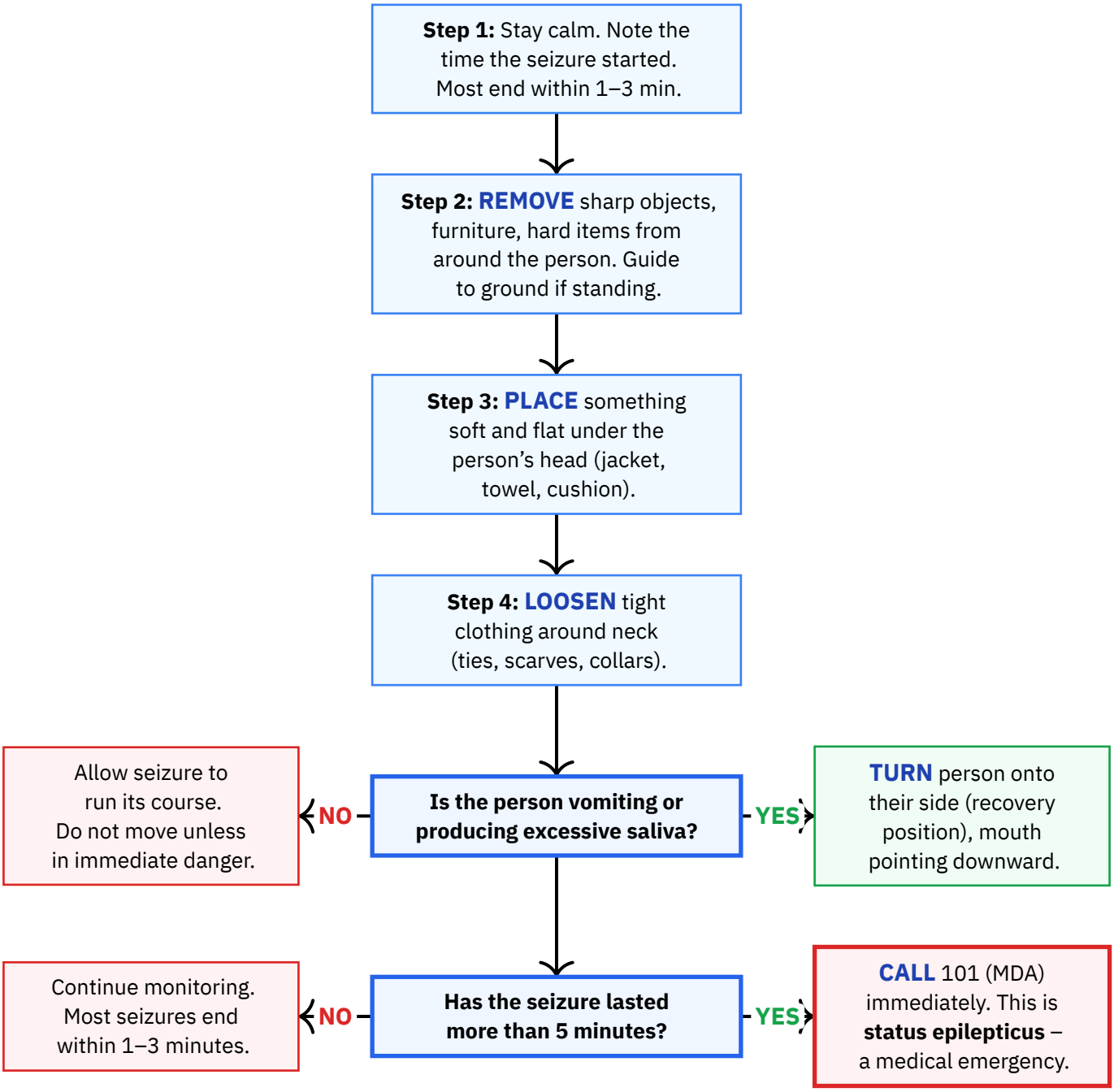
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

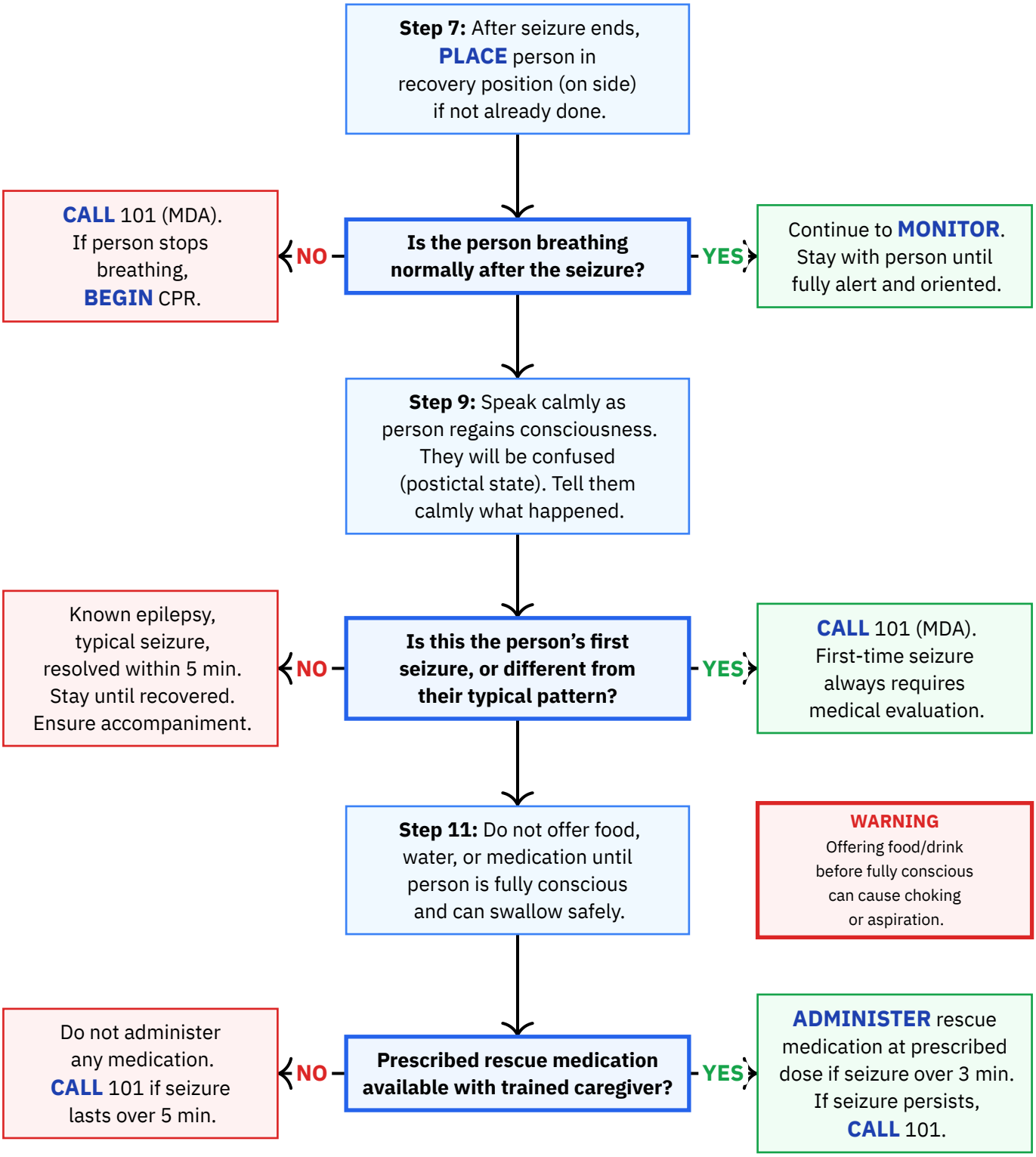
Person is having a tonic-clonic (grand mal) seizure: sudden loss of consciousness, body stiffening, rhythmic jerking of limbs, eyes rolling upward, jaw clenching, excessive salivation/foaming, possible loss of bladder/bowel control, bluish skin around lips.

Summary: Protect the person from injury during the seizure, time the seizure, place in recovery position after it ends, and call 101 if the seizure lasts more than 5 minutes or meets other emergency criteria.

During the Seizure



After the Seizure



Personal reference only — not medical advice

Reference

Call 101 (MDA) if:

- Seizure lasts more than 5 minutes
- A second seizure follows without regaining consciousness
- This is a first-time seizure
- Person has difficulty breathing after the seizure
- Person is pregnant or has diabetes
- Injury occurs during the seizure

DO NOT:

X DO NOT restrain the person or try to hold them down. You cannot stop a seizure by force. Restraining can cause fractures, dislocations, or soft tissue injury.
X DO NOT put anything in the person's mouth -- no spoons, wallets, sticks, fingers, or any object. The person cannot swallow their tongue. Objects risk breaking teeth or causing jaw injuries.
X DO NOT attempt mouth-to-mouth during the seizure. People usually resume breathing on their own once the seizure ends.
X DO NOT pour water on the person or attempt to 'wake them up'.
X DO NOT offer food, drinks, or medication by mouth during or immediately after the seizure until the person is fully alert and able to swallow safely.
X DO NOT crowd around the person -- give them space and privacy. Ask bystanders to step back.
X DO NOT move the person unless they are in immediate danger (e.g., in the middle of a road, near a ledge, next to fire or water).

Equipment needed:

— Soft padding for under the head (jacket, towel, cushion, blanket)
— Watch or phone to time the seizure duration
— Prescribed rescue medication if available (Buccolam, Stesolid, or Rivotril -- prescription only)

Source: Magen David Adom (MDA) — "Seizure First Aid Protocol (synthesised from MDA course materials, Israeli Epilepsy Association, and corroborating Israeli medical sources)"
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. No significant divergence between the two organisations for seizure management. Status epilepticus defined as seizure lasting over 5 minutes. Israeli Epilepsy Association support line: 02-5000283.

Severe Bleeding (Hemorrhage Control) — Adult

Israel — ADULT

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Severe or uncontrolled external bleeding from any cause including trauma, amputation, crush injury, or penetrating injury. Signs: rapid spurting or steady flow of blood, pooling blood, blood-soaked clothing, or a severed/partially severed limb.

Summary: Control severe external bleeding through escalating interventions: direct pressure, Israeli bandage, wound packing, and tourniquet.

Initial Assessment

Step 1: CHECK scene safety. Wear disposable gloves if available.

↓

Step 2: CALL 101 (MDA) immediately. Provide location, number of casualties, nature of injuries.

↓

Is there an embedded or impaled object in the wound?

- **YES** → Do NOT remove it. **IMMOBILISE** with padding around it. **APPLY** pressure around, not on it. Go to Step 11.
- **NO** ↓

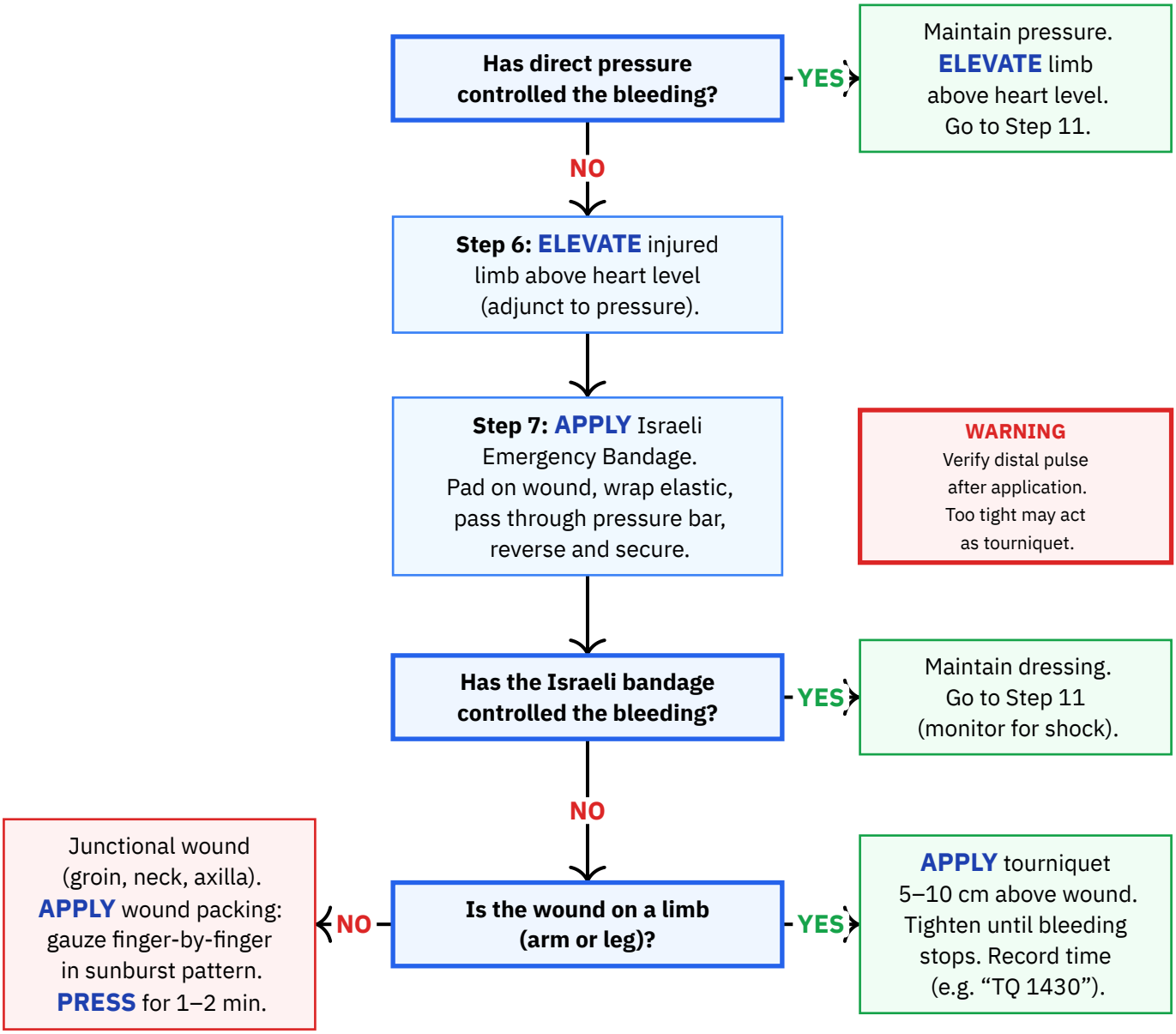
Step 4: APPLY strong, direct pressure to the wound with gloved hand or gauze. Maintain continuous pressure.

↓

Step 5: If dressing soaks through, add layers on top. Never remove blood-soaked dressing.

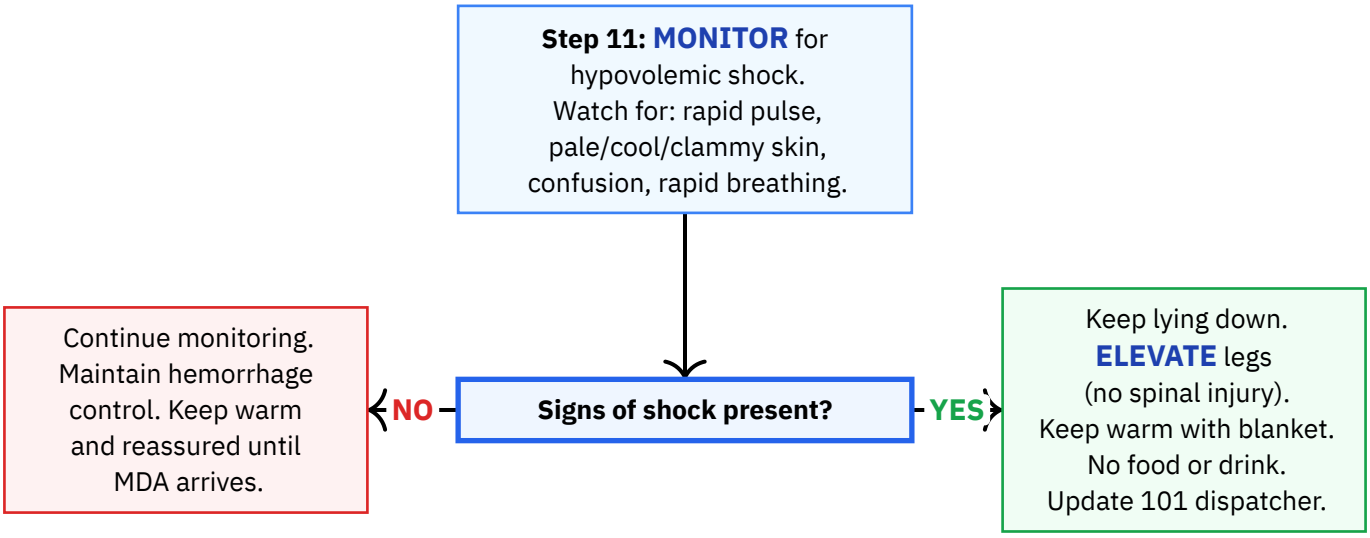
WARNING
Never lift or remove a blood-soaked dressing to check the wound.

Hemorrhage Control Escalation



TOURNIQUET WARNING: A properly applied tourniquet completely blocks blood flow and will cause significant pain – this is expected. Do NOT remove or loosen once applied unless instructed by a paramedic or physician.

Shock Monitoring



DO NOT:

X Do not remove an embedded or impaled object from the wound.
X Do not lift or remove a blood-soaked dressing to check the wound -- add layers on top.
X Do not remove or loosen a tourniquet once applied unless instructed by a paramedic or physician.
X Do not give food or drink to a casualty with severe bleeding or suspected shock.
X Do not use wound packing for penetrating stab wounds.
X Do not enlarge wound openings to facilitate wound packing.
X Do not assume a rapid pulse is caused only by anxiety or pain -- suspect significant blood loss.
X Do not delay calling 101 for severe or uncontrolled bleeding.
X Do not apply a tourniquet directly over a joint -- place it 5--10 cm above the wound on the limb.

Equipment needed:

— Disposable gloves
— Sterile gauze or clean cloth
— Israeli Emergency Bandage (pressure dressing)
— Tourniquet (CAT or equivalent)
— Wound packing material (hemostatic gauze preferred)
— Triangular bandage (supplementary)
— Blanket or coat (for shock management)

Source: Magen David Adom (MDA) — "How to Stop Bleeding (MDA 101 Public Guidance)"
URL: https://www.mdais.org/101/bleeding
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Escalation order includes wound packing (sunburst technique) from Hatzalah supplementary material.

Shock (Helem) — Adult

Israel — ADULT

CALL 101 (MDA) — CALL IMMEDIATELY IF IN DOUBT

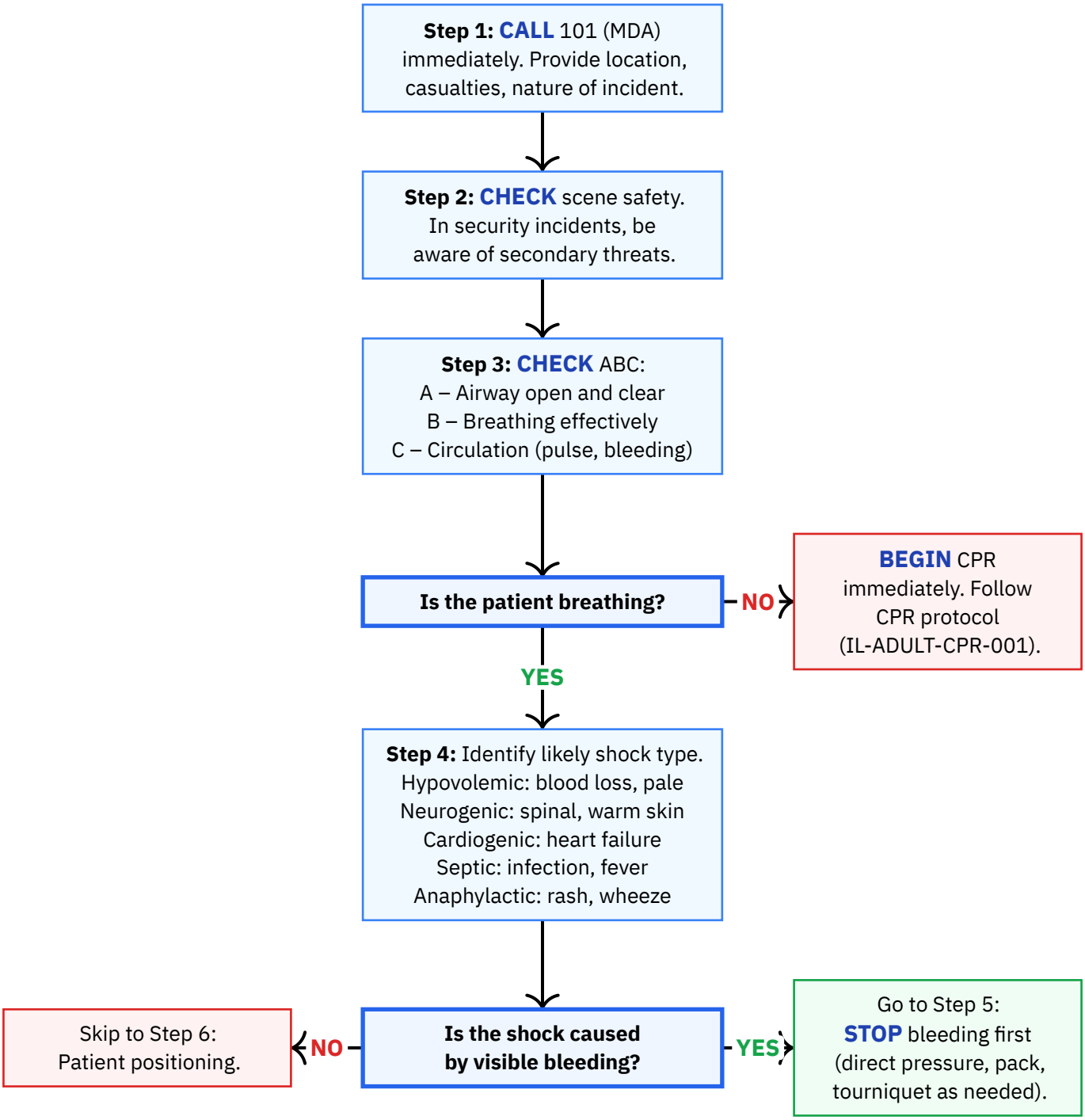
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

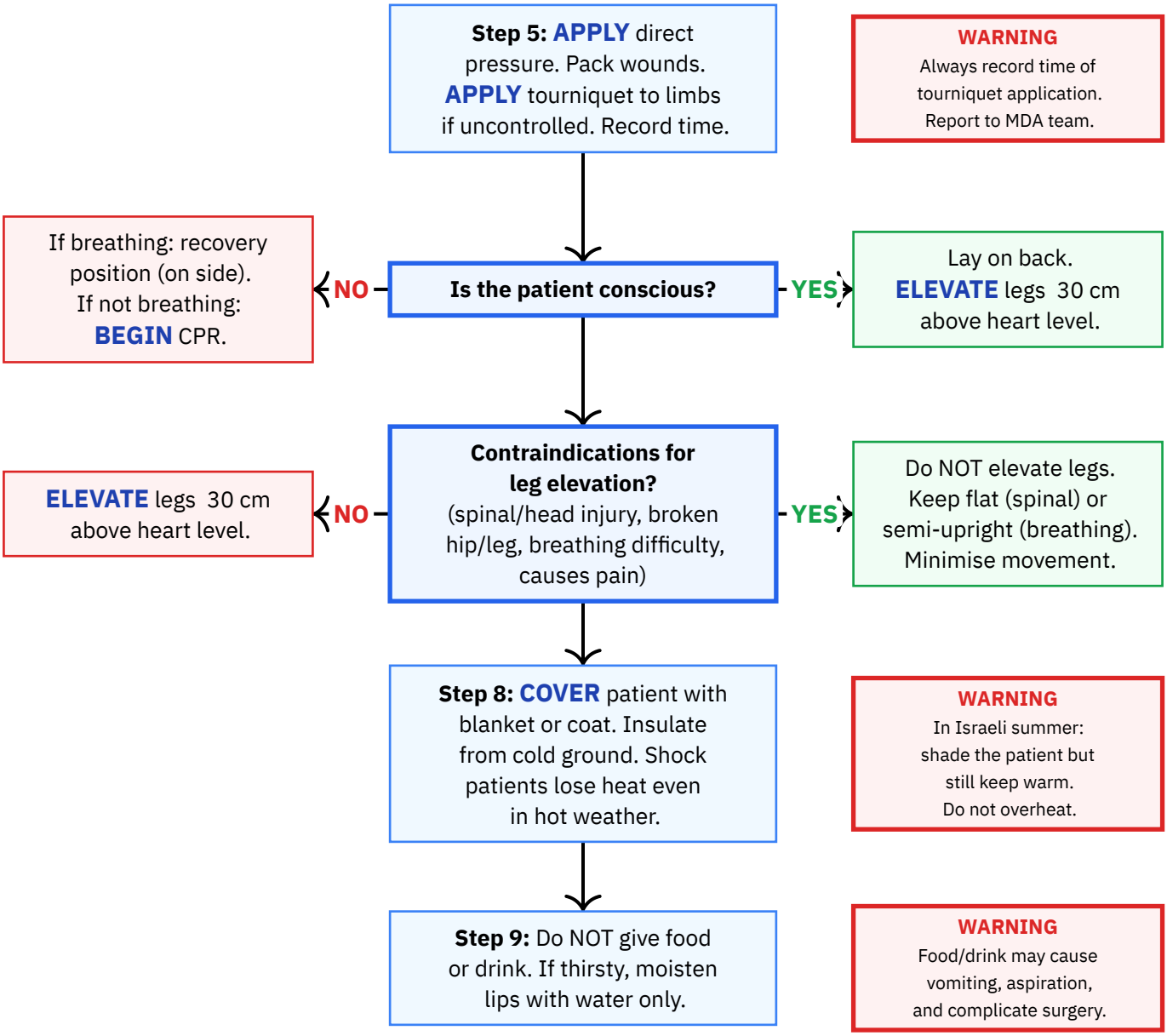
Patient presents with signs of shock: rapid weak pulse, pale/cold/clammy skin, cold perspiration, rapid shallow breathing, decreased consciousness or confusion, falling blood pressure, weakness, dizziness, fainting, nausea, vomiting, or thirst. May result from blood loss, spinal injury, heart failure, severe infection, or severe allergic reaction.

Summary: Identify shock type, call 101, perform ABC assessment, control bleeding, elevate legs 30 cm, maintain body temperature, give nothing by mouth, and monitor until MDA arrives.

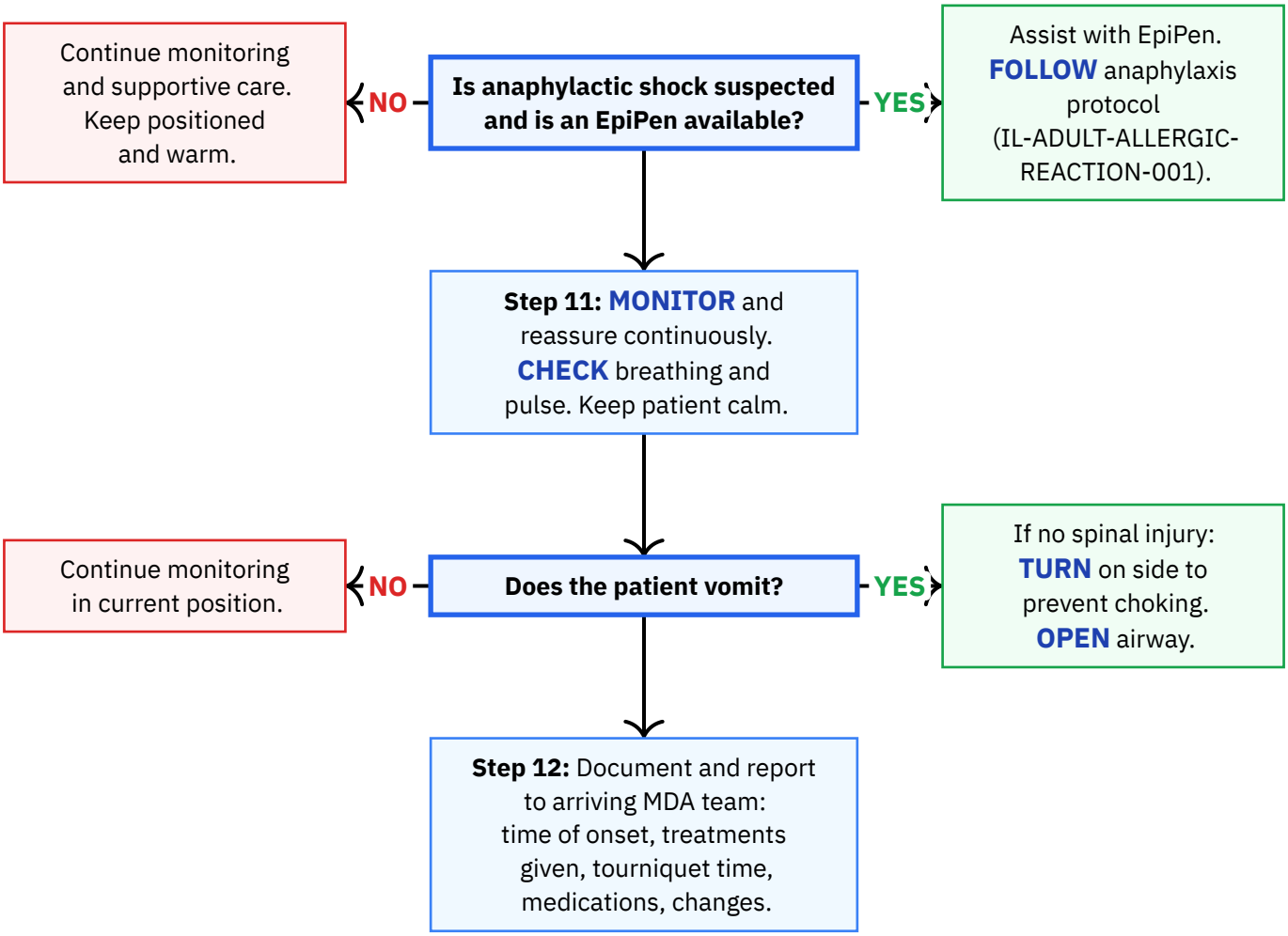
Initial Assessment



Bleeding Control and Positioning



Anaphylaxis Check and Continuous Monitoring



DO NOT:

- X DO NOT give the patient anything to eat or drink -- moisten lips only if thirsty.
- X DO NOT elevate legs if head, neck, or spinal injury is suspected.
- X DO NOT elevate legs if the patient has a broken hip or leg.
- X DO NOT elevate legs if it causes the patient pain or further injury.
- X DO NOT elevate legs if the patient has difficulty breathing -- keep them semi-upright instead.
- X DO NOT overheat the patient when maintaining body temperature.
- X DO NOT move a patient with suspected spinal injury unnecessarily.
- X DO NOT delay calling 101 -- shock is a life-threatening emergency.
- X DO NOT remove a tourniquet once applied -- leave removal to MDA paramedics.
- X DO NOT leave the patient unattended.

Equipment needed:

— Tourniquet -- widely available in Israeli first aid kits due to security context
— Israeli bandage (emergency bandage / amtza'sh) -- for wound packing and pressure
— Blanket or coat -- to maintain body temperature
— Ground insulation -- to protect from cold ground
— EpiPen (epinephrine auto-injector) -- for anaphylactic shock if patient has a prescription

Source: Magen David Adom (MDA) — "Shock (Helem) - MDA-Approved First Aid Curriculum and BLS Guide"
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Includes Israel-specific context for desert heat, security/blast injuries, and tourniquet/Israeli bandage use.

Smoke / Tear Gas Inhalation — Adult

Israel — ADULT

CALL 101 (MDA) + 102 (FIRE) — SCENE SAFETY FIRST

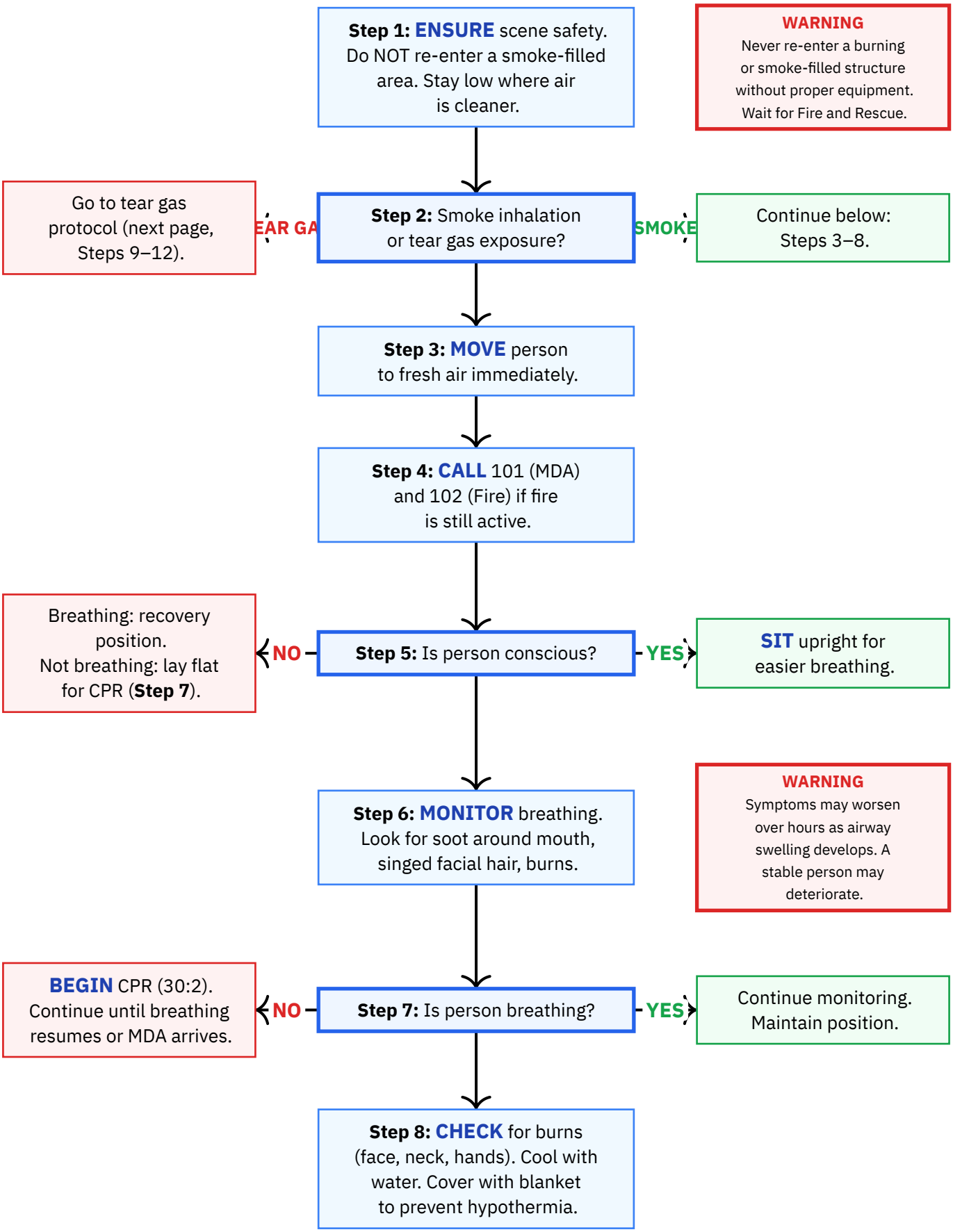
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

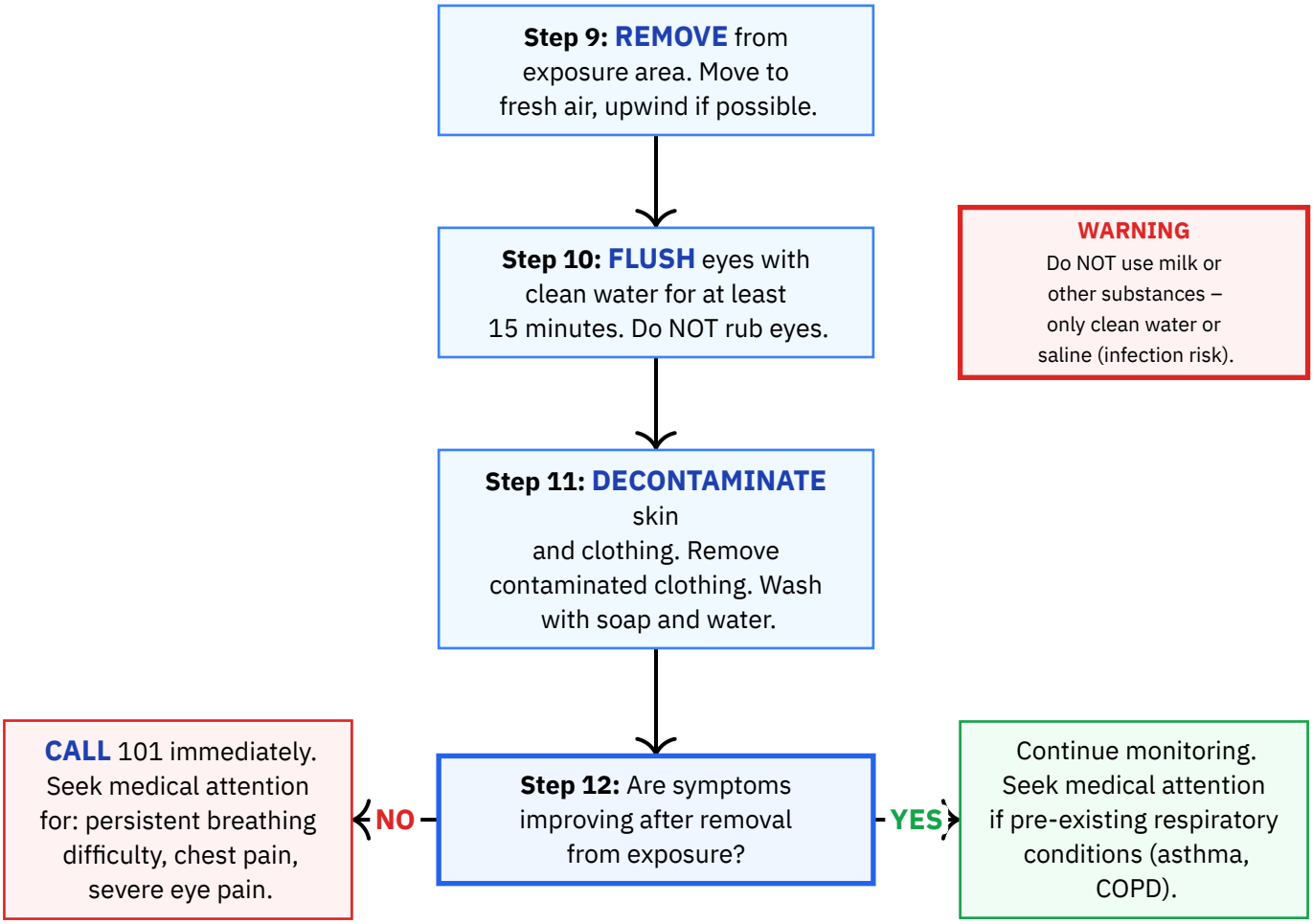
Person has inhaled smoke, fumes, or tear gas. Signs: coughing, wheezing, difficulty breathing, hoarse voice, soot around mouth/nose, singed facial hair, burns to face/mouth, confusion, eye irritation and tearing (tear gas).

Personal reference only — not medical advice

Smoke Inhalation Protocol



Tear Gas Exposure Protocol



DO NOT:

X Do NOT re-enter a smoke-filled or burning structure without proper firefighting equipment and training.
X Do NOT rub eyes after tear gas exposure -- this worsens irritation and spreads the chemical.
X Do NOT use milk or other substances to flush eyes -- use only clean water or saline due to infection risk.
X Do NOT remove clothing stuck to burns.
X Do NOT assume a smoke inhalation patient is stable -- symptoms may worsen over hours.
X Do NOT leave a smoke inhalation patient alone -- continuous monitoring is required.
X Do NOT ignore soot around the mouth/nose or singed facial hair -- indicators of potentially serious airway injury.

Equipment needed:

— Phone to call 101 (MDA) and 102 (Fire and Rescue)
— Clean water for eye flushing (tear gas) and burn cooling
— Clean cloth or non-adherent dressings for burns
— Blanket for warmth
— Soap for skin decontamination (tear gas)

Source: Magen David Adom (MDA) — Smoke and Tear Gas Inhalation Guidelines
URL: https://www.mdais.org/media/4954/mda-first-aid-guidelines-en.pdf
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Covers smoke from fires/wildfires and tear gas exposure in protest/border situations.

Snakebite — Adult

Israel — ADULT

CALL 101 (MDA) — CALL IMMEDIATELY

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Person has been bitten or suspected bitten by a snake. Signs include: puncture marks on skin, pain and swelling at bite site, redness or bruising around the wound. May progress to nausea, vomiting, abdominal pain, tachycardia, hypotension, difficulty breathing, or altered consciousness.

Summary: Immobilise the victim and bitten limb, call 101 immediately, clean wound with soap and water only, monitor for systemic reaction, and transport to hospital for evaluation and possible antivenom.

Initial Response

Step 1: CALL 101 (MDA) immediately. MDA will provide phone guidance until arrival.

↓

Step 2: KEEP victim calm and completely still. Have them lie down. Immobility slows venom spread.

↓

Step 3: IMMOBILISE bitten limb. Position below heart level. Splint against the body.

↓

Step 4: REMOVE jewellery, watches, rings, tight clothing from bite area and limb.

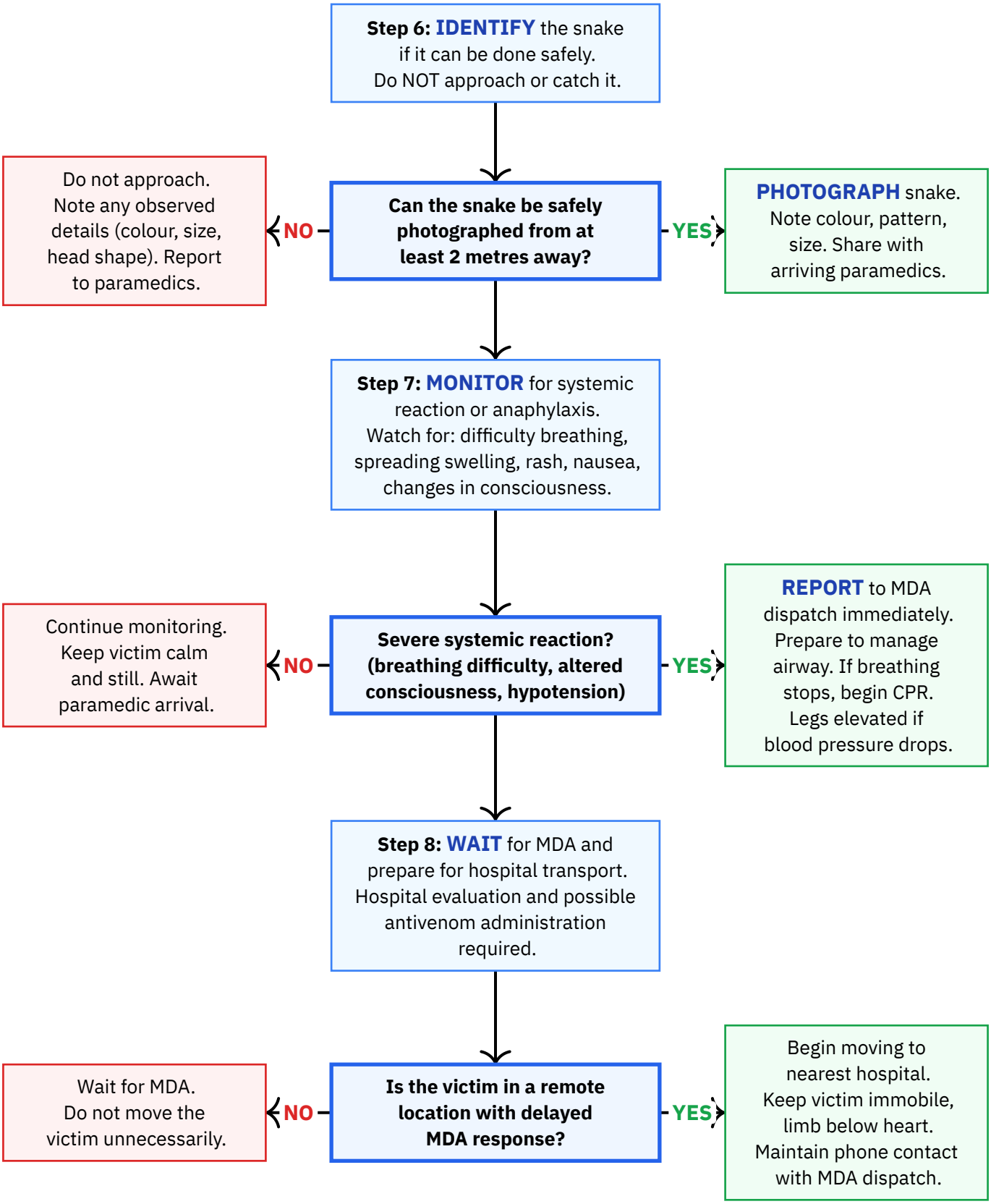
↓

Step 5: CLEAN bite wound with soap and water only. Do NOT use alcohol-based disinfectants.

WARNING
Swelling can progress rapidly. Remove all constrictive items as early as possible.

WARNING
Do NOT use alcohol-based disinfectants on the bite wound.

Identification and Monitoring



Reference

Dangerous Venomous Snakes in Israel

Daboia palaestinae
(Palestine viper)
Antivenom: available

Echis coloratus
(Painted saw-scaled viper)
Antivenom: available

Atractaspis engaddensis
(Israeli mole viper)
Antivenom: NONE

Peak snakebite season: spring/summer. Poison Center: 04-7771900

DO NOT:

X Do NOT apply a tourniquet -- it traps venom locally and causes severe tissue damage.
X Do NOT suck the venom out of the wound -- this is ineffective and dangerous.
X Do NOT cut or incise the bite area -- this increases infection risk and does not help remove venom.
X Do NOT cool or apply ice to the bite area -- this can worsen tissue damage.
X Do NOT give the victim food or drink -- this may complicate hospital treatment.
X Do NOT induce vomiting.
X Do NOT give aspirin, ibuprofen, or naproxen -- these can worsen bleeding because viper venom is often anticoagulant.
X Do NOT use alcohol-based disinfectants on the wound.
X Do NOT attempt to catch or kill the snake -- snakes are protected under Israeli law and approaching them risks additional bites.
X Do NOT allow the victim to walk or move the bitten limb -- movement accelerates venom circulation.

Equipment needed:

— Splint or improvised immobilisation material
— Soap and clean water for wound cleaning
— Phone or camera to photograph the snake

Source: Magen David Adom (MDA) — "How to Treat a Snake Bite – MDA 101 Public First Aid Guidance Series; corroborated by Israeli Ministry of Health guidelines"
URL: https://www.mdais.org/101/snake-bite
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Israel Poison Information Center 24/7 hotline: 04-7771900.

Spider Bites — Adult

Israel — ADULT

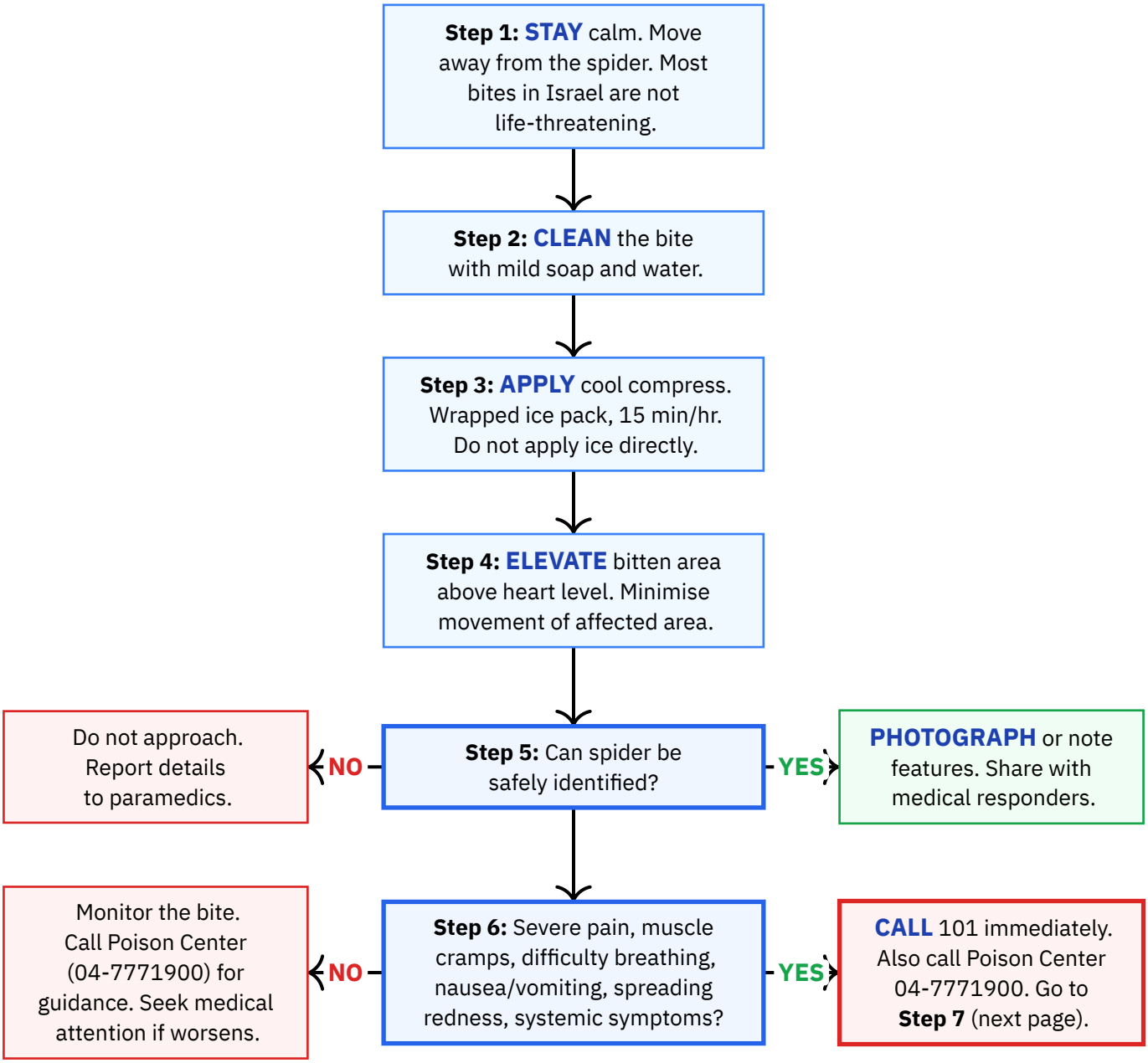
CALL 101 (MDA) / POISON CENTER 04-7771900

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

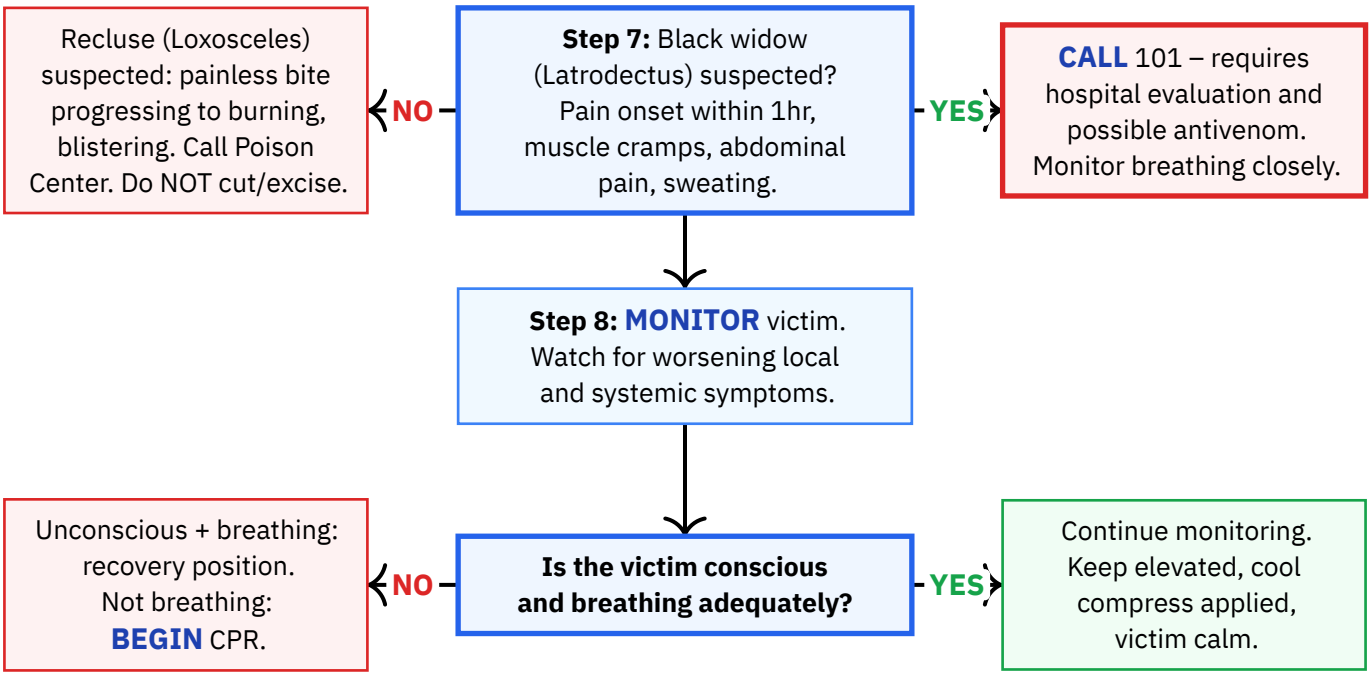
When to apply:

Person has been bitten or suspected bitten by a spider. Two species of medical significance in Israel: Mediterranean recluse (Loxosceles rufescens) – initially painless, progressive necrosis; Mediterranean black widow (Latrodectus tredecimguttatus) – neurotoxic venom, antivenom available.

Initial Care and Assessment



Species-Specific Response and Monitoring



DO NOT:

X Do NOT apply a tourniquet -- spider venom is not stopped by tourniquets and it will cause tissue damage.
X Do NOT cut, suck, or apply suction to the bite wound -- ineffective and increases infection risk.
X Do NOT apply heat to the bite -- use only cool compresses.
X Do NOT attempt to catch the spider at risk of additional bites.
X Do NOT attempt to cut out or excise a recluse spider bite wound -- early excision worsens outcomes.
X Do NOT apply ice directly to skin -- wrap in cloth first.
X Do NOT ignore initially mild symptoms -- recluse bites progress to serious necrosis over hours to days.
X Do NOT assume a bite is from a spider without confirmation -- 83.5% of cases presenting as 'spider bites' are actually other conditions.

Equipment needed:

— Phone to call 101 (MDA) and 04-7771900 (Israel Poison Information Center)
— Phone or camera to photograph the spider if safe
— Mild soap and clean water for wound cleaning
— Clean cloth or wrapped ice pack for cool compress

Source: Magen David Adom (MDA) — First Aid for Spider Bites
URL: https://www.mdais.org/media/4954/mda-first-aid-guidelines-en.pdf
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Two medically significant species in Israel: Mediterranean recluse (Loxosceles rufescens) and Mediterranean black widow (Latrodectus tredecimguttatus). Israel Poison Information Center: 04-7771900 (24/7).

Stroke (Cerebrovascular Event) — Adult

Israel — ADULT

CALL 101 (MDA) — TIME IS CRITICAL

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Person shows one or more signs of stroke: facial drooping, arm weakness, slurred speech, sudden confusion, sudden severe headache, sudden vision loss, sudden loss of balance, or sudden change in consciousness. Even a single FAST sign warrants immediate emergency evaluation.

Summary: Recognise stroke signs using FAST (Face, Arm, Speech, Time), call 101 immediately, note time of symptom onset, position the person safely, and await paramedics.

FAST Assessment and Emergency Call

Step 1: ASSESS using FAST
F – Face: ask to smile, look for one-sided drooping
A – Arms: extend both, look for drift
S – Speech: repeat sentence, listen for slurring
T – Time: note exact onset

↓

Step 2: CALL 101 (MDA) immediately. Do not wait to see if symptoms improve. Do not drive to hospital.

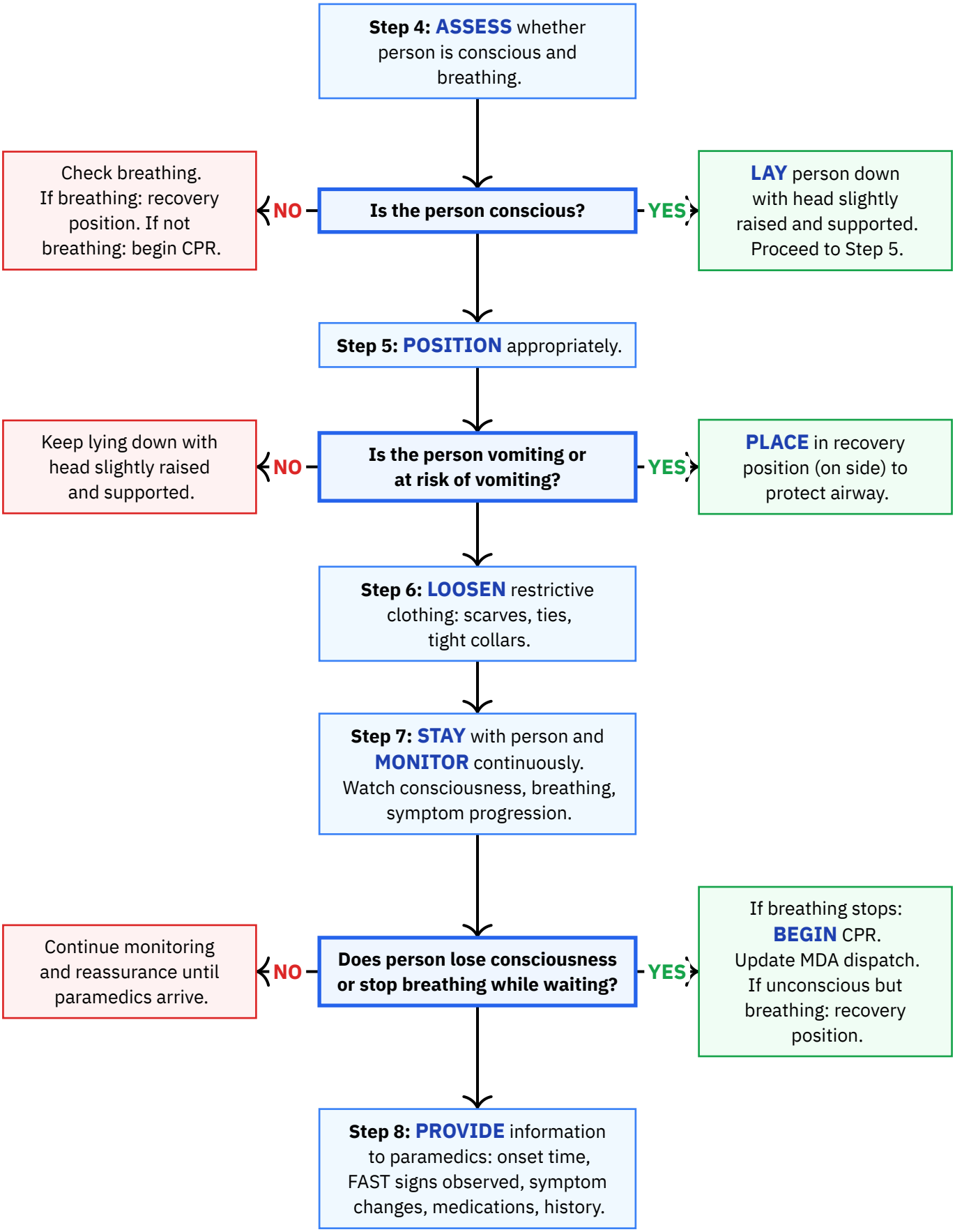
↓

Step 3: RECORD exact time of symptom onset. Critical for treatment decisions. If woke with symptoms: onset **when sleep began.**

WARNING
Every minute of delay results in 2 million neurons dying. Treatment window: 6 hours from onset. Target: hospital within 60 minutes.

WARNING
Without accurate onset time, doctors may not be able to administer time-sensitive treatments.

Positioning and Monitoring



Reference

FAST — Quick Reference

F – Face	A – Arms	S – Speech	T – Time
Ask to smile.	Raise both arms.	Repeat a sentence.	Note exact onset.
One-sided drooping?	One drifting down?	Slurred or garbled?	Call 101 NOW.

DO NOT:

X DO NOT give food or drink -- the person's swallowing reflex may be impaired (dysphagia), creating aspiration risk.
X DO NOT give any medications, especially aspirin or blood thinners -- ~15% of strokes are hemorrhagic (caused by bleeding), and aspirin can worsen hemorrhagic stroke. Only medical professionals should decide on medication.
X DO NOT delay calling 101 -- do not drive the person to hospital yourself unless absolutely unavoidable. The ambulance crew will pre-notify the hospital and begin assessment en route.
X DO NOT ignore transient symptoms -- even if symptoms resolve (possible TIA), the person still needs emergency evaluation. ~40% of TIA patients experience a full stroke within three months.
X DO NOT allow the person to eat, drink, or take medications by mouth until a medical professional has assessed their swallowing function.

Equipment needed:

— Phone to call 101 and record symptom onset time
— Pillow or support to slightly elevate the head

Source: Magen David Adom (MDA) — "Stroke Recognition and First Aid – MDA 101 Public First Aid Guidance Series; supplemented by Ynet article quoting Dr. Shfir Botner (Head of MDA Paramedic School) and United Hatzalah educational material"
URL: https://www.mdais.org/101/stroke
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Treatment window 6 hours from onset. Target hospital arrival within 60 minutes.

Tourniquet Application — Adult

Israel — ADULT

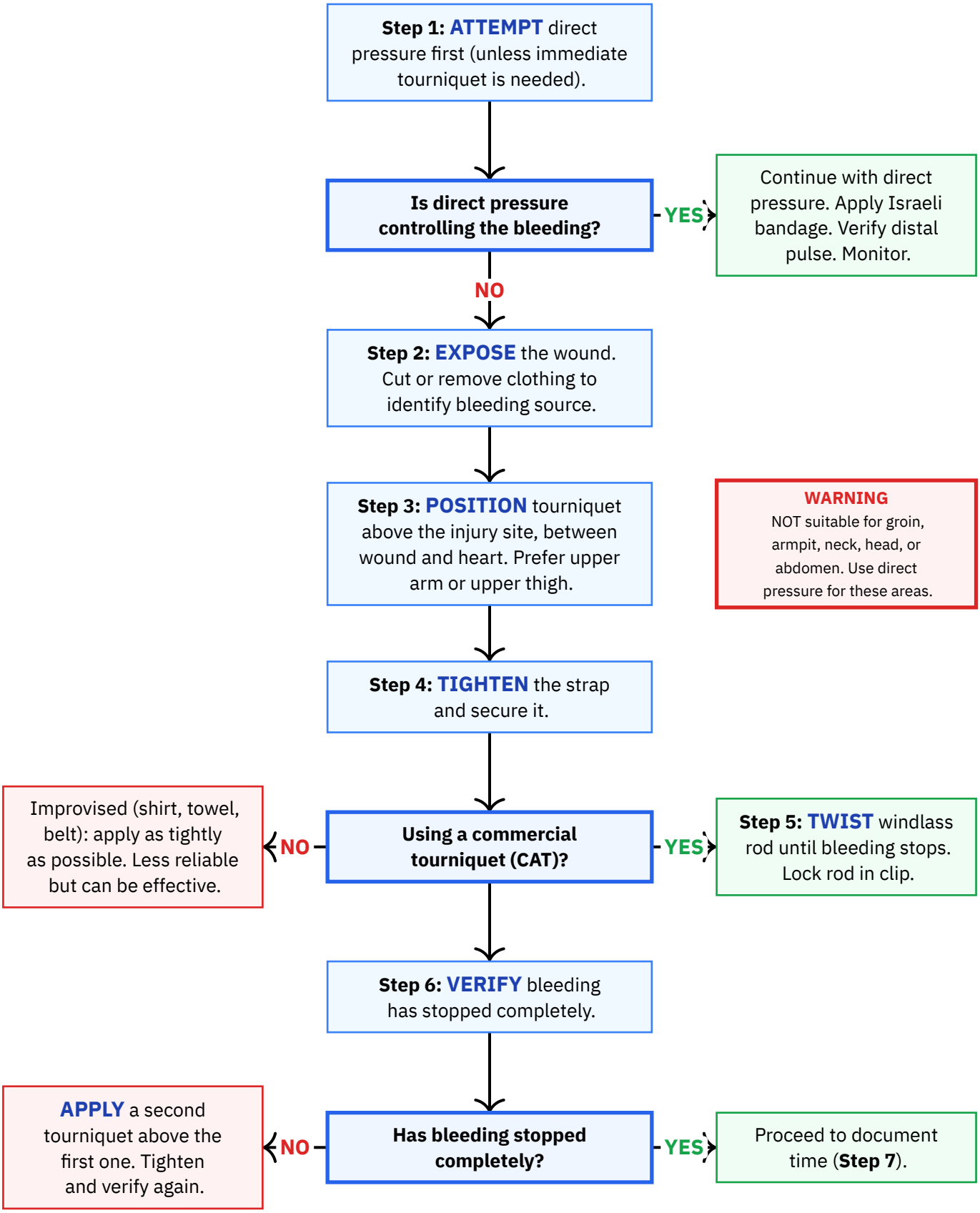
CALL 101 (MDA) — LOSS OF LIMB IS PREFERABLE TO LOSS OF LIFE

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Life-threatening bleeding from a limb that cannot be controlled by direct pressure. Specific indications: direct pressure fails, partial or complete amputation, severe crushing injury, multi-casualty situations where sustained direct pressure is not feasible.

Assessment and Application



Documentation and Monitoring

Step 7: DOCUMENT the time of tourniquet application. Write on tourniquet tab, patient's skin, or visible location.

CRITICAL
Time documentation is critical for limb viability assessment at hospital.

↓

Step 8: CALL 101. Mention tourniquet has been applied. Report location, injury, and time of application.

↓

Step 9: MONITOR the victim. Watch for shock (rapid pulse, pale/cold skin, confusion). Keep calm and still.

↓

Is the victim conscious and breathing?

- **NO** → Unconscious + breathing: recovery position. Not breathing: **BEGIN** CPR.
- **YES** → Continue monitoring and reassurance. Keep warm until MDA arrives.

DO NOT:

X Do NOT remove the tourniquet once applied -- only medical professionals should decide to release or convert.
X Do NOT apply a tourniquet to junctional zones (groin, armpit, neck), head, or abdomen.
X Do NOT apply the tourniquet over a joint -- place on the limb above the wound.
X Do NOT loosen or adjust the tourniquet to 'check if bleeding has stopped' -- this can restart hemorrhage.
X Do NOT delay tourniquet application in life-threatening hemorrhage due to fear of limb loss.
X Do NOT allow the victim to walk or move the affected limb after tourniquet application.
X Do NOT attempt tourniquet conversion -- this is a professional-level decision under medical direction only.

Equipment needed:

— CAT tourniquet (Combat Application Tourniquet) -- preferred
— Improvised tourniquet material (shirt, towel, belt) as last resort
— Scissors or Leatherman Raptor for cutting clothing
— Pen or marker to document tourniquet application time
— Phone to call 101 (MDA)

Source: Magen David Adom (MDA) — MDA First Aid Guidelines and 'First Seven Minutes' initiative
URL: https://www.mdais.org/media/4954/mda-first-aid-guidelines-en.pdf
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Approximately 500,000 people trained by MDA in 2024. MDA presents tourniquet as part of a progressive hemorrhage control sequence.

AED Operation -- Child and Infant

Israel — CHILD

CALL 101 (MDA) — USE AED IMMEDIATELY

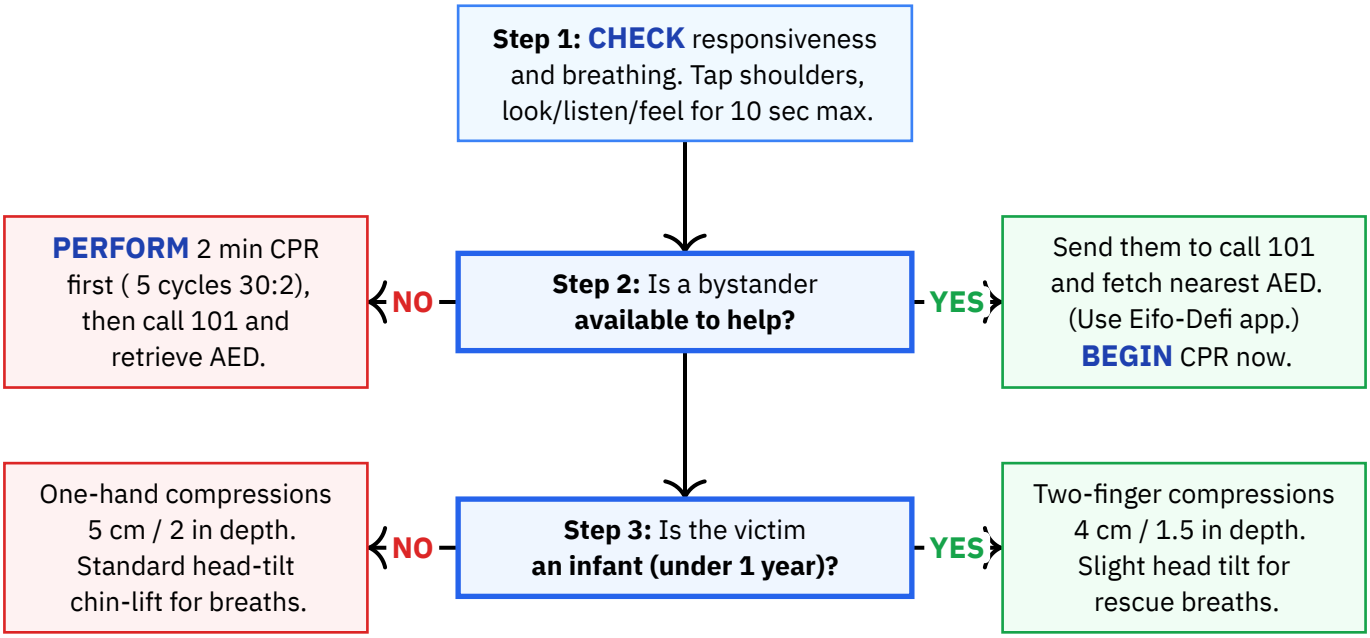
MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

Child (age 1–8) or infant (under 1 year) who is unresponsive, not breathing normally, and in suspected cardiac arrest. If alone with a child: perform 2 minutes of CPR first, then call 101 and retrieve AED (cardiac arrest in children is more often respiratory in origin).

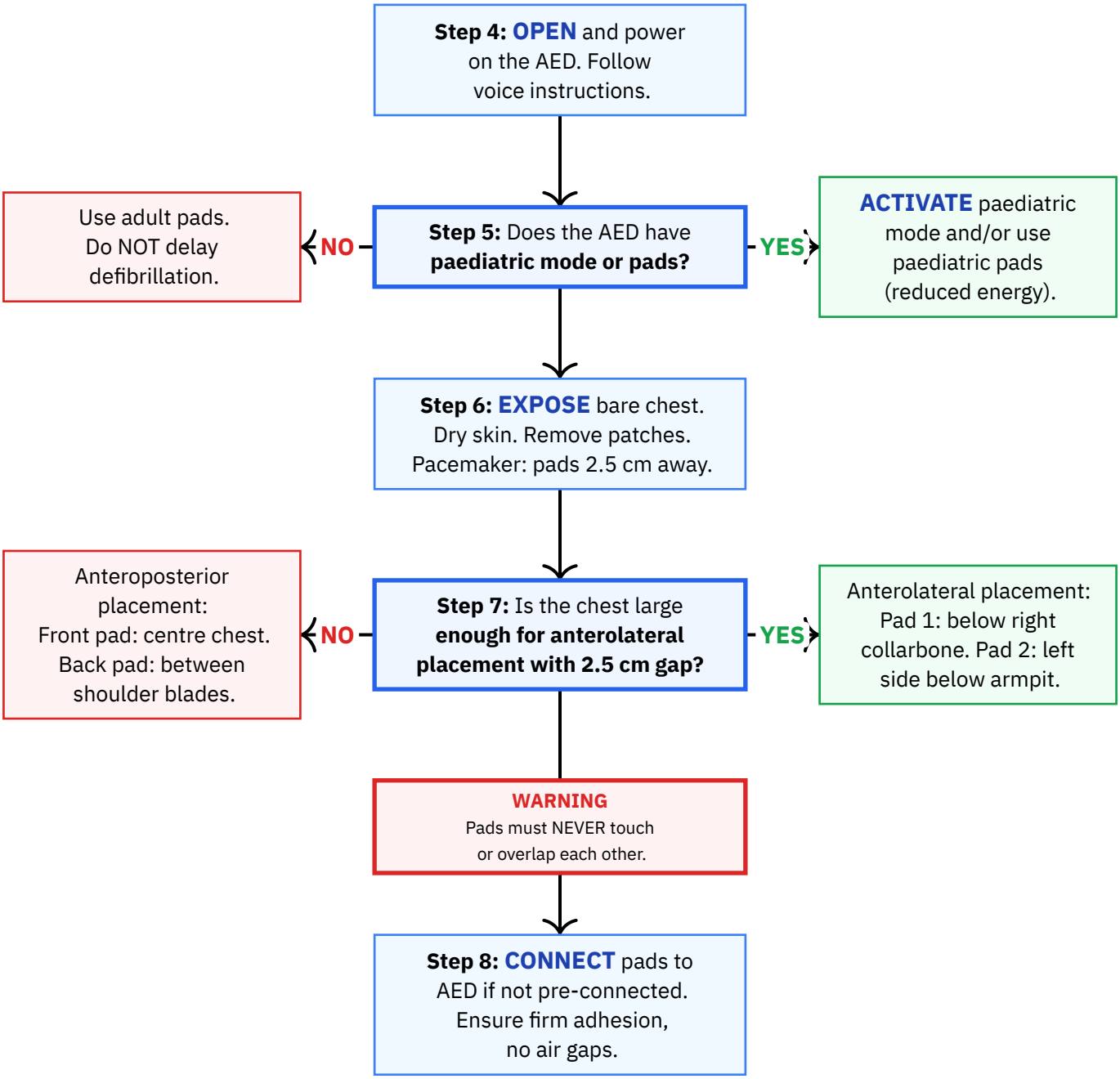
Summary: Activate paediatric mode or use paediatric pads. For small chests: anteroposterior (front-and-back) pad placement. Follow AED voice prompts. Integrate with age-appropriate CPR. Do NOT delay defibrillation for paediatric pads – use adult pads if needed.

Recognition and CPR – Steps 1-3

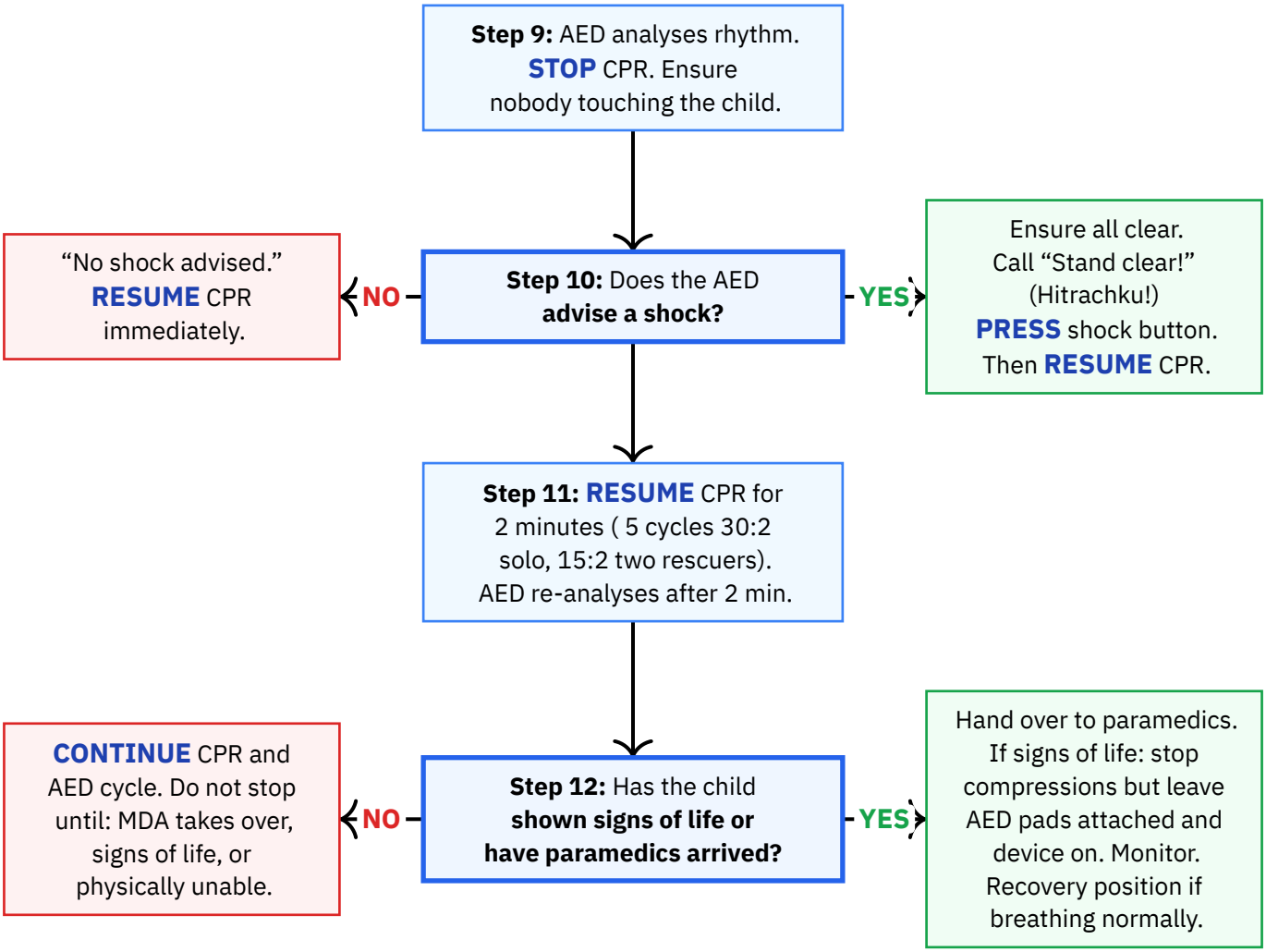


Personal reference only — not medical advice

AED Setup and Pad Placement – Steps 4-8



AED Analysis and CPR Cycles – Steps 9-12



Key Paediatric CPR/AED Parameters

Child (1–8 yr) depth: 5 cm / 2 in
Compression rate: 100–120/min
Ratio (2 rescuers): 15:2
Infant technique: 2-finger or 2-thumb
Infant (under 1 yr) depth: 4 cm / 1.5 in
Ratio (solo): 30:2
Child technique: 1-hand compression
AED re-analysis: every 2 minutes

DO NOT:

X DO NOT use AED on a conscious, breathing child -- only for cardiac arrest.
X DO NOT use AED in water or near flammable gases.
X DO NOT delay defibrillation because paediatric pads are unavailable -- use adult pads.
X DO NOT allow pads to touch or overlap -- use anteroposterior if chest too small.
X DO NOT touch the child during rhythm analysis or shock delivery.
X DO NOT turn off AED or remove pads once applied, even if child regains consciousness.
X DO NOT perform adult-depth compressions on a child/infant -- one-third chest depth only.
X DO NOT use anterolateral placement on infant/small child if pads would overlap.

Equipment needed:

— AED (Automated External Defibrillator)
— Paediatric AED pads (preferred for children under 8 / under 25 kg)
— Adult AED pads (if paediatric pads unavailable)
— Scissors or shears (to cut away clothing)
— Towel or cloth (to dry the chest)

Source: Magen David Adom (MDA) – AED Operation for Children and Infants, based on AHA 2025 Guidelines for CPR and ECC
URL: https://www.mdais.org/kursim/defibrillator
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah sources. Age categories per MDA/AHA: infant (under 1 year), child (1–8 years). Children 8+ or showing puberty signs use adult AED protocol.

Anaphylaxis (Severe Allergic Reaction) — Child

Israel — CHILD

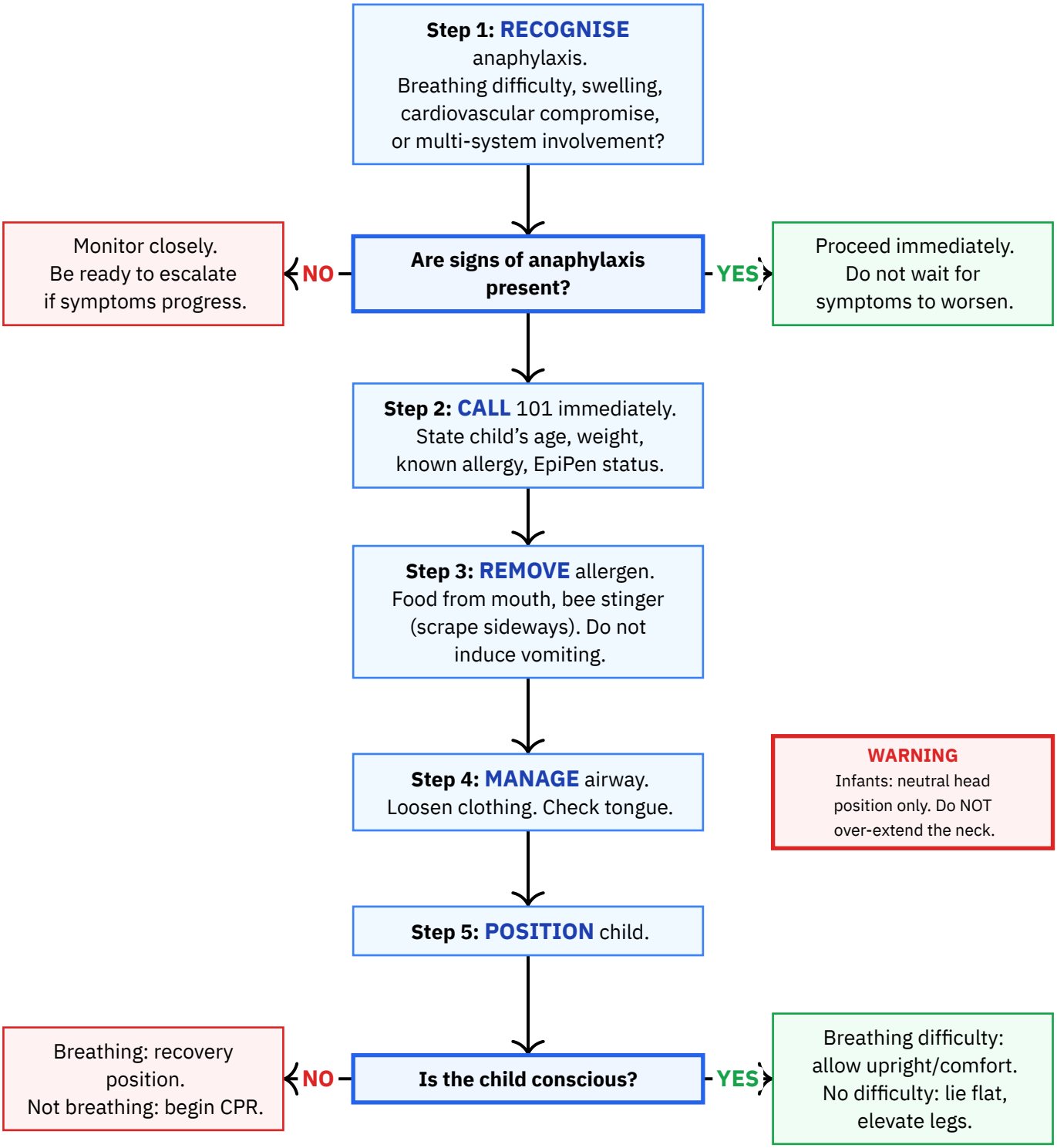
CALL 101 (MDA) — EPIPEN FIRST, THEN CALL

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

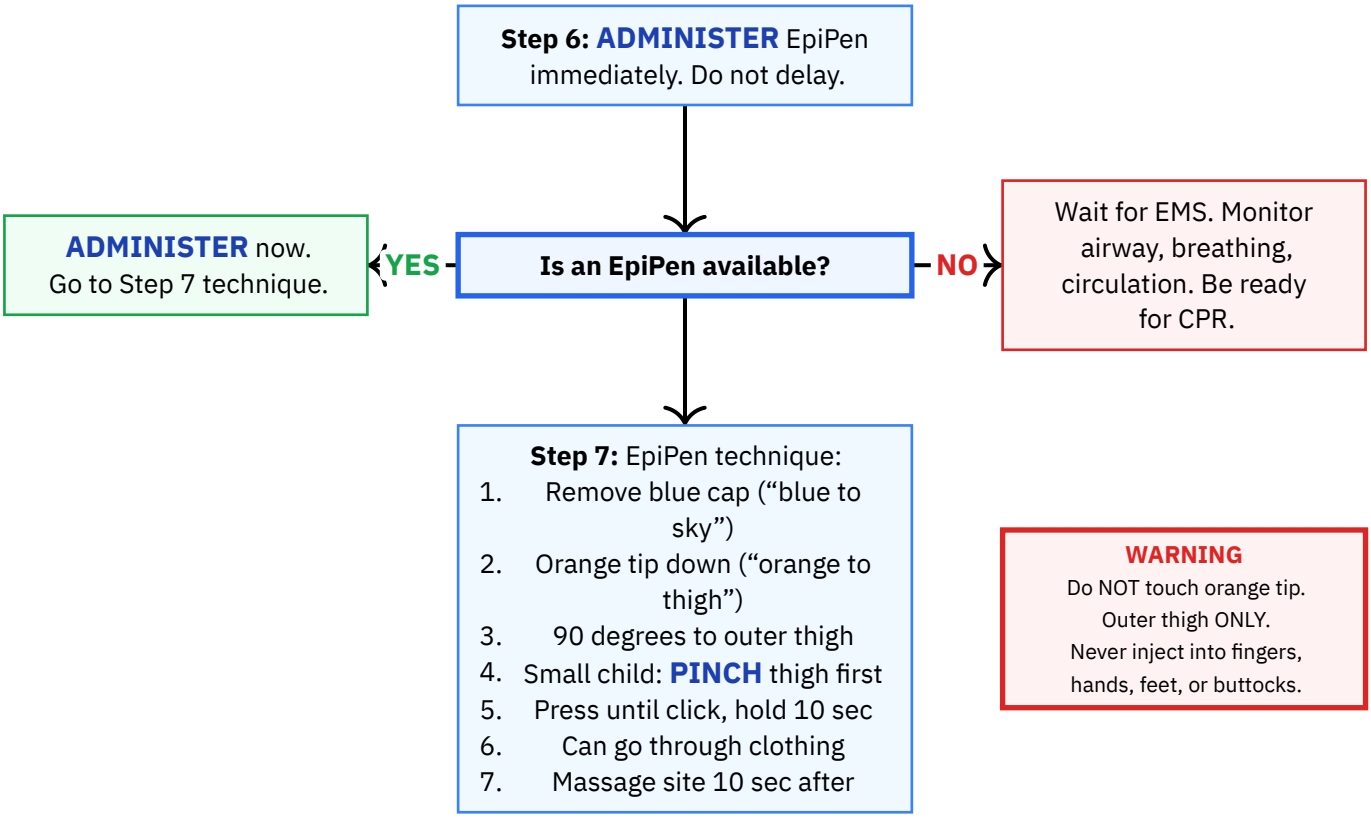
When to apply:

Child showing signs of severe allergic reaction: hives, facial/throat swelling, wheezing, stridor, difficulty breathing, weak/rapid pulse, confusion, loss of consciousness. Any involvement of breathing, circulation, or consciousness indicates anaphylaxis – do not wait.

Recognition and Initial Response (Steps 1–5)



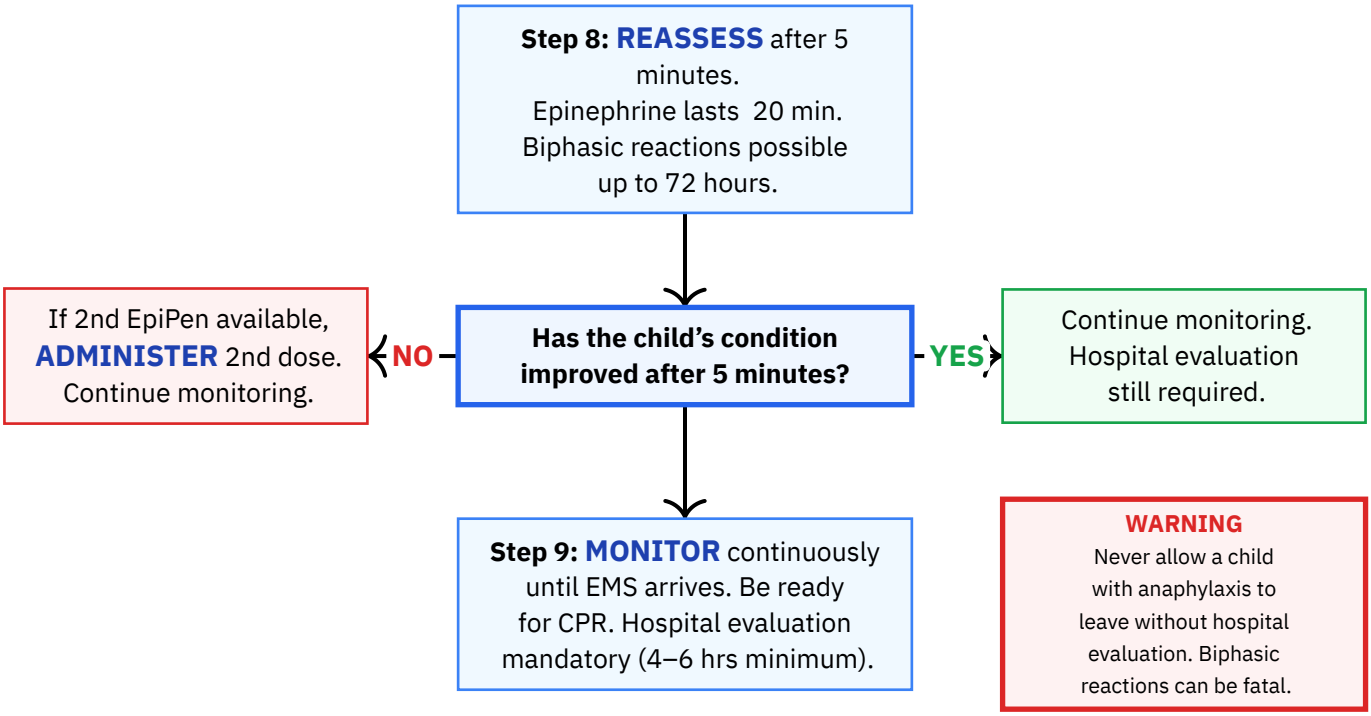
EpiPen Administration (Steps 6–7)



EpiPen Dosing by Weight

15–30 kg: EpiPen Junior 0.15 mg (green label)
Over 30 kg: EpiPen 0.3 mg (yellow label)
Under 15 kg: No auto-injector for this range, but 0.15 mg EpiPen Junior is still recommended over withholding treatment.
Note: Some Israeli allergists prescribe 0.3 mg from 20 kg – follow individual prescription.

Reassessment and Monitoring (Steps 8–9)



DO NOT:

X Do not delay epinephrine -- it is the only effective treatment for anaphylaxis.
X Do not substitute antihistamines (Benadryl) for epinephrine in anaphylaxis.
X Do not force the child to lie down if having breathing difficulty -- allow upright position.
X Do not over-extend an infant's neck -- use neutral position only.
X Do not inject epinephrine into fingers, hands, feet, or buttocks -- outer thigh only.
X Do not touch the orange tip of the EpiPen.
X Do not induce vomiting if allergen was ingested.
X Do not allow a child with anaphylaxis to leave without hospital evaluation.
X Do not store EpiPen in refrigerator, car, or direct sunlight -- Israeli heat risk.
X Do not give oral medications to a child with throat swelling or difficulty swallowing.

Equipment needed:

— EpiPen Junior 0.15 mg (green label) -- for children 15-30 kg
— EpiPen 0.3 mg (yellow label) -- for children over 30 kg
— Phone to call MDA at 101

Source: Magen David Adom (MDA) — ALS Manual 2024 / BLS Guide 2016
URL: https://www.mdais.org/media/5033/als-2024.pdf
Publication date: 2024-04-01 · **Imported:** 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah. Israeli Ministry of Education directive requires epinephrine availability in schools/ganim.

Asthma Attack First Aid -- Child

Israel — CHILD

CALL 101 (MDA) IF NO IMPROVEMENT

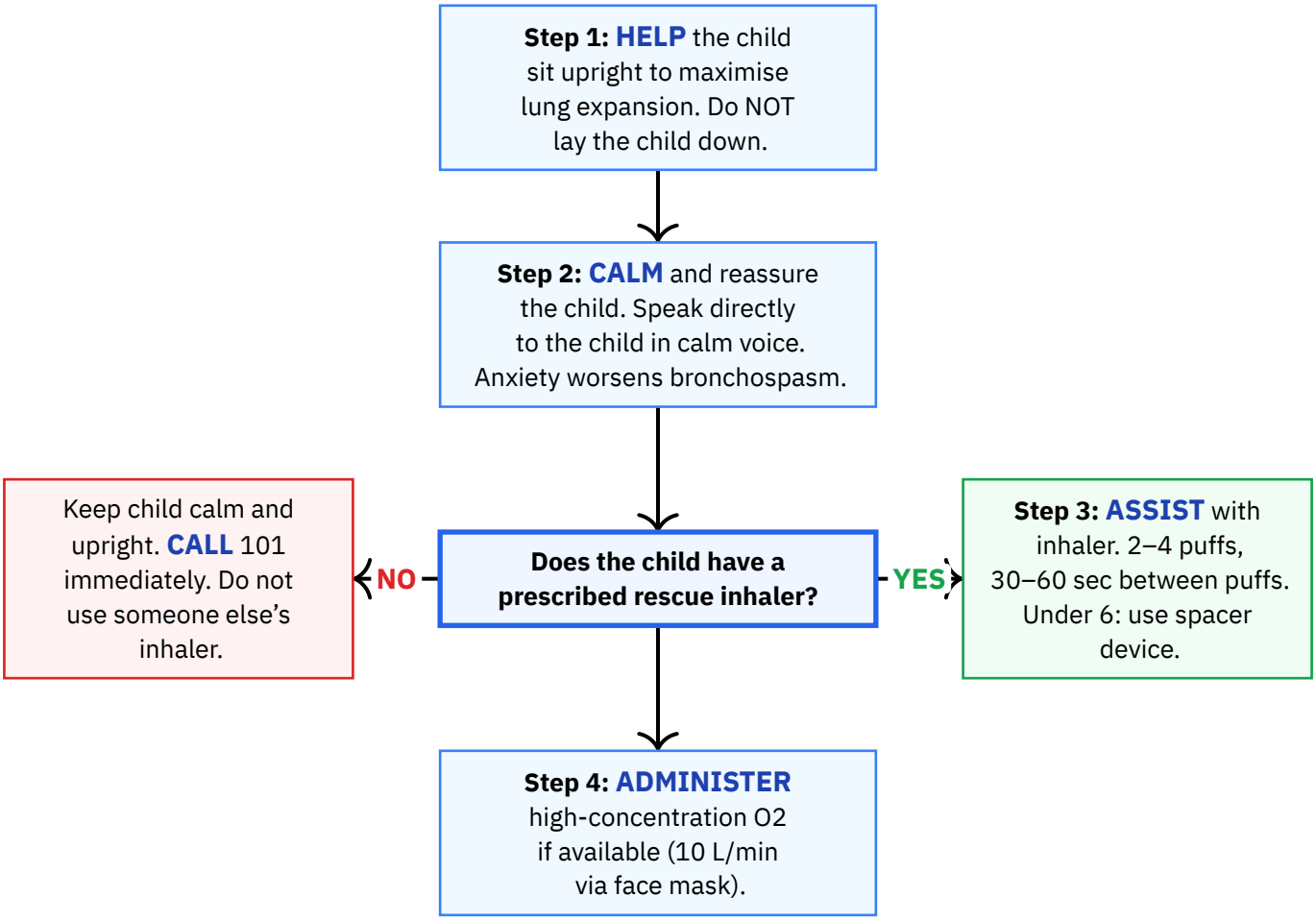
MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

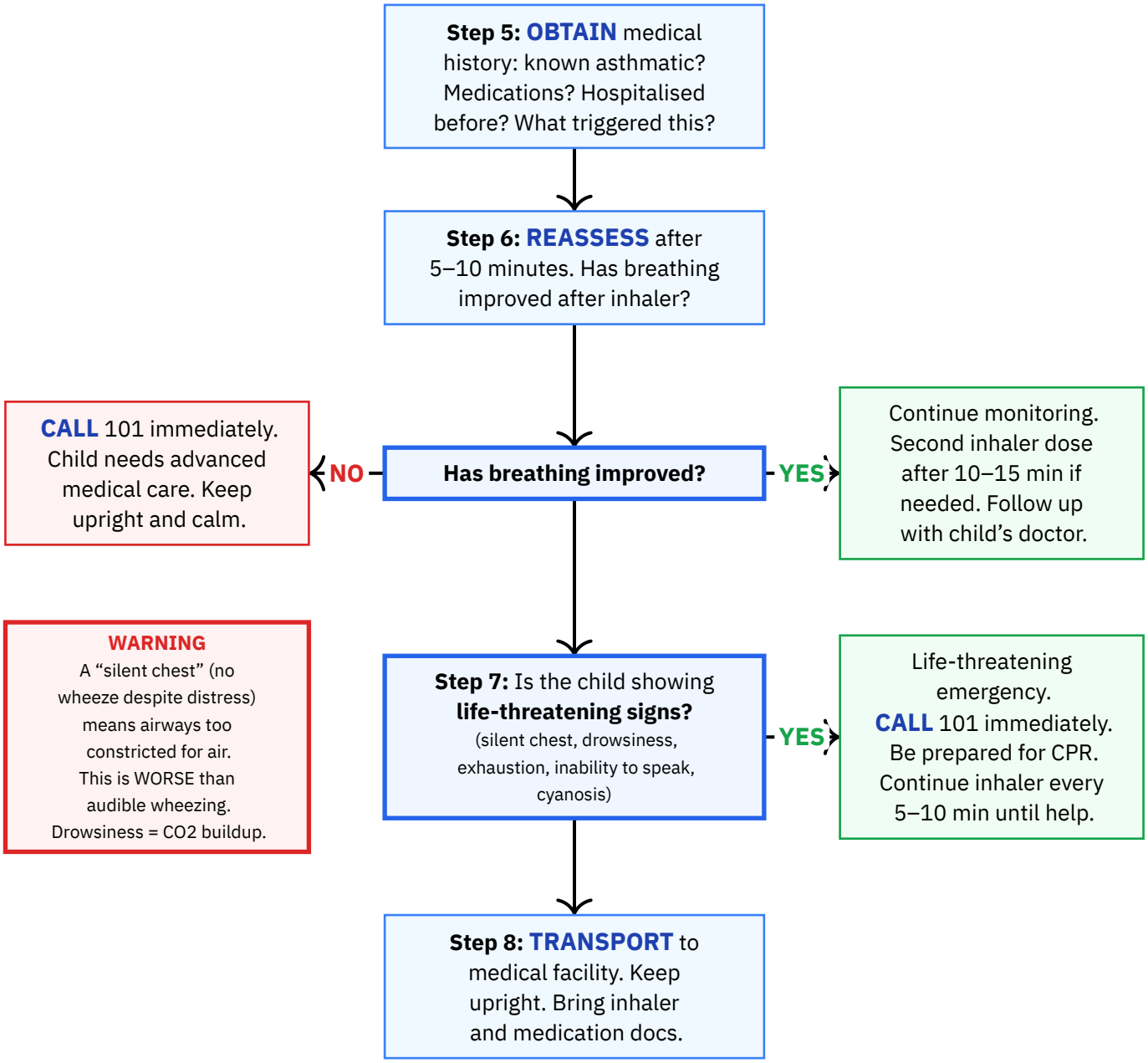
When a child is experiencing an asthma attack: persistent cough, wheezing, difficulty breathing (especially exhaling), chest tightness, lip cyanosis, accessory muscle use, intercostal retractions, nasal flaring. Asthma is the most common chronic disease in Israeli children (5–10% prevalence).

Summary: Sit upright. Calm the child. Assist with rescue inhaler (2–4 puffs, spacer for under 6). Monitor for improvement. Call 101 if no improvement or worsening. A “silent chest” is a danger sign.

Initial Treatment – Steps 1-4



Monitoring and Escalation – Steps 5-8



DO NOT:

- X Do NOT lay the child down -- upright position is essential for breathing.
- X Do NOT give cold drinks during an attack -- cold can worsen bronchospasm.
- X Do NOT leave the child alone during an attack.
- X Do NOT administer someone else's prescription inhaler without medical direction.
- X Do NOT delay calling 101 if the child is not improving.
- X Do NOT ignore drowsiness during an asthma attack -- dangerous sign of CO2 buildup.
- X Do NOT assume a quiet chest means improvement -- silent chest during distress is life-threatening.

Equipment needed:

— Rescue inhaler (Ventolin/salbutamol) -- child's own prescribed inhaler
— Spacer device (for children under 6)
— Supplemental oxygen and face mask (if available)
— Pulse oximeter (if available)
— Phone (to call 101)

Source: Magen David Adom (MDA) – ALS Professional Manual (April 2024), supplemented by Ichud Hatzalah asthma and respiratory distress guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2024-04-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah sources. Asthma is the most common chronic disease in Israeli children (5–10% prevalence).

Blast / Shrapnel Injuries First Aid -- Child

Israel — CHILD

CALL 101 (MDA) — SCENE SAFETY FIRST

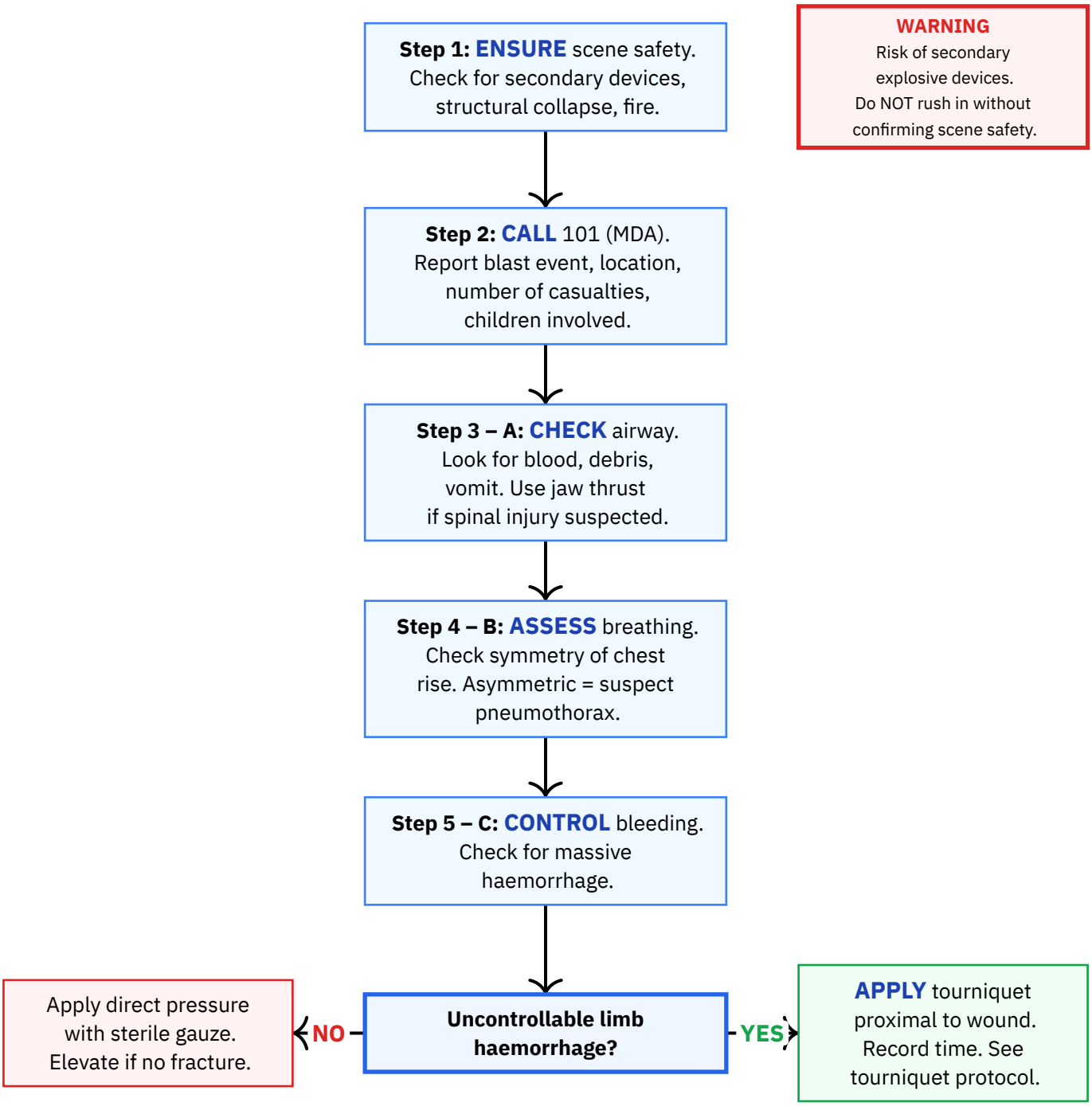
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

When a child has been injured by a blast event: rocket/missile attack, explosive device, or any explosion. Children are at higher risk due to smaller body mass, thinner chest wall, less blood volume, and greater energy transfer per kilogram. Follows PHTLS methodology (SABCDE).

Summary: Scene safety first (secondary device risk). SABCDE assessment: Scene, Airway, Breathing, Circulation, Disability, Exposure. Control bleeding. Do NOT remove embedded objects. Prevent hypothermia. Keep child with parent/caregiver.

Scene Safety and ABC – Steps 1-5



Disability, Exposure, and Care – Steps 6-10

Step 6 – D: ASSESS neurological status (AVPU). Check pupil size/reactivity.

WARNING
A silent, non-communicative child after blast may have serious injuries. Do not assume uninjured because they are quiet.

↓

Step 7 – E: EXPOSE and examine for additional wounds, burns, embedded objects, fractures. Then **COVER** immediately to prevent hypothermia.

WARNING
Do NOT remove embedded objects (shrapnel, fragments). Stabilise them in place.

↓

Step 8: COVER open wounds with sterile dressings. Build padding around embedded objects.

↓

Step 9: KEEP child with parent or caregiver. Separation compounds psychological trauma. Speak calmly and honestly.

↓

Step 10: MONITOR continuously. Keep warm and covered. Monitor breathing, pulse, consciousness. Be prepared for CPR.

DO NOT:

- X Do NOT enter an unsafe blast scene -- risk of secondary devices and structural collapse.
- X Do NOT remove embedded objects (shrapnel, fragments) -- stabilise them in place.
- X Do NOT assume a quiet child is uninjured -- children may be in psychological shock.
- X Do NOT remove a tourniquet once applied -- hospital personnel will manage removal.
- X Do NOT forget to prevent hypothermia after examination -- children lose heat rapidly.
- X Do NOT separate the child from their parent or caregiver unnecessarily.
- X Do NOT ignore asymmetric chest rise -- indicates thoracic trauma requiring urgent attention.
- X Do NOT give food or drink if significant injuries present -- may need anaesthesia at hospital.

Equipment needed:

— Tourniquet (CAT or equivalent -- see tourniquet protocol for child-specific sizing)
— Sterile gauze and bandages
— Blankets (for hypothermia prevention)
— Rigid splints (for fracture immobilisation)
— Phone (to call 101)
— Supplemental oxygen and mask (if available)

Source: Magen David Adom (MDA) – PHTLS-aligned trauma protocols, supplemented by Ichud Hatzalah missile strike response and SABCDE assessment guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2024-04-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah sources. Both follow PHTLS methodology. Both operate psychotrauma units for paediatric psychological first aid.

Burns First Aid — Child

Israel — CHILD

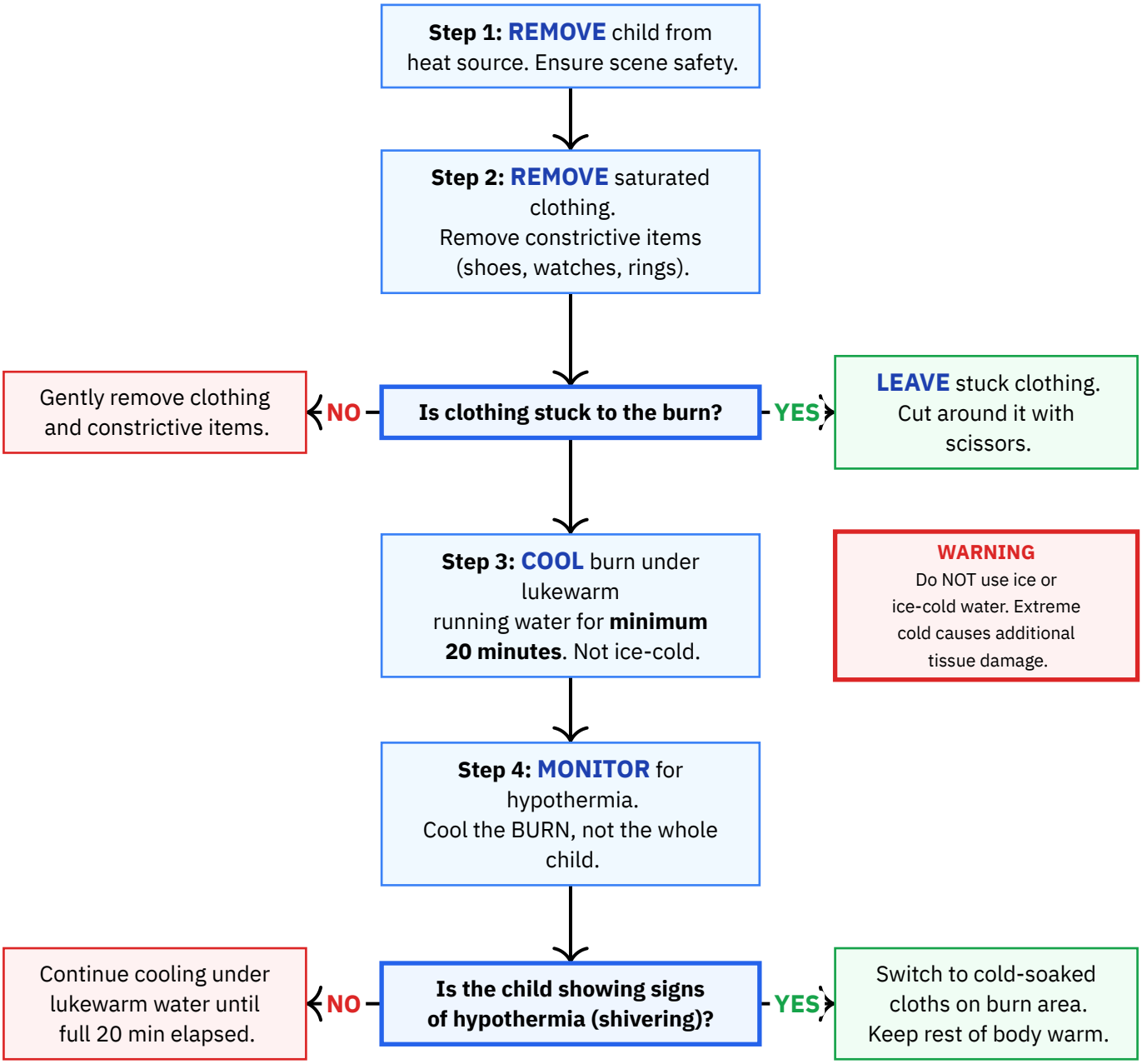
CALL 101 (MDA) FOR SERIOUS BURNS

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

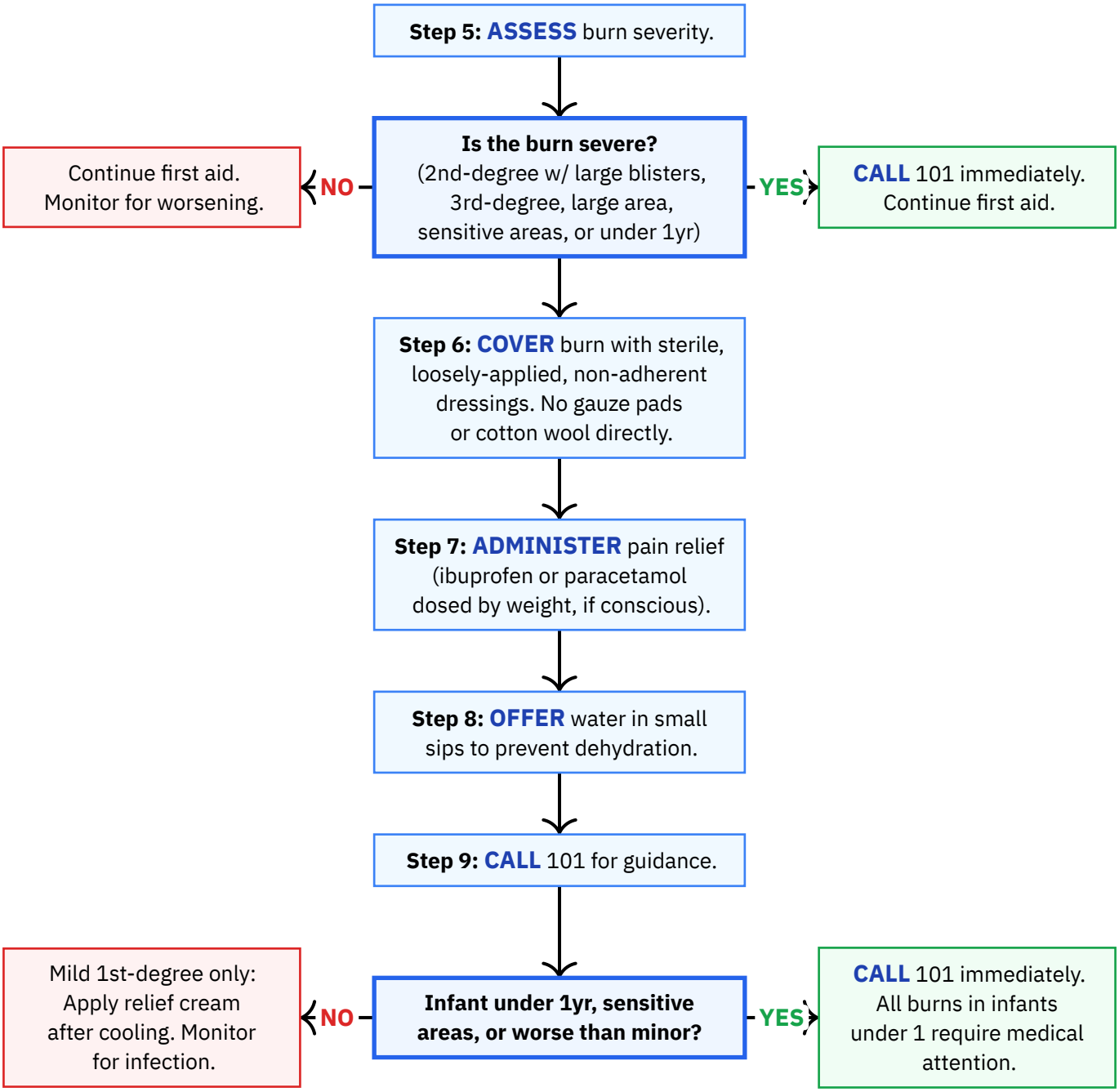
When to apply:

Child has sustained a burn from heat, hot liquids (scalds), chemicals, electricity, or radiation. 84% of paediatric burns in Israel are scalds. All burns in infants under 1 year require immediate medical attention.

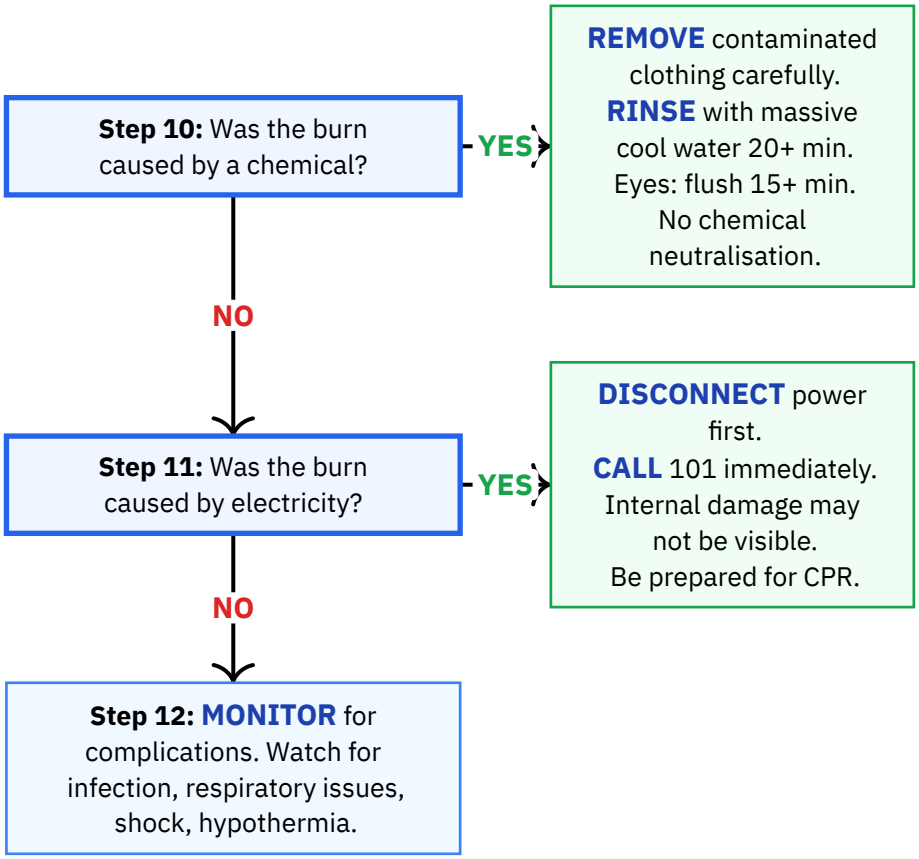
Initial Response (Steps 1–4)



Assessment and Treatment (Steps 5–9)



Chemical and Electrical Burns (Steps 10–12)



DO NOT:

X Do NOT apply ice or ice-cold water -- extreme cold causes additional tissue damage.
X Do NOT apply toothpaste, coffee grounds, raw eggs, yogurt, or butter.
X Do NOT puncture or pop blisters -- fluid protects tissue.
X Do NOT apply ointments, oils, or powders to serious burns before medical evaluation.
X Do NOT use gauze pads or cotton wool directly on burns -- fibres adhere.
X Do NOT leave the child unattended if unconscious.
X Do NOT remove clothing stuck to the burn -- cut around it.
X Do NOT immerse the whole child in cold water -- cool the burn, not the child.
X Do NOT attempt chemical neutralisation on chemical burns.

Equipment needed:

— Cool (lukewarm) running water source
— Sterile non-adherent dressings or clean dry sheet
— Scissors (to cut clothing around stuck areas)
— Blanket or towel (warmth during cooling)
— Age-appropriate pain relief (ibuprofen or paracetamol, dosed by weight)
— Drinking water (for hydration)

Source: Magen David Adom (MDA) — MDA Burns First Aid Guidelines
URL: https://www.mdais.org/101/burns
Publication date: 2026-01-01 · **Imported:** 2026-03-15 · **Last verified:** 2026-03-15
Notes: Key child difference: minimum 20 minutes cooling (vs 10 for adults). Cool the burn, not the whole child. All burns in infants under 1 year require immediate medical attention.

Choking (Foreign Body Airway Obstruction) — Child/ Infant

Israel — CHILD

CALL 101 (MDA) — COMPLETE OBSTRUCTION

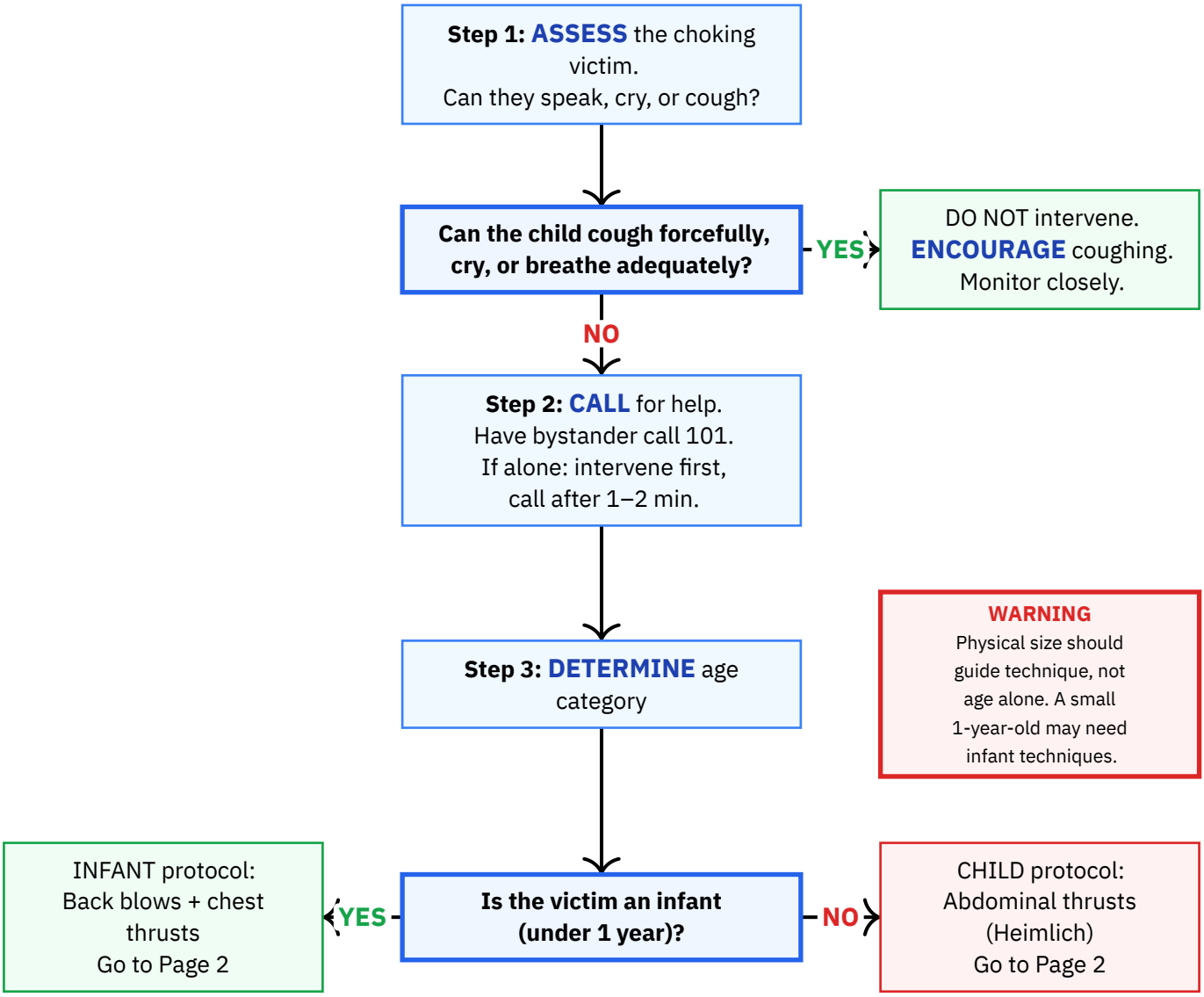
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

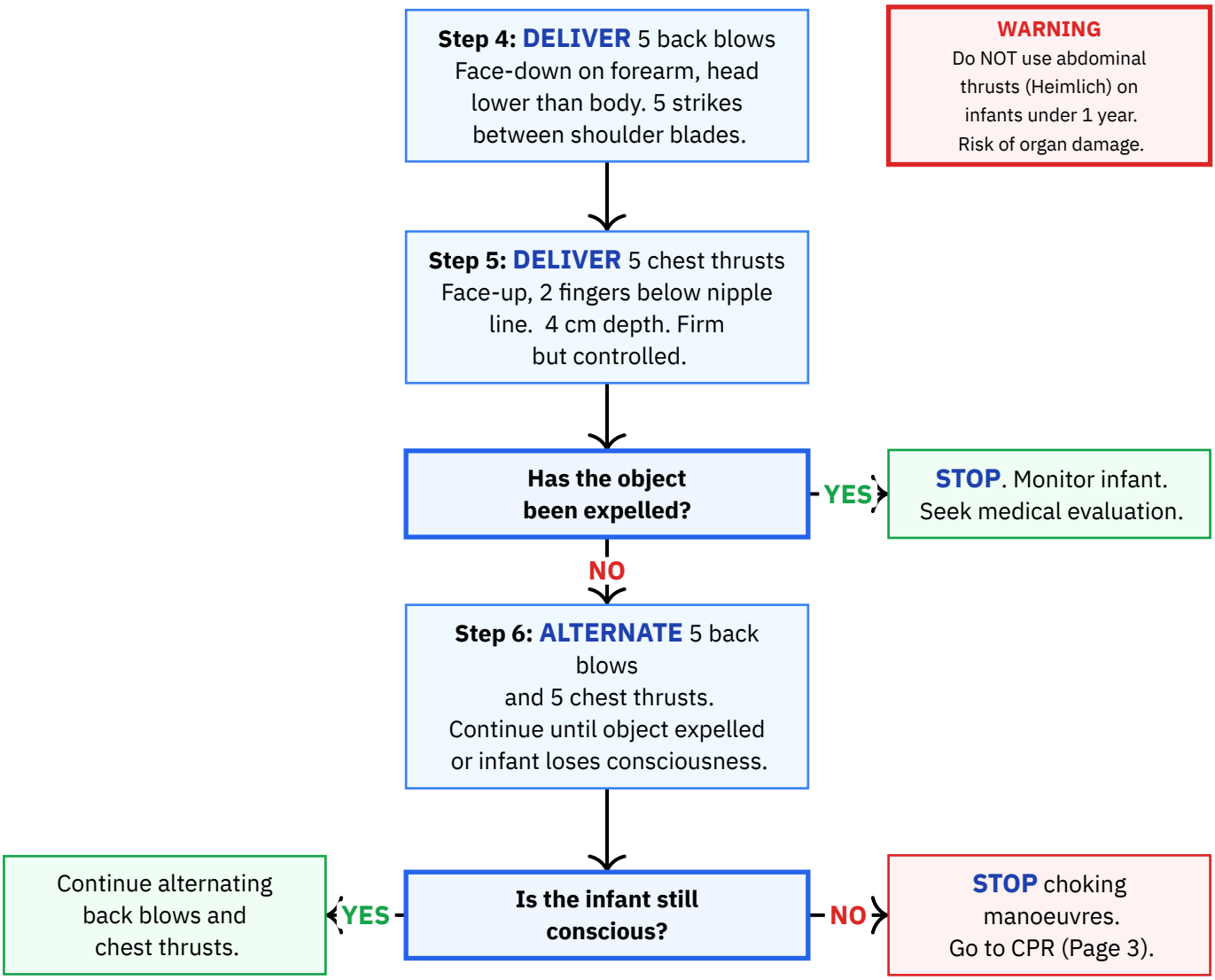
Child or infant is choking: inability to speak, cry, or make sounds; weak or absent cough; difficulty breathing; cyanosis (bluish skin); high-pitched wheezing. Note: young children may NOT grasp their throats.

Summary: Back blows and chest thrusts for infants (under 1 year); abdominal thrusts (Heimlich) for children (1–8 years); transition to CPR if victim becomes unresponsive.

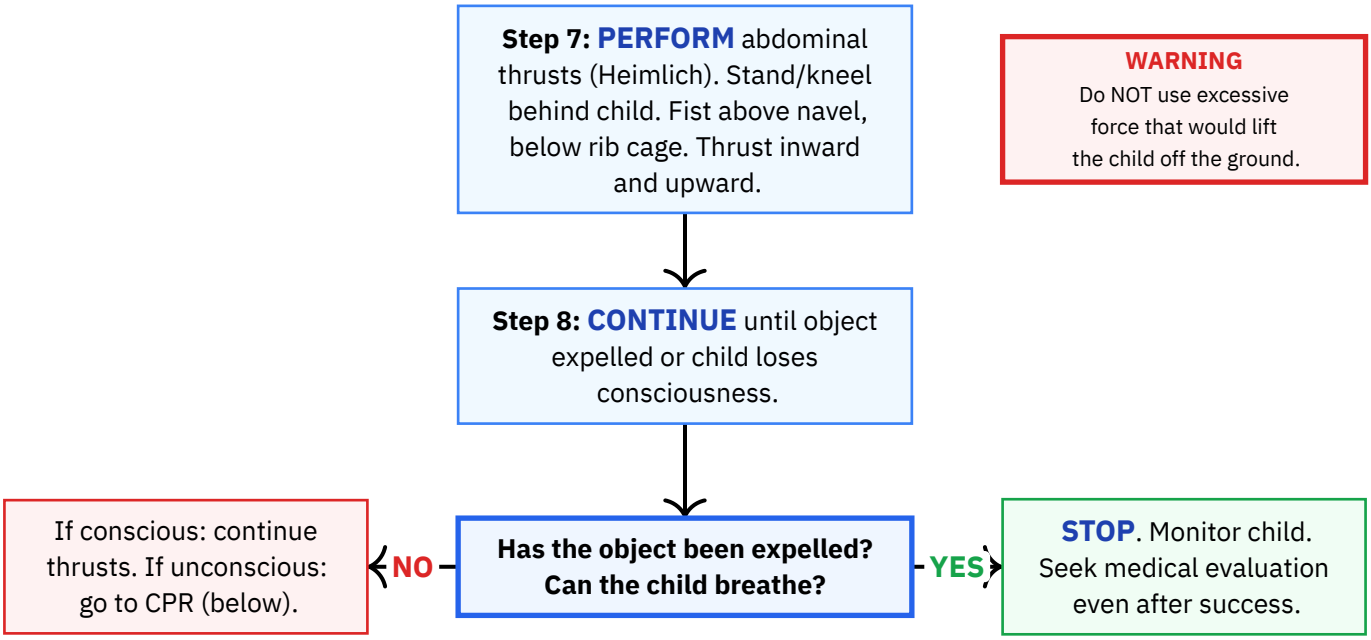
Assessment and Age Determination



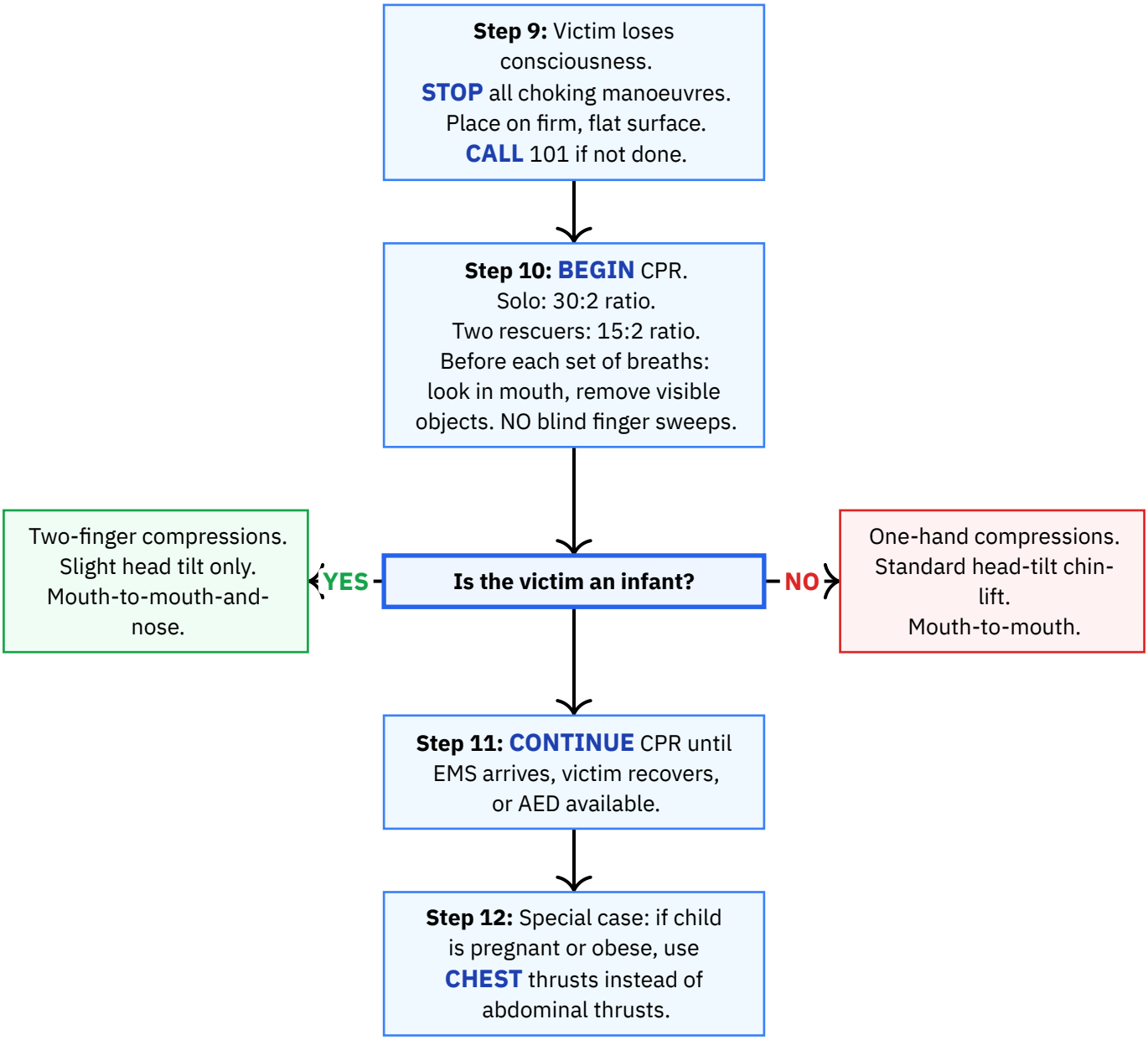
Infant Protocol (Under 1 Year)



Child Protocol (1–8 Years)



CPR Transition (Unconscious Victim)



DO NOT:

X DO NOT perform abdominal thrusts on infants under 1 year -- use only back blows and chest thrusts.
X DO NOT insert fingers blindly into the mouth (blind finger sweeps) -- only remove objects you can clearly see.
X DO NOT force anything into the mouth -- risk of pushing object deeper.
X DO NOT attempt mouth manipulation if teeth are clenched.
X DO NOT give food or fluids to a choking victim.
X DO NOT intervene if the child is coughing effectively -- allow the body to clear the obstruction.
X DO NOT use this protocol for choking caused by asthma, infection, or head injury.
X DO NOT use excessive force during abdominal thrusts that would lift the child off the ground.
X DO NOT fully extend an infant's head when opening the airway.

Equipment needed:

— AED with paediatric pads (if available, for CPR transition)

Source: Magen David Adom (MDA) — MDA 101 Public First Aid Guidance Series
URL: https://www.mdais.org/101/asphyxia
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Infant (0–1 year), child (1–8 years). Physical size should guide technique selection, not age alone.

CPR (Cardiopulmonary Resuscitation) — Child/Infant

Israel — CHILD

CALL 101 (MDA) — IF ALONE, DO 2 MIN CPR FIRST

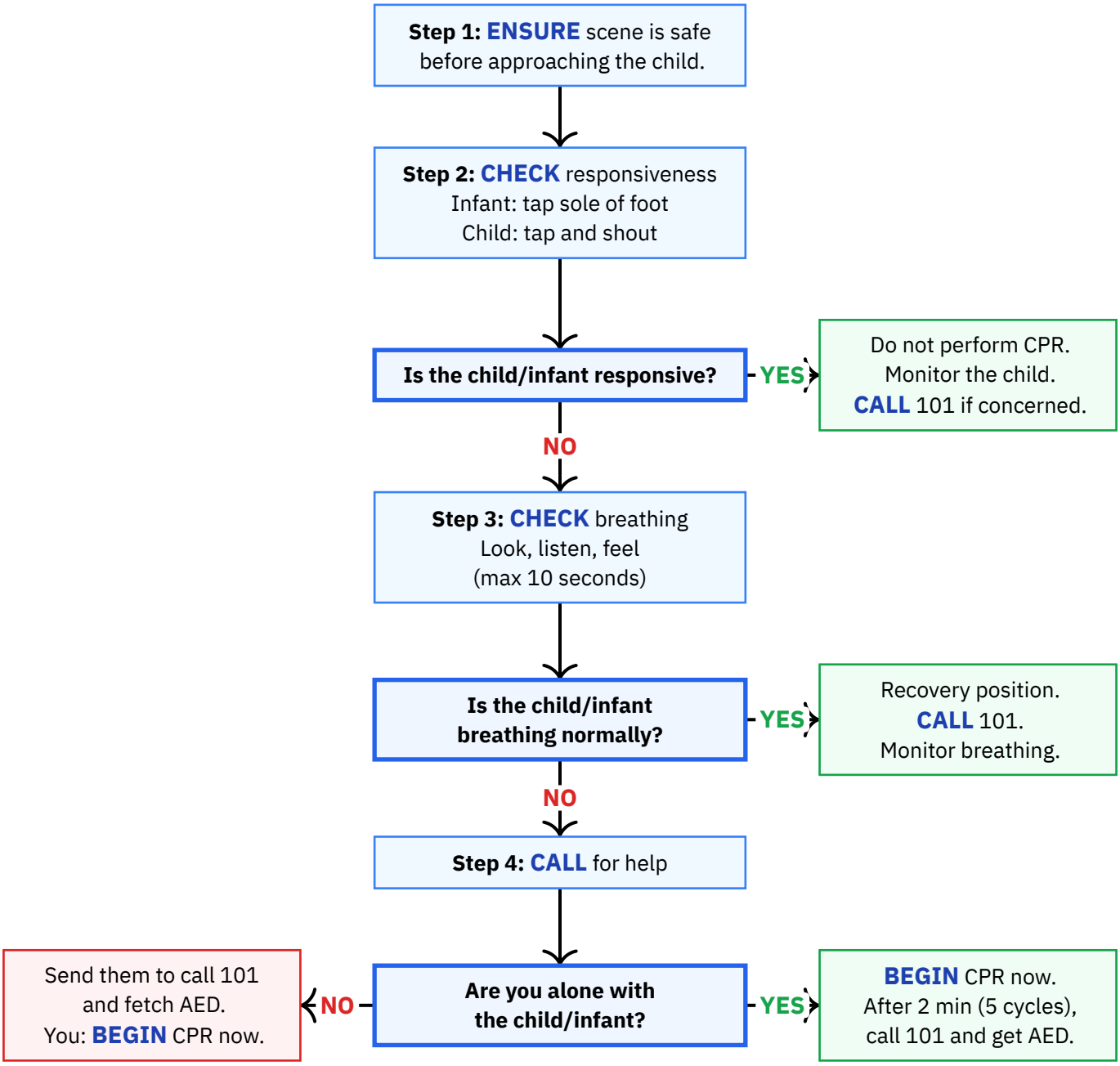
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Child (1 year to puberty) or infant (1 month to 1 year) is unresponsive and not breathing normally. In children, cardiac arrest is almost always secondary to respiratory failure – rescue breaths are critically important.

Summary: CPR protocol for unresponsive children/infants not breathing normally, following MDA guidelines aligned with ILCOR consensus. If alone, perform 2 minutes of CPR before calling 101.

Assessment Phase



CPR Technique (C-A-B)

Step 5: BEGIN chest compressions
Place child on firm, flat surface.
Rate: 100–120/min.
Allow full chest recoil.

Is the patient an infant (under 1 year)?

YES → Solo: 2-finger technique
Two rescuers: two-thumb encircling hands.
Depth: 4 cm (1/3 chest).

NO → Heel of one or two hands (size-dependent).
Depth: 5 cm (1/3 chest).

Step 6: OPEN airway

WARNING
Do NOT hyperextend an infant's neck.
Use neutral/sniffing position only.

Is the patient an infant (under 1 year)?

YES → Neutral/sniffing position.
Gentle chin lift only.

NO → Head-tilt chin-lift with mild extension.

Step 7: DELIVER 2 rescue breaths
1 second each, watch for chest rise.
Rescue breaths are ESSENTIAL in paediatric CPR.

WARNING
Compression-only CPR is NOT adequate for children/infants.
Arrest is almost always respiratory in origin.

Is the patient an infant?

YES → Seal mouth over infant's mouth AND nose.
Gentle puffs from cheeks.

NO → Pinch nose, seal mouth.
Breathe until chest rise.

CPR Cycles and Continuation

Step 8: CHECK chest rise
If no rise, reposition airway and reattempt. Consider foreign body obstruction.

↓

Does the chest rise with rescue breaths?

- **NO** → Reposition head/airway. Reattempt. If still no rise: suspect foreign body.
- **YES** → Continue CPR cycles.

↓

Step 9: CONTINUE CPR cycles

↓

Are there two or more rescuers?

- **NO** → Use 30:2 ratio.
- **YES** → Use 15:2 ratio. Infants: switch to two-thumb technique.

↓

Step 10: APPLY AED when available. Use paediatric pads if available. If only adult pads, ensure no overlap.

WARNING
If adult pads overlap on small child/infant: use anteroposterior placement (front/back).

↓

Step 11: After AED shock, **RESUME** CPR immediately. 5 cycles before reassessing.

↓

Step 12: CONTINUE CPR until: MDA arrives, child starts breathing/responding, or you are physically unable.

↓

Has the child started breathing and responding?

- **NO** → **CONTINUE** CPR without interruption.
- **YES** → **STOP** CPR. Recovery position. Monitor until MDA arrives.

Reference

Key Paediatric CPR Parameters

Compression rate: 100–120 / min
Infant depth: 4 cm (1/3 chest)
Two-rescuer ratio: 15 : 2
Lone rescuer: 2 min CPR, then call 101
Child depth: 5 cm (1/3 chest)
Solo ratio: 30 : 2
Breath duration: 1 second each
Emergency number: 101 (MDA)

DO NOT:

X Do NOT hyperextend an infant's neck when opening the airway -- use neutral/sniffing position only.
X Do NOT insert fingers into the infant's or child's mouth (risk of pushing objects deeper).
X Do NOT deliver deep lung breaths to infants -- use gentle puffs from your cheeks.
X Do NOT use excessive volume or pressure when delivering rescue breaths (risk of gastric distension).
X Do NOT skip rescue breaths in paediatric CPR -- compression-only CPR is NOT adequate for children and infants.
X Do NOT delay CPR to call for help when alone -- perform 2 minutes of CPR first, then call 101.
X Do NOT allow AED pads to touch or overlap each other on a small child or infant.
X Do NOT stop CPR to check for pulse unless there are clear signs of recovery (breathing, movement).
X Do NOT compress over the xiphoid process (the very bottom tip of the sternum).

Equipment needed:

— AED with paediatric pads or paediatric dose attenuator (if available)
— Barrier device or pocket mask (optional, for rescue breaths)
— Bag-valve-mask -- infant size (250-350 cc) or paediatric size (if available)

Source: Magen David Adom (MDA) — BLS Guide 2016 / ALS Professional Handbook 2024
URL: https://www.mdais.org/kursim/cpr-children
Publication date: 2024-04-01 · **Edition:** BLS Guide 2016 / ALS Professional Handbook 2024
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Both aligned on C-A-B sequence, compression depths, ratios, and AED guidance. If alone with the child, perform 2 minutes of CPR before calling 101.

Dehydration First Aid -- Child

Israel — CHILD

CALL 101 (MDA) FOR SEVERE DEHYDRATION

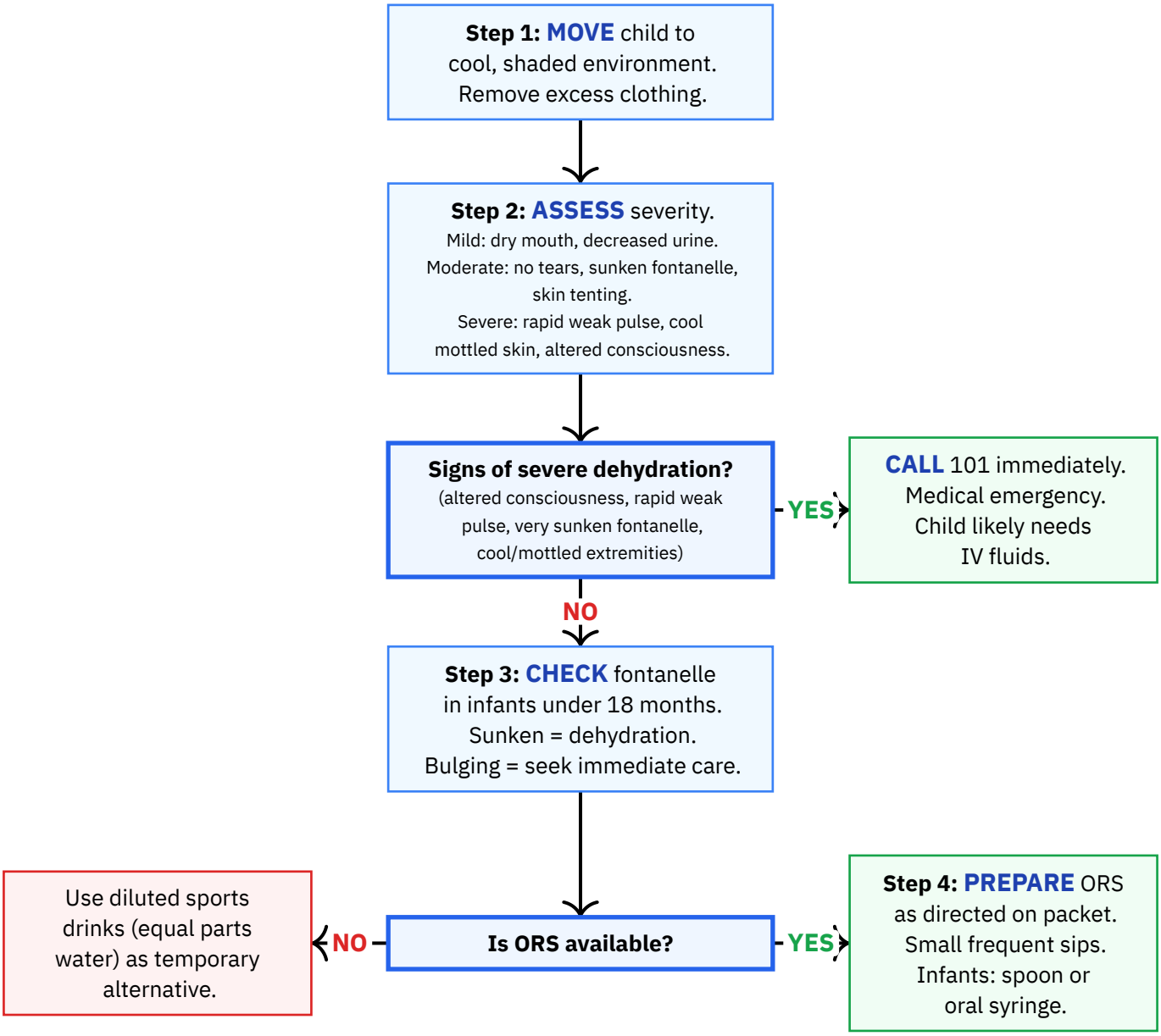
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

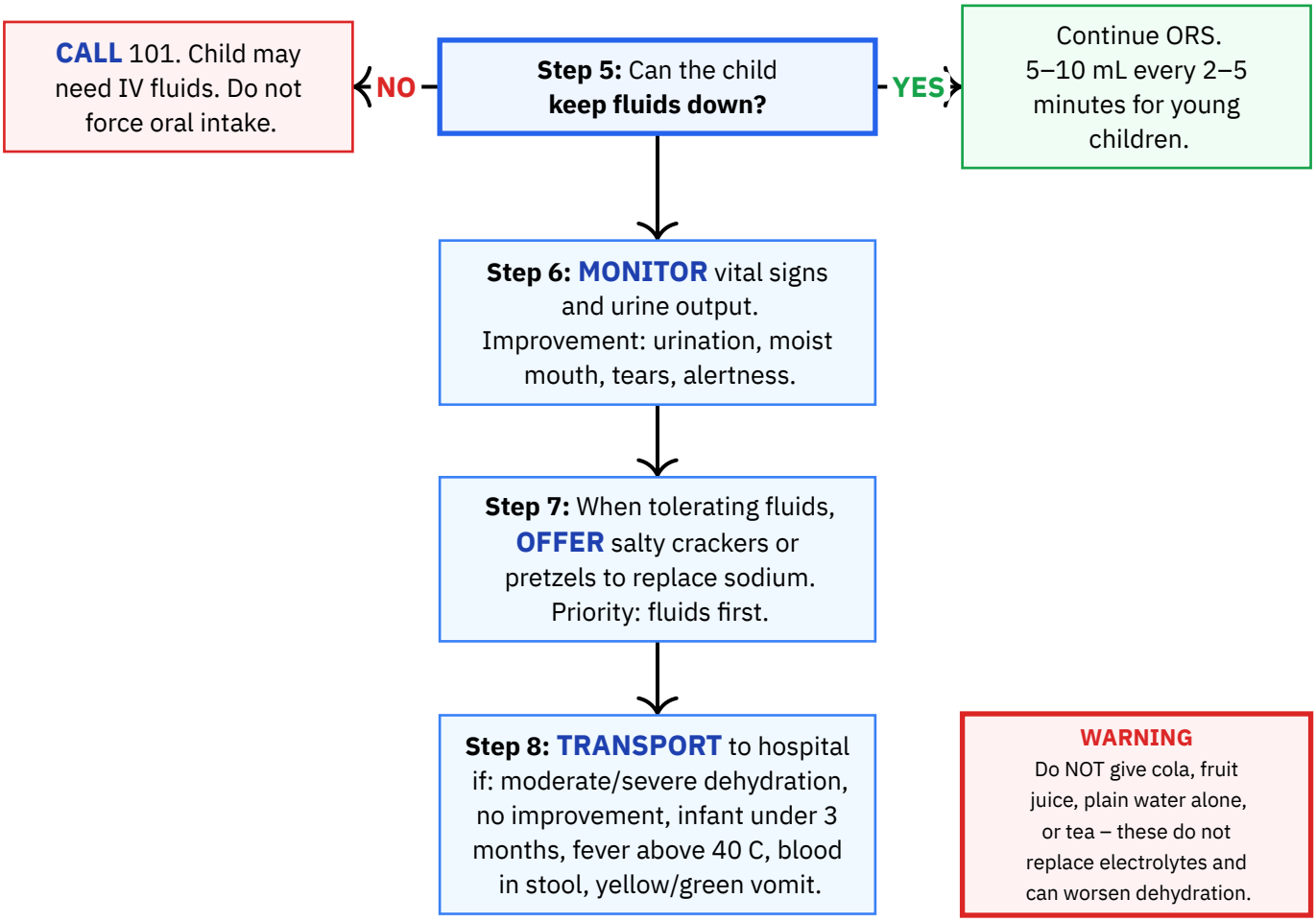
When a child shows signs of dehydration from any cause: insufficient fluid intake, excessive sweating, vomiting, diarrhoea, or fever. Particularly relevant during Israeli summer (May–October), sharav heat waves, school field trips in desert areas (Negev, Arava, Dead Sea), and during childhood gastroenteritis.

Summary: Move to cool environment. Assess severity. Check fontanelle in infants. Oral rehydration with ORS (not cola, juice, or plain water). Small frequent sips. Hospital for severe cases, all infants under 3 months, or persistent vomiting.

Assessment and Rehydration – Steps 1-5



Fluid Tolerance and Transport – Steps 5-8



DO NOT:

X Do NOT give cola, fruit juice, plain water alone, or tea -- do not replace electrolytes adequately.
X Do NOT use homemade salt/sugar solutions unless ORS is completely unavailable.
X Do NOT force large volumes of fluid at once -- small frequent sips are more effective.
X Do NOT delay calling 101 for an infant under 3 months with any signs of dehydration.
X Do NOT ignore a sunken fontanelle in an infant -- key dehydration indicator.
X Do NOT give anti-diarrhoea medications to children without medical direction.
X Do NOT stop breastfeeding -- continue breastfeeding alongside ORS.

Equipment needed:

— ORS (Oral Rehydration Solution) packets
— Spoon or oral syringe (for infant fluid administration)
— Phone (to call 101)
— Thermometer (to check for fever)

Source: Magen David Adom (MDA) – supplemented by Ichud Hatzalah hot climate injuries and Hatzalah Without Borders dehydration guide
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah sources. Israeli climate context: sharav heat waves, Negev field trips, summer months May–October.

Diabetic Emergency First Aid -- Child

Israel — CHILD

CALL 101 (MDA) — WHEN IN DOUBT, GIVE SUGAR

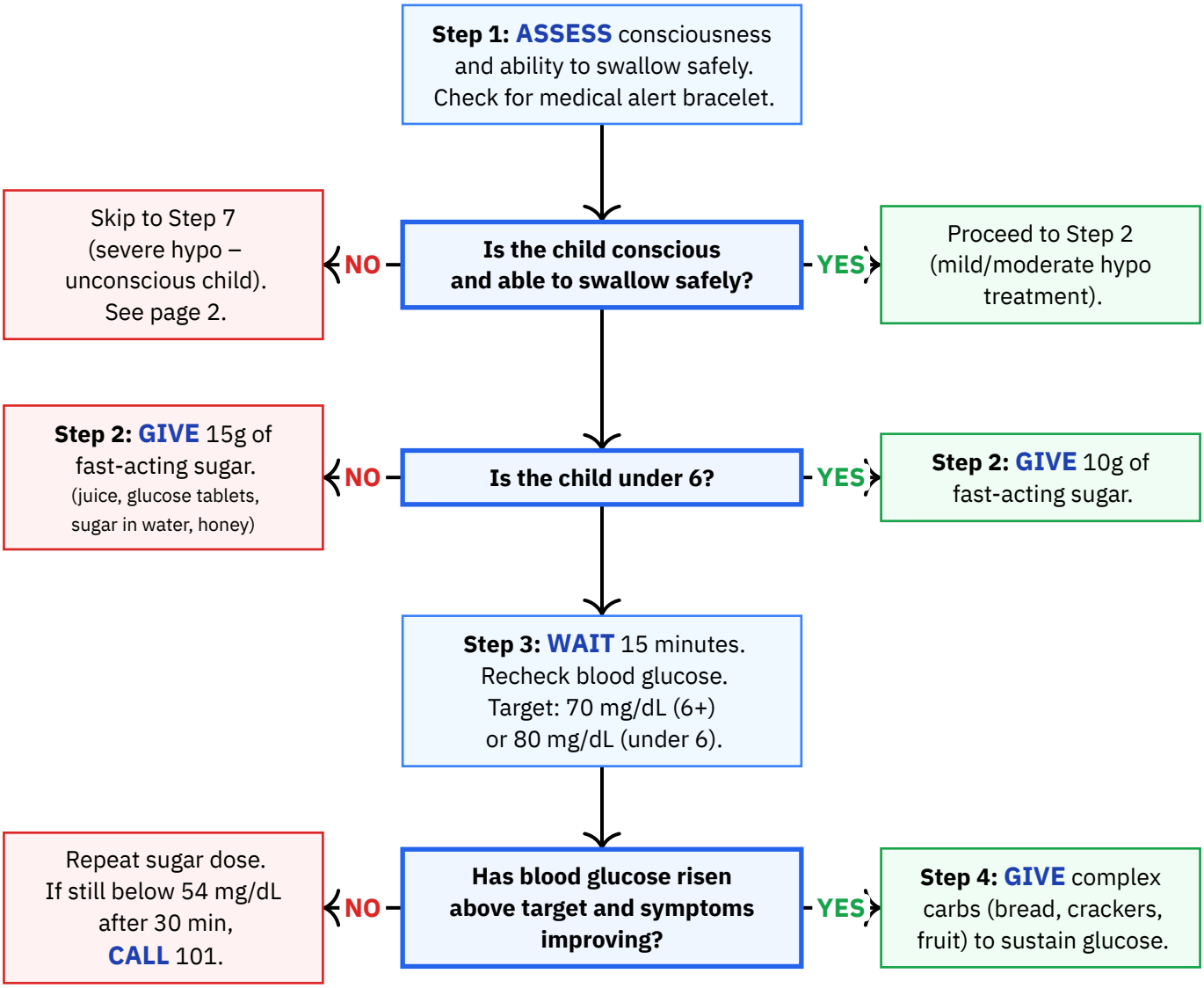
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

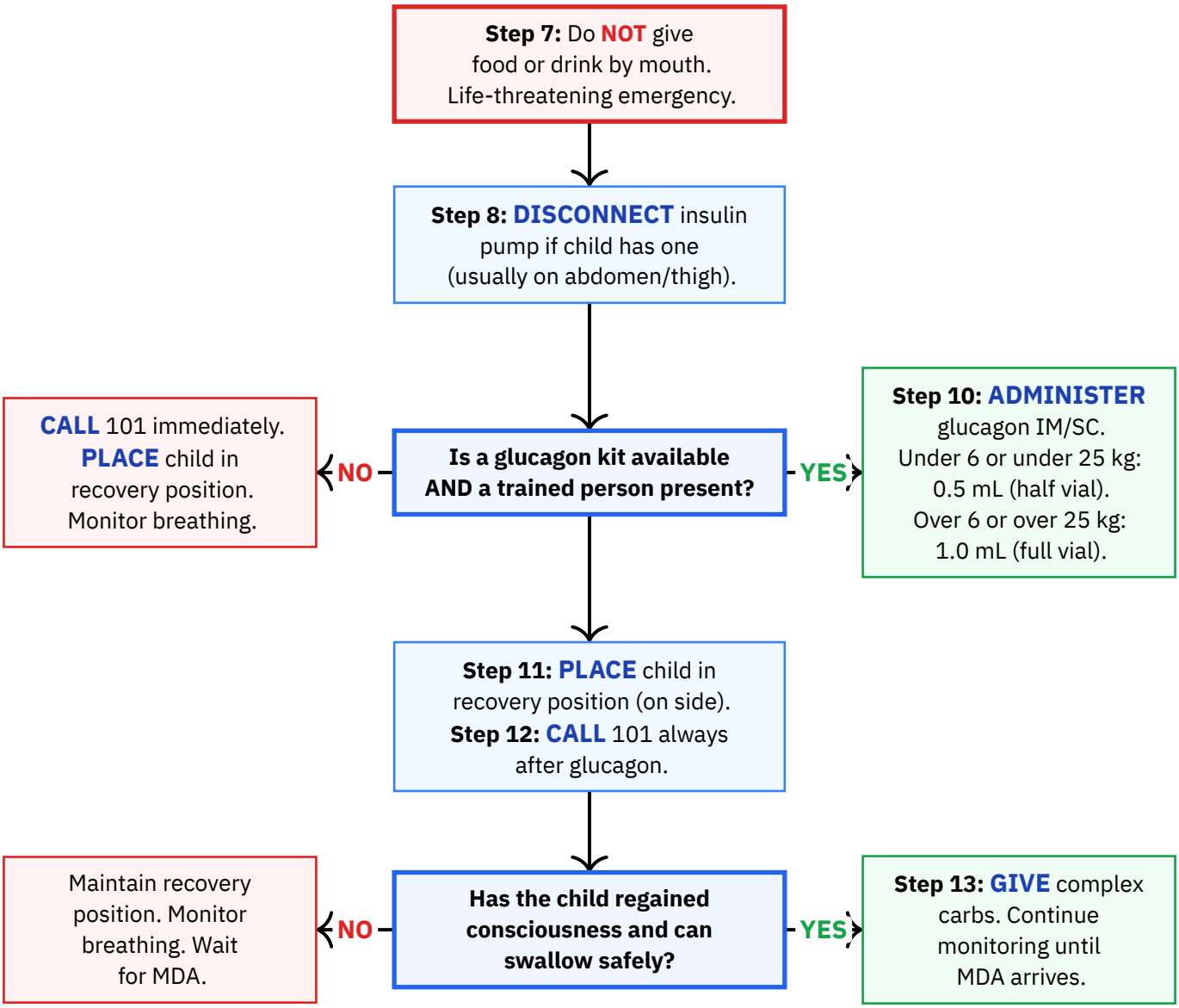
Child with known or suspected diabetes showing signs of hypoglycemia (paleness, sweating, trembling, rapid heartbeat, irritability, confusion, seizures) or hyperglycemia/DKA (excessive thirst, vomiting, rapid deep breathing, fruity breath, lethargy).

Key Principle: When in doubt whether hypoglycemia or hyperglycemia: **give sugar**. Sugar saves a hypoglycemic child's life. Sugar will not cause immediate harm to a hyperglycemic child.

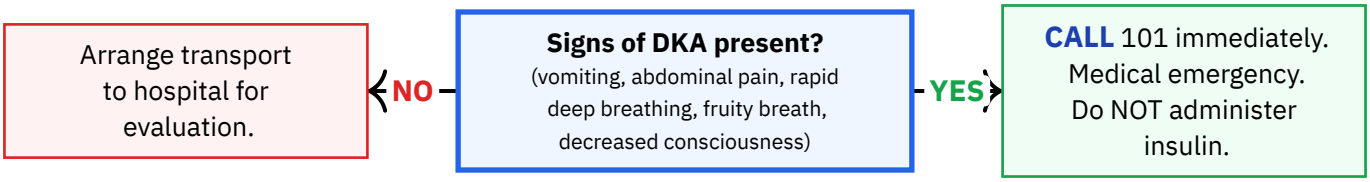
Assessment and Mild Hypoglycemia



Severe Hypoglycemia – Unconscious Child



DKA Assessment (Step 14)



Reference

Key Parameters – Rule of 15

Under 6: 10g sugar, target 80 mg/dL
Reassess interval: 15 minutes
Glucagon (under 6 / under 25 kg): 0.5 mL
Glucagon onset: 10–15 minutes

6 and over: 15g sugar, target 70 mg/dL
Call 101 if: below 54 mg/dL after 30 min
Glucagon (6+ / over 25 kg): 1.0 mL
Glucagon duration: 30 minutes

Step 15: When in Doubt – GIVE SUGAR

Giving sugar to a hypoglycemic child is **life-saving**.
Giving sugar to a hyperglycemic child will **not cause immediate harm**.
Withholding sugar from a hypoglycemic child **can be fatal**.

Only if conscious and can swallow safely. Never give anything by mouth to an unconscious child.

DO NOT:

X DO NOT administer insulin -- strictly a medical intervention, can be fatal if given inappropriately.
X DO NOT give food, drink, or oral medication to an unconscious or seizing child -- choking hazard.
X DO NOT use chocolate as initial sugar source -- fat/protein slows absorption.
X DO NOT administer glucagon unless trained and authorised.
X DO NOT delay calling 101 for severe hypoglycemia, seizures, or loss of consciousness.
X DO NOT attempt to manage hyperglycemia/DKA with first aid -- requires medical intervention.
X DO NOT use diet drinks or sugar-free products as a sugar source.

Equipment needed:

— Glucose gel syringe (15g glucose)
— Fast-acting sugar source (juice, glucose tablets, sugar cubes, honey)
— Complex carbohydrate snack (bread, crackers, fruit)
— Glucagon emergency kit (Hypo-Kit) if available and authorised to use
— Blood glucose meter (glucometer) if available

Source: Magen David Adom (MDA) – How to Treat a Diabetes Patient, corroborated by Shamir Medical Center paediatric endocrinology guidance
URL: https://www.mdais.org/101/diabetes
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Child-specific dosing (Rule of 15 with 10g modification for under-6) sourced from MDA-aligned Israeli hospital guidance.

Drowning — Child

Israel — CHILD

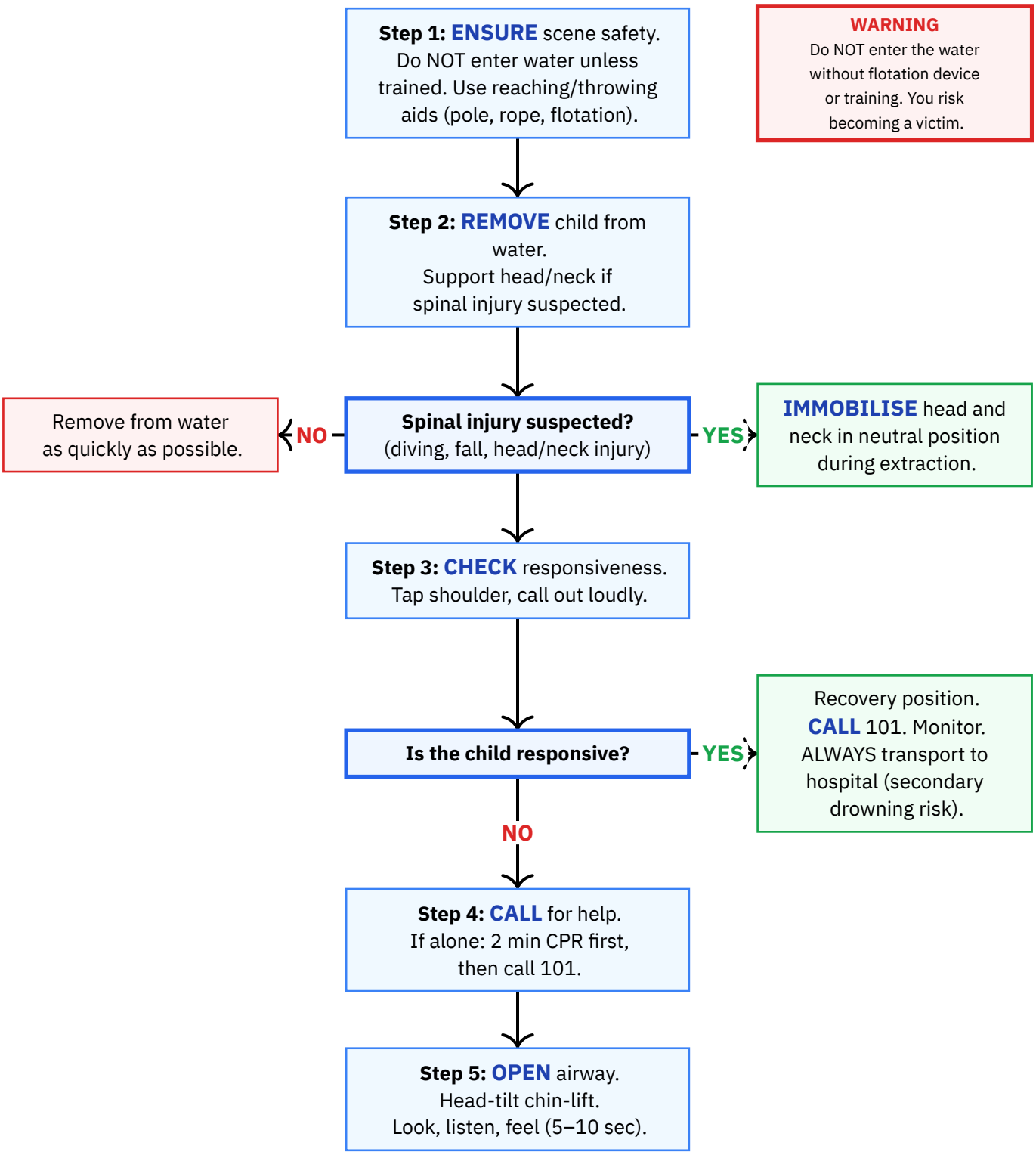
CALL 101 — RESCUE BREATHS ARE ESSENTIAL

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

A child has been submerged or immersed in water and is unresponsive, not breathing, or breathing inadequately. Drowning is the 2nd leading cause of unintentional death in children under 18 in Israel; 83% of drownings in children under 10 occur in swimming pools.

Rescue and Assessment (Steps 1–5)



CPR and Recovery (Steps 6–10)

Step 6: GIVE 2 initial rescue breaths.
Pinch nose, seal mouth.
Watch for chest rise.

WARNING
Compression-only CPR is INSUFFICIENT for drowning. Rescue breaths are ESSENTIAL – drowning is a hypoxic emergency.

Does the chest rise?

- **NO** → Reposition head. Retry. If still no rise: proceed to compressions.
- **YES** → Effective. Proceed to compressions.

Step 7: BEGIN chest compressions.
Heel of one hand, centre of chest. Depth: 1/3 to 1/2 chest.
Rate: 100–120/min. Full recoil.

Step 8: CONTINUE CPR cycles.
Solo: 30:2. Two rescuers: 15:2.
Switch every 2 min if two rescuers. Minimise pauses.

Does the child show signs of life (breathing, coughing)?

- **NO** → **CONTINUE** CPR until MDA arrives, AED available, or physically unable.
- **YES** → **STOP** compressions. Recovery position (Step 9). Monitor until MDA arrives.

Step 9: Recovery position: on side, upper knee bent.
Maintain warmth – hypothermia risk high.

WARNING
ALWAYS transport to hospital, even if child appears to recover. Secondary drowning can develop hours later.

Step 10: Hospital transport is MANDATORY. Secondary drowning and electrolyte imbalances can occur hours later.

DO NOT:

X DO NOT attempt to remove water from the lungs -- impossible and dangerous.
X DO NOT perform abdominal thrusts (Heimlich) to expel water -- not effective, risks aspiration.
X DO NOT delay CPR to attempt water removal.
X DO NOT use compression-only CPR -- rescue breaths are essential for drowning.
X DO NOT assume the child is fine if they regain consciousness -- always transport to hospital.
X DO NOT enter the water unless you are a trained rescuer with flotation device.
X DO NOT use excessive ventilation force -- risk of gastric distention in children.

Equipment needed:

— Reaching aid (pole, rope, towel) or flotation device
— Firm, flat surface for CPR
— Blanket or covering (warmth, hypothermia prevention)

Source: Magen David Adom (MDA) — MDA 101 Public First Aid Guidance; MDA 2024 drowning statistics
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Drowning is the 2nd leading cause of unintentional death in children under 18 in Israel (after traffic accidents). 83% of drownings in children under 10 occur in swimming pools.

Electrocution — Child

Israel — CHILD

DO NOT TOUCH CHILD WHILE CONNECTED TO SOURCE — CALL 101

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Electric Co:** 103

When to apply:

Child has sustained electric shock from any source: household outlet (220V in Israel), electrical cord (oral contact/biting in toddlers), faulty appliance, exposed wiring, or high-voltage source. Signs: burns at contact points, loss of consciousness, muscle rigidity, difficulty breathing, cardiac arrest.

Scene Safety and Separation (Steps 1–3)

WARNING
Touching the child while connected causes chain electrocution. You will become a victim.

Step 1: STOP – do NOT touch the child while connected to electrical source.

↓

Step 2: DISCONNECT the source.

↓

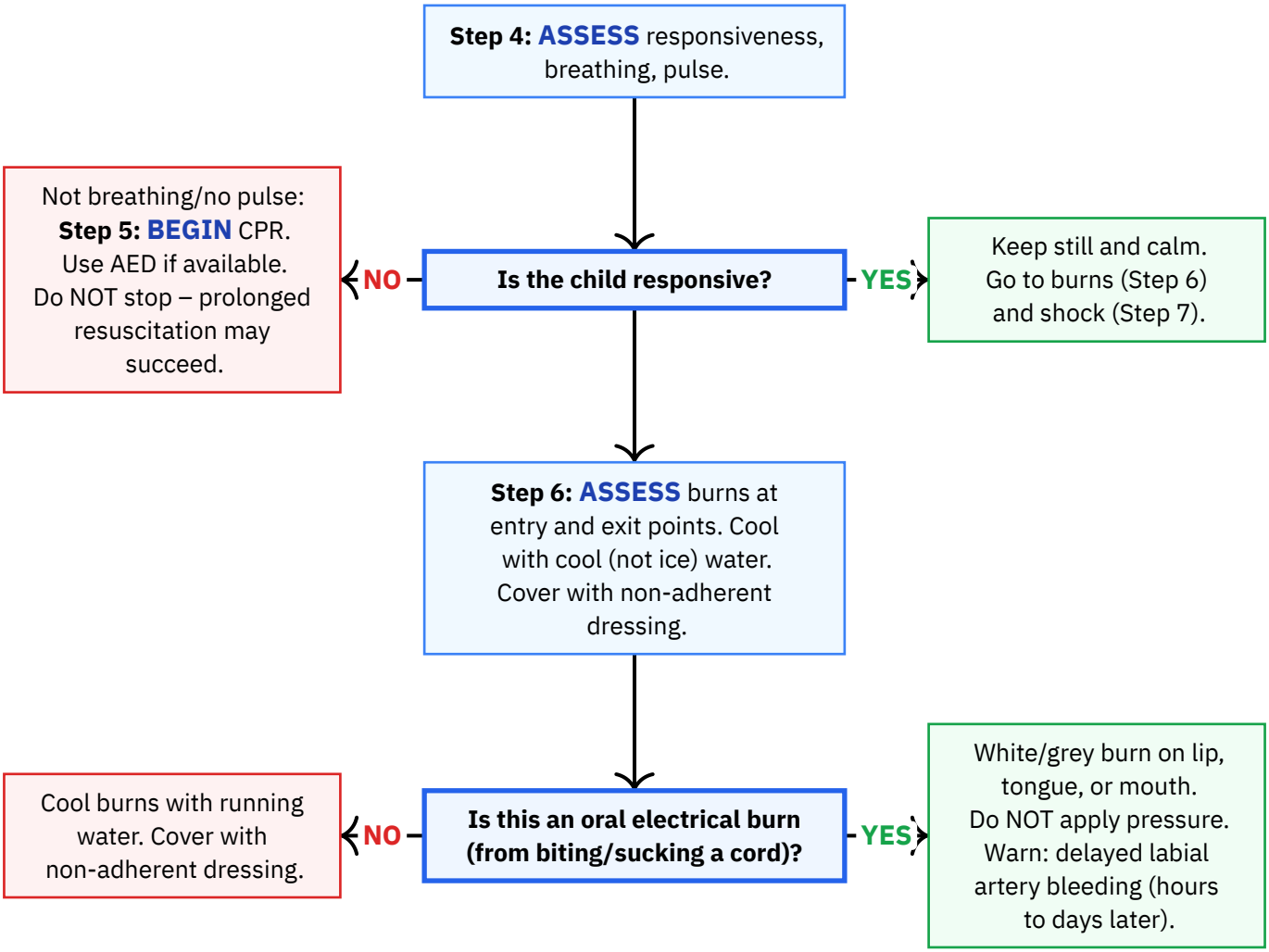
Is this a high-voltage source (power lines, transformer)?

- **NO** → Turn off circuit breaker or unplug appliance. If not possible: use dry non-conductive object (wooden broom, plastic chair).
- **YES** → Do NOT approach. **CALL** 101, 102, 103. Maintain safe distance. Wait for emergency services.

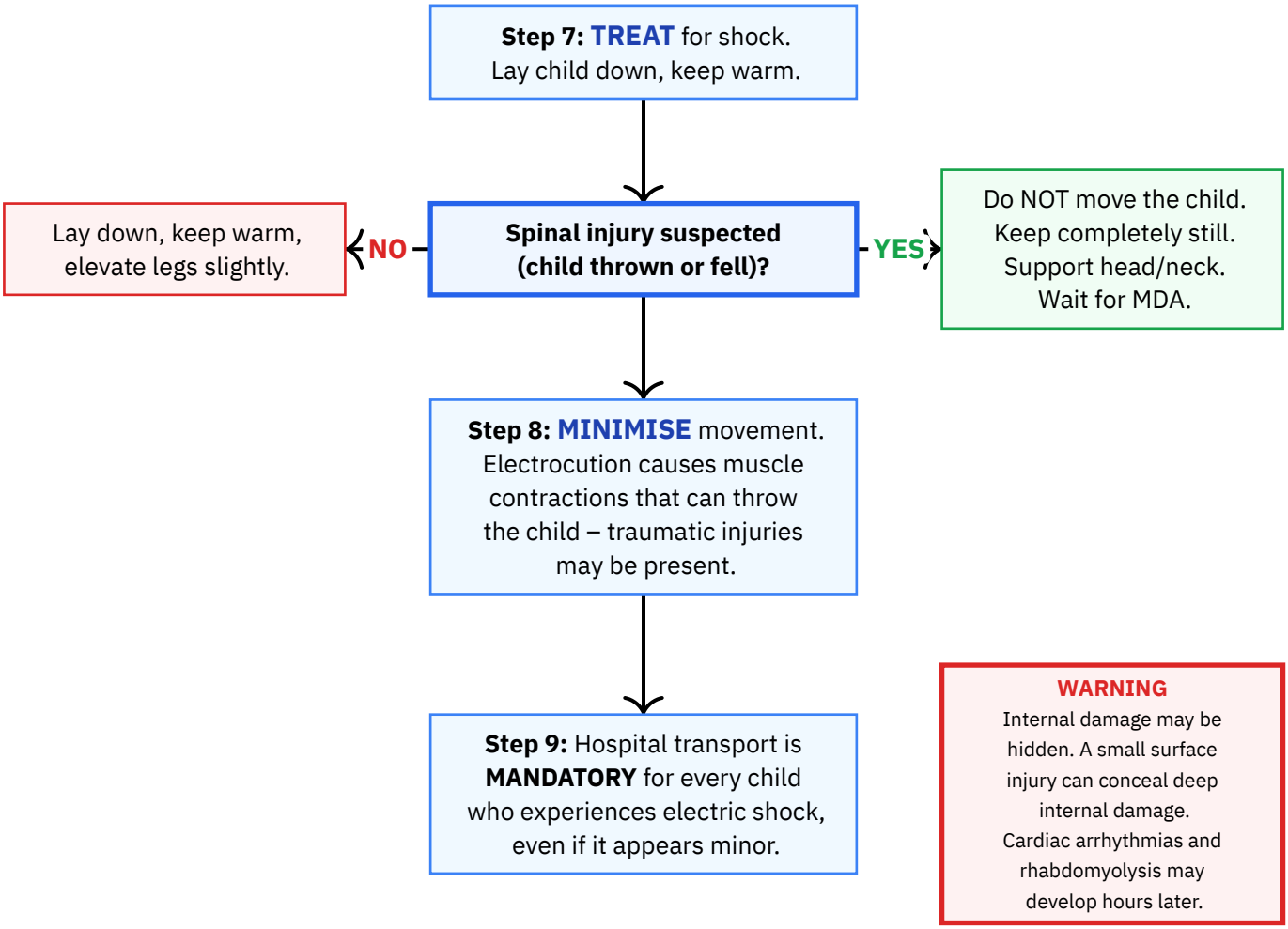
↓

Step 3: CALL 101 immediately. Provide: phone number, address, child's age, what happened, current condition.

Assessment and Treatment (Steps 4–6)



Shock and Transport (Steps 7–9)



DO NOT:

- X Do NOT touch the child while connected to the electrical source -- chain electrocution.
- X Do NOT use metal objects or wet/damp materials to separate from source.
- X Do NOT apply ice to electrical burns.
- X Do NOT apply creams, ointments, toothpaste, or home remedies to electrical burns.
- X Do NOT assume the child is fine because external injuries appear minor.
- X Do NOT delay calling 101 -- every electrocution requires hospital assessment.
- X Do NOT move the child unnecessarily -- spinal/traumatic injuries may be present.
- X Do NOT approach high-voltage sources -- call 101, 102, and 103.
- X Do NOT stop CPR based on apparent lack of vital signs -- prolonged resuscitation may succeed.
- X Do NOT give food or drink to a child in shock after electrocution.

Equipment needed:

— Non-conductive object (wooden broom handle, dry stick, plastic chair)
— AED / defibrillator (with paediatric pads if available)
— Cool running water (for burn cooling)
— Sterile non-adherent dressings (for burns)
— Blanket (for warmth and shock treatment)

Source: Magen David Adom (MDA) — MDA Electrocution First Aid Guidelines
URL: https://www.mdais.org/101/electrocution
Publication date: 2026-01-01 · **Imported:** 2026-03-15 · **Last verified:** 2026-03-15
Notes: Israeli household supply is 220V. Key child concerns: oral burns from cord-biting (under 6), delayed labial artery bleeding, lower body resistance increasing severity. Hospital evaluation mandatory for all cases.

Eye Injuries First Aid -- Child

Israel — CHILD

CALL 101 (MDA) — DO NOT RUB THE EYE

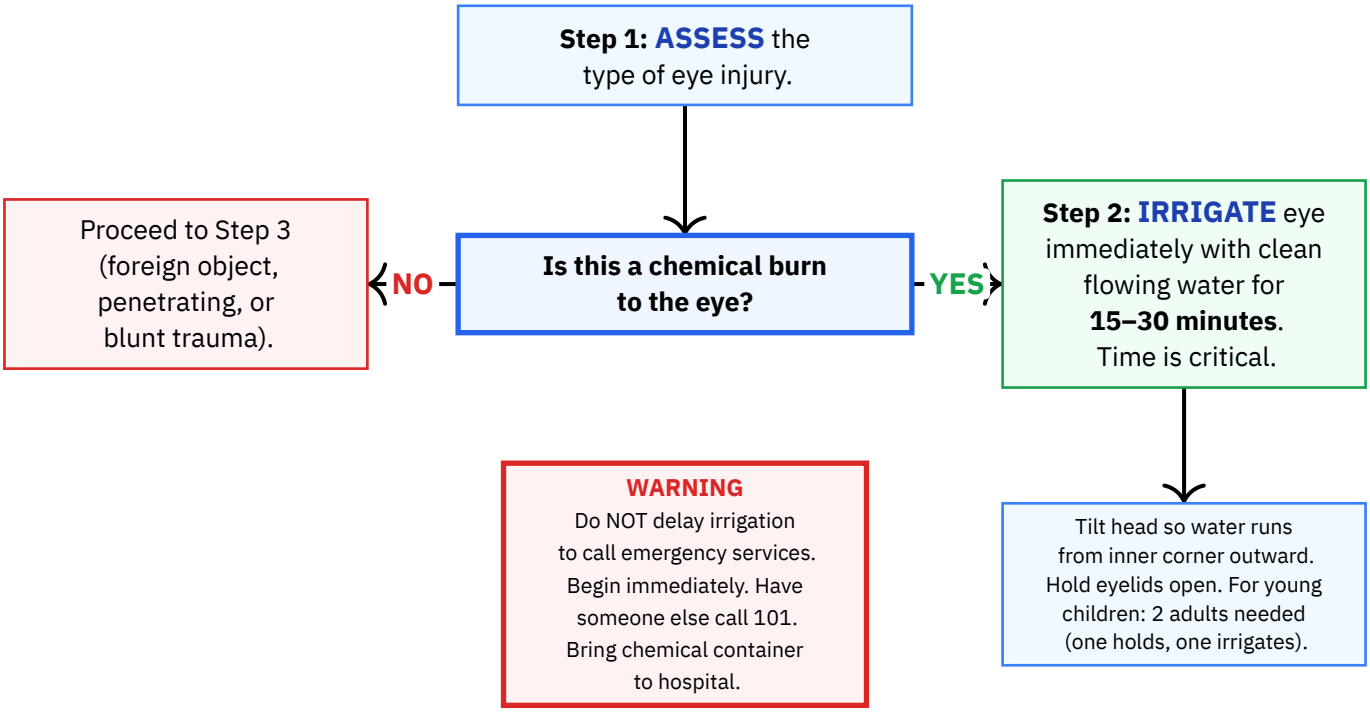
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

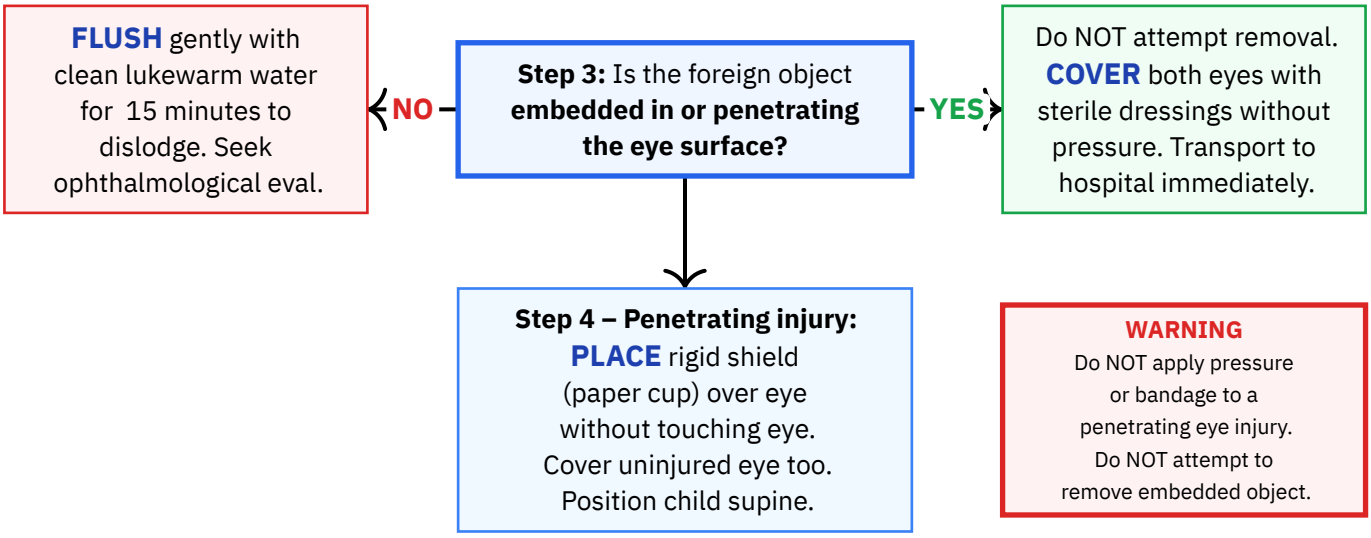
When a child has sustained any eye injury: chemical splash, foreign object, penetrating injury, blunt trauma, or thermal/UV burn. Common causes in Israeli children: playground injuries (sand, sticks), household chemicals (bleach, cleaners), toy-related injuries.

Summary: Chemical burns: irrigate immediately for 15–30 minutes. Foreign objects: flush if superficial, do NOT remove if embedded. Penetrating injuries: rigid shield, no pressure. Blunt trauma: cold compress gently.

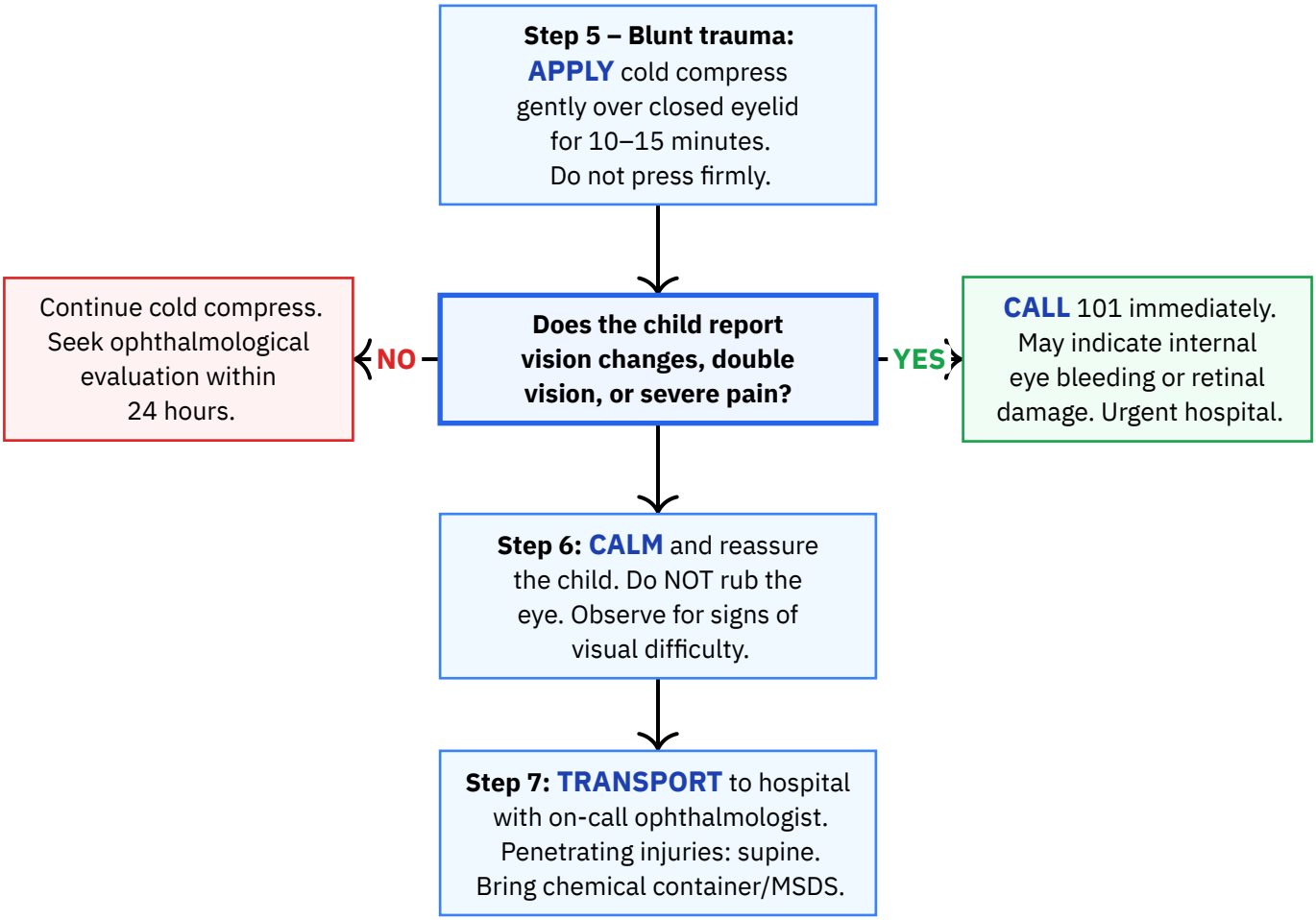
Assessment and Chemical Burns – Steps 1-2



Foreign Objects and Penetrating Injuries – Steps 3-4



Blunt Trauma and Care – Steps 5-7



DO NOT:

X Do NOT rub the injured eye -- worsens damage from foreign objects and chemical burns.
X Do NOT apply pressure or bandages to a penetrating eye injury.
X Do NOT attempt to remove an embedded object from the eye.
X Do NOT delay irrigation for chemical burns -- begin immediately.
X Do NOT use any eye drops or ointments without medical direction.
X Do NOT allow the child to rub or touch the injured eye.
X Do NOT apply antiseptic or alcohol to the eye.

Equipment needed:

— Clean water source (tap water or sterile water for irrigation)
— Rigid eye shield or paper cup
— Sterile gauze or clean cloth
— Cold compress or ice wrapped in cloth
— Phone (to call 101)

Source: Magen David Adom (MDA) – supplemented by Hatzalah Without Borders eye injury protocols (November 2021)
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah/Hatzalah Without Borders sources. Chemical burn irrigation duration 15–30 minutes per Hatzalah guidance.

Fractures and Sprains — Child

Israel — CHILD

CALL 101 FOR OPEN FRACTURES, SPINAL INJURY, OR SEVERE DEFORMITY

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Suspected fracture or sprain: severe pain worsening with movement, visible swelling/bruising, limb deformity, inability to move or bear weight, tenderness, audible snap at injury. In young children: sudden refusal to use arm/leg, inconsolable crying. Note: sprains in children may actually be growth plate fractures.

Assessment (Steps 1–4)

Step 1: **CALM** the child. Speak at eye level. Explain simply. Use distraction. Involve parent/caregiver.

↓

Step 2: **ASSESS** injury severity.

↓

Is bone visible through skin (open/compound fracture)?

- YES → Medical emergency. **CALL** 101 immediately. Go to Step 8 (open fracture management).

NO ↓

Step 3: **CALL** 101 if emergency criteria met (open fracture, spinal injury, severe deformity, loss of sensation/circulation).

↓

Suspected spinal or neck injury?

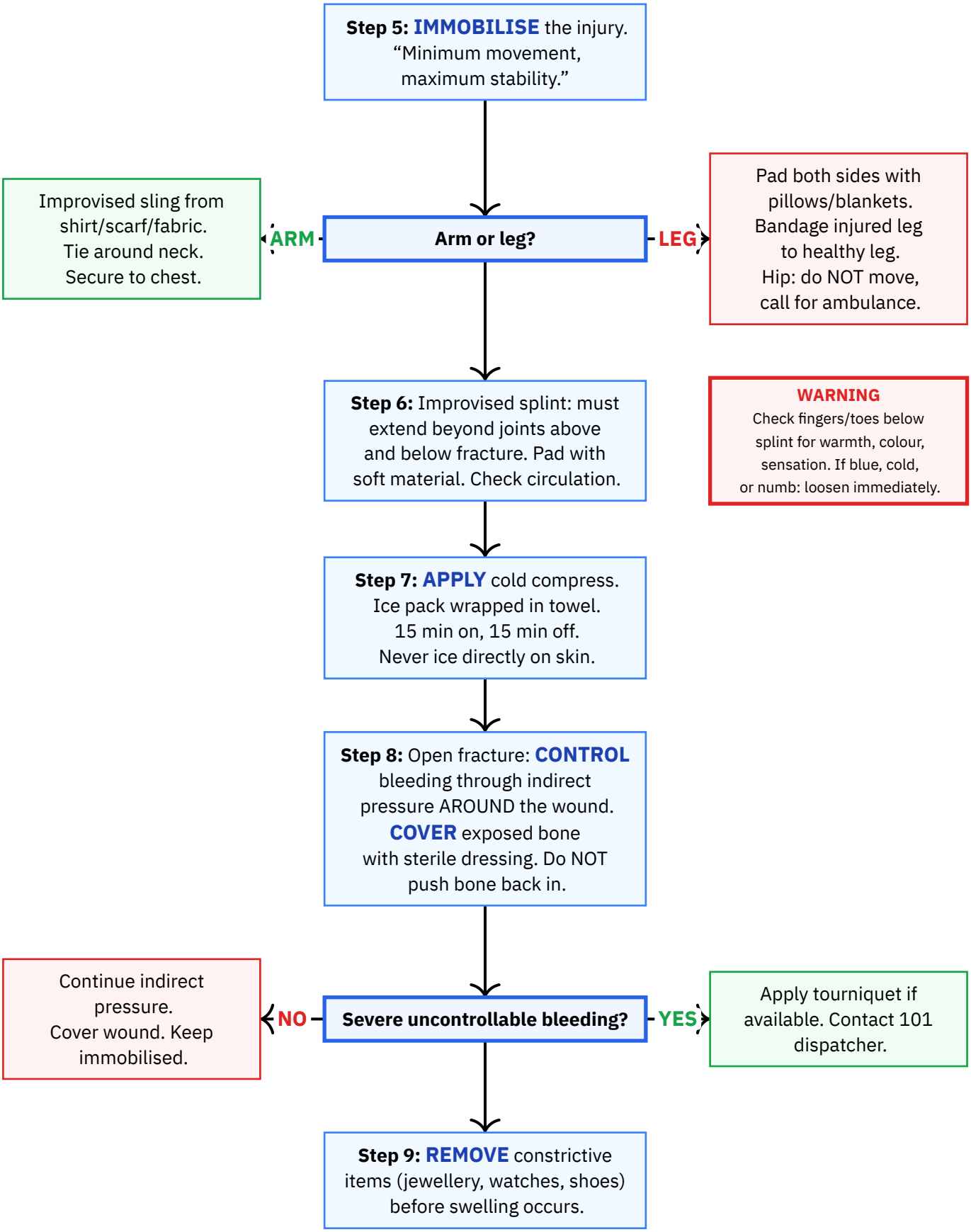
- YES → Do NOT move the child. **CALL** 101 immediately. Hold head steady. Wait for professionals.

NO ↓

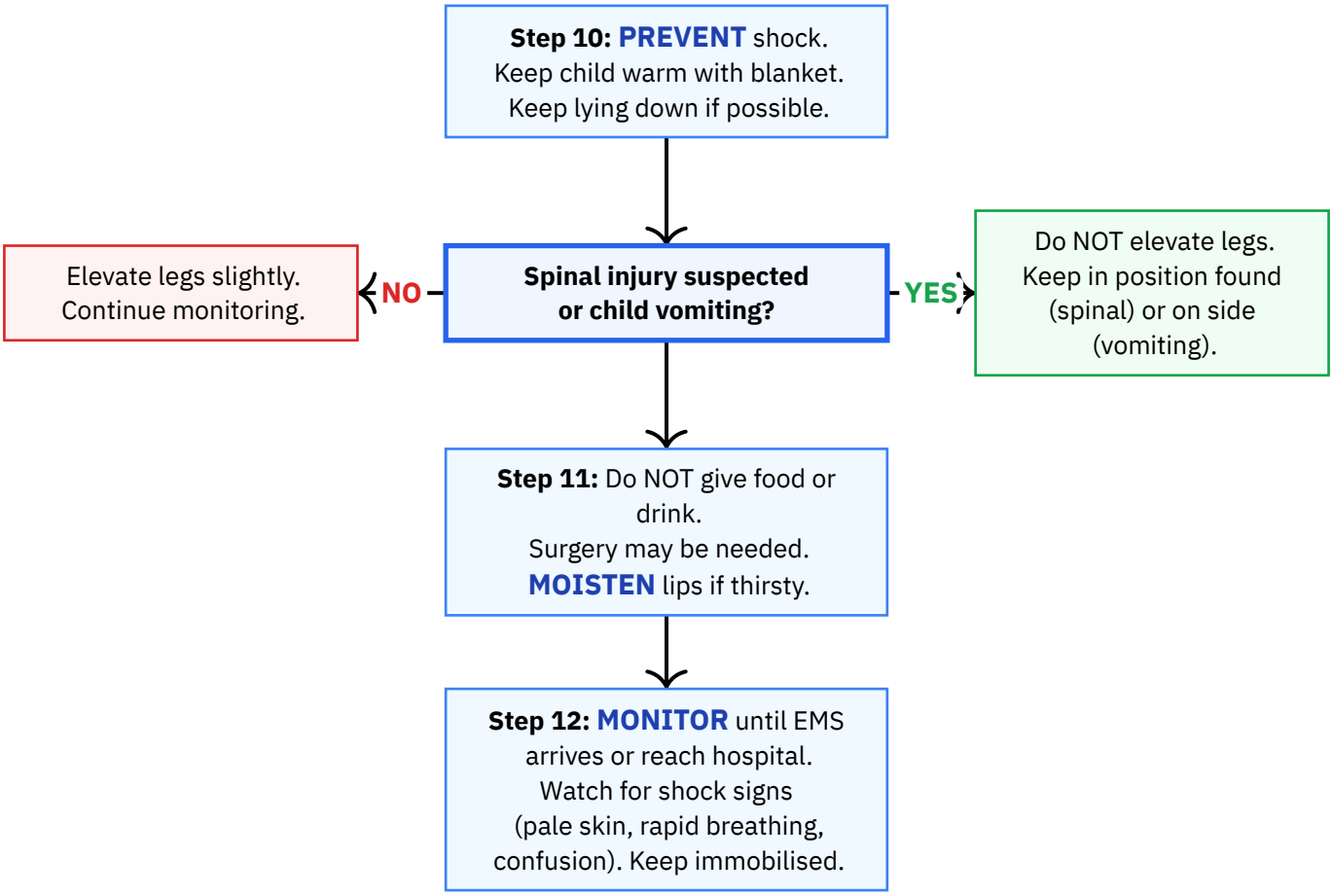
Step 4: Do NOT move or **REALIGN** the injury. Never straighten, pull, or push a bone back into place.

WARNING
Realigning can damage arteries, nerves, and growth plates – potentially affecting future bone development.

Immobilisation and Treatment (Steps 5–9)



Shock Prevention and Monitoring (Steps 10–12)



DO NOT:

- X Do NOT attempt to straighten, pull, or realign a broken bone -- damages arteries, nerves, growth plates.
- X Do NOT move the child unnecessarily, especially if spinal injury suspected.
- X Do NOT apply heat to the injury (no hot compresses, no warm water).
- X Do NOT apply ice directly to the child's skin -- always wrap in cloth.
- X Do NOT tie splints or bandages too tightly -- monitor circulation below.
- X Do NOT allow the child to bear weight on an injured leg.
- X Do NOT remove clothing by pulling over the injury -- cut it away.
- X Do NOT give aspirin to children (Reye's syndrome risk) -- use paracetamol or ibuprofen.
- X Do NOT push a protruding bone back into the wound.
- X Do NOT give the child food or drink -- surgery may be required.

Equipment needed:

— Improvised sling material (shirt, scarf, triangular bandage)
— Improvised splint (cardboard, newspapers, rulers, rigid material)
— Padding (clothing, towels, blankets) for splint and leg immobilisation
— Ice pack or cold compress (or frozen vegetables) + cloth for wrapping
— Sterile dressings or clean cloth (for open fractures)
— Bandages or fabric strips for securing splints
— Blanket for warmth and shock prevention
— Tourniquet (for severe uncontrollable bleeding, if available)

Source: Magen David Adom (MDA) — MDA 101 Public First Aid Guidance Series
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: MDA principle: “Minimum movement, maximum stability.” What appears to be a sprain in a child may be a growth plate fracture. All suspected sprains should be evaluated by a doctor.

Head / Spinal Injury First Aid -- Child

Israel — CHILD

CALL 101 (MDA) — DO NOT MOVE THE CHILD

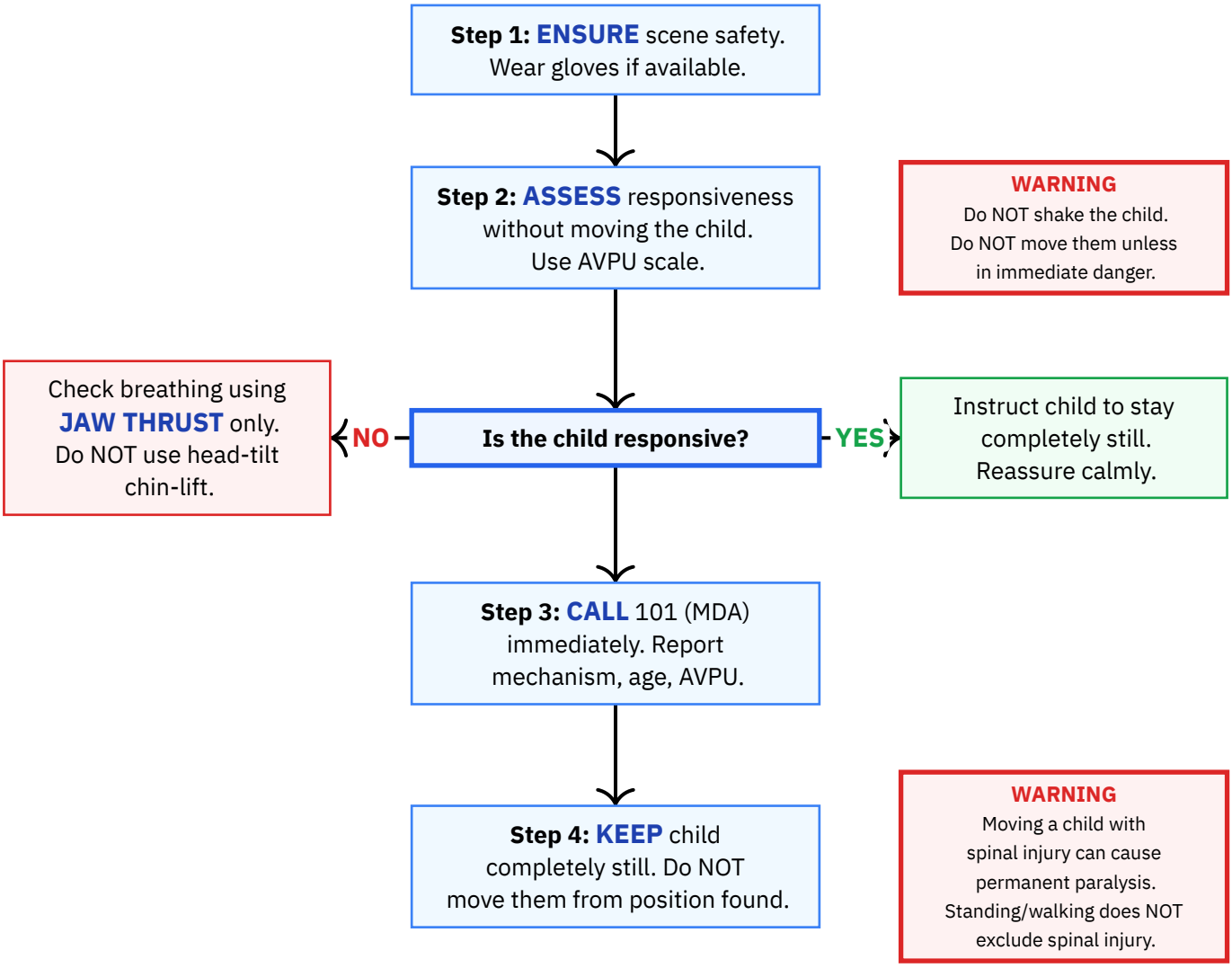
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

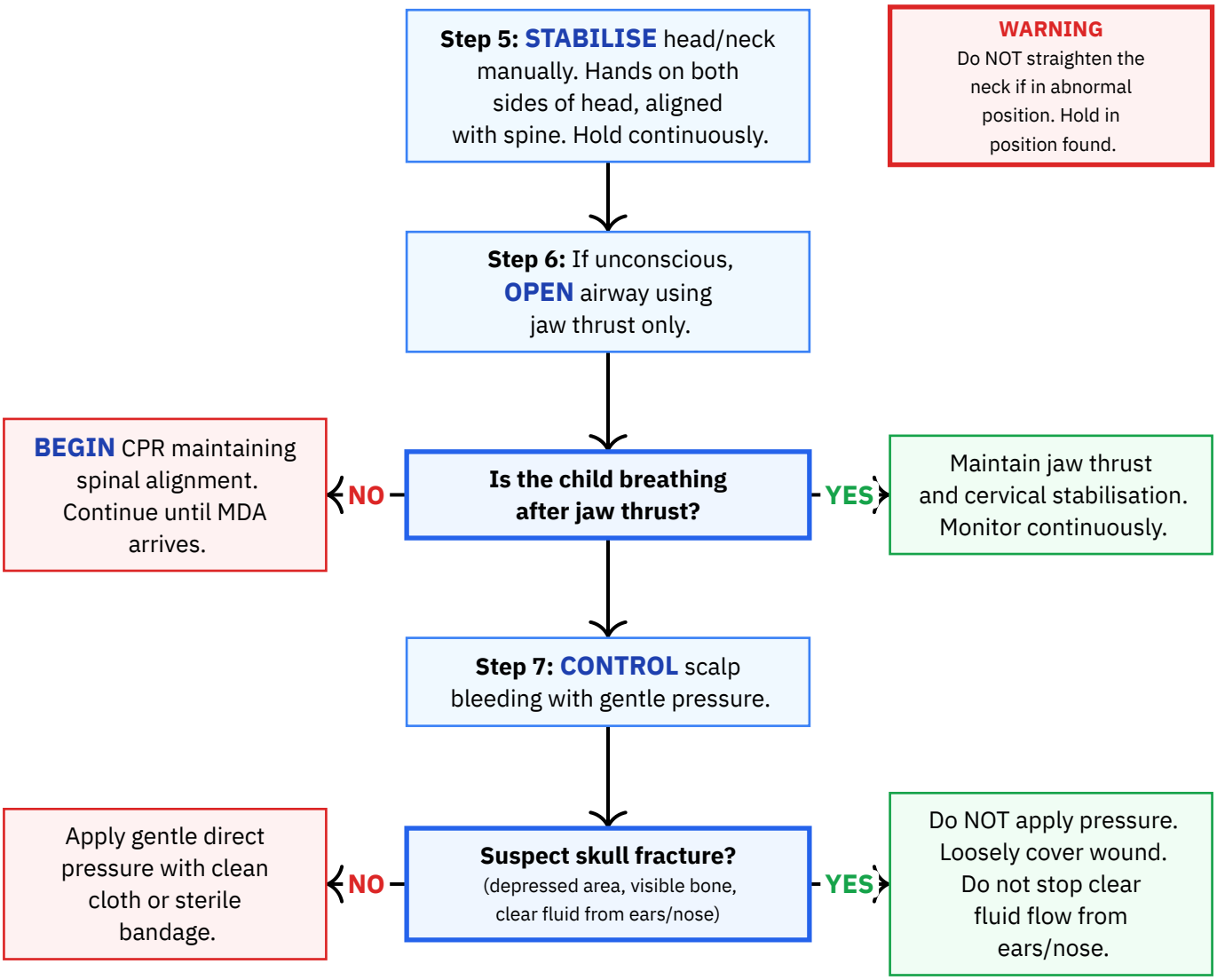
A child who has sustained or may have sustained a head or spinal injury from: falls from height (over 1 m or 2x child's height), vehicle accidents, bicycle/scooter crashes, playground falls, being struck by heavy objects, blast/shrapnel, any head impact in infants under 1 year.

Summary: Keep the child completely still. Call 101 immediately. Stabilise head and neck manually. Control scalp bleeding (avoid pressure on suspected fractures). Use jaw thrust only for airway. Monitor continuously until MDA arrives.

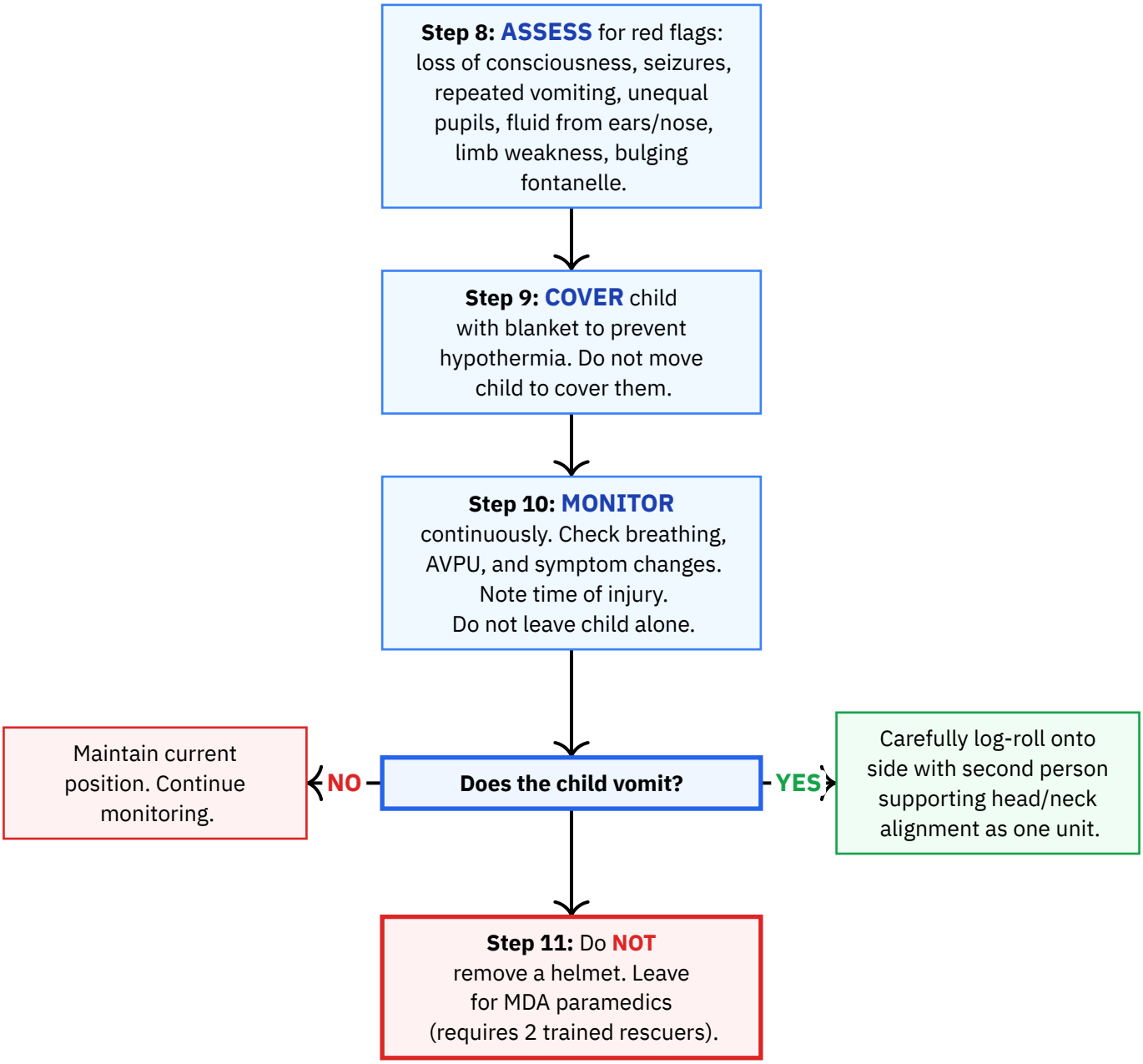
Initial Assessment – Steps 1-4



Stabilisation and Airway – Steps 5-7



Red Flags and Monitoring – Steps 8-11



DO NOT:

- X Do NOT move the child unless in immediate life-threatening danger.
- X Do NOT remove a helmet -- leave for MDA paramedics.
- X Do NOT allow the child to stand up or walk -- does NOT exclude spinal injury.
- X Do NOT give food or drink (may need surgery/anaesthesia).
- X Do NOT apply pressure to a depressed skull fracture or push back protruding matter.
- X Do NOT insert anything into a wound.
- X Do NOT attempt to straighten the neck if in abnormal position.
- X Do NOT use head-tilt chin-lift -- use jaw thrust only.
- X Do NOT shake the child to assess responsiveness.
- X Do NOT leave the child alone.
- X Do NOT stop clear fluid flow from ears or nose (may be cerebrospinal fluid).

Equipment needed:

— Disposable gloves
— Clean cloth or sterile bandages for wound control
— Blanket or coat for warmth

Source: Magen David Adom (MDA) – BLS Guide (2016), ALS Professional Manual (2024), Israeli Standard-of-Care Paediatric Head Injury Guidance
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Both follow PHTLS-based trauma assessment (ABCDE) and selective immobilisation.

Heart Attack / Cardiac Emergency — Child

Israel — CHILD

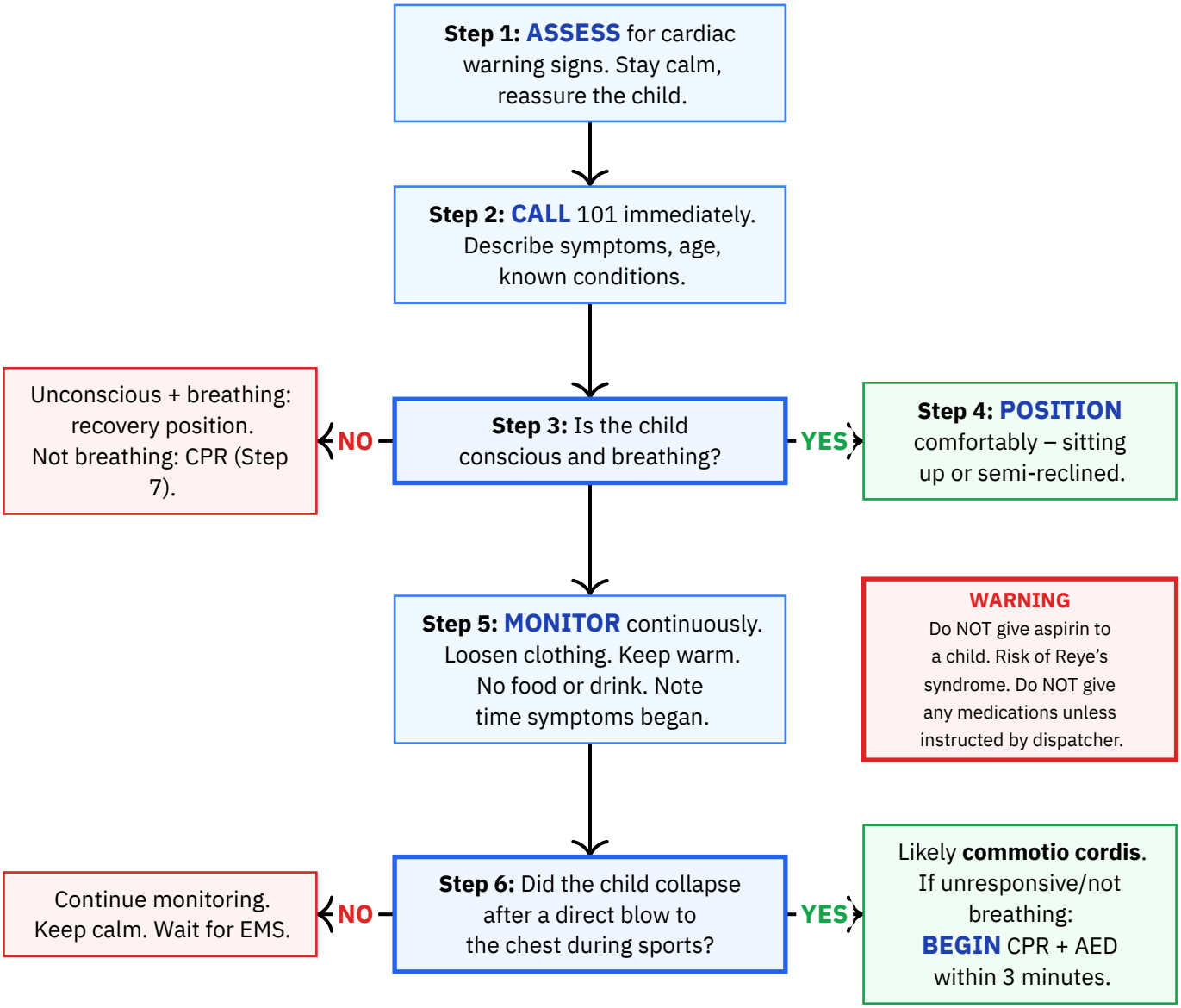
CALL 101 — DO NOT GIVE ASPIRIN TO CHILDREN

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

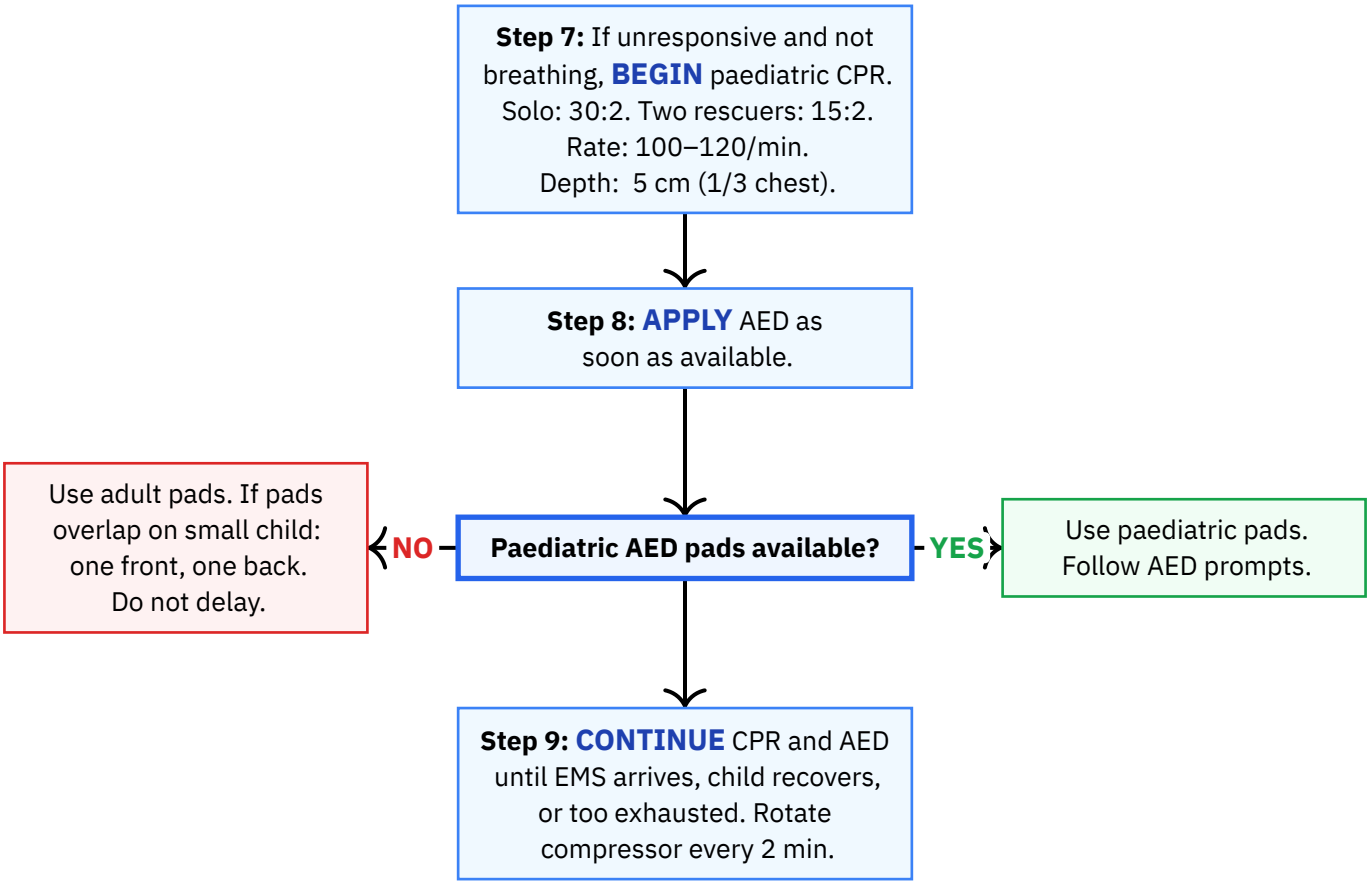
When to apply:

Child with persistent/severe chest pain (especially during exertion), syncope during activity, palpitations, unexplained breathlessness, sudden collapse during sports, cyanosis, chest pain with fever (possible myocarditis), or any cardiac symptoms in a child with known heart condition. True MI is exceedingly rare in children – cardiac causes include myocarditis, congenital defects, hypertrophic cardiomyopathy, commotio cordis.

Assessment and Response (Steps 1–6)



CPR and AED (Steps 7–9)



DO NOT:

X Do NOT give aspirin to a child -- risk of Reye's syndrome (unlike adult protocol).
X Do NOT give nitroglycerin or any cardiac medication unless prescribed and directed by a physician.
X Do NOT dismiss chest pain in a child with recent illness, known heart condition, or collapse during activity.
X Do NOT attempt to drive the child to hospital -- call 101 for ambulance with monitoring equipment.
X Do NOT leave the child unattended while waiting.
X Do NOT give food or drink (hospital procedures may be needed).
X Do NOT give any medications unless instructed by the 101 dispatcher or a physician.

Equipment needed:

— AED with paediatric pads or dose attenuator (if available)
— Phone to call 101 (MDA) or 1221 (United Hatzalah)
— MDA app (for AED locator and emergency calling)

Source: Magen David Adom (MDA) — MDA BLS Guide 2016; ALS Guide 2022; Training Syllabus 2026
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: True MI is exceedingly rare in children. Rare cardiac causes include myocarditis, congenital defects, hypertrophic cardiomyopathy, commotio cordis, Kawasaki disease, Long QT syndrome.

Heatstroke and Heat Exhaustion — Child

Israel — CHILD

CALL 101 (MDA) — HEAT STROKE IS LIFE-THREATENING

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Child shows signs of heat-related illness: intense thirst, headache, profuse sweating, pale clammy skin, nausea, vomiting, dizziness, floppiness or unusual sleepiness (young children), or severe signs such as hot dry red skin, confusion, seizures, or loss of consciousness. Children aged 0–4 are at highest risk.

Assessment and Cooling (Steps 1–5)

Step 1: ASSESS severity.

Hot dry red skin, temp above 40C, confusion, seizures, or unconscious?

- **NO** → Heat exhaustion. Move to shade, begin cooling. Call 101 if no improvement in 30 min.
- **YES** → HEAT STROKE – life-threatening. **CALL** 101 immediately. Begin aggressive cooling.

Step 2: CALL 101 or My MDA app.

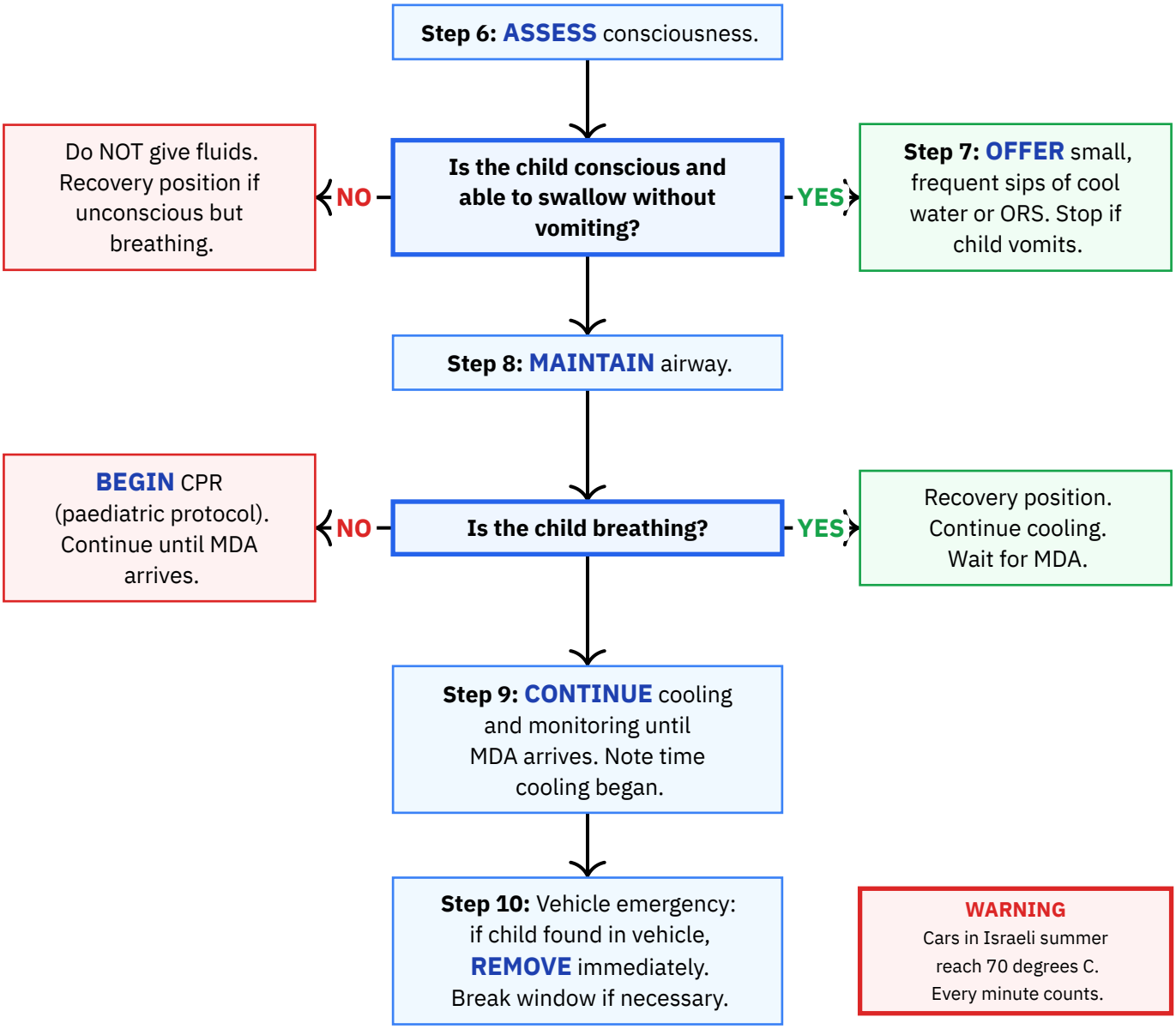
Step 3: MOVE child to cool environment immediately. Shade, A/C, or coolest location.

Step 4: REMOVE excess and tight clothing.

Step 5: BEGIN active cooling. Pour/spray tepid-cool water. Fan the child. Ice packs in armpits and groin.

WARNING
Do not use ice-cold water immersion for young children without medical supervision.

Hydration and Airway (Steps 6–10)



DO NOT:

X Do NOT give fever-reducing medications (Acamol, Nurofen, naproxen) -- they do not reduce heat stroke temperature and may cause harm.
X Do NOT use ice-cold water immersion for young children without medical supervision.
X Do NOT force fluids if the child is vomiting or has altered consciousness.
X Do NOT leave the child unattended at any point.
X Do NOT give fluids to an unconscious child.
X Do NOT delay calling 101 if heat stroke is suspected.
X Do NOT leave a child unattended in a vehicle, even for a moment.

Equipment needed:

— Cool water (for pouring or spraying)
— Fan or means of fanning
— Ice packs or cold wet cloths
— Oral rehydration solution (ORS) or cool drinking water
— Thermometer (if available)

Source: Magen David Adom (MDA) — ALS Manual 2024; public guidelines 2020–2024
URL: https://www.mdais.org/news/300820
Publication date: 2024-04-01 · **Imported:** 2026-03-15 · **Last verified:** 2026-03-15

Hypothermia First Aid -- Child

Israel — CHILD

CALL 101 (MDA) — HANDLE GENTLY

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

When a child shows signs of hypothermia (core temperature below 35 C): shivering (may be absent in infants), cold pale skin, confusion, slurred speech, lethargy. Infants may show bright red cold skin, low energy, weak cry. Children are at higher risk due to immature thermoregulation and higher surface-area-to-mass ratio.

Key Principle: “No one is dead until they are warm and dead.” A severely hypothermic child may appear dead but can recover with proper rewarming. Handle with extreme gentleness – a cold heart can develop fatal arrhythmias from rough movement.

Personal reference only — not medical advice

Rewarming Protocol – Steps 1-5

Step 1: REMOVE child from cold environment. Handle very gently.

WARNING
Minimise all movement. A cold heart is extremely sensitive and can develop fatal arrhythmias.

↓

Step 2: REMOVE wet clothing. Cut or carefully remove. Replace with dry clothing.

↓

Step 3: DRY the child. **WRAP** in warm blankets. Position horizontally. Cover head, neck, chest, groin.

↓

Step 4: CHECK ABC (Airway, Breathing, Circulation). Take up to 60 seconds for pulse/breathing assessment.

↓

Is the child breathing and has a pulse?

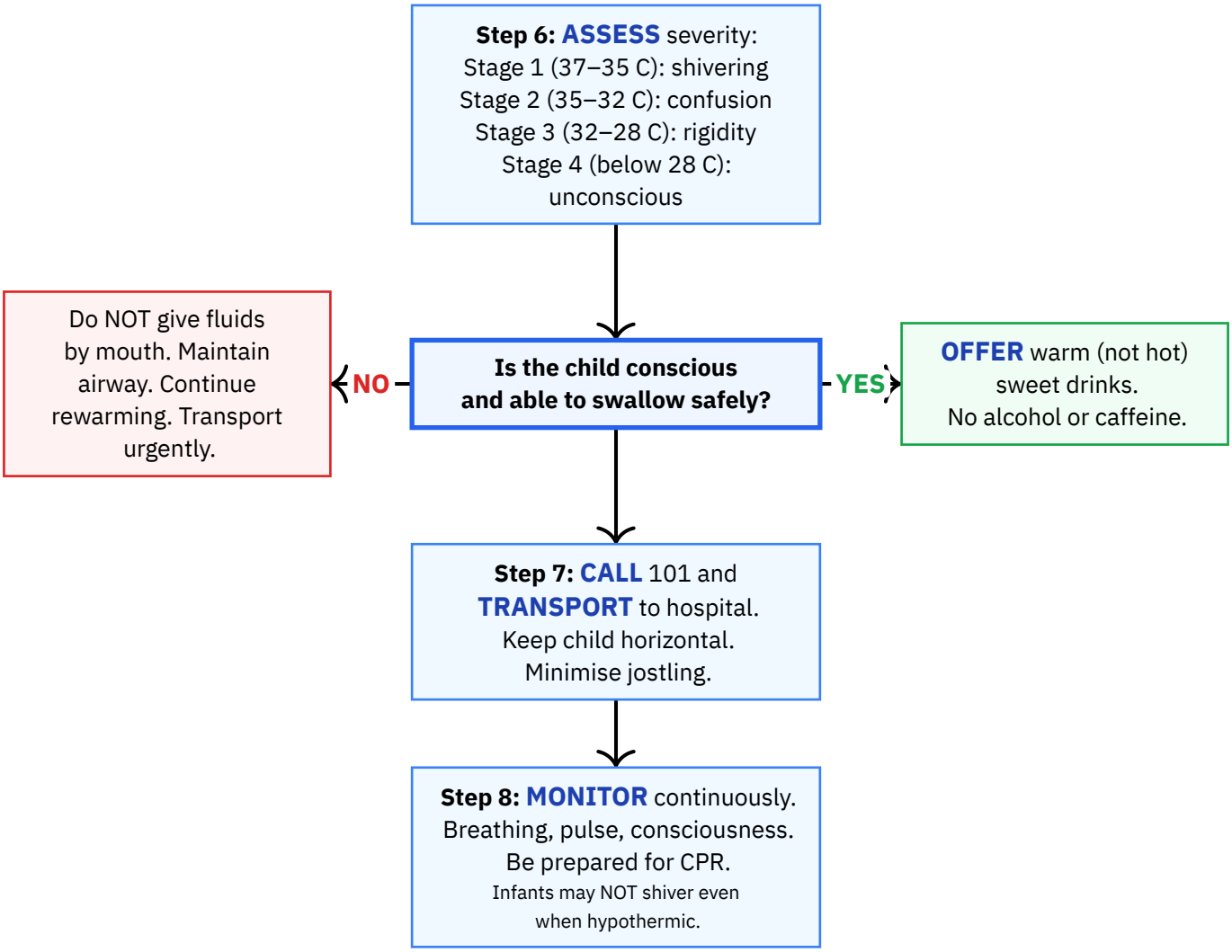
- **NO** → **BEGIN** CPR immediately. Continue CPR and rewarming until emergency services arrive.
- **YES** → Continue rewarming. Monitor continuously. Recovery position if unconscious but breathing.

↓

Step 5: APPLY warm blankets or wrapped heat packs to head, neck, chest, groin. Warm core first, not extremities.

WARNING
Do NOT apply direct heat to skin – risk of burns. Always wrap heat sources in cloth.

Severity Assessment and Transport – Steps 6-8



DO NOT:

X Do NOT give alcohol, caffeine, or allow smoking -- worsen hypothermia.
X Do NOT make excessive or rough movements -- risk of cardiac arrhythmias.
X Do NOT apply direct heat to skin without a cloth barrier -- risk of burns.
X Do NOT rewarm extremities before the core -- dangerous core temperature drop as cold blood returns to heart.
X Do NOT give fluids by mouth to unconscious or semi-conscious child -- aspiration risk.
X Do NOT assume a severely hypothermic child is dead -- 'no one is dead until they are warm and dead.'
X Do NOT use hot water immersion for rewarming in the field.
X Do NOT jostle or move the child unnecessarily during transport.

Equipment needed:

— Warm blankets or sleeping bags
— Dry clothing
— Heat packs or warm water bottles (wrapped in cloth)
— Phone (to call 101)

Source: Magen David Adom (MDA) – ALS Professional Manual (April 2024), supplemented by Ichud Hatzalah hypothermia guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2024-04-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Unified from MDA and United Hatzalah sources. Four-stage severity classification from Hatzalah incorporated.

Poisoning First Aid -- Child

Israel — CHILD

CALL 101 (MDA) — Poison Center: 04-7771900

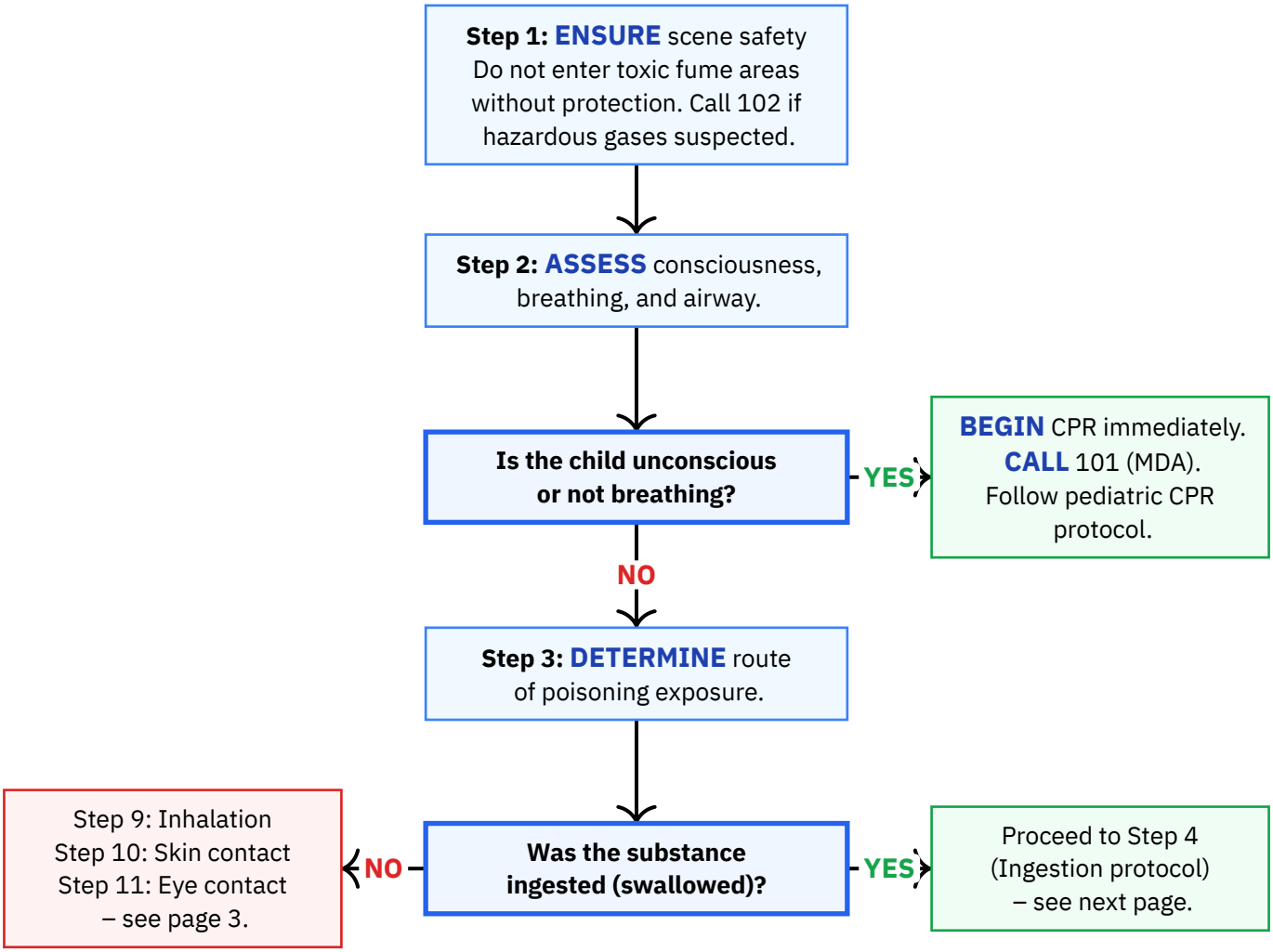
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

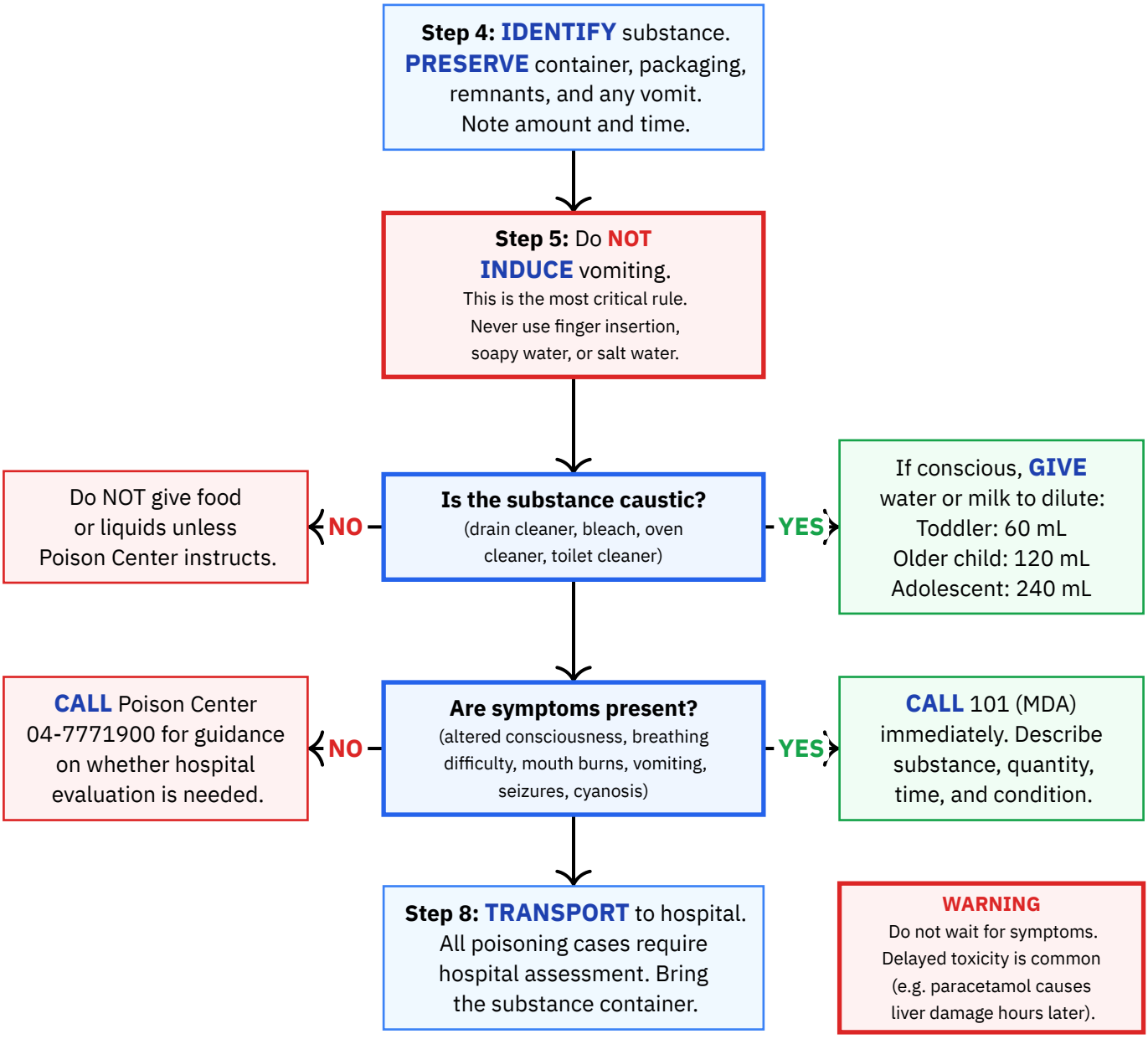
When a child is suspected of having ingested, inhaled, or come into contact with a toxic substance. Common substances: medications (49%), cleaning products (36%), pesticides, plants, alcohol. 90% of child poisoning occurs at home. Signs: altered consciousness, breathing difficulty, burns in/around mouth, vomiting, abdominal pain, drooling, unusual odour.

Summary: Never induce vomiting. Preserve the substance container. Call Poison Center (04-7771900) for non-symptomatic cases, 101 for symptomatic cases. Dilute caustic substances with water or milk if the child is conscious.

Assessment Phase



Ingestion Protocol



Non-Ingestion Exposure Routes

Step 9 – Inhalation:
MOVE child to fresh air.
If not breathing, begin CPR.
CALL 101 and 102 if hazardous gases suspected.

↓

Step 10 – Skin Contact:
REMOVE contaminated clothing.
FLUSH skin with running water for 15–20 minutes.
Do not apply creams.

↓

Step 11 – Eye Contact:
FLUSH eye(s) with running water for 15–20 minutes.
Gently lift eyelids.
Do not rub eyes.

WARNING
Never enter confined space with toxic fumes without protection. Never mix bleach with other cleaners.

Reference

DO NOT:

X Do NOT induce vomiting -- regardless of the substance. Causes additional harm (oesophageal burns, aspiration risk).
X Do NOT give activated charcoal at home unless instructed by Poison Center or physician.
X Do NOT give food to the poisoned child. Water/milk for dilution of caustic substances only.
X Do NOT attempt home remedies (raw eggs, olive oil, etc.).
X Do NOT wait for symptoms before calling for guidance -- delayed toxicity is common.
X Do NOT discard the container -- always bring it to the hospital.
X Do NOT mix cleaning products -- especially bleach with acids (releases toxic chlorine gas).
X Do NOT enter a toxic fume environment without proper protection.

Equipment needed:

— Cloth (to wipe substance from child's mouth if present)
— Water or milk (for dilution of caustic substances only, age-appropriate amounts)
— Running water source (for skin and eye decontamination, 15-20 minutes flushing)
— Container or bag (to preserve substance packaging and any vomit for medical teams)

Source: Magen David Adom (MDA) – MDA Poisoning First Aid Guidelines, supplemented by Rambam National Poison Information Center guidelines and Beterem child safety materials
URL: https://www.mdais.org/101/poisoningn
Publication date: 2026-01-01
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. 43% of poison center calls involve children under age 6. Pre-Passover cleaning season is a known spike period.

Recovery Position -- Child

Israel — CHILD

CALL 101 (MDA) FOR ANY UNCONSCIOUS CHILD

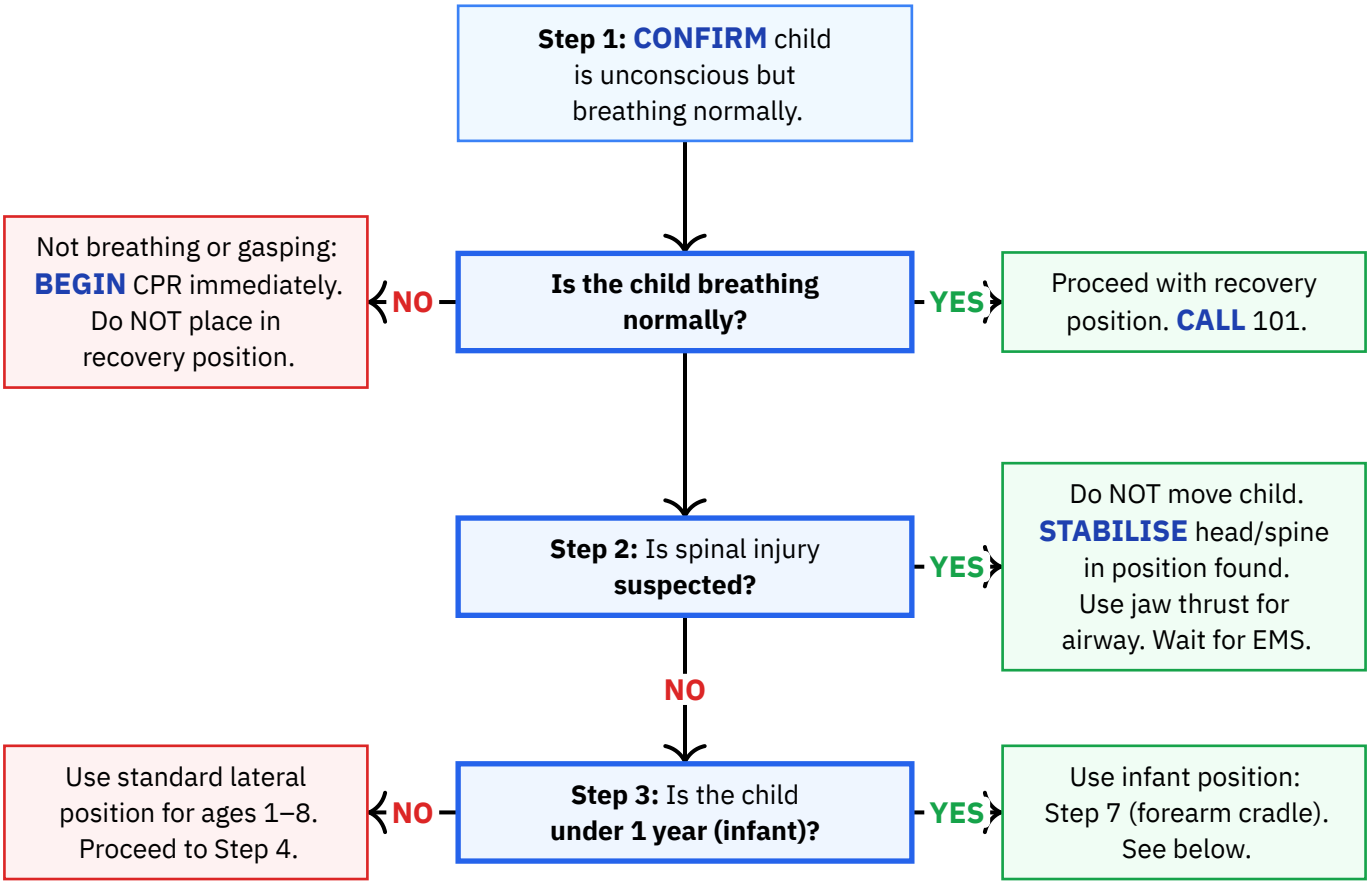
MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

When a child is unconscious but breathing normally and has no suspected spinal injury. Maintains an open airway, facilitates drainage of secretions, and reduces aspiration risk. Do NOT use if spinal injury is suspected.

Summary: Children aged 1–8: standard lateral recovery position (more gently than adults). Infants under 1: cradle/arm position (face-down on rescuer's forearm). Children have proportionally larger heads – position carefully to avoid neck flexion.

Assessment – Steps 1-3



Lateral Position (Ages 1-8) – Steps 4-6

Step 4: KNEEL beside child. Place near arm at right angle, elbow bent, palm facing up.

↓

Step 5: BRING far arm across chest. Hold back of hand against cheek. Bend far knee. **ROLL** child towards you gently.

↓

Step 6: ADJUST position. Upper leg: hip and knee at right angles. Tilt head back gently for open airway. Mouth angled downward for drainage.

WARNING
Children have larger heads proportionally. Ensure head position does not flex neck forward – this can obstruct the airway.

Infant Position and Monitoring – Steps 7-8

Step 7 – Infant (under 1): HOLD infant face-down on your forearm. Head supported by your hand, lower than body. Support jaw with fingers.

WARNING
Do NOT place infants in the standard lateral position. Use the forearm cradle only.

↓

Step 8: MONITOR breathing continuously (every minute). Cover with blanket. Be ready for CPR.

↓

Has the child stopped breathing?

- **NO** → Continue monitoring in recovery position. Maintain warmth and airway patency.
- **YES** → **ROLL** child onto back. **BEGIN** CPR immediately.

DO NOT:

X Do NOT use recovery position if child is not breathing -- begin CPR instead.
X Do NOT use recovery position if spinal injury is suspected -- stabilise the spine.
X Do NOT place infants under 1 year in standard lateral position -- use forearm cradle.
X Do NOT leave child unattended in recovery position -- monitor breathing continuously.
X Do NOT allow head to flex forward (especially in children with larger heads) -- obstructs airway.
X Do NOT forget to cover the child to prevent hypothermia.

Equipment needed:

— Blanket or coat (to prevent hypothermia)
— Phone (to call 101)

Source: Magen David Adom (MDA) – BLS Guide (2016), supplemented by Ichud Hatzalah pediatric treatment approach guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2016-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Children aged 1–8 use standard adult lateral recovery position with greater gentleness. Infants under 1 use cradle/arm position.

Scorpion Sting First Aid -- Child

Israel — CHILD

CALL 101 (MDA) FOR EVERY CHILD STING

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

When a child has been stung or suspected stung by a scorpion. All scorpion stings in children require emergency evaluation – do not “wait and see.” The deathstalker scorpion (Leiurus quinquestriatus) can be deadly for young children. Black scorpions can also be dangerous.

Summary: Call 101 immediately. Keep the child calm and still. Cool the sting site with wrapped ice. Immobilise the limb below heart level. Monitor for systemic symptoms. Transport urgently to hospital.

Immediate Response

Step 1: MOVE child away from the scorpion. Do not attempt to catch it. Photograph if safe.

WARNING
Do NOT attempt to catch or handle the scorpion. Risk of additional stings.

Step 2: CALL 101 (MDA) immediately. Report age, weight, time of sting, location on body.

WARNING
Do NOT delay calling 101 for children. They deteriorate faster due to venom-to-body-weight ratio.

Step 3: KEEP the child calm and still. Have them lie down or sit still. Reassure with calm voice.

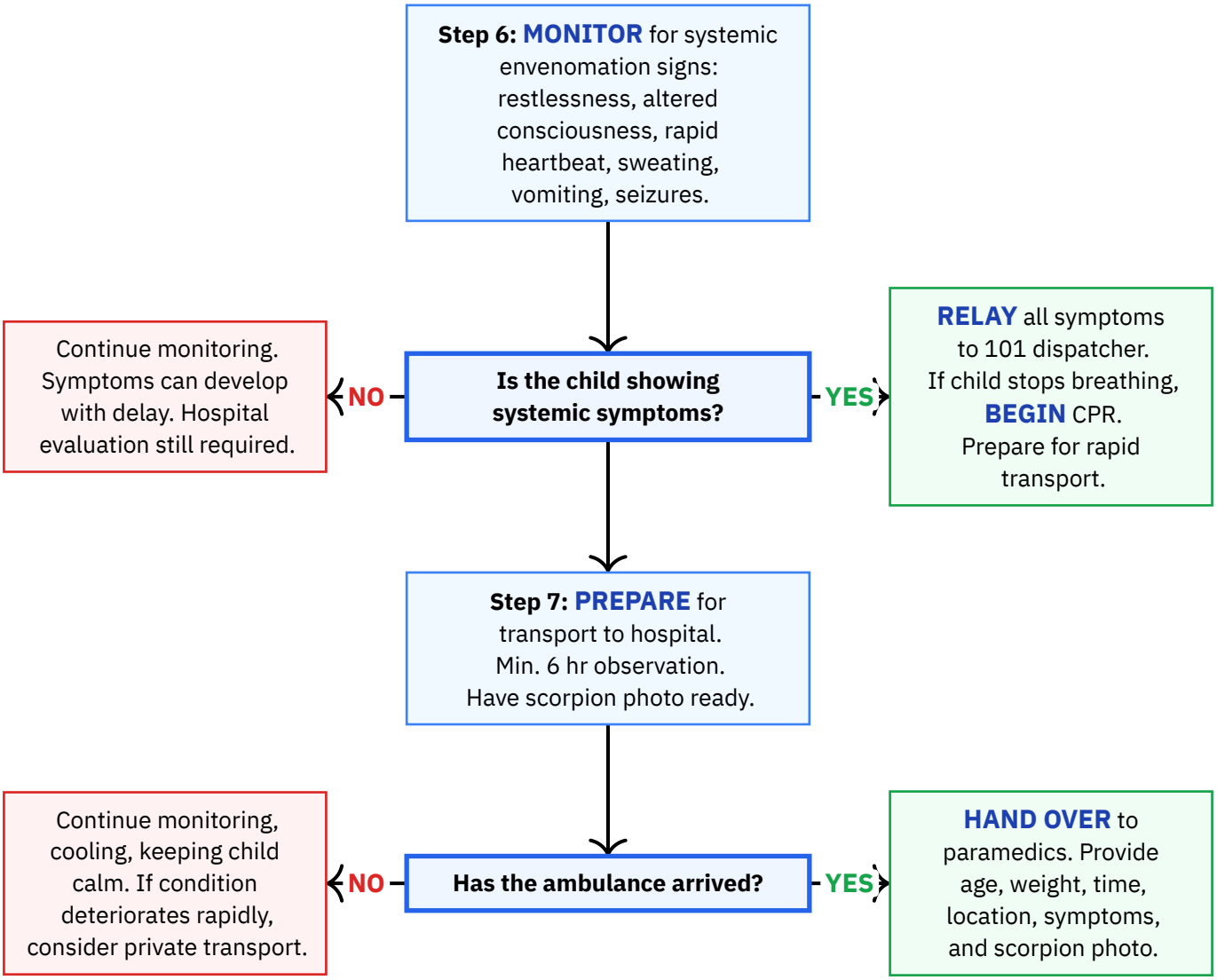
Is ice or cold compress available?

NO → Skip cooling. Focus on keeping child calm, still, and limb immobilised.

YES → **Step 4: APPLY** ice wrapped in cloth. 10 min on, 10 min off. Do NOT apply ice directly to skin.

Step 5: IMMOBILISE stung limb below heart level. Use splint if available.

Monitoring and Transport



Reference

DO NOT:

- X Do NOT apply a tourniquet -- concentrates venom and causes tissue damage.
- X Do NOT cut or incise the sting site -- causes additional injury, does not remove venom.
- X Do NOT attempt to suck out the venom -- ineffective and risks infection.
- X Do NOT apply alcohol or antiseptic to the wound.
- X Do NOT give the child medication without medical advice.
- X Do NOT underestimate a sting based on scorpion colour -- black scorpions can be lethal.
- X Do NOT delay calling 101 for children -- faster deterioration due to venom-to-body-weight ratio.
- X Do NOT apply ice directly to skin -- always wrap in cloth.
- X Do NOT allow the child to walk or move -- movement accelerates venom distribution.
- X Do NOT 'wait and see' with children -- all stings require emergency evaluation.

Equipment needed:

— Ice pack or cold compress (wrapped in cloth)
— Splint or rigid object for limb immobilisation
— Phone (to call 101 / 04-7771900 and photograph the scorpion)
— Cloth or towel (to wrap ice pack)
— Blanket or clothing (to keep child comfortable while lying still)

Source: Magen David Adom (MDA) – supplemented by Jerusalem Post articles, Israeli Ministry of Education first aid instructions, Midaat.org.il, and peer-reviewed Israeli pediatric clinical data (Paret et al. 1991)
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah sources. Deathstalker scorpion (Leiurus quinquestriatus) found throughout Israel. IV antivenom effective and safe in children with significant symptoms.

Seizures (Including Febrile Seizures) — Child

Israel — CHILD

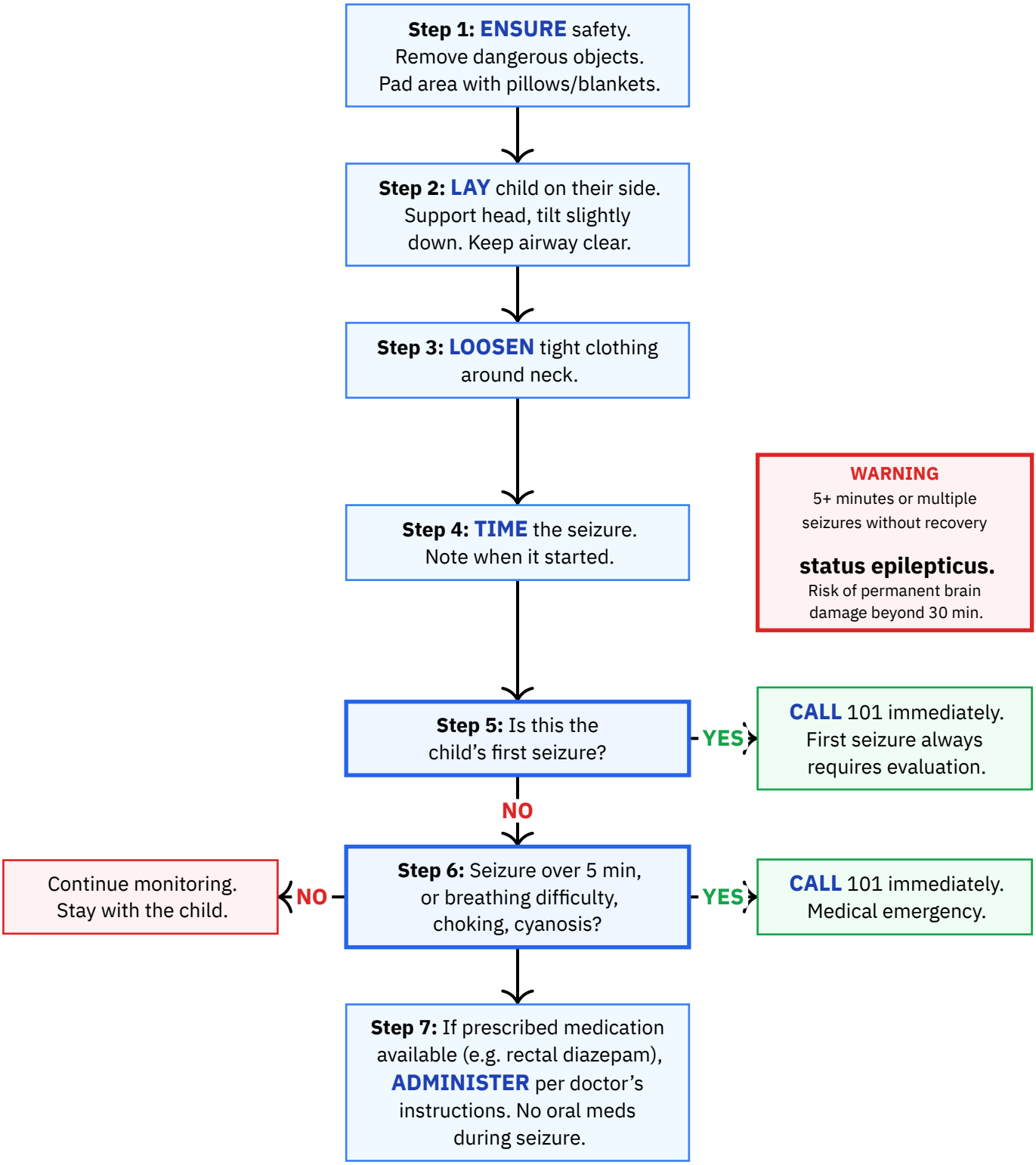
CALL 101 IF: FIRST SEIZURE, OVER 5 MIN, OR NO RECOVERY

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

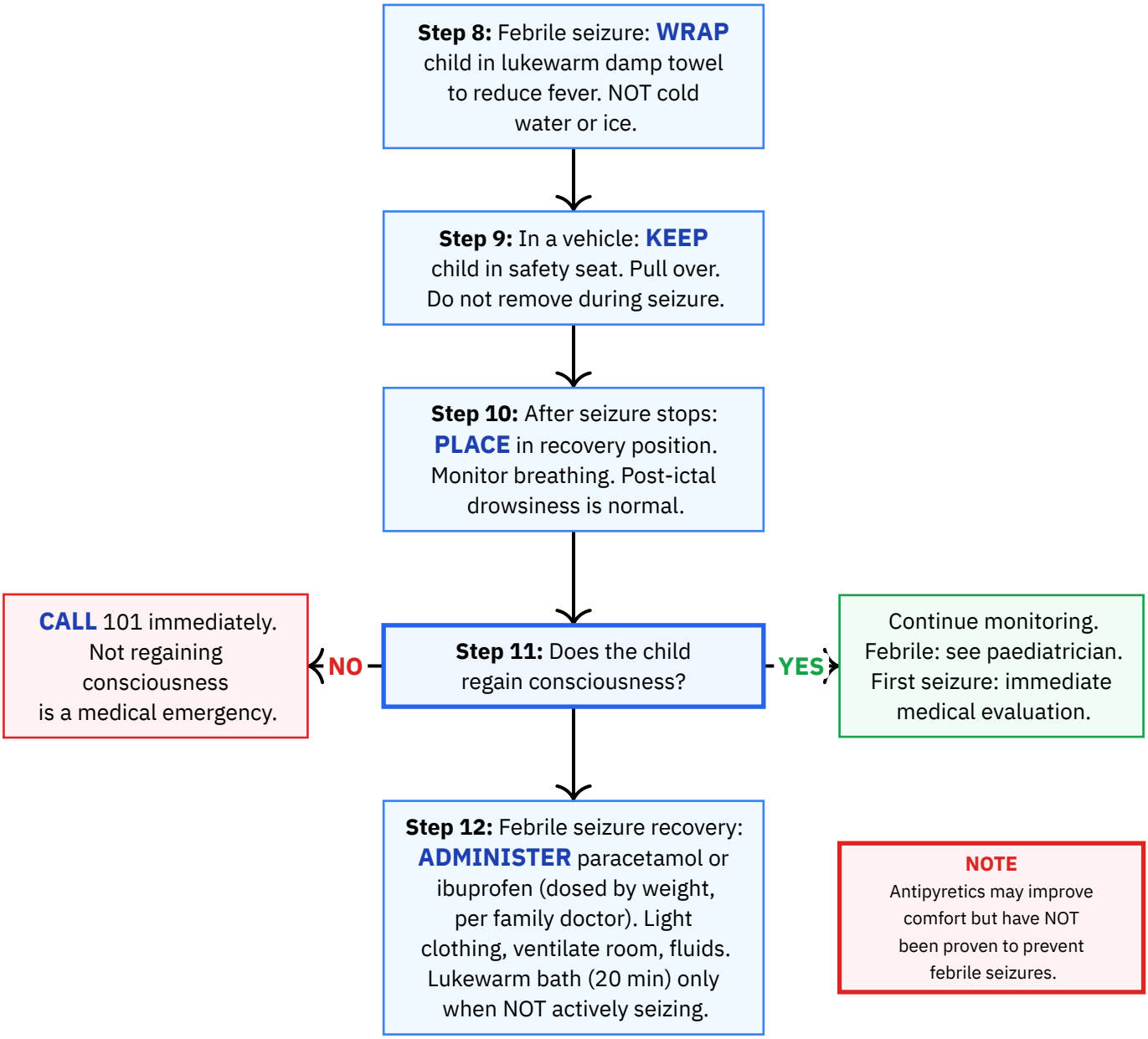
When to apply:

Child experiencing a seizure: rhythmic jerking or stiffening, unresponsiveness, eyes rolling, loss of consciousness. Febrile seizures: ages 6 months to 5–6 years, in the context of fever. A seizure lasting 5+ minutes is a medical emergency (status epilepticus).

During the Seizure (Steps 1–7)



After the Seizure (Steps 8–12)



DO NOT:

- X DO NOT force the child's mouth open -- the child will not swallow their tongue.
- X DO NOT insert anything into the mouth -- can injure teeth or jaw.
- X DO NOT attempt CPR during the seizure -- this can only cause harm.
- X DO NOT put the child in a bath or shower during the seizure.
- X DO NOT splash water on the child during the seizure.
- X DO NOT try to wake the child or restrain/hold them down.
- X DO NOT give medications by mouth during the seizure.
- X DO NOT attempt rapid cooling with cold water or ice.
- X DO NOT insert an airway device during an active seizure.
- X DO NOT attempt forced airway opening during an active seizure.

Equipment needed:

— Lukewarm damp towel (for febrile seizures)
— Pillows or blankets (to pad surroundings)
— Prescribed seizure-stopping medication if available (e.g. rectal diazepam)
— Fever-reducing medication for post-seizure (paracetamol or ibuprofen, per doctor)

Source: Magen David Adom (MDA) — MDA 101 Public First Aid Guidance Series
URL: https://www.mdais.org/101/seizures
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Febrile seizures affect children aged 6 months to 5–6 years, peak at 18–22 months, prevalence 3–4%.

Severe Bleeding (Hemorrhage Control) — Child

Israel — CHILD

CALL 101 (MDA) IMMEDIATELY

MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

Severe or uncontrolled external bleeding in a child (1–12 years). Children have significantly smaller blood volumes (a 3-year-old has only 1,200 mL total) – what appears moderate in an adult may be life-threatening in a child.

Initial Response (Steps 1–5)

Step 1: ENSURE scene safety.
Wear gloves if available.
Involve parent/caregiver.

↓

Step 2: CALL 101 immediately.
State child's age and weight.
Children deteriorate faster.

↓

Step 3: CALM the child.
Speak softly, involve parent.
Shield from sight of blood.

↓

Step 4: CHECK for embedded objects.

↓

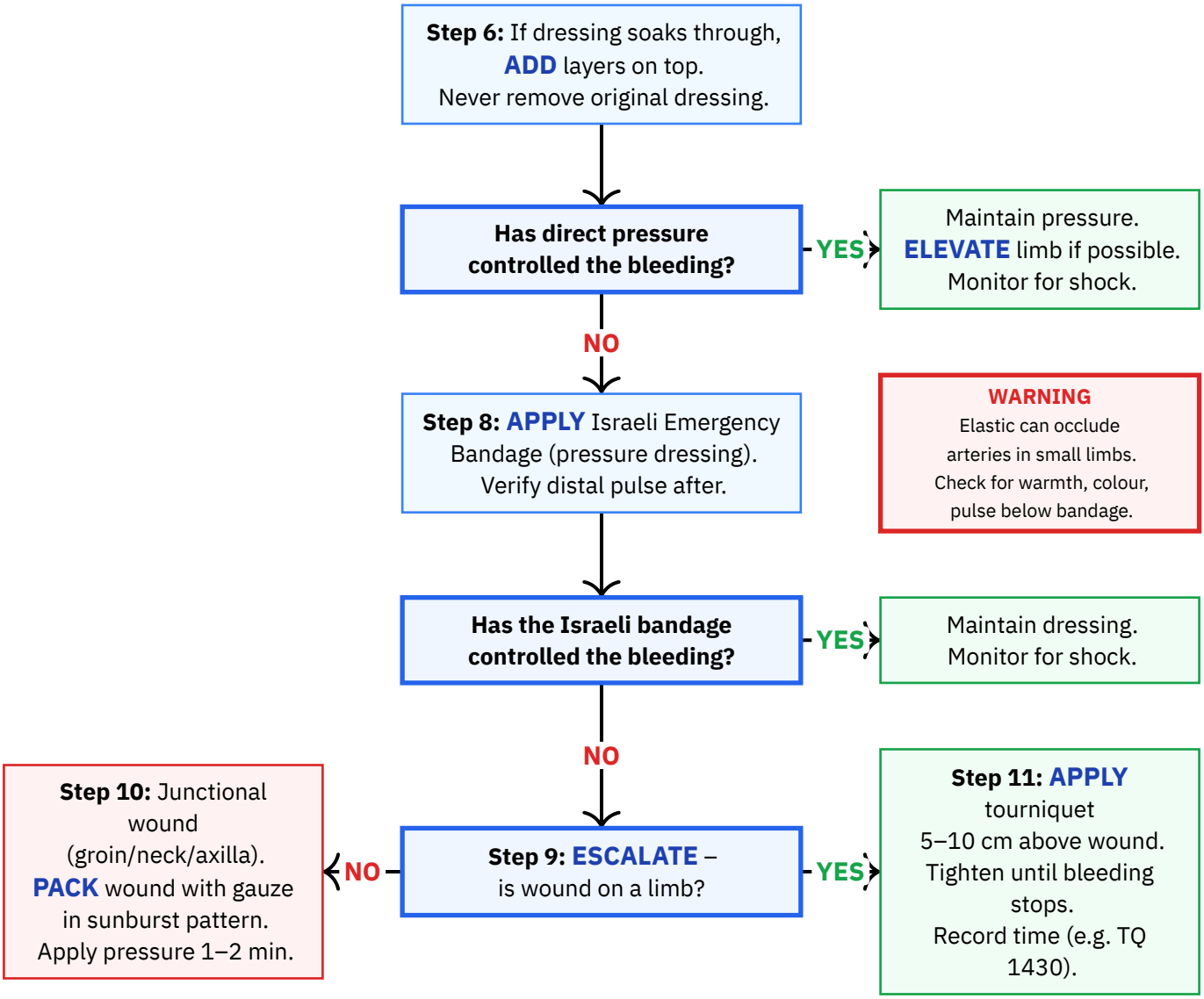
Is there an embedded or impaled object in the wound?

- **YES** → Do NOT remove it.
STABILISE with padding.
Apply pressure AROUND it, not on it.
Skip to shock monitoring.

- **NO** ↓

Step 5: APPLY strong direct pressure to the wound with gloved hand or gauze.
Maintain continuous pressure.

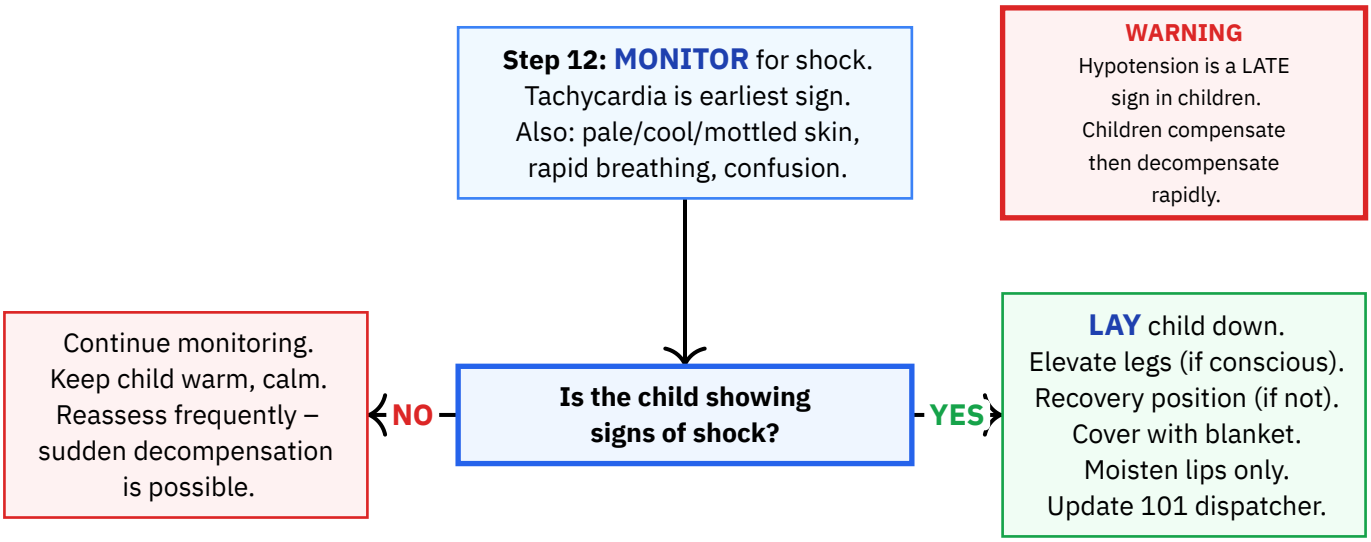
Escalation (Steps 6–11)



Tourniquet by Age

Age 2+, limb circumference 13 cm+: CAT Gen 7 tourniquet.
Age 6+, limb circumference 16 cm+: Reliable upper and lower extremity.
Under age 2: No evidence for manufactured tourniquets. Use sustained direct pressure.

Shock Monitoring (Step 12)



DO NOT:

X Do not remove an embedded or impaled object from the wound.
X Do not lift or remove a blood-soaked dressing -- add layers on top.
X Do not remove or loosen a tourniquet once applied unless instructed by a paramedic or physician.
X Do not loosen a tourniquet because the child is screaming -- pain is expected.
X Do not give food or drink to a child with severe bleeding or suspected shock.
X Do not use wound packing for penetrating stab wounds.
X Do not enlarge wound openings to facilitate wound packing.
X Do not assume tachycardia is caused only by anxiety -- suspect significant blood loss.
X Do not wait for hypotension to suspect shock -- it is a late, pre-arrest sign.
X Do not delay calling 101 for severe or uncontrolled bleeding.
X Do not apply a tourniquet directly over a joint.
X Do not attempt a manufactured tourniquet on a child under 2 years -- use direct pressure.

Equipment needed:

— Disposable gloves
— Sterile gauze or clean cloth
— Israeli Emergency Bandage (pressure dressing)
— Tourniquet (CAT Gen 7 -- for children aged 2+ with limb circumference 13 cm+)
— Wound packing material (hemostatic gauze preferred)
— Blanket or coat (for shock management and warmth)

Source: Magen David Adom (MDA) — MDA 101 First Aid Public Guidance (Bleeding Control)
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Unified from MDA and United Hatzalah. Children have 80–90 mL/kg blood volume but much smaller total volume. Compensate longer, then decompensate rapidly.

Shock (Helem) — Child

Israel — CHILD

CALL 101 (MDA) IMMEDIATELY

MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

Child showing signs of inadequate tissue perfusion: tachycardia (earliest sign), pale/cool skin, prolonged capillary refill (over 2 seconds), cold clammy extremities, rapid shallow breathing, mottled skin, altered mental status. Children have much smaller total blood volumes (newborn 240 mL, one-year-old 800 mL).

Initial Response (Steps 1–5)

WARNING
Children can appear deceptively stable during compensated shock, then deteriorate suddenly.

Step 1: CALL 101 immediately. State child's age. Shock is a time-critical emergency.

↓

Step 2: ASSESS ABC. Check airway, breathing, circulation.

↓

Is the child breathing?

- NO → **BEGIN** CPR immediately.
- YES → Continue to Step 3.

↓

Step 3: CONTROL any visible bleeding. Apply direct pressure.

↓

Step 4: POSITION the child.

↓

Is the child conscious?

- NO → **Step 5:** Spinal injury suspected? Yes: do NOT move. No: recovery position.
- YES → Lay flat on back. **ELEVATE** legs above head level.

Ongoing Care (Steps 6–10)

Step 6: KEEP child warm. Cover with blanket. Insulate from cold ground. Even in warm weather, shock causes hypothermia.

↓

Step 7: LOOSEN restrictive clothing. Belts, collars, tight waistbands.

↓

Step 8: CALM and reassure the child. Involve parent or caregiver.

↓

Step 9: MOISTEN lips with water only. Do NOT give food or drink.

↓

Step 10: MONITOR continuously until EMS arrives. Track breathing, pulse, skin colour, consciousness.

↓

Is the child's condition worsening?

- **NO** → Continue monitoring, warmth, positioning, and reassurance.
- **YES** → **UPDATE** 101 dispatcher. Maintain legs elevated / airway protected. Re-check bleeding. Prepare for CPR.

WARNING
Bradycardia (slow heart rate) is a PRE-ARREST sign in children. If bradycardia develops, be ready for CPR.

DO NOT:

X Do not sit the child upright -- reduces blood flow to the brain.
X Do not give the child anything to eat or drink -- moisten lips only.
X Do not leave the child unattended -- continuous monitoring essential.
X Do not delay calling 101 -- early IV fluids are critical for survival.
X Do not assume normal blood pressure means no shock -- children compensate then suddenly decompensate.
X Do not attribute tachycardia solely to anxiety or pain -- suspect shock.
X Do not move a child with suspected spinal injury.
X Do not remove blood-soaked dressings -- add layers on top.

Equipment needed:

— Blanket or coat (for warmth and insulation from ground)
— Sterile gauze or clean cloth (for bleeding control)
— Disposable gloves
— Pillow or folded blanket (for leg elevation)

Source: Magen David Adom (MDA) — BLS Guide 2016; MDA 101 First Aid Public Guidance
URL: https://www.mdais.org/101/first-aid
Imported: 2026-03-15 · **Last verified:** 2026-03-15
Notes: Tachycardia is the earliest and most reliable sign. Hypotension is a late, pre-arrest sign. Children compensate longer, then decompensate rapidly.

Smoke / Tear Gas Inhalation First Aid -- Child

Israel — CHILD

CALL 101 (MDA) + 102 (FIRE)

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

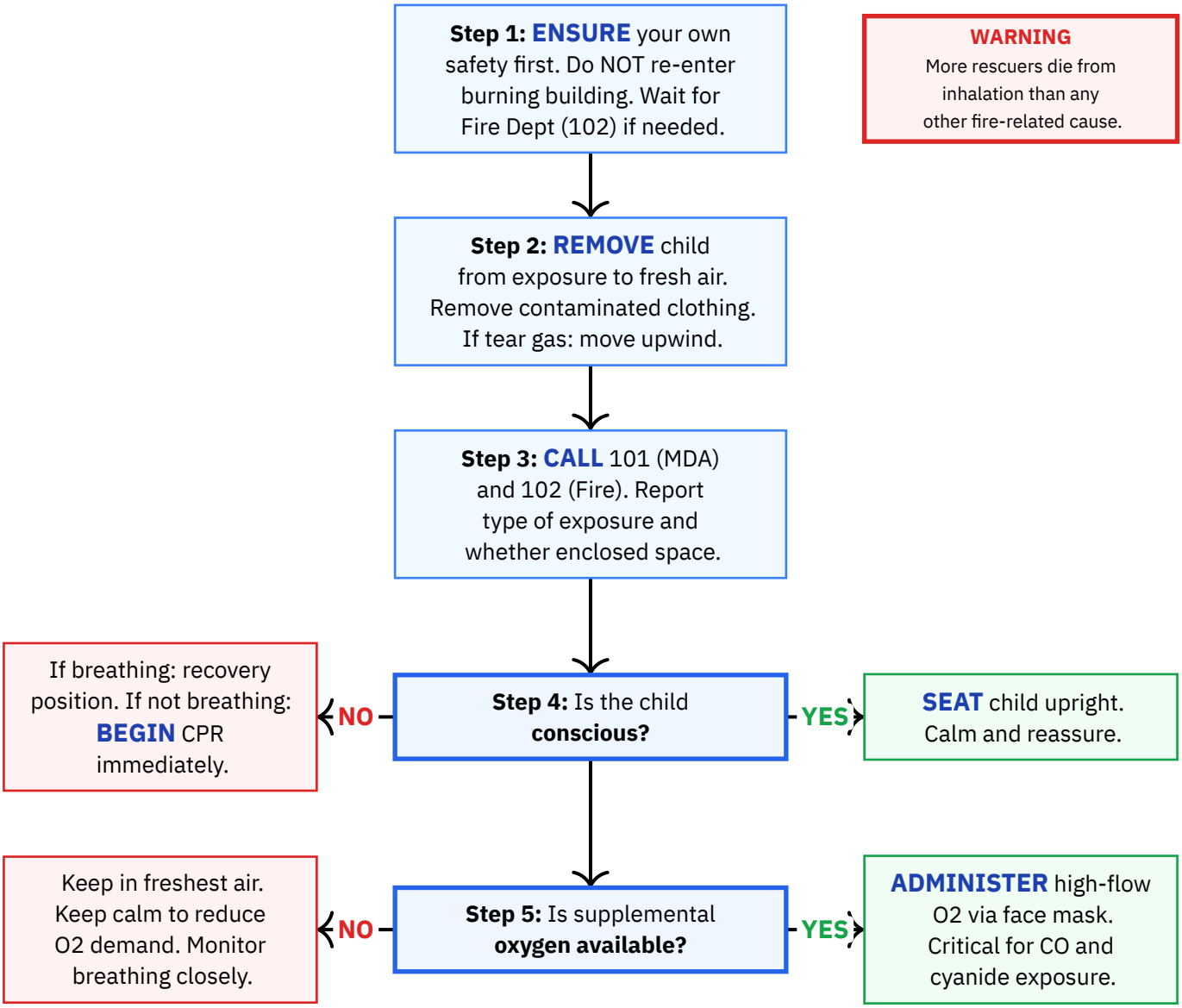
When to apply:

When a child has inhaled smoke, tear gas, toxic fumes, or combustion byproducts. Common Israeli contexts: Lag B'Omer bonfire smoke, apartment fires (winter space heaters), tear gas near residential areas, cleaning product fumes. Children more vulnerable: smaller airways, higher respiratory rate, proximity to ground-level gases.

Summary: Do NOT re-enter hazardous area. Remove child to fresh air. Call 101 and 102. Seat upright. Administer oxygen if available. Watch for stridor (airway swelling). Symptoms can be delayed by hours – all exposed children need hospital evaluation.

Personal reference only — not medical advice

Rescue and Assessment – Steps 1-5



Airway Assessment and Tear Gas – Steps 6-9

Step 6: Does the child **have stridor or severe breathing difficulty?**

- NO → Continue monitoring. Delayed onset of respiratory distress can develop hours later.
- YES → Urgent. Keep calm and upright. Do NOT examine throat. Ensure 101 called. Be prepared for CPR.

WARNING
Stridor after inhalation = danger sign. Airway may swell further and close. Do NOT examine throat.

↓

Step 7: **MONITOR** for CO (pink lips, headache, confusion) and cyanide (seizures, altered consciousness) signs. Note materials that were burning.

↓

Step 8 – Tear gas:
IRRIGATE eyes with clean water for 15 min.
WASH skin with soap.
Remove contaminated clothing.
Move upwind.

WARNING
Do NOT rub eyes after tear gas – worsens irritation.

↓

Step 9: **COVER** with blanket.
MONITOR breathing continuously until EMS.
Be prepared for CPR.
All exposed children need hospital evaluation.

WARNING
Symptoms can be delayed by hours.
A well-appearing child may deteriorate later.
Hospital evaluation required for all.

DO NOT:

- X Do NOT re-enter a burning building or hazardous area -- wait for Fire Department.
- X Do NOT ignore light smoke exposure in a child -- assess even with mild exposure.
- X Do NOT assume a well-appearing child is uninjured -- respiratory deterioration can be delayed by hours.
- X Do NOT rub eyes after tear gas exposure -- worsens irritation.
- X Do NOT give food or drink to a child with stridor or significant breathing difficulty.
- X Do NOT examine the throat of a child with stridor -- can worsen airway swelling.
- X Do NOT leave the child unattended -- monitor breathing continuously.
- X Do NOT position a child with breathing difficulty lying flat -- keep upright.

Equipment needed:

— Supplemental oxygen and face mask (if available)
— Clean water (for eye irrigation after tear gas)
— Blanket (to prevent hypothermia)
— Cool mist humidifier (if available, for stridor management)
— Phone (to call 101 and 102)

Source: Magen David Adom (MDA) – supplemented by Ichud Hatzalah inhalation poisoning and respiratory distress guidelines
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Common Israeli contexts: Lag B'Omer bonfires, winter apartment fires, tear gas near residential areas. Children vulnerable due to smaller airways and higher respiratory rate.

Snakebite — Child

Israel — CHILD

CALL 101 — TREAT EVERY BITE AS VENOMOUS

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Child bitten or suspected bitten by a snake. Treat every snakebite as venomous until proven otherwise. Primary venomous species in Israel: Palestine viper (Daboia palaestinae). Common bite locations: 63% lower limbs, 33% upper limbs. Children are at higher risk due to lower body weight.

Response Steps (1–8)

Step 1: ENSURE scene safety. Move child and bystanders away from snake (2+ metres). Photo snake from safe distance.

WARNING
Do NOT approach, chase, or attempt to catch the snake. Risk of additional bites.

Step 2: CALL 101 immediately. Report child's age, weight, bite location, time. Do NOT wait for symptoms.

Step 3: KEEP child calm and still. Have them lie down. Prevent all movement. Movement accelerates venom spread through lymphatic system.

Step 4: POSITION bitten limb below heart level. Immobilise as for a fracture.

Splint available?

- **NO** → Position limb against body. Secure with clothing if possible.
- **YES** → Apply splint. Keep limb below heart level.

Step 5: REMOVE constrictive items (rings, bracelets, watches, anklets, tight clothing).

Step 6: CLEAN bite gently with water and soap only.

Step 7: MONITOR for systemic signs: swelling, tachycardia, vomiting, difficulty breathing, altered consciousness.

Has the child lost consciousness and stopped breathing?

- **NO** → Continue monitoring. Keep calm, still, lying down. Wait for EMS.
- **YES** → **BEGIN** CPR immediately. Continue until EMS arrives.

Reference

Step 8: Hospital Transport

All snakebite victims require urgent medical evaluation, even without symptoms. Children are at higher risk due to proportionally more venom per kilogram. Antivenom (monovalent equine anti-V. palaestinae) is stocked at Israeli hospitals. Have snake photo ready for medical staff. Israeli paediatric data shows 0% mortality with appropriate care.

DO NOT:

X Do NOT apply a tourniquet -- concentrates venom and causes tissue damage.
X Do NOT cut the bite site -- causes additional injury, does not remove venom.
X Do NOT attempt to suck out the venom -- ineffective and risks infection.
X Do NOT apply ice or cold compresses -- can cause tissue damage.
X Do NOT apply alcohol, antiseptic, or any other substance to the wound.
X Do NOT give the child food or drink -- risk of vomiting, may need anaesthesia.
X Do NOT give hot liquids or alcohol.
X Do NOT chase, catch, kill, or handle the snake.
X Do NOT use a pressure immobilisation bandage -- not recommended for viper bites.
X Do NOT allow the child to walk or move -- movement accelerates venom spread.
X Do NOT wait for symptoms before seeking emergency care.

Equipment needed:

— Splint or rigid object for limb immobilisation
— Water and soap (for wound cleaning)
— Phone (to call 101 and photograph the snake)
— Blanket or clothing (to keep child warm while lying still)

Source: Magen David Adom (MDA) — MDA Snakebite First Aid Guidelines
URL: https://www.mdais.org/101/snake-bite
Publication date: 2026-01-01 · **Imported:** 2026-03-15 · **Last verified:** 2026-03-15
Notes: Primary venomous species: Palestine viper (Daboia palaestinae). Venom travels through lymphatic system, activated by movement. 0% mortality with appropriate care.

Spider Bites First Aid -- Child

Israel — CHILD

CALL 101 (MDA) FOR SYSTEMIC SYMPTOMS

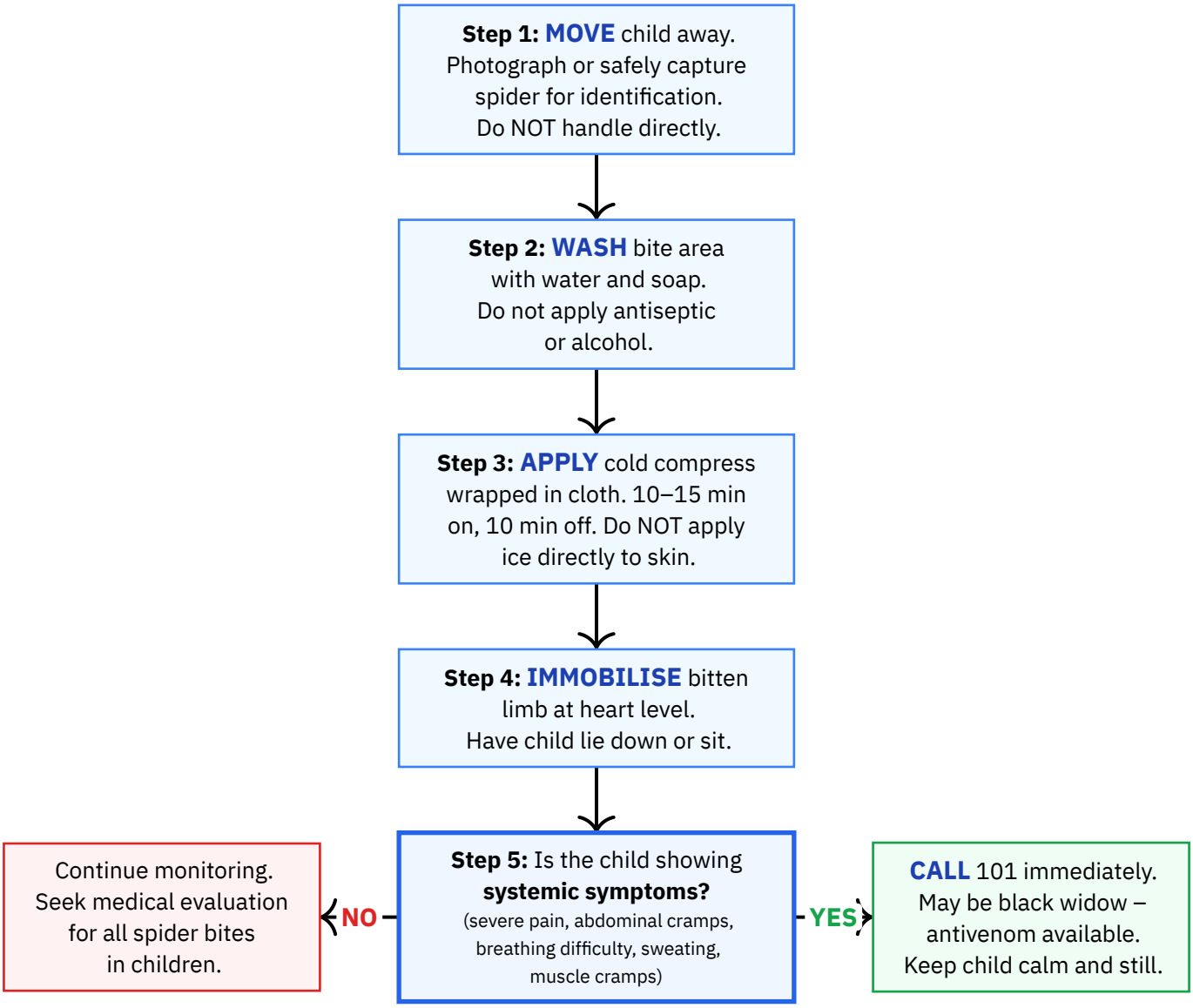
MDA: 101 **Police:** 100 **Fire:** 102 **Hatzalah:** 1221 **Poison:** 04-7771900

When to apply:

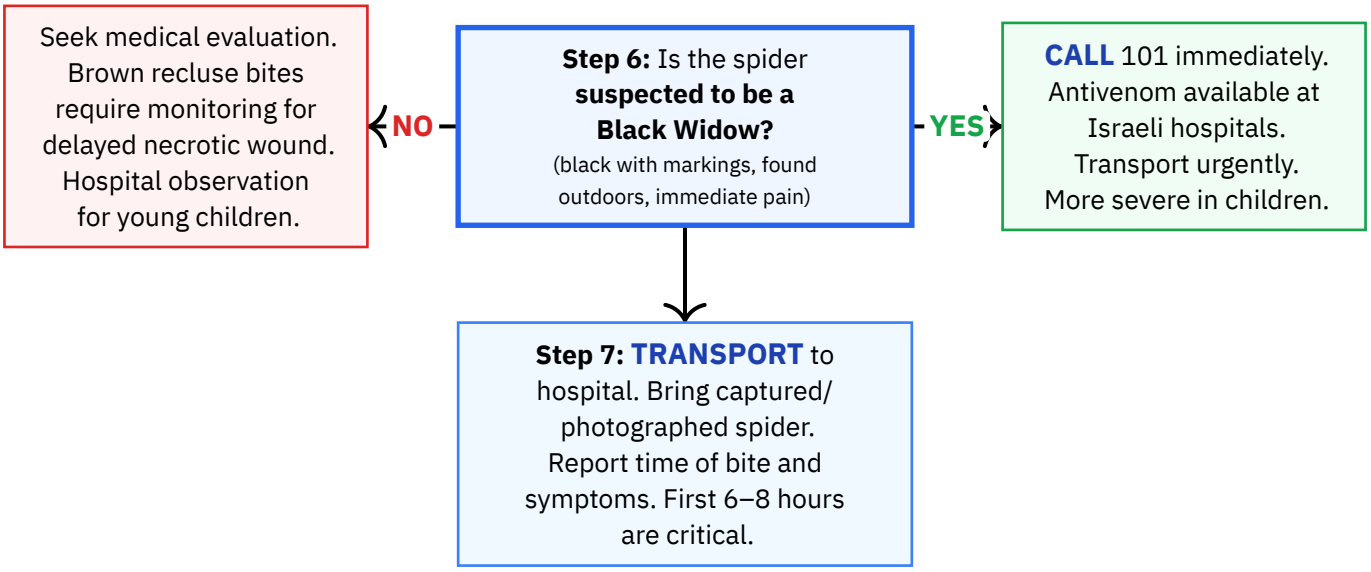
When a child has been bitten or suspected bitten by a spider. Two venomous species in Israel: Brown Recluse (wardrobes, shoes, stored clothing) and Black Widow (gardens, stone walls). Children more vulnerable due to greater venom-to-body-weight ratio. Treat all spider bites in children with medical evaluation.

Summary: Move child away from spider. Photograph spider if safe. Wash bite with soap and water. Apply cold compress. Immobilise bitten limb. Monitor for systemic symptoms. First 6–8 hours are critical. All spider bites in children need medical evaluation.

First Aid and Monitoring – Steps 1-5



Species Identification and Transport – Steps 6-7



Israeli Venomous Spiders

Brown Recluse (shshan chum)
Small, brown, long legs
Lives indoors: wardrobes, shoes
Bite barely felt initially
Effects appear hours later
Causes necrotic ulcer

Black Widow (almana shchora)
Black with markings
Found outdoors: gardens, walls
Immediate pain and burning
Rapid systemic symptoms
Antivenom available in hospitals

DO NOT:

X Do NOT apply a tourniquet -- worsens envenomation by concentrating venom.
X Do NOT cut the bite site -- additional injury, does not remove venom.
X Do NOT attempt to suck out the venom -- ineffective, risks infection.
X Do NOT apply antiseptic, alcohol, or any substance other than soap and water.
X Do NOT bandage tightly over the bite site.
X Do NOT delay medical evaluation for any spider bite in a child.
X Do NOT ignore a painless bite -- brown recluse bites are often painless initially.
X Do NOT give the child medications without medical direction.

Equipment needed:

— Water and soap (for wound cleaning)
— Ice or cold compress wrapped in cloth
— Container or glass (to safely capture the spider)
— Phone (to call 101 and photograph the spider)

Source: Magen David Adom (MDA) – supplemented by Ichud Hatzalah venomous animal guidelines, Ichilov Hospital spider bite guidance, and DrKids.co.il
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: Two venomous species: Brown Recluse and Black Widow. First 6–8 hours critical for recovery. Antivenom for black widow available in Israeli hospitals.

Stroke (Cerebrovascular Event) — Child

Israel — CHILD

CALL 101 — TIME IS BRAIN

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

Child showing: facial drooping, arm weakness/drift, slurred speech, sudden severe headache with vomiting, new-onset seizure (especially one-sided), sudden altered mental status, sudden loss of balance, sudden vision loss. Children may present atypically: seizures at onset, gradual symptoms, irritability, feeding difficulty, lethargy in young children.

FAST Assessment

Face: Ask child to smile – check for drooping/asymmetry
Arms: Ask child to raise both arms – check if one drifts down
Speech: Ask child to repeat a phrase – check for slurring
Time: Note exact time symptoms began

Note: FAST is not validated for the paediatric population and may miss atypical presentations.

Response Steps (1–7)

WARNING
In children, stroke may present differently. Seizures at onset, gradual symptoms, irritability in young children.

Step 1: APPLY FAST assessment. Also check for atypical signs: seizures, headache with vomiting, sudden behavioural changes.

↓

Step 2: CALL 101 immediately. State: suspect child stroke.

↓

Step 3: NOTE exact time symptoms began. Critical for treatment decisions. Alteplase: within 4.5 hrs. Clot removal: within 6–24 hrs.

↓

Step 4: Is the child conscious?

- **NO** → **Step 5:** Breathing? Yes: recovery position. No: **BEGIN** CPR.
- **YES** → Lay down with head and shoulders slightly elevated. Keep calm/still.

↓

Step 6: Is the child having a seizure?

- **NO** → Continue monitoring.
- **YES** → Clear area. Do NOT restrain. Do NOT insert anything in mouth. After: recovery position. Note duration/type.

↓

Step 7: MONITOR continuously until EMS arrives. Watch for changes in consciousness, breathing, symptoms. Be ready for CPR.

DO NOT:

X Do NOT delay calling 101 to wait and see if symptoms resolve.
X Do NOT give any food or drink -- stroke impairs swallowing, risk of aspiration.
X Do NOT give any medications, especially not aspirin -- if haemorrhagic, aspirin will worsen it.
X Do NOT attempt to drive to hospital -- MDA ambulances begin treatment en route.
X Do NOT assume a child cannot have a stroke -- children can have strokes at any age.
X Do NOT restrain the child during a seizure.
X Do NOT put anything in the child's mouth during a seizure.
X Do NOT rely solely on FAST -- adult screening tools are not validated for children.

Source: Magen David Adom (MDA) — MDA Stroke Recognition Protocol (FAST); AHA 2024/2026
URL: https://www.mdais.org/101/stroke
Publication date: 2024-11-18 · **Imported:** 2026-03-15 · **Last verified:** 2026-03-15
Notes: Treatment windows: alteplase 4.5 hrs (ages 28 days–18 yrs), mechanical clot removal 6–24 hrs (age 6+).

Tourniquet Application -- Child

Israel — CHILD

CALL 101 (MDA) — STOP THE BLEEDING

MDA: 101	**Police:** 100	**Fire:** 102	**Hatzalah:** 1221	**Poison:** 04-7771900

When to apply:

When a child has uncontrollable life-threatening limb haemorrhage: blast/shrapnel injuries, amputations, severe lacerations with arterial bleeding, or when direct pressure is insufficient. Children have less total blood volume and develop haemorrhagic shock faster.

Summary: CAT tourniquet effective for most children aged 2+. SWAT-T/RATS for very small limbs. Place 5–7 cm above wound, tighten until bleeding stops, record time, never remove. Transport urgently.

Application – Steps 1-5

Step 1: Is bleeding from a **limb and cannot be controlled with direct pressure?**

- NO → Use direct pressure with sterile gauze. Torso/neck: pack wound and apply max pressure.
- YES → **APPLY** tourniquet immediately. Proceed to Step 2.

Step 2: Is child's limb **large enough for CAT?** (13 cm circumference, age 2+)

- NO → Use SWAT-T or RATS. If unavailable: max direct pressure or improvised tourniquet.
- YES → Use CAT tourniquet.

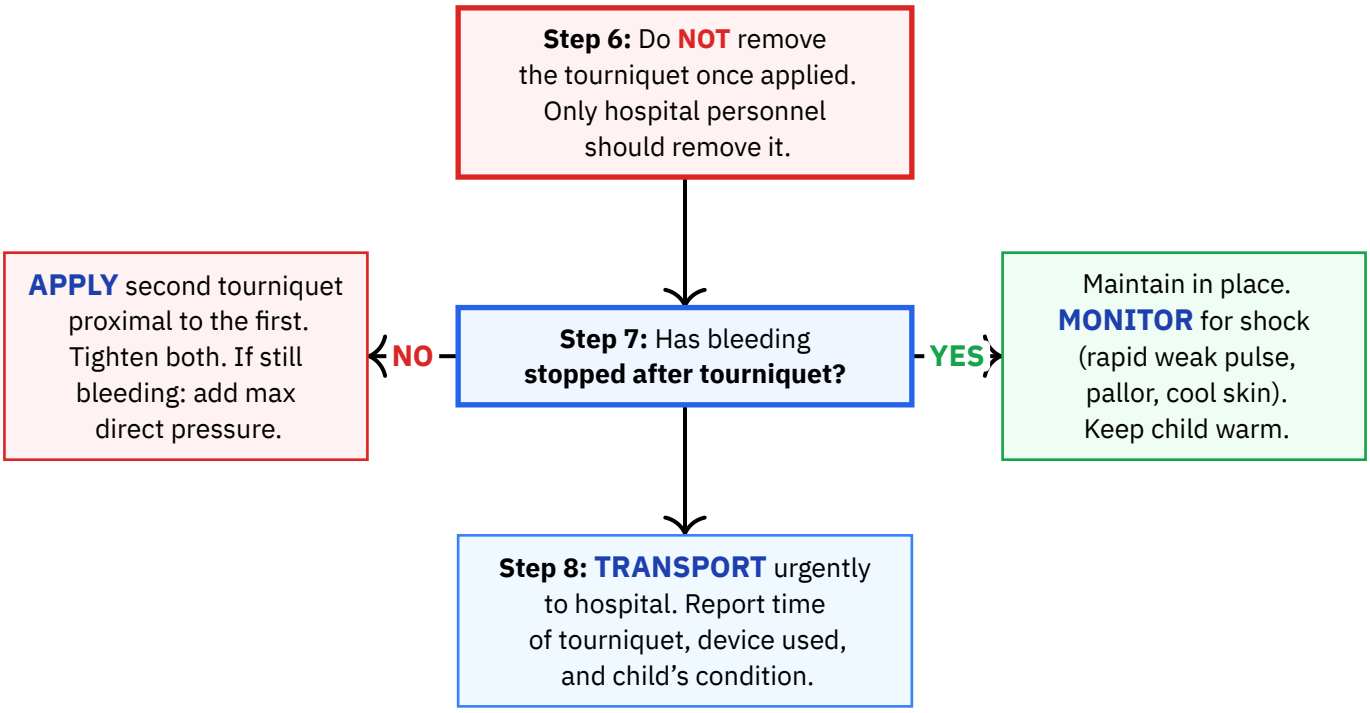
Step 3: **PLACE** tourniquet 5–7 cm above wound. Over single bone (upper arm/thigh), not over joint.

Step 4: **TIGHTEN** until bleeding stops. Turn windlass, secure in clip. Check distal bleeding has stopped.

Step 5: **RECORD** time of application. Write on tourniquet, skin, or tape. Use 24-hour format.

WARNING
Very painful. Warn child. Pain is expected. A loose tourniquet is WORSE than none (causes venous bleeding without arterial occlusion).

Post-Application – Steps 6-8



DO NOT:

X Do NOT remove a tourniquet once applied -- only hospital personnel should remove it.
X Do NOT apply a tourniquet for venomous animal bites (snakes, scorpions, spiders) -- worsens outcomes.
X Do NOT apply over a joint (elbow, knee) -- place on upper arm or upper thigh.
X Do NOT apply too loosely -- worse than no tourniquet (venous without arterial occlusion).
X Do NOT use standard CAT on very small infant limbs (under ~13 cm circumference).
X Do NOT delay tourniquet to attempt prolonged direct pressure on massive haemorrhage.
X Do NOT forget to record the time of application.

Equipment needed:

— CAT (Combat Application Tourniquet) Gen 7 -- for children aged ~2+ with sufficient limb size
— SWAT-T tourniquet -- for all pediatric sizes including very small limbs
— RATS tourniquet -- alternative for all pediatric sizes
— Improvised tourniquet materials (triangular bandage + rigid windlass) if no manufactured device
— Marker or pen (to record time of application)
— Sterile gauze (for wound packing if needed)
— Phone (to call 101)

Source: Magen David Adom (MDA) – supplemented by Ichud Hatzalah tourniquet guidance, Israeli First Aid resources, and NAEMSP/TCCC pediatric tourniquet evidence
URL: https://www.mdais.org/101/first-aid
Publication date: 2026-01-01
Imported: 2026-03-16 · **Last verified:** 2026-03-16
Notes: CAT Gen 7 effective for limb circumference 13–16 cm and above (most children aged 2+). SWAT-T and RATS effective across all pediatric sizes.